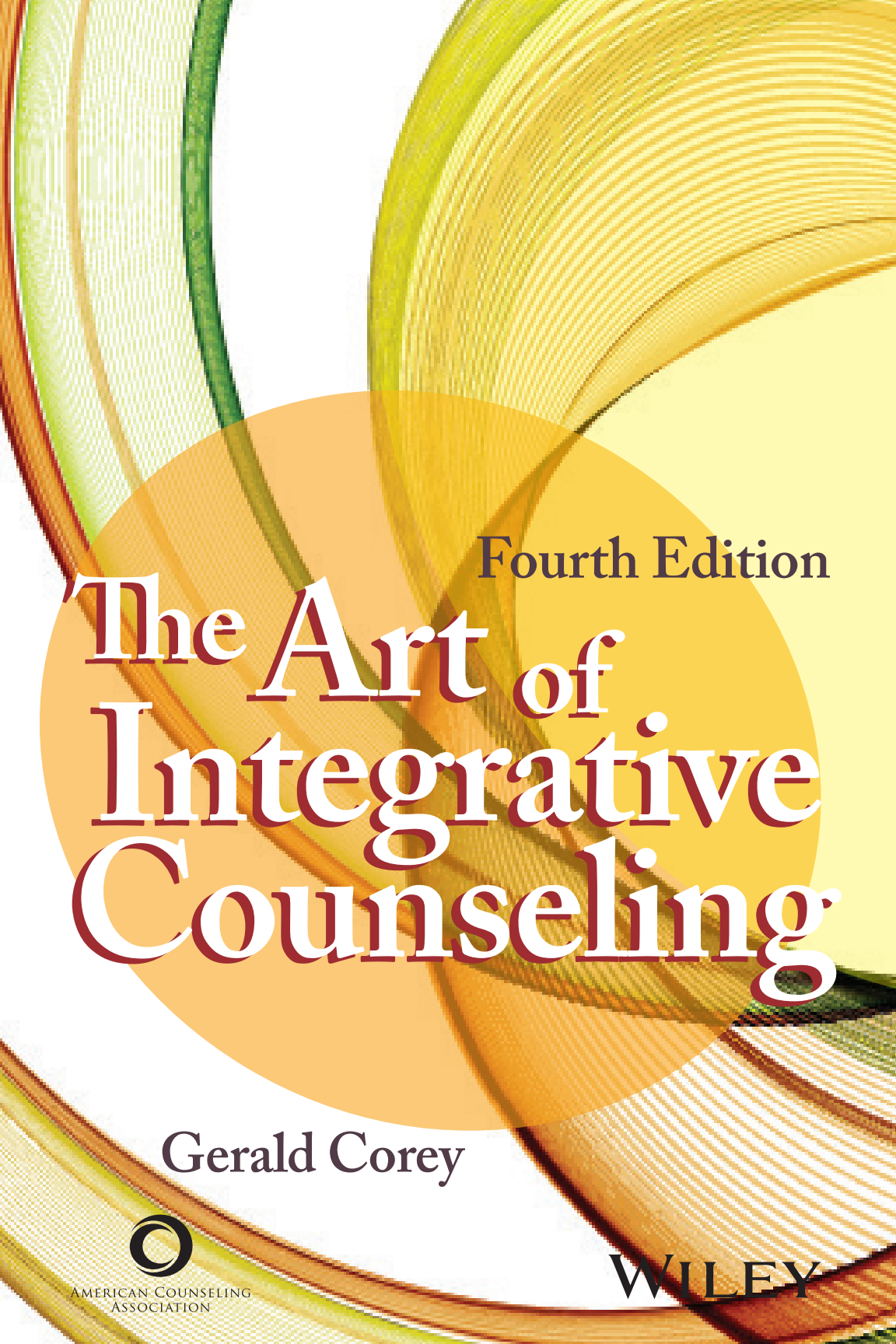
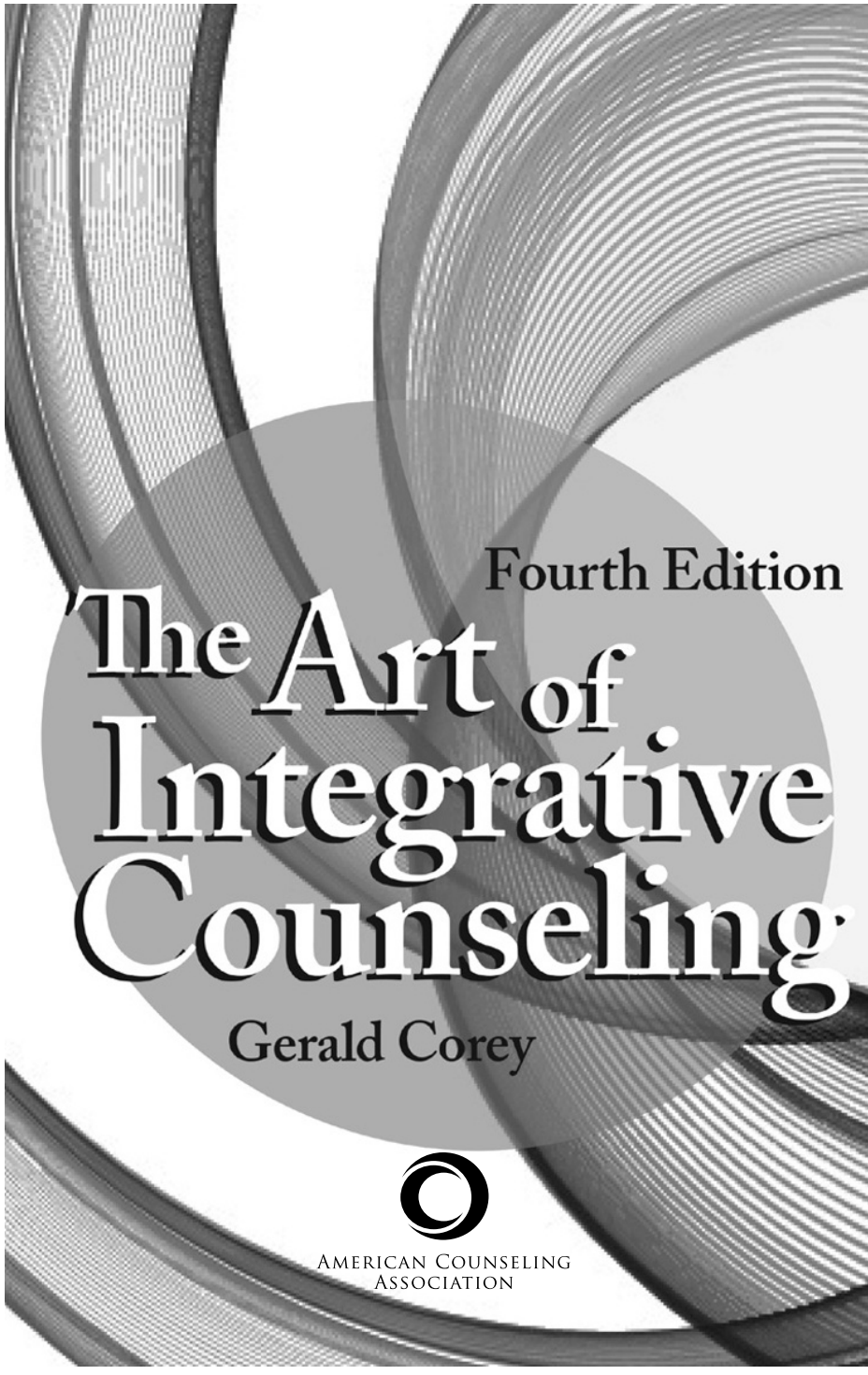
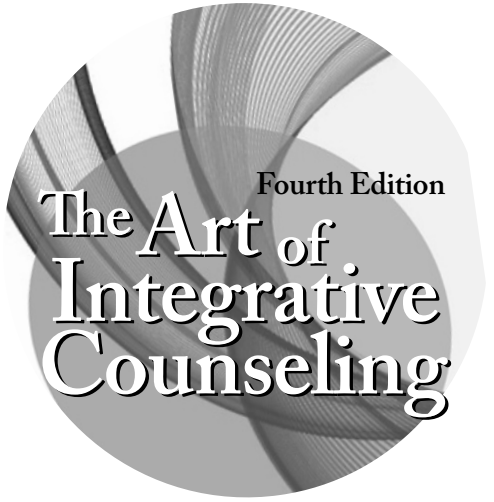

Fourth Edition
The Art of Integrative Counseling
Gerald Corey
American Counseling Association
WILEY



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To the many students I have had the privilege to know and to teach.

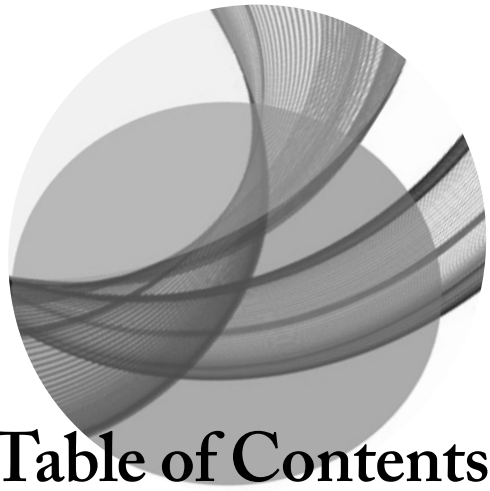


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Preface

In my books dealing with the theory and practice of counseling, I devote specific chapters to presenting an integrative approach to counseling practice and a case example of the application of my integrative perspective. *The Art of Integrative Counseling* is basically an extension of these chapters and is aimed at helping readers conceptualize the various dimensions of an integrative perspective.

About the Fourth Edition of *The Art of Integrative Counseling*

The Art of Integrative Counseling has special relevance for graduate students and new professionals, especially for those who are interested in applying counseling theories to practice . . . and for those interested in understanding how they can develop their own integrative approach to the practice of counseling. This book can be used as a supplementary text to a standard theories of counseling book, and it also can be used for courses in advanced counseling practice, counseling techniques, therapeutic procedures, practicum, and internship. This new edition deals with basic concepts and techniques from many contemporary theories, including psychoanalytic therapy, Adlerian therapy, existential therapy, person-centered therapy, Gestalt therapy, psychodrama, reality therapy, behavior therapy, cognitive behavior therapy, solution-focused brief therapy, motivational interviewing, narrative therapy, feminist therapy, and family systems therapy.

One of the trends in the counseling field is the move toward integration of various theoretical systems and approaches to counseling. Most practitioners believe that an integrative approach is a more useful guide to practice than relying on a single theory. Many standard

textbooks are based on an exploration of concepts and techniques from contemporary counseling and psychotherapy theories. However, only a few books are devoted to integrating the best from all of these theoretical models. In this book I strive to do several things: (1) describe the concepts and techniques that I most draw from in my own integrative approach to counseling practice; (2) demonstrate how concepts and techniques can be borrowed from a variety of theoretical models and applied to the counseling process from the initial to termination stages; (3) invite you to imagine that you are a client in counseling with me as I describe my approach to integrative counseling; and (4) suggest ways for you (as a counseling professional) to think about designing your own integrative approach that will serve as a foundation for what you do in your counseling practice. To encourage active learning, I ask you to put yourself in the role of a therapist at times, and to assume the role of a client at other times, as you reflect these topics.

I cannot tell you how to develop a personal integrative style that will work best for you. However, I have provided some guidelines to assist you in the task of considering which key concepts and techniques you might incorporate in your personal therapeutic style. There is no "one right way" to formulate an integrative perspective. From reading and studying this book, I hope you will acquire a framework that can assist you in systematically constructing an integrative counseling approach that works best for the professional you are and for the clients you will serve.

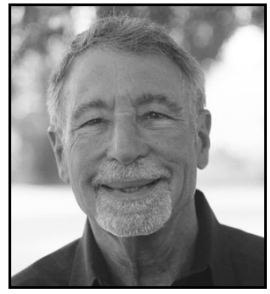
What's New in the Fourth Edition of *The Art of Integrative Counseling*?

The Fourth Edition of *The Art of Integrative Counseling* has been revised to bring the discussions up to date and to refine existing ideas while retaining the informal, personal style of writing that characterized previous editions. I present key findings from recent research on topics such as the central role of the therapeutic relationship (Chapter 2), the role of client feedback on therapy outcomes (Chapter 3), how addressing diversity is central in an integrative approach to therapy (Chapter 4), evidence-based practice as it applies to working with clients in cognitive, emotive, and behavioral ways (Chapters 6, 7, 8), trends in integrative therapies (Chapter 9), and the future of psychotherapy integration and the role of evidence-based practice (Chapter 12).

Other material new to the Fourth Edition includes an expanded treatment of diversity issues, especially the role of spirituality in counseling practice, coverage on the various forms of diversity, and more emphasis on tailoring psychotherapy to individuals from diverse cultural backgrounds (Chapter 4); a broadened discussion of the dynamics of resistance, along with the importance of respecting and reframing resistance, and more on the stages of change (Chapter 5); new material and expansion of the cognitive behavior approaches,

and how cognitive behavior therapy can provide a foundation for an integrative approach to counseling practice (Chapter 6); new sections on incorporating mindfulness and acceptance-based concepts into an integrative perspective (Chapter 8); an emphasis on flexibility and tailoring treatment to individuals and using motivational interviewing in an integrative approach (Chapter 9); more discussion of understanding the dynamics of transference and countertransference (Chapter 10); understanding the role of the past, present, and future as it pertains to counseling individuals (Chapter 11); more emphasis on solution-focused brief therapy and narrative therapy (Chapters 4, 9, 11); and an expanded discussion on termination including the use of therapist's letters, and relapse prevention strategies to help clients deal with setbacks that occur after the end of therapy sessions (Chapter 12). All of the chapters have been carefully examined to determine how the topics fit with contemporary integrative approaches, and I have given particular attention to trends in the psychotherapy integration movement.

About the Author



Gerald Corey, EdD, is professor emeritus of Human Services and Counseling at California State University at Fullerton. He received his doctorate in counseling from the University of Southern California. He is a Diplomate in Counseling Psychology, American Board of Professional Psychology; a licensed psychologist; and a National Certified Counselor. He is a Fellow of the American Psychological Association (Division 17, Counseling Psychology, and also Division 49, Group Psychotherapy); a Fellow of the American Counseling Association; and a Fellow of the Association for Specialists in Group Work. He also holds memberships in the American Psychological Association; the American Counseling Association; the Association for Specialists in Group Work; the American Group Psychotherapy Association; the American Mental Health Counselors Association; the Association for Spiritual, Ethical, and Religious Values in Counseling; the Association for Counselor Education and Supervision; and the Western Association of Counselor Education and Supervision. Both Jerry and Marianne Corey received the Lifetime Achievement Award from the American Mental Health Counselors Association in 2011, and both of them received the Eminent Career Award from ASGW in 2001. Jerry was given the Outstanding Professor of the Year Award from California State

University at Fullerton in 1991. He regularly teaches both undergraduate and graduate courses in group counseling and ethics in counseling at California State University, Fullerton. He is the author or coauthor of 16 textbooks in counseling currently in print, along with more than 60 journal articles and book chapters. Several of his books have been translated into other languages. *Theory and Practice of Counseling and Psychotherapy* has been translated into Arabic, Indonesian, Portuguese, Turkish, Korean, and Chinese. *Theory and Practice of Group Counseling* has been translated into Korean, Chinese, Spanish, and Russian.

During the past 40 years Jerry and Marianne Corey have conducted group counseling training workshops for mental health professionals at many universities in the United States as well as in Canada, Mexico, China, Hong Kong, Korea, Germany, Belgium, Scotland, England, and Ireland. In his leisure time, Jerry likes to travel, hike and bicycle in the mountains and the desert, and drive his grandchildren and friends in his 1931 Model A Ford. Marianne and Jerry have been married since 1964. They have two adult daughters (Heidi and Cindy), two granddaughters (Kyla and Keegan), and one grandson (Corey).

Gerald Corey has six books that are published by the American Counseling Association. In addition to the Fourth Edition of *The Art of Integrative Counseling* (2019), Jerry is coauthor (with Michelle Muratori, Jude Austin, and Julius Austin) of *Counselor Self-Care* (2018); he is coauthor (with Barbara Herlihy) of *Boundary Issues in Counseling: Multiple Roles and Responsibilities*, Third Edition (2015) and *ACA Ethical Standards Casebook*, Seventh Edition (2015); he is coauthor (with Robert Haynes, Patrice Moulton, and Michelle Muratori) of *Clinical Supervision in the Helping Professions: A Practical Guide*, Second Edition (2010); and he is the author of *Creating Your Professional Path: Lessons From My Journey* (2010).

Other publications by Gerald Corey, all with Cengage Learning, include:

- *Issues and Ethics in the Helping Professions*, Tenth Edition (2019, with Marianne Schneider Corey and Cindy Corey). This work has been translated into Japanese, Chinese, and Korean.
- *Groups: Process and Practice*, Tenth Edition (2018, with Marianne Schneider Corey and Cindy Corey)
- *I Never Knew I Had a Choice*, Eleventh Edition (2018, with Marianne Schneider Corey and Michelle Muratori)
- *Theory and Practice of Counseling and Psychotherapy*, Tenth Edition (and *Student Manual*) (2017)
- *Theory and Practice of Group Counseling*, Ninth Edition (and *Student Manual*) (2016)
- *Becoming a Helper*, Seventh Edition (2016, with Marianne Schneider Corey)
- *Group Techniques*, Fourth Edition (2015, with Marianne Schneider Corey, Patrick Callanan, and J. Michael Russell)
- *Case Approach to Counseling and Psychotherapy*, Eighth Edition (2013)

He has also made several educational DVD programs on various aspects of counseling practice: (1) Group video to accompany *Theory and Practice of Group Counseling* (2019); (2) *Ethics in Action*: *DVD and Workbook* (2015, with Marianne Schneider Corey and Robert Haynes); (3) *Groups in Action: Evolution and Challenges DVD and Workbook* (2014, with Marianne Schneider Corey and Robert Haynes); (4) DVD for *Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes* (2013); (5) DVD for *Integrative Counseling: The Case of Ruth and Lecturettes* (2013, with Robert Haynes); and (6) DVD lecturettes to accompany *Theory and Practice of Group Counseling* (2012). All of these video programs are available through Cengage Learning.



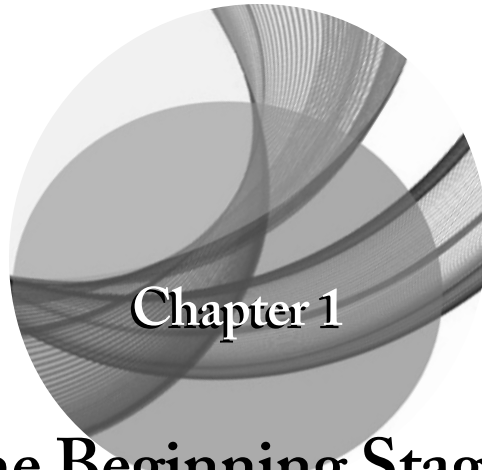
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California State University at Fullerton, contributed to the cases of Lani and Rob and Rico. Randy Alle-Corliss and I have been coteaching for years, and we worked collaboratively in designing and revising the cases of Chet and Jasper. My former students Nicholas Lazzareschi and Jeff Markow reviewed the manuscript and provided suggestions that were incorporated in the final manuscript. Special recognition goes to Amanda Connell, a valued colleague with whom I coteach a group class at California State University at Fullerton, who critiqued the entire manuscript and assisted in revising many of the cases. All who reviewed the manuscript gave particular attention to the cases and made suggestions that I used as I developed these cases. Presenting these cases was truly a collaborative endeavor.

Special appreciation goes to Carolyn Baker, the associate publisher at the American Counseling Association. Carolyn contributed her expertise by reviewing the entire manuscript, providing insightful comments on content issues and suggestions for consistency in style, and offering support throughout the revision process. A special note of thanks goes to the manuscript editor, Kay Mikel, who made sure the presentation was clear, practical, personal, and effective. A book such as this is truly a team effort, and both Carolyn and Kay are key people on this team.

—Gerald Corey



Chapter 1

The Beginning Stages of Counseling

Although you may be just starting your practicum or internship in the helping professions, already you may have been asked to answer these questions: "What is your approach to counseling?" "How does your theoretical orientation influence the manner in which you practice?" You will revisit these questions throughout your career in job interviews and self-evaluations.

This book will assist you in conceptualizing what you do as a counseling practitioner and help you clarify your theoretical orientation. I want to stimulate your thinking about the importance of developing an integrative approach to counseling practice that pays attention to what your clients are *thinking*, *feeling*, and *doing*. Combining these three dimensions is the basis for a powerful and comprehensive approach to counseling practice. If any of these dimensions is excluded, the therapeutic approach is incomplete.

It is important to examine the contemporary theories of counseling to determine which concepts and techniques you can incorporate in your approach to practice. Creating your own integrative stance is truly a challenge. You cannot simply pick bits and pieces from theories in a random and fragmented manner. Each theory represents a different vantage point from which to look at human behavior. Study all the major theories, resist too quickly embracing any single point of view, and look for a basis for an integrative perspective that will guide your practice.

A theory is a good road map for understanding the therapy process, but no one theory is best for all clients under all sets of circumstances (Norcross, Goldfried, & Arigo, 2016). Research has clearly established that psychotherapy works and is remarkably effective, but no par-

ticular theory or technique has proved to be superior to all others (Wampold, 2010). Each theory has something unique to offer you. There is a growing recognition that counseling and therapy can be most effective when contributions from various approaches are integrated (Goldfried, Glass, & Arnkoff, 2011). Goldfried and colleagues believe that evidence-based practice will increasingly become the organizing force for integration. Empirical pragmatism, rather than theory, will be the integrative theme of the 21st century. In addition, evidence increasingly supports the idea that the therapeutic alliance (the therapeutic relationship) and the therapist as a person are critical factors in determining therapy outcomes (Elkins, 2016; Hubble, Duncan, Miller, & Wampold, 2010; Keenan & Rubin, 2016); these topics are addressed in detail in Chapter 2.

The aim of this book is to assist you in acquiring your own unique perspective on counseling, which will aid you in eventually developing your integrative approach to counseling practice. To develop this kind of integration, you need to be well grounded in a number of theories, be open to the idea that some aspects of these theories can be unified in different ways, be familiar with the research literature in the field of psychotherapy, and be willing to continually test your hypotheses to determine how well they are working. In developing and conceptualizing your integrative counseling approach, consider your own personality, and think about what concepts and techniques work best with a range of clients. Choosing techniques that are suitable for particular problems and for different clients requires knowledge, skill, art, and clinical experience. It is also an art to know *when* and *how* to use a particular therapeutic intervention.

To be an effective counselor it is essential that you be willing to take an honest look at your own life. Are you willing to do for yourself what you ask clients to do? It will be difficult to inspire clients to seek help when they need it if you are not open to change in your own life. Your own self-exploration likely will be one of the most important factors in learning how to use many of the techniques in this book. You can learn many ways to creatively intervene with your clients by experiencing what it is like to be a client. (See Chapter 10 for more on the value of personal therapy for counselors in training.) As much as possible, I will ask you to put yourself in the shoes of the client. The "Becoming a Client" sections in this book are useful sources of self-exploration and offer strategies for intervening with your own clients.

An Integrative Theoretical Approach: An Overview

This book represents my own integrative approach to counseling. I define an *integrative approach* to counseling as being rooted in a theory or theories, with techniques systematically borrowed from other approaches and tailored to a client's unique needs. I am not suggesting that you adopt my conceptualization of theory applied to practice. You

will develop your own integrative style based on your personality and the kinds of clients you expect to counsel. By describing my personal orientation to counseling, I hope to provide a framework to assist you in designing a theoretical orientation that makes sense to you.

The early history of counseling was full of theoretical wars as practitioners argued over the "best" way to bring about personality change. Many practitioners and scholars were resistant to psychotherapy integration, often to the point of being blind to alternative theories and ignoring effective methods from other theoretical schools. However, since the 1980s most therapists have seriously considered integrating the best from the various schools. Therapists now acknowledge the limitations inherent in specific theories and the potential value of other theoretical systems. Most practitioners use some form of integration today, and research supports the efficacy of an integrative approach (Norcross et al., 2016). For example, two popular therapeutic approaches are cognitive behavior therapy and motivational interviewing. Naar and Safren (2017) devote an entire book to ways of combining these strategies for maximum effectiveness.

As a student, you can begin the process of developing a style tailored to your own personality by familiarizing yourself with the major approaches to therapeutic practice. Then choose one theory to study in some depth and branch out from there in your search for an integrative style. I recommend that you study in depth the one theory that comes closest to your worldview and values, and use this theory as a foundation for developing your personal orientation. Being grounded in a theory provides an anchor for making sense of what you are doing as a counselor. Without a theoretical foundation you are liable to flounder, and neither you nor your client is likely to experience productive results.

Attempting to practice without having an explicit theoretical rationale is like trying to build a house without a set of blueprints. The foundation of a house needs to be sturdy and strong to support the structure. If you operate in a theoretical vacuum and are unable to draw on theory to support your interventions, your attempts to help people change will have uncertain outcomes. Theory provides a foundation for understanding human nature, developmental processes, learning processes, function and dysfunction, and purposes and motivations. This model is not a rigid set of structures; rather, it provides a general framework that enables you to make sense of the many facets of the counseling process and gives direction to what you do and say.

You need to believe in the basic philosophy of the theory you embrace and that the treatment you are delivering will be effective. Therapists providing a treatment that they find interesting and fits for them are likely to be more effective than therapists who deliver a treatment not to their liking (Wampold, 2010). The treatment needs to be consistent with your beliefs, values, and personality. Ask yourself: "Which treatment delivered by me will be most effective?" (p. 49).

I draw on concepts and techniques from most of the contemporary counseling models. I then adapt them to a style that fits me personally, taking into account the universal thinking, feeling, and behaving dimensions of human experience. I typically ask clients to think about the decisions they have made about themselves. Some of these decisions may have been necessary for their psychological survival as children but may now be ineffective. I encourage clients to pay attention to their "self-talk" and to ask themselves these questions:

- How do your problems reflect the assumptions you make about yourself, about others, and about life?
- How do you create your problems by the thoughts and beliefs to which you cling?
- How can you begin to free yourself by critically evaluating the statements you repeat to yourself?

These interventions help clients think about events in their lives, how they have interpreted those events, and what they need to do cognitively to change their belief systems.

Once clients begin thinking about their problems, they often feel stuck due to unexpressed and unresolved emotional concerns. I encourage clients to experience the range of their feelings and to talk about how certain events have affected them. The healing process is facilitated by using techniques that engage clients' feelings and allow them to feel listened to and understood. In addition to encouraging clients to pay attention to their beliefs and decisions, I ask clients questions about their feelings and behavior:

- Do you ever feel so overwhelmed by your feelings that you think there is little or nothing you can do about it?
- Do your feelings tell you anything about how you see life or suggest how you protect yourself from perceived danger?
- How do your feelings fuel your actions?
- What habit or routine do you do every day without thinking, and how does that work for you?
- What behaviors or actions do you avoid?
- Do you initiate some actions and then immediately regret them?

Thinking and feeling are vital components in the helping process, but eventually clients must express themselves in the behaving or acting dimension. Clients can spend countless hours gaining insights and venting pent-up feelings, but eventually they need to get involved in a program of change. Their feelings and thoughts can then be tested and adapted to real-life situations. If the helping process includes a focus on what people are doing, there is a greater chance that clients also will be able to change their thinking and feeling. Using an integrative counseling style, interaction among these three dimensions occurs throughout the counseling process.

It is crucial to help clients consolidate what they are learning by encouraging them to apply new behaviors to situations they encounter every day. Some strategies I use are contracts, between-sessions assignments, action programs, self-monitoring techniques, support systems, and self-directed programs of change. (These strategies are discussed in some detail in Chapters 8, 9, and 12.) These approaches all stress the role of commitment on the part of clients to practice new behaviors, to follow through with a realistic plan for change, and to develop practical methods of carrying out this plan in everyday life.

Clearly defining and articulating a set of values and a philosophy of life are essential to moving through life with balance and conviction. Ultimately, the most meaningful counseling perspective for you to use is one that is an extension of your values and personality. As your philosophy of life evolves, so will your philosophy of counseling. Developing a personalized approach that guides your practice is an ongoing process, and your personal approach to counseling will undergo continuous revision. Reflecting on your own values, life experiences, and philosophy of life is a good starting point.

Becoming the Client: Put Yourself in the Shoes of the Client

The Initial Counseling Session

The main tasks of the initial session include developing a therapeutic alliance, gaining a picture of the client's problems and concerns, determining the focus of treatment, building motivation, and providing information about the therapeutic process so the client can make informed decisions (Naar & Safren, 2017). The initial interview focuses on key areas, including identifying information, the presenting problem, relevant recent history, current functioning, and expectations of therapy (Maniacci & Sackett-Maniacci, 2019).

One of my goals in writing this book is to include you experientially in the process of deciding which aspects of current theories you might incorporate into your own philosophy of counseling. I ask you to "become the client" now and reflect on integration from that perspective. Imagine that you are the client and we are about to have our first session. As a therapist, I realize that the first few minutes are critical in setting the tone. I begin by explaining the confidential nature of our work together, including the limitations of confidentiality. By explaining a few of the foundational aspects and guidelines of therapy, I hope to provide a sense of safety that will encourage you to talk freely. To promote trust and rapport, I ask what you expect from this session today.

What I most want to do is to listen to your story. Meeting and valuing you as a person is essential to positive change. To create a working therapeutic relationship, I try to make a good connection with you by being present—being fully open to and affected by you. My aim is to

establish a positive relationship by listening, responding with clarity and empathy, demonstrating respect for your capacity to understand yourself, and exhibiting faith, hope, and caring. Here are some questions I am likely to ask you during the first session. Reflect on your responses to each of these questions:

- What brings you here? What has been going on in your life recently that prompted you to seek professional help at this time?
- What expectations do you have of therapy? Of me? What are your hopes, fears, and reservations? What goals do you have for yourself through therapy?
- Could you give me a picture of some significant turning points in your life? Who have been the important people in your life? What significant decisions have you made? What are some of the struggles you've dealt with, and what are some of the issues that are current for you?

To the extent possible, I avoid preconceived notions about what our dialog will consist of or how the therapy process will unfold. My hope is that you will share your present thoughts and feelings as they arise in this session. You may not be sure what you want from counseling, and you may have ambivalent feelings about being with me. You may be uncertain about how being in counseling will change your relationships at home. This lack of certainty is a good place to begin. I want to give you a chance to express your expectations, hopes, concerns, reservations, and doubts about making a commitment to the counseling process. This is one way I get to know you and connect with you.

I am not consciously thinking about applying a specific set of techniques. I adapt my interventions to fit your needs as a client. I take into account an array of factors about you, including your readiness to confront an issue, your stage in the change process, your cultural background, your value system, and your trust in me. I want to help you identify and experience whatever you are feeling, understand how your assumptions are influencing how you feel and behave, and encourage you to experiment with new behaviors.

Informed Consent

For the informed consent process and the initial sessions, I draw heavily on an Adlerian approach, largely because Adlerians strive to create a democratic spirit that permeates the entire counseling process. Educating you (the client) about the counseling process, addressing your questions, and clarifying your expectations are routes to ethical and effective counseling practice. The challenge is to create a balance between giving you too much or too little information. In the first session, I give you a well-written informed consent document that provides much of this information without overwhelming you. The main agenda is to provide you with the opportunity to talk about what you hope to gain from being in counseling. For you to feel safe enough to meaningfully express yourself, you need to have at least

some minimal information about the nature of the therapeutic relationship. What would you want to know about how counseling works as you begin as my client? What do you consider essential to know before you make a commitment to this professional relationship? Here are some topics we might explore as part of the therapeutic contract during our early sessions:

- Why is confidentiality essential to our work together, and what are the limits of confidentiality?
- How does the therapeutic process work?
- What is my primary role as a therapist?
- What is expected of you as a client, both during the sessions and outside of sessions?
- How can you and I become collaborators?
- What is the approximate length of the counseling process? How long will it take for you to begin to feel better and act more effectively?
- How will we know when it is time to end our work? Who has the right and the responsibility to terminate?
- What are your rights and responsibilities as the client?
- What are some of the benefits and risks of therapy that you can expect?

During this first session, let's assume that you say this to me: "One of the troubles I have is trusting myself. I always think about what others expect of me. I've tried to please others for so long that I don't know who I am most of the time." This is a good opportunity for me to briefly educate you about a key task of therapy: challenging you to begin to examine the ways in which you think, feel, and act.

I want to establish a collaborative working relationship with you. (In Chapter 2, I discuss in detail how I strive to create an effective therapeutic alliance.) I spend time explaining my view of the therapy process and how it works. By demystifying the therapeutic process, I am conveying the message that you are largely in charge of the direction your therapy will take. I operate on the assumption that you are the expert on your own life. I encourage you to look within to find your own answers; I won't try to provide easy solutions or answers. I will provide guidance and support as you strive to develop your own answers. As an outcome of our work together, I hope to help you increase your awareness of your choices and investigate ways that you limit or restrict yourself.

By this time, we have talked about the nature of counseling, the purpose and limitations of confidentiality, the procedures that we may employ, and the benefits and risks of the process. What questions and reactions do you have? In later sessions we will discuss specific therapy procedures I believe may be appropriate as we uncover your particular problems. You will help choose the techniques used in dealing with your problems.

Informing you about the counseling process is an important professional responsibility and an ethical requirement of all the major mental health professions. To what extent has this discussion helped you feel informed and a partner in the therapeutic venture?

Initial Assessment

Next, I begin the process of conducting an assessment of your circumstances and sharing my impressions with you. Early in this phase, I assess for risk factors such as suicidality, as well as determine whether we should begin with an approach of stabilization or exploration. An initial therapeutic path of stabilization may be needed if you are in crisis, have substance misuse issues, or suffer from emotional dysregulation. Understanding your current coping strategies, strengths, relationships, and meaning in life is also beneficial.

Assessment consists of evaluating the relevant factors in your life to identify themes for further exploration in therapy. Adlerians are skilled in asking questions and leading clients to find answers (Maniacci & Sackett-Maniacci, 2019). I am likely to employ an Adlerian technique as part of your assessment by asking you some version of "The Question": "How would your life be different if you did not have this problem?" "What would you do differently if you did not have this symptom or problem?" "How would your life be different if you didn't have these issues, concerns, or problems?" If you answer that "nothing would be different, except the symptoms would be gone," I suspect the symptoms may be physiological or organic even though they manifest themselves as psychological complaints. However, if you say that "if it weren't for this depression, I would get out more and see my friends," I suspect your problem serves the purpose of helping you to avoid something you perceive as necessary but from which you would like to retreat. Such a statement may reveal your concern about the possibility of being a good friend or being welcomed by your friends. Raising such questions can be a good catalyst to stimulate your reflection on what it might be like for you if you were able to change some problematic area in your life.

I am also interested in learning something about your family of origin during the initial assessment. Understanding and assessing your family of origin will reveal patterns of interpersonal behavior and communication you learned in your family that are likely to be repeated in other interactions outside your family. As a way of learning about the influence of your family on the person you are, I may ask you to identify what you learned from interacting with your parents, from observing your parents' interactions with each other, and from observing how your parents interacted with each sibling. This is not simply a question-and-answer interview; rather, it is an assessment that enlists you as a partner in the therapeutic process (Naar & Safren, 2017).

Assessment is an ongoing process. It will not be completed during the intake interview, nor will fixed judgments be made. Assessment can be linked directly to the therapeutic process, forming a basis for developing methods of evaluating how well my interventions are working to achieve your goals. At this time, I encourage you to tell your story. As you do, I listen not only to the content but also to the manner in which you present the story of your life. I am interested in learning about strengths and resources you have to draw on in addressing your life concerns. This kind of assessment goes beyond simply understanding your problems. Together we can build on the positive patterns your life story reveals.

Therapeutic Time Limitations

In the 1970s, the field of psychotherapy began to shift from long-term psychotherapy to more problem-focused, short-term therapies such as cognitive behavior therapy. If you are my client in a setting in which brief therapy is the standard, it is especially important for me to be clear about the number of sessions the agency allows or for which your insurance will pay. If an agency policy specifies that you can be seen for only six sessions, you have a right to know this from the outset. If we are working from a short-term therapy approach, I will always keep in mind the short duration of our work together. The goal is to help you learn, as quickly and efficiently as possible, the coping skills you need to live in self-directed ways.

Practicing brief therapy requires that we set clear and realistic treatment goals, that our sessions are structured with a clear focus, and that I assume an active role in making interventions. At times, even one session can be enough to get you back on track. For example, you may be uncertain about pursuing a doctoral program, and a single session could assist you in clarifying ambivalence and making a plan. Hoyt (2009) describes single-session therapy as the most common length of treatment. Practitioners with varying theoretical orientations suggest the utility of single-session therapy in certain cases. Short-term therapies are increasing in popularity (Norcross, Pfund, & Prochaska, 2013); indeed, short-term therapies have become the treatment imperative (Norcross & Beutler, 2019). Time-limited brief therapy refers to a variety of time-sensitive, goal-directed, efficiency-oriented methods; these methods can be incorporated in any theoretical approach (Hoyt, 2015).

The limitation of time can assist us in establishing realistic goals. Toward the end of each session, I will ask you the degree to which you see yourself reaching the goals you have established, and I will also ask you to take a few moments to fill out a rating scale for the session. (This written rating scale is described by Duncan, Miller, and Sparks [2004] in *The Heroic Client*.) As a part of *feedback-informed therapy*, I will ask you to rate four areas:

Relationship: To what degree did you feel heard, understood, and respected?
Goals and topics: To what degree did we work on and talk about what you wanted to work on and talk about?
Approach or method: To what degree is my therapy approach a good fit for you?
Overall: To what degree was the session right for you today?

By reviewing the course of treatment, you are in a position to identify what is and is not working for you in the counseling process. I will include you by asking for your feedback on the progress of treatment and your experience in the therapeutic process. If I listen to feedback from you, together we can modify what we are doing in our therapy sessions, making the therapeutic process a collaborative endeavor. More information on using feedback from clients to improve therapeutic outcomes can be found in *Feedback-Informed Treatment in Clinical Practice: Reaching for Excellence* (Prescott, Maeschalck, & Miller, 2017).

Even though the emphasis these days is on brief therapy, my approach is more in line with brief intermittent therapy. I want to focus on and address the major concerns in your life today, but I expect we will have brief, ongoing checkups over time, somewhat like a yearly medical checkup, if you like that idea. My overriding goal is to increase the chances that you will not continue to need a therapist. If I do my work well, eventually I hope to put myself out of business. I am open to exploring termination issues with you at any point during the counseling process. As my client, would you want to know about these matters from the start? To what extent do you think discussing termination early on could be helpful to you?

Each chapter features two cases that demonstrate my integrative approach to counseling with a variety of clients. The cases are meant to highlight the application of key principles and themes in the chapter. I have made an effort to keep these cases relatively brief, emphasizing the interventions I made with each client. As you read and reflect on each case, think of how you might work with each client.

Initial Sessions and the Case of Gwen

Gwen is a 56-year-old, married, African American woman presenting with difficulty sleeping, stress, isolation associated with her job, and a history of anxiety and depression.[1] Gwen is the oldest of five children, and after her parents' divorce, she took on the responsibility of caring for her younger siblings. As the eldest child, Gwen learned very early that she was responsible not only for herself but also for all those in need

[1]The material about this client is adapted from the case of Gwen, which is described in my textbook, *Theory and Practice of Counseling and Psychotherapy* (Corey, 2017). This case appears in each of the theory chapters in that book, and Kellie Kirksey, PhD, demonstrates her way of working with Gwen using each of the theories.

around her. She often sacrificed her own desires in an effort to please others. She knows how to stand up for herself, but too often she takes on the role of helper and loses her sense of personal meaning and identity.

Gwen has been married to Ron for 31 years and states that they have problems at times, but basically they have a good relationship. They have three adult children. Gwen has a master's degree in accounting and is employed at a large firm as a CPA. She reports being the only woman of color at her job. Because she is the only one speaking up for issues of diversity, gender, and racial equality at her workplace, she often feels isolated and tired. She does not have enough time to spend with friends or to do the things she once enjoyed because of her long work hours. Gwen is also the primary caretaker of her mother, who resides with her and is in the advanced stages of dementia.

Intake Session

This is Gwen's first time in formal counseling, and she is seeking counseling because she is having difficulty staying focused at work and is generally feeling sad and overwhelmed. Gwen also reports experiencing a great deal of anxiety. Gwen begins by saying she is tired of the stressors she has been holding inside. She states that she has held everything together for everyone far too long. During this initial session, I address the relevant aspects of informed consent and begin an ongoing process of educating Gwen on how the therapeutic process works.

Gwen says she feels a heaviness in her heart, which is associated with all that is expected of her at work and with her family, what she has not accomplished, and where she is heading. A number of life concerns bring Gwen into counseling. A few of her concerns relate to her work. She experiences mounting tension on the job, and when she asserts her opinions, she is labeled as emotional and angry. The more tension she experiences at work, the less she engages at home. An additional concern is that her mother is losing her memory.

Gwen has lived for a long time believing therapy was for weak people and certainly not for an African American woman of strong faith. Gwen has not been in therapy before, and I need to establish a climate of trust and respect so she feels it is safe to express her feelings and experiences. As we develop our working alliance, I help Gwen understand that she has already begun the change process by doing something different and coming in for therapy.

The first step of our journey is to build a working alliance based on mutual respect. I follow the lead provided by Gwen of what is troubling her and attempt to work within the framework of what she says she wants. At each step along the way, I show empathy and compassion for her challenges as she works to rebuild self-trust and reconnect to her own sense of personal power and value.

Questions to Facilitate the Initial Session

It is to be expected that Gwen has some anxiety about initiating therapy. I want to provide her with the opportunity to talk about what it is like

for her to come to the office today. That in itself provides the direction for much of our session. I structure the initial session so that she can talk about her expectations and about her fears, hopes, and ambivalent feelings. At this early stage of working with Gwen, I do not have a clear idea of where our journey will take us, for much depends on how far Gwen wants to go and what she is willing to explore. I begin by giving her a chance to say what she was thinking and feeling as she came to her initial therapy session. I then ask some of these questions, giving her time to respond to each one:

- What prompted you to call for an appointment now, given that you have been feeling this way for some time? What has occurred, changed, happened? Was there any event that precipitated your calling?
- What do you want me to know about you?
- What are your experiences of sexism and racism, and how do they play into what you are experiencing right now?
- In what areas is your life going well?
- What do you wish was different?
- In what areas are you presently experiencing conflict or struggle?
- What were you experiencing as you were getting ready to come to this session?

These questions open the discussion of what Gwen is experiencing as she makes her first contact with me. They are central to the assessment process, which begins during the initial session. A guiding question in the back of my mind during this assessment is, "What does this client most need to understand about herself to grow and to deal more effectively with her present relationships?"

Establishing a Therapeutic Relationship With Gwen

From the beginning, I want Gwen to understand as much as possible about our mutual responsibilities as partners in this therapeutic endeavor. Informing Gwen about how the therapeutic process works, obtaining her informed consent, acknowledging that I am both a different gender and a different race than she (and that I will need her help in knowing how these affect her life), and establishing a therapeutic alliance are key matters to attend to early in our sessions. Because the therapeutic relationship is central to the outcome of therapy, this is something I begin to establish at our first meeting. This relationship is critical at the initial stages of therapy, but it must be maintained during all stages if therapy is to be effective.

In establishing the therapeutic relationship, I am influenced by the assumption that therapy is a joint venture, not something that I *do* to a passive client. I tend to ask myself questions such as these: "To what degree am I able to listen to and hear Gwen in a nonjudgmental way?" "Am I able to empathize and care for her?" "Do I have the capacity to enter her subjective world without losing my own identity?"

I begin by being as honest as I can with Gwen, especially about my limitations. I invite Gwen to raise any questions that will help build our working alliance.

Theories During the Early Stage of Counseling

At the beginning of the counseling relationship, my concern is to establish a foundation that will provide safety for clients to be able to undertake the risks necessary in making fundamental life changes. I incorporate many constructs from the person-centered approach in my therapeutic style. For example, I believe Gwen will tell me a great deal about what she wants from life if I can really listen with deep understanding. Although I see my overall style as being active and directive, I first and foremost want to see the world from Gwen's vantage point. She will provide me with leads if only I care enough to carefully listen and observe. From the person-centered approach, I value the emphasis on striving to experience Gwen's world from her subjective perspective and trusting in her basic wisdom. This approach places emphasis on the quality of our relationship, which I believe is the curative factor that brings about healing and change. I want to approach meeting Gwen with as much presence, openness, and interest as I am able to bring to the initial counseling session. What I most want to do is assist her in creating her own agenda rather than being too quick to present my agenda to her. I believe Gwen is the expert on her own life. Consistent with the spirit of person-centered therapy, I believe that Gwen, with good support, has the capacity to identify what she wants in her life and that she will be able to chart her own course.

I assume that Gwen cannot be understood without considering the various systems that affect her: family, social groups, community, church, and other cultural forces. For the counseling process to be effective, it is critical to understand how Gwen influences and is influenced by her social world. (As multicultural, feminist, and family therapists have emphasized, the human condition needs to be understood within the context of a system, which includes the social and cultural framework.) Ignoring either the internal or the external perspective in understanding Gwen's life experience will restrict my effectiveness with her.

Use of Techniques

I introduce ideas and techniques, and I let Gwen know that she is free to say what is helpful to her and what does not work for her in our sessions. In deciding on techniques to introduce, I take into account an array of factors about Gwen. Some of the areas I consider include her readiness to confront an issue, her stage in the change process, her cultural background, her value system, and her trust in me.

I typically begin and end each session by asking Gwen about the session. I depend on regular feedback to make the process truly collaborative and to ensure that Gwen's therapeutic goals are primary.

My role as a therapist is to be as fully present as possible so I can effectively integrate therapeutic approaches that will assist Gwen on her journey of transformation as she moves toward a state of optimal functioning and balance.

Reflection Questions

Reflect on what you would most hope to accomplish with Gwen during the initial session. Consider these questions:

- What would you focus on with Gwen during the assessment process?
- What specific aspects of the informed consent process would you most want to address at the first session?
- How competent would you feel in working with Gwen given any of the differences between you? What differences would you see as being most important to address?

Initial Sessions and the Case of Chet

Chet, age 42, decided to seek counseling after a crisis in his marriage, which led to his wife (Amelia) filing for a divorce. Married for 15 years, he was utterly shocked when his wife said that she felt a strong need to become more independent and that their marriage was no longer fulfilling. Although there were periods of conflict in their relationship, Chet never imagined the situation was so desperate that Amelia would decide to pursue a divorce.

Amelia is in her own personal therapy and also participates in a women's group. Chet wanted Amelia to join him in marital therapy, but she refused. He is concerned that her dissatisfaction with their relationship has been stirred up by what she hears in her women's group. Chet comes from a culture that honors family stability, and divorce is frowned upon. He is angry with Amelia for going against traditional family roles and values, and he has a hard time understanding why she is not willing to work on keeping their marriage intact. Chet feels helpless about the situation, and often feels panic and extreme sadness.

I am especially interested in what Chet most hopes to get from our work together. At the initial session, Chet asks how counseling can help him get out of his depression, anxiety, and hopelessness. One of his expectations is that he will reveal his problems and that I will then give him advice. He admits to feeling anxious about approaching his first counseling session, but he adds that he is scared about being lonely and lost when it comes to creating new relationships. In response to my question about what he expects to get from counseling, he replies, "I want to know how to cope with the breakup of my 15-year marriage. I really need to know how I can deal with my depression, anger, and anxiety over the divorce."

The Therapeutic Contract

I begin to establish a therapeutic alliance with Chet by formulating a working contract that will give some direction to his sessions. I discuss with him what I see as my primary responsibilities and functions, as well as Chet's responsibilities in the process. I want him to know at the outset that I expect him to be an active party in this relationship, and I tell him that I function in an active and directive way. (This is characteristic of most of the cognitive behavior and action-oriented therapies.)

Early in the counseling process, it is essential that I get some sense of what Chet wants from counseling and from life. Although his responses are vague at first, I work with Chet to be as specific and concrete as possible regarding how he views his life situation and what he expects from the therapy process. (This process is especially important in Adlerian therapy, behavior therapy, cognitive behavior therapy, reality therapy, and feminist therapy. Goals are discussed further in Chapter 3.)

Establishing a therapeutic contract, which is a part of informed consent, is critical at the beginning of the counseling process, but timing must be considered. I have explained both confidentiality and the limits of confidentiality. In my assessment of Chet's concerns, I am evaluating both suicidality and homicidality. Chet's description of feeling hopeless, angry, and depressed following a long-term relationship breakup are all potential areas of concern for self-harm and harm to others, and they warrant heightened attention both at the beginning of treatment and throughout the process of therapy as the need arises. Hopelessness can be particularly dangerous, and I work to instill hope right from the beginning, even in small amounts. I also begin with a stabilization process to evaluate Chet's current coping strategies, and I teach him some mindfulness relaxation exercises. These new coping strategies are designed to enable Chet to manage his current thoughts and feelings, and they will be helpful in future experiences as our work deepens.

Educating Chet about how he can get the most from his therapy is important, but I need to be careful not to overwhelm him with too much information or interfere with his typical style of presenting himself. I strive to listen carefully and to let Chet know what I am hearing without talking too much. Being fully present in the therapy session and giving Chet my sincere attention will yield dividends in our subsequent work together. If I listen well, I will get a good sense of why Chet has come to therapy. If I fail to listen accurately and sensitively, there is a risk I will be too eager to attack the first problem he mentions. Patience is critical in learning to listen before designing and implementing an intervention.

Initial Assessment

During the first two sessions, I conduct an assessment of Chet, which helps me to conceptualize his case and implement a treatment plan.

I develop hypotheses about Chet, and the two of us discuss them. Diagnosis is often a part of this assessment process, and it consists of describing behavior and thinking about its meaning. In this way, diagnosis becomes a process of thinking *about* Chet *with* him. I think of diagnosis as a general descriptive statement identifying Chet's style of functioning. Based on the process of assessment over a period of time, I develop hunches about Chet's behavioral style, and I may share these observations with him as part of the therapeutic process. Accurate diagnosis is important in some clinical settings, but it is not the defining point of therapy in general, nor the goal of the initial session.

From the first couple of sessions, I encourage Chet to spend time reflecting on what he would like to change in his life and what he wants from therapy. My main message is that change will come about to the degree that he becomes an active participant in the therapy sessions. I reinforce the importance of taking small steps that will result in the changes he desires.

Existential Themes

Some existential themes surface even in our initial session. Chet has generated stories to make sense of himself and his world. It takes several sessions for him to reveal these key themes in his life. Although Chet feels that he has made bad choices in the past, I want him to realize that he does not have to be the victim of his past experiences and that he has the capacity to redesign his future. Chet's anxiety is a vital part of living with uncertainty and freedom. Because there are no guarantees in life, Chet can expect to experience some degree of healthy anxiety, aloneness, and guilt. These conditions are not self-destructive in themselves, but the way in which Chet copes with these existential conditions is critical.

Listening to Chet's Story

During the early phases of Chet's therapy, I do not dwell exclusively on his problems. One of my tasks as a counselor is to assist Chet in freeing himself from problem-saturated stories and to help him re-author his story about himself and his relationships. I strive to establish a collaborative approach with a special interest in listening to Chet's story, searching for times in his life when he was resourceful, and assisting him in separating himself from identifying with his problem. From the very beginning of our work together, I listen for times when Chet is not bogged down with his problems and when he was able to draw upon some strength. Chet has many personal resources and strengths to help him make fundamental changes in his life. Thus, I draw on the strength-based approaches to counseling that will assist Chet in recognizing his personal competencies. He is more than a set of problems. Chet will discover and come to appreciate his personal resources and how to draw from them as he faces the task of re-creating his life story.

Reflection Questions

Solution-focused therapists use exception questions to discover clients' strengths, and narrative therapists focus on unique events that suggest alternatives to problem-saturated stories that enable clients to create new stories about their life experiences. Consider these questions in working with Chet during the initial session.

- What are a few salient points you would want to address with Chet in educating him about the therapy process?
- Chet says, "I want to know how to cope with the breakup of my 15-year marriage. I really need to know how I can deal with my depression, anger, and anxiety over the divorce." What are you likely to say to Chet in response to what he tells you?
- How might you help Chet begin to free himself from problem-saturated stories and help him re-author his story about himself and his relationships?

Concluding Comments

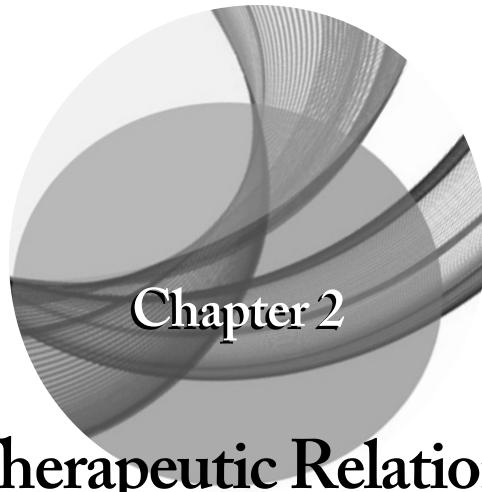
I am convinced that an integrative perspective is needed to effectively counsel the diverse range of clients who seek counseling. Each therapeutic approach has useful dimensions, and accepting the validity of one model does not necessarily imply a rejection of seemingly divergent models. It is not a matter of a theory being "right" or "wrong," for every theory offers a unique contribution to understanding human behavior and has unique implications for counseling practice. Pluralism in society necessitates choosing an integrative approach to counseling practice.

Of necessity, discussion of the theoretical orientations mentioned in this chapter has been brief. For a more elaborate discussion of the various theoretical approaches, see *Theory and Practice of Counseling and Psychotherapy* (Corey, 2017).

After reading this chapter on the beginning of counseling, take time to reflect on the following questions as a way of clarifying your thoughts:

- At this point, what is the main theory you would choose as a framework for practice?
- What are the main challenges you face in developing your personalized approach to counseling?
- What aspects of informed consent would you want to be sure to address with a client during the initial counseling session?
- What do you consider to be the main advantages and disadvantages of time-limited therapy or brief therapy?

- What experiences of being a client provided you with the most valuable lessons about counseling others?
- Do you think it is essential to arrive at a diagnosis of a client during the initial session? Why or why not?
- To what degree do you believe that assessment and diagnosis should be a collaborative endeavor?
- What are some advantages and disadvantages of designing your integrative approach based on the thinking, feeling, and doing dimensions?



Chapter 2

The Therapeutic Relationship

It is essential to use skills and techniques effectively—and to have a theoretical base from which to draw a range of techniques—but these abilities are meaningless in the absence of a therapeutic connection that is characterized by mutual respect and trust, an orientation central to many approaches from Adlerian therapy to the modern approaches that incorporate neuroscience in counseling (Miller & Dillman Taylor, 2016). The quality of this person-to-person encounter is what leads to positive change. Because therapy is a deeply personal relationship, I place central emphasis on the client–counselor relationship. My attitude toward the client and my personal characteristics of honesty, integrity, and courage are what I have to offer.

The existential approach is primarily concerned with the basic conditions of being human, and therapy is viewed as a shared journey. This approach to therapy provides the foundation upon which I conceptualize the therapeutic relationship. Both existential and person-centered approaches emphasize the personal characteristics and attitudes of the therapist. Existential therapists strive to be authentic and self-disclosing in their therapy work. Although existential therapists can be creative in developing techniques and may apply techniques from many other theoretical orientations, practitioners are not bound by specific techniques. Their interventions are guided by a philosophical framework about what it means to be human. Therapy is a journey taken by counselor and client, a journey that delves deeply into the world as perceived and experienced by the client. This type of quest demands that the therapist be in contact with his or her own phenomenological world as well.

Developing a Working Alliance

The *therapeutic alliance* refers to the therapist and the client working together to achieve agreed upon goals. Many studies demonstrate that the quality of the therapeutic alliance is a robust predictor of therapy outcomes and behavior change and is often cited as a general factor related to successful therapy (Meichenbaum, 2017; Naar & Safren, 2017). The importance of the client–therapist relationship is a common factor among all approaches to counseling, yet some models place more emphasis on the quality of the relationship than do others. This is especially true of the existential, person-centered, and Gestalt approaches, and it is also true of feminist therapy and the postmodern approaches (solution-focused therapy and narrative therapy). These relationship-oriented approaches (sometimes referred to as experiential and humanistic approaches) are all based on the premise that the quality of the client–therapist relationship is primary, with techniques being secondary. Key to establishing this relationship is the therapist's ability to understand the experiential world of the client and to empathically communicate this understanding to the client. Other therapeutic approaches, such as the cognitive behavior therapies, acknowledge that a therapeutic alliance is a necessary, but not a sufficient, condition for change.

The therapist does not establish the working alliance alone. This is a collaborative effort of client and therapist, and they co-create the therapeutic relationship (Cain, 2010, 2016; Meichenbaum, 2017; Naar & Safren, 2017). The quality of the therapeutic relationship is of critical importance to therapy outcomes (Hubble et al., 2010; Keenan & Rubin, 2016; Kottler & Balkin, 2017). Researchers have repeatedly confirmed that the client–therapist relationship is central to therapeutic change and is a significant predictor of both effectiveness and retention of therapy outcomes (Elkins, 2016; Keenan & Rubin, 2016; Miller, Hubble, & Seidel, 2015; Naar & Safren, 2017). An effective therapeutic relationship fosters a creative spirit that focuses on developing techniques aimed at increasing awareness, which enables clients to change their thinking, feeling, and behaving. One of the most comprehensive discussions of the therapeutic relationship can be found in *Psychotherapy Relationships That Work* (Norcross & Lambert, 2019).

Part of the therapeutic relationship involves listening to client feedback. It is essential to use client feedback to inform, guide, and evaluate the treatment process (Duncan, Miller, Wampold, & Hubble, 2010). Active client participation and experiencing meaningful change in an early stage of treatment are the most potent contributors to a positive outcome. The success of psychotherapy is best predicted by client contributions and the therapy relationship (Norcross et al., 2016). One of the best ways to improve the effectiveness of psychotherapy is by taking direction from clients through client-directed, outcome-informed therapy (Duncan et al., 2004; Miller, Hubble, Duncan, & Wampold, 2010; Miller et al., 2015).

Here are some assumptions I hold and some key notions that are central to my definition of effective therapy. These statements are the foundation of my personal metatheory.

- The quality of the person-to-person encounter in the therapeutic situation is the catalyst for positive change.
- The therapist's role is rooted in his or her way of being; attitudes are more important than the techniques employed.
- The counselor's chief role is to be present with the client, both physically and emotionally, during the therapeutic hour.
- The basic work of therapy is done by the client; the therapist's job is to create a climate in which the client is encouraged to try out new ways of being.
- The therapist can best invite the client to change by modeling authentic behavior.
- The therapist must adapt her or his techniques and relationship style to meet the needs of the client.
- The therapist's attitudes and values are at least as critical as his or her knowledge, theory, or techniques, but it is unethical for the therapist to impose those values on the client.
- The therapist has an ethical responsibility to acquire and use professional knowledge and skills in providing counseling services to a diverse range of client populations.
- To function optimally, the therapist must have a good connection with the client and be centered within him- or herself.
- A therapist who is not sensitively tuned in to his or her own reactions to a client runs the risk of becoming a technician and is likely to miss key pieces of information.
- An effective therapeutic alliance enables the client to experience the safety necessary for risk-taking behavior.
- Awareness emerges within the context of authentic engagement between therapist and client.

These somewhat overlapping notions give a sense of the paramount importance of the therapeutic relationship. If you operate within a relationship-oriented framework, you will be much less anxious about using the right technique or about stimulating clients to think, feel, or act in a specific manner. Of course, you will most likely utilize a wide range of techniques designed to enhance the capacity of clients to savor their experience.

As a therapist, you might conceive of your role as that of a consultant. This role is stressed in both behavioral and solution-focused approaches. It is also at the center of life coaching, an increasingly popular focus for some in the helping professions. Your clients tell you what they want, and you serve as a resource person, helping them explore ways to achieve their goals. As a consultant, you teach your clients specific strategies they can use in a variety of situations.

The person of the therapist is a critical ingredient in the therapy process (Cain, 2010, 2016; Hubble et al., 2010; Keenan & Rubin, 2016;

Wampold, 2010). Who you are as a person will influence your clients' progress in therapy. We invite our clients to grow by modeling authentic behavior. The therapist who possesses wide knowledge, both theoretical and practical, yet lacks human qualities of compassion, caring, good faith, honesty, authenticity, and sensitivity will have difficulty making a significant difference in the lives of clients.

Self-Disclosure in the Therapeutic Relationship

Self-disclosure is one avenue for both letting clients know you better and of being "real" in your work; it is a clear pathway toward establishing a therapeutic relationship with clients. If you keep yourself hidden during the therapeutic session or if you engage in inauthentic behavior, clients will remain guarded and persist in their own inauthentic ways. You can help clients become more trusting and open by selectively disclosing your own responses when it may enhance the therapeutic process. Of course, this disclosure does not mean an uncensored sharing of every fleeting feeling or thought. Rather, it entails a willingness to share persistent reactions with clients, especially when this sharing can facilitate clients' progress.

Therapists' disclosures of their reactions in the here and now, especially as they pertain to interactions with clients, are generally most useful. Therapist self-disclosure is a value shared by a number of theoretical orientations, including Adlerian therapy, existential therapy, the person-centered approach, Gestalt therapy, reality therapy, and feminist therapy. Existential therapists use disclosure in the service of their clients, and these disclosures are appropriately timed and are shared with clients in respectful ways (Yalom & Josselson, 2019). Gestalt therapists are encouraged to be self-disclosing regarding their feelings in the therapeutic relationship (Yontef, Jacobs, & Bowman, 2019). And my Adlerian colleague Jim Bitter and Rebekah Byrd (2011) provide an excellent description of how self-disclosure and storytelling can be combined for use with individuals, groups, couples, or families.

The traditional or classical model of psychoanalytic therapy discourages therapists from engaging in self-disclosure in the therapeutic relationship. Therapists functioning from a classical psychoanalytic model typically assume an anonymous stance, sometimes called the "blank-screen" approach, and engage in very little self-disclosure, maintaining a posture of neutrality. By doing so, therapists are attempting to foster a transference relationship in which their clients will make projections onto them. If the therapist reveals little and rarely shares personal reactions, it is assumed that whatever the client feels toward the therapist is largely the result of projections associated with other significant figures from the past. These projections, which have their origins in unfinished and repressed situations, are considered the very essence of the psychoanalytic endeavor.

Self-disclosure is one way to diminish clients' likelihood of projecting unrealistic reactions onto therapists. If therapists are not mysterious

figures and clients know something about their therapists, they are less likely to include therapists in their projections. Contemporary relational psychoanalytic therapy is based on the assumption that the therapeutic relationship is of central importance in facilitating client change. The contemporary relational version of psychoanalytic therapy allows for appropriate self-disclosure on the part of therapists, and therapists are encouraged to share their reactions to what is happening in the here and now of the therapeutic situation.

From the person-centered approach, I appreciate the emphasis given to immediacy and to an open discussion of how both parties are experiencing the therapeutic relationship. Immediacy entails self-disclosure within the therapy session in the here and now about a therapist's feelings and reactions to the client or about any aspect of the client–therapist relationship (Gelso, 2011). Perhaps the most important type of self-disclosure is the kind of here-and-now immediacy that focuses on what is transpiring between you and your client. This could be especially useful in cases in which clients are involved in a transference relationship with you. If you are having a difficult time listening to a client, for example, it could be useful to share this information. You might say: "I've noticed at times that it's very difficult for me to stay connected to what you're telling me. I'm with you when you talk about yourself and your own feelings, but I tend to get lost as you go into great detail about all the things your daughter is doing or not doing." In this statement, you are sharing your reactions about what you hear when your client tells stories about others, but the client is not being labeled or judged. Letting clients know how you are perceiving and experiencing them is an important form of immediacy. Selectively discussing some of your reactions toward them may be useful, especially if you encourage clients to discuss the feedback you give them.

Although I caution against indiscriminately revealing your personal problems to a client, sharing some aspects of yourself can be a powerful way to connect with your client. Judging the appropriate amount of self-disclosure can be difficult, even for seasoned counselors. In determining the appropriateness of self-disclosure, consider *what* to reveal, *when* to reveal, and *how much* to reveal. Either disclosing too little or disclosing too much can be problematic. If you try too hard to be genuine and self-revealing, you may burden your client with inappropriate personal details. It is critical that what you reveal is appropriate and timely, and that it is done for the client's benefit. If your feelings are very much in the foreground and inhibit you from fully attending to a client, it may be helpful to share how you are being affected with your client. Gelso (2011) recommends that therapists think deeply and empathically about what their client needs and that therapists monitor their own needs and conflicts carefully, use self-disclosure infrequently and in small doses, and always think about what benefits the client. To determine whether your self-disclosures are in the service of your clients, ask yourself these questions:

- What motivated me to disclose?
- To what extent do my disclosures help clients talk more honestly and specifically about themselves?
- To what extent might my disclosures facilitate clients' progress toward their treatment goals?
- Does what I share about myself with clients help them put their problems into a new perspective or help them consider new alternatives for action?
- To what extent is my self-disclosing helping clients translate their insights into new behavior?

Self-disclosure does not mean telling clients detailed stories about your personal past or present problems. Too much sharing of yourself can easily distract both you and your clients from productive self-exploration. One of the main risks of counselor self-disclosure is shifting the focus to the counselor's needs and away from the client's needs. Sometimes merely letting a client know that you have similar personal issues or similar feelings about a life situation, without going into elaborate detail, can do a great deal to enhance the therapeutic relationship. Your disclosures can help a client accept that his or her reactions are normal and can result in a sense of being understood by you. Admittedly, you are vulnerable when you share your own experiences, feelings, and reactions. Yet can you expect your clients to be willing to be vulnerable in front of you if you remain unknown?

Self-disclosure is not recommended with all clients, and cultural factors need to be considered. Some clients may view self-disclosure as inappropriate in a professional relationship with an authority figure. Cultural norms inhibit many Asian American clients from revealing personal concerns to those outside the family. There are often similar pressures on Latino, American Indian, and African American clients, military clients, and those from various other cultures. Counselors need to realize that cultural forces are most likely operating when clients are slow to disclose personal details. Talking about themselves personally to a professional counselor whom they do not know may be uncomfortable for these clients. Therapists' self-disclosures must be measured and may best be pursued as an exchange of cultural norms regarding this new relationship.

Your Role as a Counselor in Creating a Working Relationship

Personal Characteristics of the Counselor

What counselor attitudes would be conducive to establishing good rapport with clients, and what behavior would you want to model for clients? The person-centered approach has contributed greatly to an understanding of the central role of the therapeutic relationship in the healing process. Carl Rogers (1957, 1961, 1980) did pioneering

work on three personal characteristics of the therapist that form the essence of the therapeutic relationship: (a) congruence or genuineness, (b) unconditional positive regard and acceptance, and (c) accurate empathic understanding. Known as the therapeutic core conditions, these factors are a basic part of all theoretical orientations. Congruence, positive regard, and empathy, along with establishing a collaborative therapeutic alliance, have been shown to be predictive of client improvement (Cain, 2010, 2016; Keenan & Rubin, 2016).

Congruence means that in your role as a therapist you are real; you are genuine, integrated, and authentic during the therapy hour. You are without a false front; your inner experience and outer expression of that experience match. You can openly express feelings, thoughts, reactions, and attitudes that are present in the relationship with your client.

In your therapeutic work, you need to communicate your caring as well as a genuine interest in your client. This caring is unconditional in that you do not judge your client's feelings, thoughts, and behavior as good or bad. You value and accept the client without placing stipulations on this acceptance. Acceptance recognizes a client's right to have her or his own beliefs and feelings. One of your main tasks is to understand your client's experiences and feelings with sensitivity and accuracy as they are revealed in the moment-to-moment interaction during the therapy session.

It is imperative that you strive to sense your client's subjective experience, particularly in the here and now. Empathy requires a deep and subjective understanding of the internal world of the client and a sense of personal identification with the client's experience. By tuning in to your own feelings, you are able to share the client's subjective world. Empathic understanding implies that you will sense your client's feelings as if they were your own without becoming lost in those feelings. This empathy on your part deepens the client's self-understanding and helps the client clarify his or her beliefs and worldview.

Action-Oriented Approaches and the Therapeutic Relationship

Even the action-oriented approaches (such as behavior therapy, cognitive behavior therapy, rational emotive behavior therapy, reality therapy, and solution-focused brief therapy) view the quality of the client–therapist relationship as a core element of effective therapy. Unlike a person-centered therapist who is not particularly concerned about techniques, action-oriented therapists draw upon techniques designed to produce specific changes. For example, behavior therapists believe clients are mainly helped by the skillful application of specific behavioral interventions that have been validated by evidence-based research. However, the successful application of these research-supported techniques requires a good working alliance. Beyond a working relationship, which is monitored throughout the duration of counseling, the behavior therapist is expected to be skilled in designing

interventions that will help clients change in the direction they choose. (A more extensive discussion of the role of techniques in assisting clients in changing the way they are thinking, feeling, and behaving is presented in Chapters 6, 7, and 8.)

Establishing the Therapeutic Relationship

If your clients are to feel free to talk about their problems, you must provide attention, active listening, and empathy. Person-centered therapists assume that clients must sense your respect for them, which you demonstrate by your attitudes and behaviors. You reveal an attitude of respect when you are concerned about your clients' best interests, view them as being able to exercise control of their own destiny, and treat them as unique individuals rather than as stereotypes. Your clients will benefit greatly by your acknowledgment that their concerns are important and that it takes courage to talk about themselves. You demonstrate your attitudes toward your clients through your behavior in a session. Some of these behaviors include the following:

- Actively listening to and understanding clients
- Acknowledging their desire to change
- Suspending critical judgment
- Expressing appropriate warmth and acceptance
- Communicating that you have an understanding of their world as they experience it
- Communicating that you respect their values, even though they may differ from your own
- Providing a combination of support and challenge
- Assisting clients in cultivating their inner resources for change
- Encouraging clients to take specific steps to bring about change

This list of attitudes and behaviors is central to the person-centered approach. Ask yourself how well you are able to pay attention to others, to fully listen to them, and to empathize with their situation. Assess the qualities you possess that will either help or hinder you in assuming the internal and subjective frame of reference of clients. Because these qualities exist on a continuum, it is best to think about the degree to which you see yourself possessing these attitudinal and behavioral characteristics. Consider how you would rate yourself on these questions:

- How do you attend to what clients are telling you both verbally and nonverbally? Do you pay attention mainly to what clients disclose, or do you also notice the way they deliver their messages?
- How do you let others tell their story? Do you get impatient and want to interrupt? Do you encourage clients to tell stories in great detail to satisfy your own curiosity? Do you have a tendency to get lost in the details of their story and miss the essence of their struggle?

- How often are you able to detect the core messages when clients speak? How do you check with clients to make certain you are understanding them?
- How do you keep clients focused on issues they want to explore? Can you keep your own centeredness, even when clients may seem very fragmented or are making demands on you?
- To what degree can you set aside your own biases for a time and attempt to enter the client's world? For example, if you consider yourself a self-sufficient or independent woman or man, are you willing to accept the client who tells you she is willing to be subservient to her husband?
- How do you communicate your understanding and acceptance to clients?
- To what extent do you work nondefensively when you detect signs of resistance from clients? How do you use this resistance as a way of helping clients explore their issues more deeply?

Although it may seem deceptively simple to merely listen to others, the attempt to understand the world as others see it is demanding. Respect, genuineness, and empathy are best considered "states of being," not techniques to be used on clients. *Being* genuine, rather than *acting* genuine or using genuineness techniques, is key to forming a successful therapeutic relationship (Gelso, 2011). Person-centered therapists establish a working relationship with clients by being genuine and respectful in observable ways and by recognizing that the relationship is a two-way process in which clients' interests assume priority.

Helping Clients Gain a Focus

People who seek assistance are often overwhelmed by their problems. By trying to talk about everything that is troubling them in one session, they also may manage to overwhelm you. A focusing process is necessary to provide direction for the therapeutic efforts. To achieve this focus, make an assessment of the major concerns of the client. You could say to a client who presents you with a long list of problems: "Unfortunately, we won't be able to deal with all your problems in one session. What was going on in your life when you finally decided to call for help?" Other focusing questions are: "At this time in your life, what seems most pressing and troublesome to you?" or "If you could address only one problem today, which one would you pick?" Alternatively, you can help clients cluster their concerns from the list into a few themes that are more manageable.

Once clients determine what concerns they are willing to explore, you can collaborate with them in designing a treatment contract. By focusing on what is salient in the present, you assist clients in clarifying their own problems and establish a direction for you to design other interventions. Your power to heal others is the result of a process of genuine dialog with your clients.

Becoming the Client: Sharing in a Collaborative Partnership

Therapy as a Joint Venture

Throughout this book I invite you to assume the role of a client from time to time. The aim of asking you to become the client is to personalize the material and provide you with an experiential sense of how integrative therapy can be applied.

Research supports the value of the client as an active agent, but Bohart and Tallman (2010) have noted that the client is often the neglected common factor in psychotherapy. The importance of client involvement as key to a successful outcome has been clearly established in empirical research (Gonzales, 2016). Imagine that you and I are beginning a therapeutic relationship. Let's expand on the idea that therapy is a joint venture with both of us working together toward common goals. I see many advantages to your assuming an active role as a client, and I encourage you to participate as fully as possible in all phases of assessment and treatment. Therapy at its best is an active process of working with the client as an active self-healer. Your own resources, life situation, and readiness for counseling are critical to a positive outcome. A number of therapeutic approaches stress the notion of therapy as a collaborative partnership, including feminist therapists, Adlerians, cognitive behavior therapists, behavior therapists, motivational interviewing practitioners, solution-focused therapists, and narrative therapists.

If you appear to be getting little from the therapeutic relationship, I examine my part in this outcome by asking myself about my involvement with you as your therapist. In addition, I explore with you how you might be contributing in part to your lack of progress. I recognize that I cannot make you want to change, but I can create a climate in which together we are able to look at the advantages and disadvantages of making changes. Both of us share the responsibility for creating an environment that is supportive of change.

The Relationship During the Early Stage of Counseling

During the first few sessions my main task is helping you define and clarify your problems. My aim is to build a relationship with you that will encourage you to reveal your story, focus more clearly on what you want to change, and attain a new perspective in dealing with your problems. What do you think your role should be in this process?

You might be seeking professional assistance because you realize you are not dealing with problem situations satisfactorily. You may seek counseling because you struggle with self-doubt, feel trapped by your fears, or suffer from some form of loss. You may need to heal from psychological wounds. And you might seek help not because you feel plagued by major problems but because you are not coping

with daily challenges as effectively as you would like. You may find yourself in a meaningless job, experience frustration because you are not living up to your own goals and ideals, or feel dissatisfied in your interpersonal life. You realize that you are not managing your life as well as you might. Why have you come for counseling?

Creating a Therapeutic Climate

How open are you to self-exploration? The kind of climate I am able to create during the initial sessions is crucial to a good therapeutic relationship. I can make the mistake of working too hard, asking too many questions, or offering quick solutions. If we can create a collaborative partnership, I can greatly assist you by teaching you how to assess your own problems and search for your own solutions. How much responsibility are you willing to assume, both inside and outside the sessions? (These aspects are central to both behavior therapy and reality therapy.) You will learn to identify and clarify problem areas and acquire problem-solving skills to use in a variety of difficult situations in everyday living. In a sense, from the very first meeting, I can be most helpful to you by encouraging you to look within yourself for resources and strengths you can draw on to better manage your life.

Considering the Cultural Context

You may be frustrated and angry due to societal factors such as discrimination in your workplace, perhaps because of your age, gender, disability, race, ethnicity, religion, or affectional orientation. It would be a disservice if I were to encourage you to settle for injustices in an oppressive environment. Instead of merely solving your presenting problem, I can begin supporting you in your efforts to take action within your community to bring about change. To accomplish this, I may need to learn a lot more about what oppression is like for you and others who experience similar discrimination; I may have to prepare myself to assume a variety of helping roles—educator, advocate, social change agent, and influencer of policymakers.

I must respect the purpose for which you initiated therapy. Paying careful attention to what you tell me about what you want is the foundation of our work. My task then is to encourage you to weigh the alternatives and to explore the consequences of what you are doing with your life. Even though oppressive forces may be severely limiting the quality of your life, you are not merely the victim of circumstances beyond your control. What can you do to improve your situation? Together we may be able to discover new courses of action that will lead to a change in your situation.

As I engage you in identifying and assessing your problems, I must avoid a stance of "blaming the victim." (As both feminist and family systems therapy stress, the source of your problems may be within your environment rather than due to an internal conflict on your part.) It may be that you come to me not to resolve internal conflicts but to better understand and deal with external stressors in your environ-

ment. I am aware that self-reliance and independence may not be a part of your worldview, and interdependence may be a core value in your life. It can be useful to put you in contact with external resources within the community that you can utilize in meeting the demands of daily living. You may need my services and my guidance to establish links to resources within your community. Some of these resources include arranging for legal assistance or assisting you in coping with day-to-day survival issues such as getting a job, arranging for child care, or taking care of an elderly parent. This is a vital part of the community approach to change.

Treating all clients in the same manner and using the same basic interventions will most likely restrict the effectiveness of therapy. The "one size fits all" notion hampers creativity and can lead you to feel misunderstood. What can you tell me about your basic values and beliefs? Understanding your cultural background helps me establish a therapeutic working relationship. Although it is not necessary that I have an in-depth understanding of your culture and worldview, I must know some of your basic beliefs and values if I hope to make a significant contribution to your change. I have an affirmative responsibility to engage diversity in a manner that will enlarge my understanding of difference. If I am not aware of the central values that guide your behavior and decisions, you will soon pick up on this and will not likely return for further sessions. It is not my job as your counselor to teach you my values but to assist you in clarifying your own values. I must respect your right to embrace values that are different from mine. It is my responsibility to manage any value conflict between us and not to view this as your problem.

I am open to a discussion with you regarding some of the similarities and differences in our background (such as ethnicity, age, disability, and gender, to mention a few). This allows us to be alert to the ways that our differences might influence our work together. Throughout the course of therapy, I encourage you to bring up any differences between us that may be hampering our work together.

Understanding the Family Context

From a family systems perspective, individuals are best understood within the context of their relationships and by assessing their interactions within an entire family. I cannot fully understand you by viewing you only from an individual frame of reference. It is necessary to work with you from a person-in-environment context, from a social embeddedness perspective, which includes having some appreciation of your family's past and present influence on you. What can you tell me about your family history and patterns?

From the family systems models, your problems are viewed as an expression of a dysfunction within a family, and these dysfunctional patterns often can be identified across several generations. The central principle of family systems therapy is that the client is connected to living interpersonal systems and that change in one part of the system

reverberates throughout other parts. Actions by any individual family member will influence all the others in the family, and their reactions will have a reciprocal effect on the individual. Therefore, as your counselor, it is not possible for me to accurately assess your concern without observing the interaction with and mutual influence among other family members, including the broader contexts in which you and your family live. To focus primarily on studying your internal dynamics without adequately considering interpersonal dynamics yields an incomplete picture of you. Your family provides a primary context for understanding how you function in relationship to others and how you behave.

Developing a Therapeutic Alliance With Sidney

Sidney is a 25-year-old, African American, male graduate student in mathematics. He is seeking counseling because he is feeling depressed. Sidney explained, "I can't put my finger on why I am feeling like this." He went on to report feeling irritable, sluggish, and having a tendency to isolate himself. In our first meeting, Sidney seems uncomfortable discussing how his past is influencing his present feelings. He also speaks in general terms and does not identify why he is coming to counseling.

Building the Therapeutic Relationship

I am convinced that one of the most significant factors determining the degree to which Sidney will attain his goals is the quality of the therapeutic relationship that he and I can create. I believe Sidney will get the most from his therapy if he knows how the therapeutic process works. I am mindful of the potentially harmful effects of the power differential in my relationship with Sidney and strive to build mutuality and a sense of partnership in the therapeutic endeavor. Although Sidney does not have a great deal of confidence in making decisions about his life, I will not serve him well if I short-circuit the process of assisting him in finding resources within himself.

One way of deepening the therapeutic relationship within our first session is to honor Sidney's fear of being vulnerable within our relationship. I disclose to Sidney that I feel somewhat lost, like I am missing a big piece of his story. I note that most African American men experience oppression, targeting, profiling, and numerous other forms of discrimination on a regular basis. Sometimes the natural dysphoria that results can feel like depression. My aim in this first session is not to push Sidney into telling me about his past and the significant events that brought him into counseling. Instead, I attempt to facilitate a process in which he feels safe to share when he is ready.

Once I disclose my feeling of lack of clarity on why Sidney is seeking therapy, he discloses his reasons for not sharing his past. Sidney discusses his sexual orientation and his intimate relationship with his male partner. We discuss his fear of being judged by me for identifying as gay. I am a White, male therapist, so we talk about ways he can feel safe within our

relationship. Throughout our discussion, I reflect his feelings of hesitation within the therapeutic relationship, his excitement within his romantic relationship, as well as his fear that his romantic relationship is in jeopardy. Sidney's past involved relationships that ended due to infidelity. These relationships have negatively influenced his self-esteem and self-image. The happiness and excitement he feels resulting from his current relationship scares him. Sidney reports feeling as though he did not deserve this happiness and that it was only a matter of time before this relationship ended. Sidney came into the session when this feeling of inevitability became overwhelming and caused him to experience depression.

I am most influenced by the person-centered approach, which places primary value on the client–therapist relationship. My genuine transparency, unconditional positive regard, and empathy influence Sidney's feeling of safety and his willingness to share his past. I began listening to his story during the first session, but more time is needed for him to share the many significant events and turning points in his life. In subsequent sessions, Sidney fleshes out his story. My aim is to listen to his story and to look for both present and past events that he can interpret in new ways. Sidney's life story influences both what he notices and what he remembers. In this sense, his story influences how he will perceive the future.

My main focus during our early work together is on being real, on accepting Sidney's feelings and thoughts, on demonstrating my unconditional positive regard for him, and on respecting him as a person. Because I trust Sidney's ability to find his own direction, I encourage him to participate with me in planning and structuring the sessions. If I am able to listen carefully and reflect what I am hearing, and if I am able to deeply empathize with his life situation, I believe that eventually Sidney will begin to clarify his struggles and work out his own solutions to his problems. Although he is only somewhat aware of his feelings at the initial phase of therapy, he will move toward increased clarity as I accept him without conditions and without judgments. I strive to create a climate of openness, trust, caring, understanding, and acceptance. If I am able to create this kind of climate, he can use our relationship to move forward and grow.

Reflection Questions

Reflect on how you might intervene with Sidney to enhance your therapeutic relationship with him. Consider these questions:

- Sidney is afraid to tell you he is gay and in a relationship with a male. How might you create a safe environment with Sidney?
- What aspects of Sidney's experience might you emphasize to deepen your therapeutic relationship with him?
- What might be difficult for you, if anything, about working with a gay African American male?

Developing a Therapeutic Alliance With Rita

Stages of Counseling With Rita

Rita initially sought therapy because of her nagging feeling of guilt and being convinced that she is inadequate as a mother. I ask Rita to reflect on any thoughts and feelings she might have about our previous sessions. She lets me know that she is not accustomed to looking at herself, nor is she comfortable being the focus of attention. We'll need to explore this further because much of the therapeutic process depends on clients paying increased attention to what they are experiencing and doing. Together we explore how Rita can assume an active role by thinking about topics she wants to bring up in her sessions and by deciding on some ways she wants to be different in her daily life. Rather than assume total responsibility for the direction of her therapy, I look for ways to collaborate with Rita on the direction of our work. We start with what Rita is presently experiencing, and I respect her need to set her own agenda and pace. As a teacher and a consultant, I am assisting her in learning about her own therapeutic process so she can continue her personal growth after therapy has ended.

Listening to Rita's Story

I make the assumption that many of Rita's problems have been created by her restricted and self-defeating vision of herself and her world. Part of our work together will be to look for inner resources that will enable her to create a new story for herself. From the narrative perspective, my commitment is to help her rewrite the story of her life. Through this collaboration, she can review and reframe events from her past and write a new story for her future. At this phase of our work, I am also influenced by the feminist therapy notion that our collaboration will be aimed at freeing Rita from the influence of oppressive elements in her social environment and empowering her to become an active agent who is directing her own life.

I did not begin the initial session by asking Rita a series of questions pertaining to her life history, but in this session I will ask questions to fill in the gaps in her story. Inquiring about her life history gives a comprehensive picture of how she views her life now, as well as events that she considers significant in her past. Through the use of questions that challenge her to separate herself from an identity that is linked to problems, I assist her in the process of re-authoring her life story. As a between-sessions assignment, I suggest an autobiographical approach and ask Rita to write about the critical turning points in her life. This will include events from her childhood and adolescent years, relationships with parents and siblings, school experiences, current struggles, and future goals and aspirations. I ask her what she thinks would be useful for her to recall and focus on, and what she imagines would be useful to me in gaining a better picture of her subjective world. In this way, Rita does some reflecting on her life experiences outside of the session, she takes an active role in deciding what her

personal goals will be for therapy, and I have a sense of where and how to proceed with her.

Helping Rita Externalize Her Problem

Rita's autobiography provides me with significant clues to the unfolding of the story of her life. During the early sessions, I encourage her to separate her identity as a person from her problems. I am influenced by narrative therapy in understanding the value of externalizing a client's problem. Narrative therapy insists that the person is not the problem, and I view Rita's problems as something separate from her. She presents many problems that are of concern to her, yet we cannot address all of them at once. When I ask her what one problem most concerns her right now, she replies, "Guilt. I feel guilty so often over so many things that I don't do. No matter how hard I work at what is important to me, I generally fall short of what I expect of myself, and then I feel guilty."

To help Rita view her problem of guilt as being separate from who she is as a person, I invite her to engage in a simple art therapy activity by drawing her guilt on paper. Together we process the colors, shapes, meanings, and experiences. Depending upon her readiness, we may take it further and give the guilt a voice and have a dialog using Gestalt therapy methods. After Rita has processed her art, I may use here-and-now self-disclosure to share how I am experiencing her process. This type of self-disclosure, when genuine and relevant, often serves to strengthen my relationship with my client. I then ask questions about how her guilt occurs and ask her to give examples of situations where she experiences guilt. I am interested in charting the influence of the problem of guilt. Using elements of solution-focused therapy and the framework of narrative therapy, I ask Rita questions aimed at finding exceptions to her feeling guilty:

- Has there ever been a time when guilt could have taken control of your relationship, but didn't? What was it like for you? How did you do it? What does it say about you that you were able to do that?
- How do you imagine your life would be different if you didn't have the problem of guilt?
- Can you think of ways that you can begin to take even small steps to separate yourself from unnecessary guilt?

My questioning is aimed at discovering moments when Rita hasn't been dominated or discouraged by the problem of guilt. This provides a basis for considering how life would be different if guilt were not in control. I assure her that there may be times when feeling some guilt is appropriate and can alert us that it is time to change our actions. However, we want to work together so that she can let go of unnecessary or overly burdensome guilt.

As our therapy proceeds, I expect Rita to gradually find that she has more control over her problem of guilt than she believed. As she is able to distance herself from these problematic themes (such as guilt), she will be less burdened by her problem-saturated story and will discover a range of options. She will likely focus more on the resources within herself to construct the life she wants.



Reflection Questions

Reflect on how you might intervene with Rita to enhance your therapeutic relationship with her. Consider these questions:

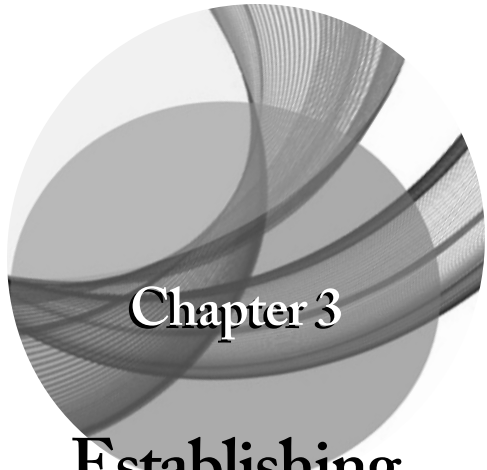
- Rita's problem-saturated story contains a theme of "guilt." What are some strategies you might employ in assisting Rita in separating her identity from this problem-saturated story and exchanging it for a liberating story?
- If Rita were to ask what you think of her, how would you be inclined to answer her?
- Would you want to give Rita answers to her problems? Why or why not? How do you think your answers might either help or hinder Rita in finding her own way?

Concluding Comments

The single most important element in becoming a competent counselor is your way of being. If you can be fully present and be yourself, you can be a catalyst for clients to engage in introspection, relevant self-disclosure, and risk-taking. Knowing who you are is the starting point for developing your own view of counseling. Techniques are secondary to your personal vitality and your ability to establish and maintain a growth-producing therapeutic relationship. Although skills and techniques are useful, they must be sensitively adapted to the particular client and context. Techniques are merely tools to amplify emerging material that is present and to encourage exploration of issues that have personal relevance to clients. Research increasingly supports the personal and interpersonal dimensions of therapy (such as presence and experiential encounter), and not the techniques employed, as the critical factors responsible for positive therapeutic outcomes (Schneider & Krug, 2017). As a therapist, one of your main functions is to provide encouragement to clients in making decisions regarding how they want to live.

After reading this chapter about the therapeutic relationship, take time to reflect on the following questions as a way of clarifying your thoughts on the kinds of relationships you want to create with your clients:

- How do you view the role of techniques in the counseling process?
- What emphasis do you place on the client–therapist relationship? If you had to describe in a few words what you consider a "working relationship" to be, what would you tell your client?
- How important do you consider your self-disclosure to be in your work as a counselor? How do you assess when it might be helpful to your client for you to self-disclose? What kinds of disclosures are you most likely to make? Are there some disclosures you are not likely to make to clients?
- To what degree do you agree that counseling is a collaborative venture? What are some specific things you are likely to say and do as a counselor during the initial session to establish a collaborative relationship? What are some things you might say to clients about what they can expect of you as their counselor? What would you most expect of them as clients?
- If you were a client in counseling, what kind of relationship would you want and expect from your counselor?
- Carl Rogers identified three therapeutic factors that contribute to successful therapy outcomes: congruence, unconditional positive regard, and empathy. In what ways are these core conditions basic to all theoretical models?
- Are you able to be empathic and respectful of clients? What work do you need to do to improve in these areas?
- To what degree are you a good listener? Do you sometimes find yourself starting to think about your response when you are listening to a person (or a client)? In a conversation, are you able to devote your full attention to what the other person is saying?
- What things would you most want to attend to during the early stage of counseling to create a safe environment for clients to explore their deepest concerns?



Chapter 3

Establishing Therapeutic Goals

All theoretical orientations address the central role of goals as a factor in successful outcomes. Some therapies focus on expressing feelings, others stress identifying cognitive patterns, and still others concentrate on changes in behavior. I believe combining a thinking, feeling, and acting approach is most useful when integrating theory and technique.

In working with clients, establish target goals in each of the areas of their functioning, especially those that clients view as problematic. Even with the same client, you will at times be focusing on different dimensions of functioning such as changing beliefs, exploring a range of feelings, or implementing behavioral changes. It is essential to be flexible in defining with your clients a variety of goals that provide direction for their therapy. Without clear goals, the counseling sessions will most likely not be productive.

The counselor's theory greatly influences the types of goals established as well as the methods used to reach those goals. In discussing counseling goals with clients, address goals pertaining to the therapeutic process, especially general goals that influence the way you work as a counselor. For example, you would probably agree that it is desirable for clients to strive to know themselves and to make choices about the way they want to live.

Many process-oriented goals are part of the therapeutic endeavor, such as clients engaging in personal reflection, self-disclosure, risk-taking, and doing work outside the therapy sessions to bring about change. It is useful to articulate these process goals early in the counseling relationship.

Both counselor and client should clearly understand the goals and the desired outcomes of their work together. Work together with your

clients to set specific, measurable, manageable goals. Consider cultural determinants and the client's cultural definition of psychological health when defining therapeutic goals. The person-in-relation and the cultural context are essential aspects when developing appropriate goals for the counseling process (Corey, Corey, & Corey, 2019).

Overview of Goals From Various Theoretical Perspectives

The goals of counseling are almost as diverse as are the theoretical approaches. Goals include restructuring the personality, uncovering the unconscious, finding meaning in life, curing an emotional disturbance, replacing faulty beliefs with constructive beliefs, developing trust in oneself, increasing social interest, becoming more self-actualizing, reducing anxiety, shedding maladaptive behavior and learning adaptive patterns, gaining more effective control of one's life, designing creative solutions to life's challenges, re-authoring one's life story, finding exceptions to problems, finding inner resources, acquiring emotional competence, reducing ambivalence to change, becoming aware of and reducing the influence of gender-role socialization, acquiring personal empowerment, and creating new patterns of relationships within a family system. Given this broad range of goals, the perspectives of the client and the therapist on goals will have an impact on the therapeutic interventions chosen. Who sets the goals of counseling is best understood in light of the theory from which you operate, the type of counseling you offer, the setting in which you work, the problems of the client, and the characteristics of your client.

Goals of Some of the Theoretical Perspectives

A brief overview follows that describes some key goals associated with each of the various theoretical perspectives that have the potential for developing an integrative approach to counseling practice.

Psychoanalytic Therapy

A primary goal is to make the unconscious conscious. Other approaches also talk about helping clients move from out of awareness into awareness, but the psychoanalytic model focuses on those nonconscious processes that have been part of the person for a very long time. Successful outcomes of psychoanalytic therapy result in significant modification of an individual's personality and character structure. This goal is accomplished through interventions such as maintaining the analytic framework, free association, interpretation, dream analysis, analysis of resistance, and analysis of transference.

Existential Therapy

The principal goal is to challenge clients to recognize and accept the freedom they have to become the authors of their own lives. Therapists encourage clients to examine the ways in which they are avoiding

their freedom and the responsibility that accompanies it. They focus on the larger issues in living a fully functional life: the meaning of existence, life and death, existential anxiety, love and will, courage and creation, and the emergence of the experience of really being in the world. People are regarded as struggling, feeling, thinking beings who have hopes and fears and who wrestle with creating meaningful lives (Yalom & Josselson, 2019).

Person-Centered Therapy

A major goal is to provide a climate of safety and trust in the therapeutic setting so that the client, using the therapeutic relationship for self-exploration, can become aware of blocks to growth. Because this approach stresses the client–therapist relationship as a necessary and sufficient condition leading to change, it minimizes directive techniques, interpretation, questioning, probing, diagnosis, and collecting history. Person-centered therapists do not hide behind their professional role; they are naturally themselves in relations with their clients (Cain, 2010).

Gestalt Therapy

The central goal of Gestalt therapy is gaining awareness. Gestalt therapy is an exploration rather than a direct attempt to change behavior or to cure symptoms (Yontef et al., 2019). The goal is to attain awareness so clients can expand their choices. Awareness includes knowing the environment and knowing oneself, accepting oneself, and being able to make contact. As Miriam Polster notes, awareness is the alpha and omega of experience (Polster & Polster, 2000). Clients are helped to pay attention to their own awareness process so they can fully experience contact with self, others, and the environment. Awareness that one is even in an experience is the first step; awareness after the experience is necessary for meaning. Gestalt therapists create a safe emergency for their clients so that individuals may experiment within therapy before enacting change in life outside of therapy. It is within this safe relationship that clients learn to be responsible and selectively and discriminatingly make choices.

Behavior Therapy

The cornerstone of behavior therapy is the identification of specific goals at the outset of the therapeutic process. The general goals are to increase personal choice and to create new conditions for learning. An aim is to eliminate maladaptive behaviors and to replace them with more constructive patterns. The client and therapist collaboratively specify treatment goals in concrete, measurable, and objective terms.

Mindfulness

Mindfulness, a part of the "third wave" of behavior therapy, is another way to focus awareness and to become fully present in life; it can lead to the development of self-compassion, a form of self-care when facing difficult situations. Mindfulness practices focus on moment-to-moment experiencing and assist clients in developing an attitude of open

awareness and acceptance of what is rather than being self-critical. When we acknowledge our shortcomings without critical judgment, we can begin to treat ourselves with kindness. We can intentionally activate goodwill toward ourselves and others when we experience anger, anxiety, or depression. Some neuroscience research supports the relationship between mindfulness-based interventions and improved emotional regulation, attention, cognition, and awareness (Miller & Dillman Taylor, 2016).

Cognitive Behavior Therapy

Cognitive behavior therapy (CBT) is an educative therapeutic approach in which clients acquire cognitive, behavioral, and emotional regulation skills. A key component of CBT is assisting clients in learning how to evaluate and change long-standing behaviors by identifying their automatic thoughts and introducing cognitive restructuring strategies to help clients recognize, challenge, and modify unhelpful beliefs and thoughts. The client and the counselor together investigate the validity of the client's thoughts and beliefs, a process known as collaborative empiricism (Naar & Safren, 2017). Changes in beliefs and thought processes tend to result in changes in the way people feel and how they behave. Clients learn practical ways to identify their underlying faulty beliefs, to critically evaluate those beliefs, and to replace them with constructive beliefs.

Reality Therapy

The overall goal is to help people find better ways to meet their needs for survival, love and belonging, power, freedom, and fun. Clients are assisted in examining what they are doing and thinking to assess whether this is getting them what they want. Changes in behavior tend to result in the satisfaction of basic needs. Clients are expected to identify long-term goals by addressing where they would like to be in their life in the future, say, one year from now (Wubbolding, Casstevens, & Fulkerson, 2017).

Motivational Interviewing

People who seek therapy are often ambivalent about making behavioral changes, and their motivation may ebb and flow during the course of therapy. A central goal of motivational interviewing is to increase internal motivation to change based on the personal goals and values of clients. The therapist helps clients formulate specific goals by encouraging them to explore their ambivalence about changing.

Narrative Therapy

Narrative therapists invite clients to describe their experience in fresh language, which tends to open new vistas of what is possible. The heart of the therapeutic process involves identifying how societal standards and expectations are internalized in ways that constrain and narrow the kind of life clients are capable of living. Using these unique events and other points of entry in the co-construction of preferred outcomes,

clients are able to re-author their stories. The goal is for clients to experience a heightened sense of personal agency to act in the world.

Solution-Focused Brief Therapy

Clients establish their own goals and preferences in a climate of mutual respect, dialog, inquiry, and affirmation. Much of the therapeutic process involves clients thinking about their future and what they want to be different in their lives. Both solution-focused therapy and narrative therapy rely heavily on preferred stories and preferred outcomes. Identifying those preferred outcomes is central to setting a course in therapy.

Feminist Therapy

The main goals of feminist therapy include equality, challenging patriarchy and sex-role stereotypes, balancing independence and interdependence, self-nurturance, empowerment, social change, and valuing and affirming diversity. Feminist therapists believe gender is central to therapeutic practice and that understanding a client's problems requires adopting a sociocultural perspective. Clearly, social advocacy is a central part of this approach to working with clients.

Goals From an Integrative Perspective

Despite the diversity of therapy goals, most approaches share some common features even though they differ in their assumptions about the best way to achieve these goals. To some degree, they have the goal of identifying what clients want and then helping them modify their thoughts, feelings, and behaviors. Although individuals may see advantages to making life changes, they also may have many concerns and fears about changing. Reluctance to change is viewed as a normal and expected part of the therapeutic process. All the theories have something to contribute to understanding a client's ambivalent feelings about making changes. Motivational interviewing is a useful way to explore a client's ambivalence about changing because it promotes acceptance and compassion for ambivalence (Naar & Safren, 2017).

As can be seen in the overview of diverse approaches, each theory focuses on a particular dimension of human experience as a way to change other facets of personality. In my own integrative approach, there is room to incorporate most of these goals. I work with clients to help them identify a range of specific goals that will provide a framework for their work. At different stages in therapy, different goals assume prominence. The priority of the client's goals will influence the current direction of therapy, but all of the goals will continue to be relevant throughout the course of the client's therapy.

The main goal of therapy endorsed by most theoretical orientations is to assist the client in bringing about changes within an individual; however, these changes often have repercussions on the systems of which the individual is a part. Family systems approaches have a broader goal than merely bringing about change within the individual.

In family therapy, and in feminist therapy as well, the goal is to bring about change within systems. When therapy is successful, the family often learns about patterns that have been transmitted from generation to generation or learns ways to detect and solve problems that keep members stuck in dysfunctional relational patterns. Incorporating concepts from systems models into an integrative approach broadens counseling to deal with changing individuals and systems.

An Adlerian Perspective on Goal-Directed Behavior and Goal Setting

I find the Adlerian approach particularly valuable in establishing therapeutic goals. Adlerians believe that all behavior is goal-directed and future-oriented. The Adlerian view of goal-directed action is similar to many neuroscience conceptualizations of behavior (Miller & Dillman Taylor, 2016). A major goal in counseling is to help individuals identify mistaken goals and private logic, understand the purpose and reduce the distress of negative symptoms, and increase the client's social interest, which leads to courage, finding happiness, and achieving a true sense of belonging in the world. Change is aimed at all three levels: cognitive, emotional, and behavioral. Adlerians are mainly concerned with challenging clients' mistaken notions and faulty assumptions. They are not merely concerned with symptom reduction but rather with replacement therapy, or developing a constructive life in place of a dysfunctional one. Adlerians view establishing goals as a mutual endeavor between two equals. Therapy is likely to fail if either the client or the therapist impose goals (Maniacci & Sackett-Maniacci, 2019).

Several key Adlerian concepts have special relevance for the process of establishing goals, both for therapy and for life: the goal-directedness of behavior, struggling with feelings of inferiority, striving for superiority, and social interest. Alfred Adler's system emphasizes the social determinants of behavior. This "socioteleological" approach implies that we are primarily motivated by social forces and strive to achieve specific goals. Adler's view is that we create both short- and long-term goals that motivate our behavior and influence our personality development. It is our long-term goals, in particular, that guide our movement toward what we perceive as completion and perfection.

Another key concept from the Adlerian approach that is directly related to the process of forming goals is the notion of social interest. Social interest is the individual's positive attitude toward other people in the world that involves a sense of identification and empathy with others. As an antidote to social isolation and self-absorption, social interest leads to courage, optimism, and a true sense of being connected to all of humanity—past, present, and future. Our happiness and success are largely related to social connectedness. As social beings, we have a need to be of use to others and to establish meaningful relationships in a community. We cannot be understood in isolation from our social context. We are primarily motivated by a desire to belong. Only within the group can we actualize our potential.

The ultimate goal is to replace a life in distress or pain or dysfunction with a more effective and meaningful one. Symptom reduction is still important, but it is useless without an even stronger effort to enact a preferred and functional life. My colleague and friend Jim Bitter, who is a well-known Adlerian, offered the following descriptions of key qualities of the good life (adapted from Bitter & Carlson, 2017):

- Feeling that you count and belong is better than feeling alone, isolated, or useless; in the end, the meaning you have in life is what you contribute through work or engagement with others to the whole of humankind.
- The experience of loving and being loved is essential to the good life; all of us need at least one person with whom we give and receive love.
- When constrictions are removed from individuals, people experience life more fully.
- Communication works best when it is open, clear, congruent, and responsive rather than closed, defensive, or dismissive.
- Although it is better to say what one thinks and feels than to be blocked in silence, tact and timing are important factors to consider.
- Reality exists, and complaining about it, wishing it were different, or ignoring it only makes matters worse.
- Most people spend about 80% of every day doing things they would rather not do. It is important not to miss the 20% that brings joy to life.
- Knowing is usually better than not knowing, and facing one's problems is usually better than avoidance. Acting responsibly, no matter how hard that may seem, is the best choice most of the time.
- It is better to feel something—even sadness or hurt—than to feel nothing at all; indeed, in most cases, you can't have positive feelings without also occasionally experiencing negative ones.
- Acting and taking a chance is better than the feeling of being constricted or immobilized.
- Having goals, options, and hope is the antidote to discouragement; it is also the antidote to anxiety and depression.
- It is better to understand and be appreciative of one's gender, affectional/sexual orientation, culture, and ethnicity than to reject it, hide it, or be cut off from it.
- Everything can always be different: Write yourself or your family a new story with a counterplot to the one that is keeping you stuck.
- It is better to risk optimism, faith, and courage than to live in fear.

Becoming the Client: Establishing Counseling Goals

In this section, I identify basic considerations in the process of formulating meaningful counseling goals by asking you to become the client again. Keep in mind that goal setting for your therapy will be a collaborative process. How can I work with you to identify clear and personal goals

to guide the sessions? What are some of the obstacles to formulating concrete goals, and how can we best deal with these obstacles?

To begin with, I have some general goals that are congruent with my philosophy of counseling that will influence the direction of our sessions. One major goal is to establish a therapeutic relationship that will serve as a foundation for all of our work together. Another goal is to operate from a guiding philosophy that will allow for systematic exploration of the personal goals you identify as being central to you. I want to teach you a framework for resolving the problems you bring to counseling, with special emphasis on teaching you how to deal successfully with future problems. What goals do *you* have for our work together? Can you identify one major goal you have for therapy?

What we talk about in our sessions will be based on personal goals you identify. Perhaps you have given a good deal of thought to what you want out of life, yet you could still benefit from identifying, clarifying, and discovering better ways of reaching your goals. It is possible that you have a generalized notion of what you want to get from your counseling. For example, you might tell me, "I'd just like to feel better about myself. I don't like myself very much." It probably would not be of much help to you if I simply asked you to be more concrete. By asking self-reflective questions such as those that follow, we may be able to clarify what you want for yourself.

- If you were feeling better about yourself, what would you be feeling?
- Imagine that you felt better about yourself. What would you be telling me about who you are and how your life is?
- Can you give me some idea of what goes on in your head when you feel bad about yourself?
- How does your current behavior either help or hurt you?
- Tell me about a particular area in your life that you'd like to feel better about. Is there one special area that you wish could be different?

By following your lead and gently asking you to say more about specific times that you have felt good or what situations in particular you most struggle with, both you and I will gradually get a clearer picture of what you want.

Your goals for counseling will be much more meaningful if you define them for yourself. As I listen to you, my interventions are aimed at getting you to state your goals in such a manner that we both know what you want and have a frame of reference to understand the degree to which you are attaining your goals.

The action-oriented therapies (behavior therapy, cognitive behavior therapy, rational emotive behavior therapy, reality therapy, solution-focused brief therapy, and motivational interviewing) provide useful concepts for identifying specific goals at the outset of the therapeutic

process. In helping you to achieve your goals, I assume an active and directive role. Although you generally determine what behavior will be changed, I typically suggest ideas of how this behavior can best be modified. In designing a treatment plan, I will borrow techniques from a variety of therapeutic systems and apply them to your unique situation. Therapeutic flexibility and versatility are required to effectively achieve a diverse range of goals.

Drawing on the behavioral approaches, reality therapy, and solution-focused brief therapy, you and I identify goals that can be measured or monitored. The goals of reality therapy include behavioral change, better decision making, improving significant relationships, enhanced living, living more mindfully, and more effective satisfaction of psychological needs. These action-oriented therapies provide a specific focus of directional change that enable us to evaluate our work together. Can you think of a personal goal that fits one of these categories?

I also ask you to consider long-range goals. What would you most like to be able to say about yourself or your life situation one year from now? Five years from now? The art of developing goals consists of breaking these long-range goals into relatively short-term objectives that lead you in the direction you want to move. The category of goals we might discuss would encompass the full spectrum of your thinking, feeling, and behaving—with a particular focus on specific areas within each of these domains that you most want to change. Together we will continue this process of delineating personal goals by identifying specific steps you are willing to take, both in the session and outside the office, to get what you want from life. Indeed, I expect that your goals will change as counseling progresses. As you learn how to best use the counseling sessions, other concerns may become evident.

Various Types of Therapeutic Goals

Because counseling is a collaborative partnership, I do not accept full responsibility for deciding what the focus of our therapy sessions will be. It is not my job to decide for you how you should live your life. My job is to help you formulate workable goals and to provide you with the tools you'll need to achieve your goals. There are three general categories of goals: those in the cognitive realm, the emotional dimension, and the behavioral realm.

Cognitive Goals

First, let's examine some possible goals in the cognitive realm that you'd be interested in exploring in your counseling sessions. I operate on the assumption that your thinking influences how you feel and act. If you want to change your emotions and certain behaviors, one route is to identify problematic cognitions and change these. My aim is to listen for some of your underlying beliefs, especially thought patterns that seem to present difficulty for you. I would not identify what I think are faulty beliefs; rather, I would ask you to identify beliefs that you

hold that do not serve you well or that lead to problematic behavior. Because you are likely preparing to become a counselor, or are already engaged in some type of counseling practice, reflect on these statements to determine the degree to which you hold similar beliefs pertaining to yourself as a counselor-in-training:

- I must always function competently and perfectly. There is no room for making mistakes.
- I really need the continual affirmation from my clients if I am to feel worthwhile as a professional.
- It is essential that I have the right technique for every problem situation a client presents or else I will look incompetent.
- I am fully responsible for the progress or lack of progress that my clients make.
- If a client does not turn up for a second session, it is most likely due to my ineptness in making good contact with this person at the first meeting.

In listening to self-statements such as these, I strive to focus on core beliefs that potentially result in problematic emotional and behavioral consequences for you. For instance, are you convinced that you are completely responsible for your clients' therapeutic outcomes? Your core beliefs have a number of consequences. You may worry excessively about your clients and experience a great deal of stress in your work. Not only does the belief that you are totally responsible for client outcomes cause you anxiety, this belief can actually contribute to client dependency. Working together it is important that you and I determine specific cognitive goals that will guide your work in therapy. For example, you may decide to challenge your beliefs about the need to assume total responsibility for those with whom you work.

Emotional Goals

I invite you to be as specific as possible in identifying emotional concerns. You might say that you are frightened of intimacy. First of all, I want to know what intimacy means to you. I also want to know if this is a situation that you want to change. You may shy away from getting too close to people because of a host of fears, yet keeping distant could well be acceptable to you. It is not my job to urge you to take risks in this area if you do not want to be different. However, if you'd like to be able to experience closeness with a few people and not flee from intimacy, this could become a target goal for our work.

As my client, I ask you about the realm of emotions you are experiencing, especially feelings that you view as problematic. I tend to notice any bodily changes that may indicate emergent feelings. Rather than interpret what you are feeling, my tendency is to ask you what you are experiencing. What did you just become aware of as you were talking? I notice that you just teared up. Can you say what that is about? What are you experiencing right now? I find that it is most

useful to begin work with the affective realm and then move to an exploration of thoughts and behaviors. However, sometimes individuals feel safer beginning with the cognitive realm while we develop a trusting therapeutic alliance in the initial stages. We can then build upon that foundation to engage in more vulnerable emotional work. Utilizing an integrative approach enables flexibility to most effectively meet your unique needs.

I draw heavily from emotion-focused therapy to assist you in getting closer to your moment-by-moment experiencing. I encourage you to identify and experience your feelings. By asking you to notice what you are aware of in the present moment, you are better able to move into whatever realm is salient for you—be it awareness of a thought, a bodily state, or feelings. You need to come into contact with your feelings and to intensify them in order to find a pathway to personal change. You will not change by simply talking about your feelings, or by trying to understand the origins of problematic feelings, or by changing the way you think. If it is appropriate, we might use a range of experiential techniques to identify and deal with affective goals that are important to you.

Behavioral Goals

In addition to setting cognitive and affective goals, it is essential that we identify concrete behavioral goals. Let's assume that you let me know that you would like to take better care of yourself. You realize that too often you feel driven by outside forces rather than feeling in charge of yourself. You contribute to this sense of constant pressure by telling yourself that you don't have time for yourself. Using elements of behavior therapy, reality therapy, and rational emotive behavior therapy, I invite you to examine the choices you are making about your body and your overall wellness. I ask you to examine your present level of physical and psychological well-being. What balance do you want to achieve in areas such as rest, exercise, diet, and ways you spend time? Which of the following behavioral goals might fit for you as a client?

- I am willing to keep a record of what I do in my work to determine whether there are some patterns that I want to change.
- I am willing to make more time for rest and sleep.
- I want to create a better balance between work and leisure.
- I am willing to ask for help when I feel overwhelmed.
- I want to find an exercise program I would profit from and enjoy.
- I want to change my diet. I am willing to start by monitoring what and how much I eat and drink to determine if there are areas I want to change.

Identifying counseling goals may seem like a simple matter, but counseling is not a linear process of resolving a single problem. You are a complex and integrated being, and any one problem you have

is best seen as one aspect of the larger picture. Counseling is a more complex and interesting process than simply resolving problems.

Clarifying Counseling Goals With Joleen

Joleen seeks therapy because her lack of confidence gets in the way of her relationships and feeling good about herself. In general, she says that she is dissatisfied with her physical appearance and energy level. During the beginning stages, I assist Joleen in getting a clearer grasp of what she most wants from therapy as well as seeing some steps she can begin to take to attain her objectives. When Joleen talks about her goals for therapy, her descriptions are quite broad, so I work with her on describing her goals in more concrete terms. When she looks in the mirror, she says she does not like what she sees. She would like to have a better self-image and be more confident. I am interested in knowing specifically what she does not like, the ways in which she now lacks confidence, what it feels like for her to confront herself, and what it is like to initiate this topic with me. Another area she would like to improve upon is taking the initiative to make contact with friends. She tends to wait for people to ask her to get together, which does not often result in meeting her needs for relationships with others.

I suggest that Joleen write in a journal to keep track of how she is doing in meeting her goals. Journal writing can bring clarity to her work in the therapy sessions, and this practice is a good way to extend the influence of what we do together into her daily life. If Joleen experiences difficulties in applying what she is learning in therapy to daily life, writing about it will be an excellent way to figure out alternative strategies. She can bring to her sessions the essence of some of what she writes in her journal.

Joleen is able to make progress toward her self-defined goals because she is willing to become actively involved in challenging her assumptions and in carrying out behavioral exercises, both in the sessions and in her daily life. For instance, Joleen establishes a number of goals that she is interested in pursuing in the counseling sessions. She does not like her appearance, and she would like better relationships with friends.

At the beginning of most sessions, Joleen and I discuss at least briefly what she wants from this particular session. I ask these questions:

- What are you aware of as you approach this session?
- How do you want to use your time in here today? What is it that you want to talk about?
- What are you hesitating to explore yet think it would be important for you to talk about?

With these focusing questions, the responsibility is with Joleen to determine what her therapy goals are for each session, including her

long-term goals. A few specific examples of Joleen's long-term goals include:

- To achieve self-acceptance and a healthier self-image
- To become a more confident individual
- To create meaningful and fulfilling relationships with others

Each of these goals has subsequent smaller and clearly defined objectives, with corresponding techniques and interventions designed to make these goals and objectives successful. For example, an objective of increasing assertiveness works toward the goal of creating meaningful relationships with others. Interventions may include psychoeducation about communication styles and behavioral rehearsals in sessions to practice effective ways of communicating with others. Joleen's goals are interrelated; as she makes progress in some areas, she will likely experience positive effects in the other areas. Assertiveness training benefits all three goals.

Goal setting is an ongoing process that is best defined with the client. It is my job to teach her how to become a collaborator in selecting both short- and long-term goals and to be a partner in deciding how she wants to use her therapy hour.

Reflection Questions

Reflect on how you might intervene with Joleen to assist her in establishing goals for her therapy. Consider these questions:

- When you ask Joleen what she wants to get from therapy, she states one of her goals is "to achieve self-acceptance and a healthier self-image." What would you say to Joleen about this goal?
- If Joleen talked about her goals in broad and general terms, how would you help her become more specific?
- Assume that Joleen asks for your advice regarding what goals she should have for her therapy, what would you say to her?

Clarifying Counseling Goals With Aaron

Aaron initiates counseling because he realizes that he is not taking care of himself in many ways, which is resulting in a good deal of dissatisfaction in his life. He would like to do much better in taking care of himself than he typically is doing. He feels stressed out a good deal of the time and does not manage these stressors well. He is lax when it comes to exercising, and he realizes this is taking a toll on him. Although he would like to make strides in his self-care, Aaron

just does not seem to be motivated to take steps to change his current patterns. He realizes there are steps he can take (such as mindfulness practices) to get better control of his stress, yet he is ambivalent about putting forth the energy to make these life changes.

I let Aaron know that determining goals is a joint project that we will work on together. I won't be making decisions for him about what we will explore. I like the Adlerian notion of goal alignment, which means that goal setting involves a mutual, collaborative process. In addition, I value the cognitive behavior approaches that emphasize collaboration between therapist and client in determining goals. It is Aaron's responsibility to define the target areas for us to explore in our sessions, and it is my responsibility to guide him in narrowing his goals so we both have a clear picture of how to proceed. Establishing and refining goals takes time, effort, and thoughtful reflection on Aaron's part. The goals he chooses to explore highlight the areas Aaron is most interested in changing and provide direction for the counseling process.

I borrow from reality therapy in asking him to look at the direction of his life. What is his life like at this time? What seems to be working for him? What feelings, thoughts, and actions are not moving him in the direction he would like to evolve? I might well ask Aaron to project his life one year into the future and describe what he would like to say that he has become or achieved. My aim in doing this is to invite him to evaluate what he is presently doing to determine whether his actions are getting him what he wants. To help him pinpoint what he wants, I ask him these questions:

- If you were already the person you wish to be, what kind of person would you be?
- What would you be doing if you were living your life more fully?
- Is what you are doing at this time taking you closer to or further from your goals?

This line of questioning focuses Aaron on the process of critically thinking about where he is now and where he would like to go in the immediate future. Self-evaluation is the cornerstone of the practice of reality therapy.

By consistently expecting Aaron to engage in a self-evaluation process, I help him create his own agenda based on an inner inventory of his own actions, cognitions, and feelings. When he decides for himself that his present behavior is not working, he will be much more open to participating collaboratively in designing goals that will meet his needs. Once clear goals are established in a collaborative fashion, meaningful evaluation of the progress of therapy can be charted. At each session, it will be critical that Aaron and I spend a brief amount of time assessing the degree to which counseling is helping him attain his goals. If his goals seem to lose vitality, this is a sign that we need to take another look at what he most wants to explore in our sessions.

Reflection Questions

Reflect on how you might intervene with Aaron to assist him in establishing goals for his therapy. Consider these questions:

- Aaron realizes that he is not taking care of himself in many ways, which is resulting in a good deal of dissatisfaction in his life. Although he would like to improve his self-care, he says that he is not motivated to put forth the effort it will take. What would you say to Aaron?
- What theory (theories) would you find useful in assisting Aaron in formulating clear and meaningful personal goals?
- What will you tell Aaron about determining goals as being a joint project that you work on together?

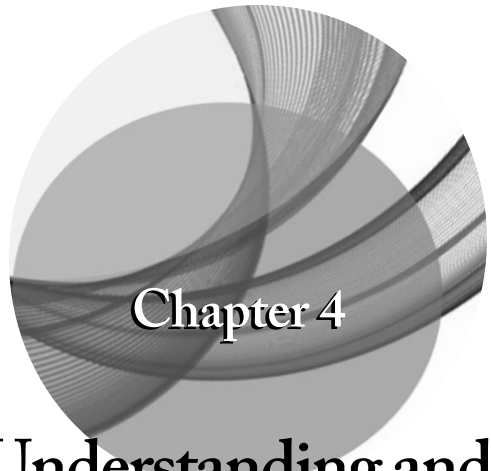
Concluding Comments

It is important to be flexible in defining with your clients meaningful goals that provide direction to therapy. Carefully consider the purposes for which your clients seek counseling and then collaboratively design specific, clear, and realistic goals to guide the therapeutic process. Your job is to assist your clients in getting a better grasp of what they want to accomplish in therapy with you. It is not your role as a therapist to identify goals for your clients. If clients do not seem to be making progress toward a given goal or goals, perhaps the goals are not personally meaningful because they have been prescribed for them by others. Developing a collaborative partnership is of the utmost importance when clients struggle with defining their own goals.

In reflecting on the construction of your own integrative approach to counseling, develop a systematic way of incorporating cognitive, affective, and behavioral goals as a starting point for therapy. After reading this chapter on establishing therapeutic goals, take time to reflect on the following questions to clarify your thoughts on the kinds of goals you want to establish with your clients.

- As a counselor, how could you make establishing therapeutic goals a collaborative process?
- How does your theoretical orientation influence your view of therapy goals?
- What are your goals for the therapeutic process?
- What would you do if your goals for therapy conflicted with your client's goals?
- How could you best manage a value conflict between you and your client?
- What challenges do you expect to face in assisting your clients in the development of specific goals for the therapeutic process?

- How could a client's therapy goals be used as a way to evaluate therapy outcomes?
- What importance do you place on expecting your clients to develop both short- and long-term goals?
- How could you help clients who have ill-defined goals formulate meaningful and clear goals for their therapy sessions?



Chapter 4

Understanding and Addressing Diversity

In this chapter I examine the role of diversity in an integrative counseling model. Intersection diversity characteristics to consider include ability or disability, affectional orientation, age, gender, race, culture, language, spirituality/religion, national origin, and socioeconomic status. As you read, think about how open you are to learning about diversity as it pertains to your counseling practice. Keep in mind what will be required for you to become a diversity competent counselor.

Being able to draw on techniques from various theoretical models seems especially necessary in working with a diverse range of client populations. Harm can come to clients who are expected to fit all the specifications of a given theory, whether or not the values espoused by the theory are consistent with their own cultural values. Rather than stretching your client to fit the dimensions of a single theory, it is best to make your theory and practice fit the unique needs of the client. To adapt your therapy to each client implies that you understand your client on a deep level. Norcross and Beutler (2019) claim that tailoring psychotherapy to the person, creating a new and responsive psychotherapy for each client that considers client characteristics, including contextual factors, results in a more efficient and effective therapy. Norcross and Beutler support the principle that people and cultures differ and should be treated as such. "Evidence-based pluralism reigns as integration infuses diversity and flexibility into psychotherapy" (p. 549). Regardless of your theoretical orientation, you need to acquire *cultural competence*, which calls for you to possess knowledge about various cultures, awareness of your own cultural heritage, and skills to assist diverse clients in meeting their needs within the realities of their culture. To work effectively with clients,

therapists must embrace change by promoting empowerment and social justice (Comas-Diaz, 2019).

My Introduction to Multicultural and Diversity Perspectives

In *Creating Your Professional Path: Lessons From My Journey* (Corey 2010), I describe how I was introduced to multicultural and diversity perspectives. In my student years in a doctoral program in counseling in the 1960s, there were few discussions of how culture influences the assessment and treatment process, and no multicultural courses were offered. When I began my work as a counselor and in university teaching, I focused on counseling from an individualistic perspective, with little attention to the cultural context of the therapeutic process. Let me share a bit about the influence of my family experiences on understanding how culture affects us.

I am a first-generation Italian American, and I learned about cultural diversity first from my family of origin. Both of my parents were from Italy, yet neither of them seemed to think it important that I learn to speak Italian at home. I do not remember conversations about cultural matters, and there did not seem to be an interest in passing along cultural traditions.

Being around a nuclear and extended immigrant Italian family, I learned at an early age that people see the world through different eyes and express themselves in different ways. I came to appreciate differences rather than judge them. My extended Italian family encountered some discrimination, yet some of these family members harbored their own prejudices as well. I observed some of my relatives making disparaging remarks about certain other groups of people and learned that racism can be both subtle and blatant. Somehow, I developed compassion for people who were the target of any form of discrimination. Perhaps this was partially due to my own childhood experiences of feeling I was on the outside. I am not sure how I developed compassion for others and a nonjudgmental way of being, but moving in that direction seemed right to me.

To a great extent, I learned about cultural diversity from my wife, Marianne, who had immigrated to the United States from Germany at the age of 19. We were married in Germany when I was 27, but both of us experienced some turmoil about whether or not to get married, especially Marianne. We were from two different cultures: she grew up in a rural town on a farm, and I grew up in the city of Los Angeles; she was Protestant, and I was Catholic; and marriage meant that Marianne would leave her family in Germany and make her home in California. At that time, cultural and religious differences were more significant in society than they are today. With the help of several significant people in our lives, we were able to work through the differences that almost separated us. We realized that we had many core values in common,

which helped to put our differences into perspective. We were married in the Worms Cathedral in Germany, and both of our families attended the wedding, which was a clear endorsement for us.

Each summer we spent time in Marianne's village in the Rheinpfalz, which gave me an opportunity to experience the values associated with living in a small, rural town in another country. Living in two cultures has helped me to appreciate the complexities of culture and various ways of perceiving the world. Without a doubt, Marianne has been my primary mentor in teaching me about culture—and she has been one of the major forces in challenging me to think about ways of understanding diversity and how to incorporate this understanding into counseling practice.

These early experiences taught me valuable lessons that were missing from my graduate program. I learned the importance of being open to what life can teach us, the significance of having respect for contrasting perspectives, the value of trying to understand the diverse worldviews of the clients I will meet, and the importance of understanding those who have been oppressed or discriminated against. Even if you have grown up in a restricted environment, you can learn about people with a worldview different from your own. Differences in worldviews from our clients often lead to communication problems with clients, misdiagnosis, and even premature termination of therapy by clients (Comas-Diaz, 2019). By understanding how your own cultural background has contributed to who you are, you have a basis for understanding other viewpoints. Through concerted efforts on your part, it is possible to expand your current attitudes and views.

Perspectives on Multicultural Concerns

Only in recent times has the counseling profession begun to consider the influence and impact of racial and cultural socialization on clients. Many clients have come to distrust mental health professionals who are associated with the establishment or with social service agencies because of a history of unequal treatment. Clients from oppressed groups may be slow to form trusting relationships with counselors from the dominant culture, and mental health professionals may have difficulty identifying with these clients if they ignore the history behind this distrust. Mental health practitioners from all cultural groups need to honestly examine their own assumptions, expectations, and attitudes about the therapeutic process.

Considering How We Are Alike and How We Are Different

We cannot afford to ignore diversity perspectives if we hope to practice competently, yet we need to be careful about generalizing to any particular population of clients. Individuals within a particular group may differ more than individuals from various groups. I like the perspective taken by Clemont Vontress (2013), a pioneer in the field

of multicultural counseling, who suggests a conceptual approach to counseling in which the proper focus is on the individual rather than on the individual's race, ethnicity, or cultural background. Vontress maintains that we need to recognize that people are more alike than different. He encourages counselors-in-training to pay attention to the universal commonalities of clients first and then to address areas of differences.

Among those with cultural differences are clients with various abilities, disabilities, or serious medical issues. It would be insulting, for example, to assume that a client with a visual impairment or who uses a wheelchair is seeking treatment to deal with adjustment issues related to the disability, especially if this condition is not new. These clients are likely to seek therapy regarding the same issues as those without such limitations, such as relationship problems, anxiety, or striving for higher levels of self-actualization. A good guideline in dealing with all people is a *person first* mentality: see the person before seeing the difference. A helpful starting point is to consciously change your language patterns and shift perspectives to speak of a client *with* autism spectrum disorder, an individual who struggles *with* borderline personality disorder, or a person *with* dementia. This pertains to cultural considerations, diagnoses, and behavioral concerns as well. Gestalt therapy pays particular attention to language patterns, and this awareness can be integrated smoothly with other therapies to promote multicultural sensitivity and respect.

Paying Attention to the Individual

Vontress (2013) emphasizes basic human conditions that transcend culture, such as existential concerns about living, loving, and dying. A human-to-human encounter is therapeutic for all clients, regardless of their cultural background. How this encounter occurs, however, is culturally determined. Keep in mind that different cultures ascribe varying meanings and definitions to the universal existential concerns. Although mental health problems are best understood in a cultural context, it is essential to remember that each client is a unique individual. The central challenge we face when working with clients who differ from us is to find a way to pay attention to what is significant to them and to understand their world. We can accomplish this goal by listening to what our clients are expressing and respecting what we hear.

Paying attention to the individual, however, does not free the counselor from the responsibility of studying the multitude of ways that so many aspects of the dominant culture discriminate against some groups and the real psychological effects that result from oppression. McGoldrick, Giordano, and Garcia-Preto (2005) note that the dominant culture also is made up of many subcultures, and they suggest that individuals cannot fully know themselves outside of knowing the history and values of the culture into which they were born.

Diversity as Central in the Counseling Process

We need to be aware of how diversity plays out in counseling interactions if we are to practice ethically, competently, and effectively. The *Code of Ethics* of the American Counseling Association (2014) infuses diversity and multicultural considerations throughout the document. According to Paul Pedersen (2008), the multicultural perspective seeks to provide a conceptual framework that both recognizes the complex diversity of a pluralistic society and suggests bridges of shared concern that link all people, regardless of their differences. This perspective looks at both the unique dimensions of a person and how this person shares themes with those who are different. Mere knowledge of certain cultural groups is not enough; it is important to understand the variability within groups. Each individual must be seen against the backdrop of his or her cultural group, the degree to which he or she has become acculturated, and the level of development of racial identity. Pedersen defines culture broadly to include variables such as race/ethnicity, gender, age, socioeconomic status, religion, sexual orientation, and disability. Using this framework, all counseling can be considered multicultural because each of us comes from a unique blend of cultures and identities. By defining culture broadly, Pedersen is able to view culture as the "thousand persons" we all have collected from various sources who follow us wherever we go and who influence all of our decisions. To have a sense of cultural self-awareness, it is necessary for us to have access to and be able to speak with those inner voices.

Pedersen (2008) states that it is no longer possible for therapists to ignore their own cultural context or the cultural context of their clients. Whether practitioners pay attention to cultural variables or ignore them, culture will continue to influence both the client's and the therapist's behavior, and the counseling process as well. Counselors who ignore culture will provide less effective services. Understanding the role that diversity plays in the therapeutic process is essential to successful outcomes. Cultural factors are an integral part of the counseling process, and these factors influence the interventions we use with our clients. Pedersen believes that multicultural awareness can increase the quality of our life, and it can also make our job easier. Adopting a perspective that cultural differences are positive attributes that add to relationships expands our ability to work with diverse client groups. It is important to view the tapestry of culture woven into the fabric of all helping relationships not as a barrier to break through but as a garment that provides comfort in our clients' search for meaning.

Being an effective counselor involves reflecting on how your own culture influences you and your interventions in your counseling practice. This awareness is a critical factor in your becoming more sensitive to the cultural backgrounds of the clients who seek your help. Understanding and addressing diversity evolves from three primary

practices. First, as a counselor, you must be aware of your own assumptions, biases, and values about human behavior, and of your own worldview as well. Second, you need to become increasingly aware of the cultural values, biases, and assumptions of diverse groups in our society, and come to an understanding of the worldview of culturally different clients in nonjudgmental ways. Third, you need to begin developing culturally appropriate, relevant, and sensitive strategies for intervening with individuals and with systems (Sue & Sue, 2016). Comas-Diaz (2019) believes that to become culturally competent you need to become aware of your worldview, examine your attitudes toward cultural differences, be open to learning about different worldviews, and develop multicultural skills. Becoming culturally competent is a continuing process, not a destination that we reach once and for all. It is possible to carry our biases around with us and yet not fully recognize how they influence our view of the therapeutic process. It takes consistent effort, honesty, and vigilance to monitor our biases and values so they do not interfere with establishing and maintaining effective working relationships with our clients.

Cultural differences are real, and they influence all human interactions. Counselors may misunderstand clients of a different gender, race, ethnicity, age, ability/disability, social class, or affectional orientation. If counselors fail to integrate these diversity factors into their clinical practice, they are infringing on the client's cultural autonomy and basic human rights, which will reduce the chance of establishing an effective therapeutic relationship. It is important to understand the intersectionality of various aspects of diversity rather than focusing on a single dimension. For some clients, religious or spiritual values may be important. Other clients may focus on gender or age discrimination. By paying attention to what a client is saying, counselors can discover which aspects of a client's identity are most salient for this person at this time.

Counselors do not have to share the experiences of the client to be effective. It is not possible for us to be similar to our clients in all respects, but we have an ethical responsibility to acquire the competence to work effectively with clients who differ in significant ways from us. Although we are not expected to have the same life experiences and problems as our clients, it is critical that we understand and respect the world of our clients. I contend that well-trained practitioners, even though they differ in many ways from their clients, are capable of providing effective counseling, especially if these differences are explored with their clients.

At this point I recommend that you take an inventory of your current level of awareness, knowledge, and skills that have a bearing on your ability to function effectively in multicultural situations by reflecting on these questions:

- To what extent are you aware of how your own culture influences the way you think, feel, and act?

- What could you do to broaden your understanding of both your own culture and other cultures?
- To what degree are you able to identify your basic assumptions, especially as they apply to diversity in culture, race/ethnicity, gender, class, religion, ability/disability, language, and affectional orientation?
- In your professional work, how are your assumptions likely to affect the manner in which you function with clients who are significantly different from you?
- How flexible are you in applying the techniques you use to the specific needs of your clients?
- How prepared are you to understand and work with individuals from different cultural backgrounds? With individuals with diverse gender or affectional orientations?
- What life experiences have you had that will help you to understand and make contact with individuals who have a different worldview from yours?
- Can you identify any areas of bias that could inhibit your ability to work effectively with people who are different from you? If so, what steps might you take to challenge your biases?

The subject of attending to multicultural and diversity perspectives in counseling is far more complex than this brief introduction allows. Some useful books for in-depth research on this subject include *Counseling for Social Justice* (Lee, 2018), *Multicultural Issues in Counseling: New Approaches to Diversity* (Lee, 2019), and *Multicultural and Social Justice Counseling Competencies: A Blueprint for the Profession* (Ratts & Butler, in press).

Counseling LGBTQI People

When we think of diversity competence, perhaps what comes to mind most readily is being able to serve clients from various cultures. However, the concept of human diversity encompasses more than cultural factors. It encompasses all forms of oppression, discrimination, and prejudice, including those directed toward age, gender, ability, religious affiliation, and affectional orientation. When serving LGBTQI individuals, counselors must be aware of their attitudes and beliefs that can either enhance or inhibit their counseling effectiveness with these populations. A counselor who works with LGBTQI people has a responsibility to understand the special concerns of these individuals and is ethically obligated to develop the knowledge and skills to competently deliver services to them. Working with LGBTQI individuals often presents a challenge to counselors who hold conventional values. The mental health system had finally begun to treat the *problems* of LGBTQI people rather than treating *them* as the problem.

Heterosexism is still alive and well in our society. Heterosexism is a worldview and a value system that can undermine the healthy functioning of the affectional orientations, gender identities, and behaviors of LGBTQI individuals. Counselors need to understand that heterosexism pervades the social and cultural foundations of many institutions and often contributes to negative attitudes and discrimination toward people who are not heterosexual or do not meet the socially accepted standards of stereotypical gender roles and behaviors. When working with LGBTQI clients, it is paramount that counselors are aware of their own privilege, marginalization, and potential biases, which could have a negative impact on the counseling relationship (Ginicola, Filmore, & Smith, 2017). Counselors can take a significant step toward gaining competence by challenging their own personal prejudices, biases, fears, attitudes, assumptions, and stereotypes regarding sexual orientation to understand the ways in which discrimination and multiple forms of oppression are manifested in society toward LGBTQI people. In his thoughtful article, "Ethics, Religion, and Diversity," Gregory Moffatt (2018) develops the message that counselors have the task of helping people who are hurting, and it is not their place to evaluate who is worthy of professional help based on culture, age, religion, or sexual orientation. Moffatt cannot believe that equity for LGBTQI clients still remains a topic for debate. I appreciate Moffatt's emphasis that counselors cannot ethically pick and choose who they will help and that he calls for counselors to avoid imposing their values in working with a diverse range of clients.

A further complicating factor to keep in mind when working with clients from the LGBTQI community is that they may have internalized negative societal views on some level. Several theories work well in an integrative manner with treating this sort of internalized shame. Psychoanalytic methods could help by bringing the unconscious self-views to the conscious level. Rational emotive behavior therapy can be useful in challenging and replacing irrational beliefs with a healthier and more self-accepting set of beliefs. Gestalt interventions can be utilized to process the internalized introjects (messages) and to initiate a dialog between the conflicted parts of the individual. In addition to their theoretical base, counselors serving LGBTQI clients need to possess the understanding of which skills and techniques are most appropriate for these populations, where they should be used, and how they should be tailored for each client (Ginicola, Filmore, & Smith, 2017). A key intervention counselors can make is providing a climate that allows clients to talk about what most matters to them. Goodrich and Ginicola (2017) capture the essence of a therapeutic climate: "Providing a space for clients that is safe, nonjudgmental, and affirming helps all clients develop their authenticity and acceptance of self; for LGBTQI+ clients this space may be a rare oasis in a desert of rejection and marginalization" (p. 104).

A recommended resource for a more comprehensive discussion on this topic is *Affirmative Counseling With LGBTQI+ People* (Ginicola, Smith, & Filmore, 2017).

Recognizing the Spiritual Domain

Effective counseling addresses the body, mind, and spirit. Today there is widespread interest in the role of spirituality in both assessment and treatment. Evidence for this interest is found in the many books and articles written on incorporating spiritual and religious values in counseling, including those published by the American Counseling Association (ACA) and the American Psychological Association (APA). Part of being culturally competent involves being able to work effectively and ethically with clients' spiritual and religious concerns. In your work with culturally diverse client populations, you will need to be prepared to address your clients' spiritual concerns.

There is also a growing interest in *positive psychology*, which considers topics such as humility, virtue, forgiveness, gratefulness, altruism, and hope—all of which can be considered dimensions of spirituality. Psychologists have given more attention to negative emotions than to positive emotions, and there has been a historical focus on studying pathology, weaknesses, and suffering. The advocates of positive psychology call for increased attention and research on positive emotions, health, hope, courage, contentment, happiness, well-being, perseverance, resilience, tolerance, and human strengths (Weiten, Dunn, & Hammer, 2018). Both humanistic psychology and positive psychology are based on common principles. Humanistic psychology focuses on a set of philosophical assumptions about what makes life meaningful. Positive psychologists explore factors that make people happy, focusing on human strengths and how people can flourish in daily life. Positive psychology emphasizes wellness and personal strengths, such as positive emotions, engagement, relationships, meaning, and accomplishment. Assessing strengths is critical for a balanced and holistic clinical practice. Psychotherapy focuses on the cultivation of wellness as much as it does on the alleviation of distress (Rashid & Seligman, 2019).

Many of the themes stressed in both positive psychology and humanistic psychology are grounded in spiritual values. Spirituality might very well be at the center of an integrative approach. Spirituality and religion are critical sources of strength for many clients, may be their bedrock for finding meaning in life, and can be instrumental in promoting healing and well-being. Because spirituality is an important component of mental health, incorporating spirituality into counseling can be important for many client populations. Currier, Pearce, Carroll, and Koenig (2018) explored military veterans' preferences for incorporating spirituality in counseling and found that 15% to 20% of veterans were highly interested in spiritually integrated care. Military veterans generally endorsed the integration of spirituality in psychotherapy as being somewhat important for them. Research findings suggest that religious faith or spirituality can either help or hinder recovery from mental health conditions that might prompt military veterans to seek counseling. These findings support the need

for a client-centered approach with veterans in which clinicians are not ignorant of religious/spiritual concerns but do not assume that this domain must be targeted in every case.

Clients may sometimes discover that they need to reexamine their values. It is essential that you remain open and nonjudgmental and recognize that there are multiple paths toward fulfilling spiritual needs. It is not your role as a counselor to prescribe any particular pathway. It is unethical to attempt to convert clients to a particular religious or spiritual set of values. You can, however, assist clients in exploring their own values to determine the degree to which they are living within the framework of this value system. It is important that you monitor yourself for subtle ways you might be inclined to impose certain values in your counseling practice, either toward embracing a particular spiritual perspective or abandoning it. It is critical to keep in mind that it is the client's role to determine what specific values to retain or modify.

Assessing Your Own Spirituality

Your own value system can influence many facets of your counseling practice. You have an ethical responsibility to be aware of how your attitudes, beliefs, and values affect your work and make sure you do not unduly influence your clients. If you hope to assist your clients with their spiritual concerns, you must both acknowledge and be comfortable with your own spiritual and religious beliefs. Take a moment to reflect on these questions:

- What role does spirituality or religion play in your life?
- To what extent does religion or spirituality provide you with a source of meaning?
- Has religion been a positive, negative, or neutral force in your life? How might your answer to this question affect your ability to work effectively with clients who want to explore spiritual or religious concerns?

Even if spiritual and religious issues are not the focus of a client's concern, these values may enter into the sessions indirectly as your client explores moral conflicts or grapples with questions of meaning in life. Can you maintain objectivity when spiritual and religious values are explored in counseling sessions? How do you think your values will influence the way you counsel? How would you answer direct questions from your clients about your religious/spiritual beliefs? Can you maintain a healthy awareness of the similarities and differences between your worldview and that of your client? If you have little belief in spirituality, have little respect for organized religions, or identify yourself as a secular humanist, can you empathize with clients who view themselves as being deeply spiritual or who feel committed to the teachings of a particular religion? If you are convinced that having

a meaning in life hinges on accepting certain religious beliefs, can you be of help to clients who do not share your conviction?

Considerations in Counseling Nonreligious Clients

Along with understanding the role of spirituality and religion for many clients, it is equally important to honor the beliefs and concerns of individuals who identify as nonreligious. Sahker (2016) focuses on ethical and clinical considerations in providing therapy with people who are nonreligious. He estimates that between 16% and 23% of American adults and 33% of adults under the age of 30 claim to be nonreligious. Individuals experiencing spiritual struggles may seek therapy when deciding to leave the religion of their family of origin. These clients may be seeking a safe place to discuss their doubts and distress related to internal conflicts involving nonbelief. Some who have left the religion of their family of origin experience rejection by family members, and clients often want to express and explore the pain they experience as a result of their choice. Some individuals have no ties to religion, others may claim that neither spirituality nor religion is important to them, and still others are atheists. Counselors who attempt to foist their values onto their clients by trying to convince them of the importance of accepting a spiritual or religious path are not practicing ethically. Nonreligious people are not valueless or immoral people. The meaning that some people seek in spirituality, nonreligious people may find in philosophy or even in psychology itself. Effective practice with nonreligious clients may be less complex because the psychological and values assessment is mostly internal rather than being formed by an outside entity. The assessment process is crucial in identifying any religious or spiritual concerns, both positive and negative, perspectives on morality and values, and assessing whether these beliefs, past or present, provide them with meaning or with distress. If clients say they want to talk about religious or nonreligious matters, these concerns should be addressed in their therapy.

Two highly recommended books for more on this topic are *Integrating Spirituality and Religion Into Counseling: A Guide to Competent Practice* (Cashwell & Young, 2011) and *Spirituality in Counseling and Psychotherapy: An Integrative Approach That Empowers Clients* (Johnson, 2013).

Becoming the Client: Understanding How to Work With Diversity

In this section, put yourself in the role of a client with me as your therapist and imagine how some of the differences between us are likely to influence our relationship. You bring up the point that you and I are different. I inquire about *how* we are different and what this means to you. Reflect for a few minutes on what you would say to me about how you and I are different. How do you think these differences are likely to affect our work together? Some differences between client

and counselor cannot be ignored. Yet more important than the specific ways that you and I differ is the matter of which differences are salient for you. I cannot assume that I automatically know the meaning of our differences, even if they seem obvious. By asking what differences particularly stand out to you and what meaning these differences hold for you, my expectation is that this discussion will help you decide if I am the counselor for you.

The reality of differences in the therapeutic relationship poses a question that students often ask: Should these differences be specifically addressed, and if so, is it the counselor's or the client's place to initiate this discussion? If you were my client, would you be inclined to point out some differences between us, especially if you thought these differences might get in the way of your progress in therapy? Or would you prefer that I take the lead to identify certain differences and ask you how you feel these differences might affect our therapeutic relationship? The crucial factor for me is understanding what differences are salient for you and what meaning you attach to such differences. As your counselor, I need to create a safe environment that will enable you to bring up your concerns about how we are different so that together we can explore how these differences are likely to influence the therapeutic work. For example, I am an 80-year-old, White male who has been afforded certain privileges. I have a doctoral degree and a number of academic and professional credentials. I have been active in the profession for more than 56 years as a teacher and author. Which of these factors, if any, might lead you to wonder if I could understand and counsel you effectively? It might be a mistake if I single out our age differences, or our racial differences, or our educational differences, or our gender differences. Indeed, it may be how some of these diversity factors intersect with each other. If I were to assume that you would have difficulty because of my age, I might very well be wrong. Thus, I think it is more productive to ask at the end of an initial session, "How has it been for you to work with me during this session?" Again, what is crucial is that you feel safe enough to raise any of your concerns about working with me.

Try this short exercise. Imagine yourself as a Black client coming to see a White counselor. What are your concerns? Now imagine that you are a White client coming to see a Black counselor. What are your concerns? Imagine being a White client coming to see an Asian counselor who was born and raised in China but has come to the United States and studied the counseling processes used here. What are your concerns? Okay, one more time. Imagine you are a member of the LGBTQI community coming to see a heterosexual-identified counselor. What are your concerns? Now suppose that you and your heterosexual partner are coming for couples counseling with a counselor who identifies as transgender (male-to-female), has been married for 20 years to a heterosexual-identified woman, and who has teenage children. What are your concerns? If you are a member of the dominant culture, you may find it difficult to imagine what the issues and

concerns might be, but there will be some concerns if you are honest with yourself. Members of marginalized cultures and groups almost always know what the issues will be because they confront their life inside the dominant culture every day.

Theories Applied to Understanding Diversity Perspectives

Practitioners who draw from any of the contemporary therapeutic models would do well to reflect on the underlying values of their theoretical orientation. Traditional theories of counseling have been criticized by some practitioners for having major limitations with respect to working with culturally diverse clients. Some writers in multicultural counseling allege that most contemporary theories of therapy and therapeutic practices are grounded in individualistic assumptions frequently associated with the values of mainstream U.S. culture (Sue & Sue, 2016). Some of these values include autonomy, independence, self-determination, and becoming your own person. Feminist and multicultural scholars are critical of the theories that tend to promote independent models of the self that emphasize personal control and the preeminence of individual rights and responsibilities. Instead, these scholars give priority to the values of mutual interdependence, connection to others, and communal responsibilities. I believe that the contemporary theories can be expanded to incorporate a multicultural and diversity component. I agree with Comas-Diaz (2019) who contends that regardless of your theoretical orientation you can incorporate a multicultural perspective into your practice. Many of the key concepts of the traditional theories can be adapted to a cultural framework that has meaning for diverse client groups. Multicultural theory is based on the premise that assessment and treatment must take into account the cultural experiences and contexts that shape clients' identities. Multicultural counseling utilizes techniques drawn from various therapeutic approaches, yet it is necessary to adapt these methods to the cultural values and expectations of individual clients.

As a counselor, you need to be able to assess the special needs of your clients. Depending on the individual client's ethnicity and culture and on the concerns that bring this person to counseling, you will have to show flexibility in utilizing diverse therapeutic strategies. Some clients will need more direction and guidance; others will be very hesitant in talking about themselves in personal ways, especially early in the counseling process. What may appear to be resistance is very likely to be the client's response to years of cultural conditioning and respect for certain values and traditions. It is important to be familiar with a variety of theoretical approaches and to have the ability to adapt your techniques and relational style to fit the person-in-environment. Working from an integrative perspective enhances your ability to be flexible and to adapt to different circumstances, which is useful in working with a diverse set of clients (Stricker, 2010).

Contributions From Theoretical Systems

In this section, I summarize a number of theoretical systems from the vantage point of their contributions to understanding diversity. I have included key concepts that I find most useful in understanding and working with clients from a multicultural perspective and some main points that I incorporate in my integrative counseling practice.

Contributions of Psychodynamic Therapy

Psychoanalytically oriented therapy can be appropriate for culturally diverse populations if techniques are modified to fit the settings in which a therapist practices. All of us have a background of childhood experiences and have experienced developmental crises throughout our lives. As a therapist, you can help your clients review environmental situations at the various critical turning points in their lives to determine how certain events have affected them either positively or negatively. To the credit of the psychoanalytic approach, the value of intensive psychotherapy is highlighted as part of the training of therapists. This helps therapists become aware of their own sources of countertransference, including their biases, prejudices, and racial or ethnic stereotypes.

Contributions of Adlerian Therapy

The Adlerians' focus on social interest, on belonging, and on the collective spirit fits well with the value systems of many client populations. Collectivist cultures that stress the welfare of the social group and that emphasize the role of the family will find the Adlerian focus on social interest to be congruent with their values. Adlerian practitioners are flexible in adapting their interventions to each client's unique life situation. Adlerian therapy has a psychoeducational focus, a present and future orientation, and is a brief, time-limited approach. All of these characteristics make the Adlerian approach suitable for working with a wide range of client problems.

If culture is defined broadly to include age, roles, ability/disability, affectional orientation, and gender differences, there will be cultural differences even within a family. The Adlerian approach emphasizes the value of subjectively understanding the unique world of the individual. Culture is one significant dimension for grasping the subjective and experiential perspective of an individual. Adlerians use a lifestyle assessment to learn more about the individual they are counseling. Maniacci and Sackett-Maniacci (2019) claim that the lifestyle process helps clients teach therapists about their world. "Working with clients from diverse cultures is much like taking a personal tutorial in multiculturalism with each new client" (p. 89).

Contributions of Existential Therapy

Vontress (2013) contends that the existential approach is perhaps the most applicable of all approaches for working with culturally diverse clients because of its focus on universal themes, or the common ground that we all share. We are all multicultural in the sense that we are the

product of many cultures. All of us, regardless of our cultural background, share in the dilemmas of existence and must grapple with the ultimate concerns of freedom, isolation, meaninglessness, and death (Yalom & Josselson, 2019).

The existential approach emphasizes presence, the I/Thou relationship, and courage, qualities that demonstrate a great deal of respect for the client and an interest in deeply understanding the client's world. This approach can be effectively applied with diverse client populations with a range of specific problems and in a wide array of settings (Schneider & Krug, 2017). Existential therapists assist clients in critically evaluating the source of their values and making a choice rather than uncritically accepting the values of their culture and family. Clients in existential therapy are encouraged to examine the ways their present existence is being influenced by social and cultural factors.

Contributions of Person-Centered Therapy

An emphasis on core conditions provides the person-centered approach with a framework for understanding diverse worldviews. Empathy, being present, and respecting the values of clients are essential attitudes and skills in counseling culturally diverse clients. Person-centered counselors convey a deep respect for all forms of diversity and value understanding the client's subjective world in an accepting and open way. The core therapist conditions of empathy, congruence, acceptance, and positive regard are nearly universal and are likely to have a constructive impact on clients regardless of cultural differences. This theory is well-suited to counseling clients in a diverse world because it places a high priority on understanding the unique characteristics of each individual (Cain, 2010).

Contributions of Gestalt Therapy

There are many opportunities to apply Gestalt experiments in creative ways with diverse client populations. Gestalt experiments can be tailored to fit the unique way in which an individual perceives and interprets his or her culture. Experiments are done with the collaboration of the client and with an attempt to understand the background of the client's culture. Gestalt therapists strive to approach each client in an open way without preconceptions. They check their understanding of the client in dialog with the client. Gestalt practitioners who have truly integrated their approach are able to apply their skills in a flexible way by adapting their methods to each individual. They are concerned about how and which aspects of the client's background become central, or figural, and what meaning the client places on these figures.

Contributions of Behavior Therapy

Behavioral approaches can be appropriately integrated into counseling with culturally diverse client populations when culture-specific procedures are developed. The approach emphasizes teaching clients about the therapeutic process and stresses changing specific behaviors. In designing a change program for clients from diverse cultures, effective practitioners conduct a functional analysis of the problem situation.

This assessment includes the cultural context in which the problem behavior occurs, the consequences both to the client and to the client's sociocultural environment, the resources within the environment that can facilitate change, and the impact that change may have on others in the client's surroundings. By developing their problem-solving skills, clients learn concrete methods for dealing with practical problems within their cultural framework. Behavior therapists are increasingly seeing the value of adopting a more culturally responsive approach in working with a diverse range of client populations, including becoming more aware of their own biases and learning more about how their clients are influenced by their cultural experiences (Antony, 2019).

Contributions of Cognitive Behavior Therapy

Cognitive therapy does not work in an arbitrary way on changing a client's beliefs, nor does it impose a therapist's beliefs on the client. Clients are assisted in examining their own beliefs to determine if they are fostering a sense of well-being. If clients discover certain beliefs are not accurate, they are encouraged to consider an alternative set of beliefs (Beck & Weishaar, 2019). In the cognitive behavior approaches, therapists function as teachers who encourage clients to learn skills to deal with the problems of living. The emphasis is on changing specific behaviors and developing problem-solving skills rather than on expressing feelings. Clients who appreciate the educational dimensions of a helping relationship and who are looking for action plans and behavioral change will be receptive to this therapy because it offers concrete methods for dealing with their problems.

Pamela Hays (2009, 2016) developed *culturally responsive CBT* (CR-CBT) to assist counselors in providing services that are more responsive to the needs of clients from diverse cultures. This approach adapts CBT techniques to align with the client's cultural context, cultural identity, and preferences. CR-CBT offers a useful framework for a strengths-oriented, culturally responsive approach to the assessment and treatment process. Hays (2009) believes there is an "almost perfect fit" between cognitive behavior therapy and multicultural therapy because these perspectives share several important assumptions that contribute to an integrative framework:

- Interventions are tailored to the unique needs and strengths of the individual.
- Clients are empowered by learning specific skills they can apply in daily life (CBT) and by acknowledging the cultural influences that contribute to their uniqueness (multicultural therapy).
- Inner resources and strengths of clients are activated to bring about change.
- Clients make changes that minimize stressors, increase personal strengths and supports, and establish skills for dealing more effectively with their physical and social (cultural) environments.

The cultural assessment considers psychological and cultural factors that are related to a client's problems and well-being, but Hays (2016) suggests that therapists avoid challenging a client's core cultural beliefs unless a client is clearly open to this. By emphasizing collaboration over confrontation, counselors can focus on their clients' culturally related strengths in developing helpful ways of thinking to replace unhelpful cognitions.

One aspect of the cognitive behavior therapies that I especially appreciate is providing clients with a framework for thinking about their thinking. Within the framework of their cultural values and worldview, clients can explore their beliefs and provide their own reinterpretations of significant life events. This allows therapists to guide clients in a manner that respects clients' underlying values. This dimension is especially important when counselors do not share the same worldview and cultural background as their clients.

Contributions of Reality Therapy

Many of the key concepts of reality therapy can be applied when working with a diverse range of clients. I especially value the straightforward approach of asking clients to look at what they are doing to determine the degree to which their actions are satisfactory to them. Once clients decide what thoughts, feelings, and behaviors they want to target for change, reality therapy offers practical procedures in designing action plans to bring about these changes.

Wubbolding (2000, 2011, 2017) believes the confrontational style used with Western clients must be modified to fit the cultural context of people other than North Americans. In Japanese culture, for example, assertive language is not appropriate, and communication is less direct. The reality therapist's tendency to ask direct questions may need to be softened, with questions being raised more elaborately and indirectly. It may be a mistake, for example, to ask individualistic questions built around whether specific behaviors meet clients' needs. For Japanese clients, directly asking "What do you want?" can sound intrusive. An alternative might be "What are you seeking?" Flexibility in using techniques is a foremost requirement in working with culturally diverse clients, and key concepts and procedures must be tailored to fit each individual.

Contributions of Solution-Focused Brief Therapy

Solution-focused brief therapists learn from their clients about their experiential world rather than approaching clients with a preconceived notion about their worldview. The therapist-as-expert is replaced by the *client-as-expert*, especially when it comes to what the client wants in life and in therapy. This nonpathologizing stance moves away from dwelling on what is wrong with a person and emphasizes creative possibilities. Clients become actively involved in resolving their problems, which makes this a very empowering approach. The therapist attempts

to create an atmosphere of understanding and acceptance that enables individuals from a wide variety of cultures to utilize their resources to make constructive changes.

Contributions of Motivational Interviewing

Therapists who practice motivational interviewing strive to experience the world from the client's perspective without judgment or criticism. Motivational interviewing emphasizes reflective listening, which is a way for practitioners to better understand the subjective world of the client. Expressing empathy is foundational in creating a safe climate for the client to explore his or her ambivalence about change. When a client is slow to change, perhaps there are compelling reasons associated with the cultural background and cultural values for the client to remain unchanged. This stance of understanding the client's world is basic when applying motivational interviewing in multicultural situations.

Contributions of Narrative Therapy

With its emphasis on multiple realities and the assumption that what is perceived to be true is the product of social construction, narrative therapy is a good fit for individuals with diverse worldviews. Narrative therapists operate on the premise that problems are identified within social, cultural, political, and relational contexts rather than existing within individuals, which makes this approach especially relevant for counseling culturally diverse clients. The sociopolitical conceptualization of problems offers understanding of cultural notions and practices that produce dominant and oppressive narratives. Practitioners deconstruct, or take apart, the cultural assumptions that are a part of a client's problem situation. Clients are able to understand how oppressive social practices affect them, which allows for the possibility of creating alternative stories.

Contributions of Feminist Therapy

Feminist therapy is intentionally multicultural, which makes this approach ideally suited to responding to the needs of diverse individuals. The emphasis in feminist therapy is on tailoring treatment to the individual client, collaboration on the goals of therapy, social advocacy, and creating a strong working alliance. All of these factors are central to working in a diversity context and all are empirically linked to positive treatment outcomes (Brown, 2018).

Gender stereotypes influence societal practices, discrimination, and individual beliefs. For example, it is a stereotype that women are more interested in relationships than in professional accomplishments. The truth is that many women are challenging this belief. They are concerned about the quality of their relationships, and at the same time, they want to accomplish professional goals they set for themselves. Gender stereotypes are powerful forces of social control, but role expectations that are oppressive and that limit women's achievement can

be challenged. Disrupting the status quo and advocating for change can be a daunting task, but adhering to traditional gender roles can be detrimental to individuals, couples, and society (Polanchek & Shaw, 2015). Even though cultural pressures toward adopting specific gender roles are strong, we are not cemented into a rigid way of being. Counseling can be instrumental in helping clients challenge role expectations that restrict them and determine whether the costs of having adopted certain roles are worth the potential gains.

Many of the social action and political strategies that call attention to oppressed groups have equal relevance for women and for ethnic minorities. Counselors who subscribe to the assumptions underlying feminist therapy demonstrate their belief that counseling should free individuals and increase their range of choices. At the individual level, feminist and other social justice therapists work to help individuals recognize, claim, and embrace their personal power. A related goal is to help individuals come together to strengthen collective power. Through empowerment, clients are able to free themselves from the constraints of their gender-role socialization and other internalized limitations and to challenge ongoing institutional oppression.

The recent emergence of the "#Me Too" movement is a challenge to individual and institutional oppression and discrimination that feminist therapists, as well as others, throughout the United States are championing.

Contributions From Counseling Theories Outside of the Mainstream

New counseling theories outside of the mainstream are emerging that address multicultural counseling. In some ways, the challenges first defined in multicultural counseling have become generic processes for quality counseling in general. Yet the mainstream counseling models are not nearly specific enough to meet the needs of marginalized communities. New models, specifically developed for a given population, are regularly emerging.

Frederick Phillips (1990) developed NTU (pronounced "in-too") psychotherapy as a holistic philosophy and lifestyle that comes complete with clarity about cultural identity, especially for Black males, and an Afrocentric conceptualization of human behavior and functioning. It is spiritually based and assists clients in becoming aligned with the natural order and spiritually balanced. The values stressed in this model include interconnectedness and interdependence, cultural awareness in the present and an understanding of cultural roots, balance, harmony, and authenticity. The therapy is family-focused and competence-based, and it is enacted collaboratively with the client through five stages:

1. *Harmony:* The counseling relationship is established through collaboration and mutual respect with a focus on listening to the client and making sense of his or her worldview.

2. *Awareness:* The counselor (or healer) and the client work together to define the client's reason for seeking counseling. Nothing is addressed that is not what the client is seeking.
3. *Alignment:* The client works toward mutual understanding and respect between himself (or herself) and his (or her) family.
4. *Actualization:* The client puts into practice the changes made during the previous phase.
5. *Synthesis:* The counselor and the client reflect on the counseling process and prepare for termination.

None of these phases is linear. They more accurately reflect a communal circular journey—a spiritual journey—that moves in and out of phases as needed. Models similar to NTU can be developed for every culture and even for cultures within cultures.

Understanding Win-May From a Diversity Perspective

Win-May is a 24-year-old Asian American woman, born in Boise, Idaho, whose parents are from South Korea. She is currently an undergraduate student studying nursing, and she plans to work in a nearby hospital after completion of her degree. Win-May sought counseling because "I feel like I am not myself . . . like my life is not my own." As I explored the meaning behind Win-May's statement, she explained that her parents are from a culture that values success through education in areas that yield financial security and stability. She told me that when she applied to college, her parents gave her the options of majoring in engineering, political science/pre-law, biology/pre-med, or nursing. She reports, "I know that I want to help people, but I do not want to be a lawyer or doctor so I chose nursing." She adds that the pressure she feels from her parents to be successful, to always follow the rules, to earn good grades, to find a successful husband, and to take care of her parents in later life is resulting in anxiety and depression.

In our counseling sessions, a primary focus has been her fear of being authentic in relationships. Win-May talks about the difficulty she experiences in striving to be her true self, largely because of her fear of being judged by her parents and family. She lets me know that a value in her culture is to respect and follow the advice of her elders. If she were to deviate from the plans her parents have for her, this would be seen as being disrespectful and selfish. However, she is experiencing a conflict between wanting to be accepted by her parents and wanting to pursue her own career ambitions. Win-May does not see a way to resolve this clash of values.

The sessions with Win-May came to critical point when she decided that she no longer wanted to be a nurse. Instead she wants to pursue a graduate program in counseling. She did not identify with the nursing profession, had failed several assignments, and did not attend some

of her classes. She says, "It is physically painful to get out of bed and put the energy into sitting in classes that I do not care about." Win-May decided to change her undergraduate major to psychology, with a long-range goal of becoming a graduate student in counseling. Because she did not want to have a conflict with her parents, she continued telling them that she was a nursing major. Eventually she wants her parents to know about her decision, but she does not want to risk ruining the relationship. She is convinced that her parents would not understand what the counseling profession is and that it would be seen as a profession beneath the medical and law fields.

Win-May expressed fears and uncertainty about her ability to succeed in graduate school. She explained, "I do not know if I am smart enough to be a graduate student." If I go against my parents' wishes and become a graduate student in a profession they may not agree with, and I fall flat on my face . . . I don't know how I would live with the embarrassment." One of her biggest fears is not being "smart enough" to both deal with the demands of graduate school and to become a counselor.

In counseling with Win-May, I drew from several theoretical orientations. From person-centered therapy, we explored the implications of looking within for answers and valuing an inner locus of control. From existential therapy, we discussed the anxiety associated with making her own choices. From a multicultural perspective, we considered the possibilities of respecting her cultural and family values but perhaps modifying those values. From cognitive therapy, I used Socratic questioning to encourage Win-May to assess the validity of her conclusion that she is not smart enough to pursue her dreams. I ask her where she acquired this belief and to consider how her behavior and self-perception might be more positive if she were to replace these beliefs with self-enhancing beliefs.

In one of our sessions, I employed a popular technique from solution-focused brief therapy. "Win-May, if you were to go to sleep tonight and wake up tomorrow with the conflicts resolved in your relationship with your parents, what would that be like for you?" Win-May took a deep breath and began to cry. She claimed that it would mean that her family "was finally *my* family." She expressed that she would feel accepted, loved, and free. If she woke up and her conflicts were resolved, her parents would see and experience a completely different person. Through our work together, Win-May decided that expressing her true self to her parents was a priority she would focus on in her counseling sessions.

Borrowing from the Adlerian approach, I invited Win-May to *act as if* she were living by her internal values and being true to herself, for just one week. I asked her to simply imagine interactions with different people several times each day in which she was being her authentic self. When she returned for her next counseling session, we talked about what this experience was like for her and how she could

build upon what she had learned from this exercise. We engaged in several behavioral rehearsals of dialogs she might have at some time with her parents. In doing this, Win-May talked with me in the role of her parents as I mainly listened. We then processed what it was like for her to express herself in various ways to her parents. I also "became Win-May" and talked to her, and she took on the role of her parents and responded to me the way she wished they would talk to her. I cautioned Win-May not to be in a hurry to have an actual discussion with her parents. It was important for Win-May to know that the decision is her own with regard to approaching her parents, and not my agenda for her. Being her genuine self involves making existential choices, living with the anxiety of the responsibility of her freedom, and dealing with the consequences of whatever she chooses. Eventually, she did approach her parents and was able to have several exchanges with them about her struggles: wanting to be accepted as their daughter yet realizing she was questioning some of the values of her family and culture, especially those values pertaining to education and career goals.

With the semester coming to an end, Win-May came back to counseling for a follow-up session. She explained in greater detail how she approached her parents and the resulting relational changes. Win-May mentioned that through discussions with her parents she learned that ultimately her parents only wanted her to live a happy and fulfilled life. Because of this hope for their daughter, they placed pressure on her to choose a career that they thought would be in her best interests. Win-May told me that she was able to explain to her parents that the counseling field is closely tied to her identity and would keep her fulfilled through life's journey. After several productive discussions with her parents, Win-May expresses that she received her parent's blessing to pursue the career she wanted and that facing this conflict brought her entire family closer. Through receiving affirmation from her parents, Win-May feels less pressure and anxiety in making decisions: "I finally feel like I can accept myself."

Reflection Questions

Reflect on how you might work with Win-May from a diversity perspective. Consider these questions:

- Win-May states that she does not see a way to resolve the clash of values between her parents and herself with respect to her educational and career goals. How would you work with her on this struggle that is causing her anxiety?
- One of Win-May's fears is thinking she is not smart enough to succeed in graduate school. What cognitive interventions might you make in helping her explore this fear?
- I made a number of interventions with Win-May. Which intervention did you find most interesting and why?

Understanding Gabriel From a Diversity Perspective

I had seen Gabriel, a 22-year-old immigrant from El Salvador, for two counseling sessions. On the third session, he said he is wondering if I can help him because he sees us as being different. For several years, Gabriel was in a youth gang, and he learned how to survive in an environment in which his life was in constant danger. When I inquire about *how* we are different and what this means to him, he mentions that we live in different worlds and he wonders if I can understand how his upbringing makes it difficult for him to express feelings and to be vulnerable as a man. More important than the specific ways that he and I differ is the matter of which differences are salient for Gabriel. I cannot assume that I automatically know the meaning of our differences, even if they seem obvious. I ask him what differences particularly stand out to him, what meaning these differences hold for him, and how these differences might affect his work with me.

Gabriel is not sure that I can empathize with his experience as a man growing up in his cultural background in Central America. He wonders about our age differences, the differences in our life experiences, and the fact that we belong to two different cultures. Although he does not identify himself as being an oppressed person, he does seem to be oppressed in some respects. Because of our differences in gender-role socialization, Gabriel hesitates to bring up some of his struggles with how a man "should be." I let him know that it might be difficult for me to understand some aspects of his life, and also let him know that I will tell him when I am having trouble grasping his subjective perspective. I do want him to know that I would hope to appreciate and understand how challenging it has been for him to question and change certain roles that he has been playing for most of his life. Should he want to continue with me as his counselor, I also invite him to tell me whenever he feels that our differences are getting in his way. Gabriel says that he is glad that he brought up his concerns, that he likes how I am responding to him, and that he wants to give more of a chance to counseling with me.

In working with Gabriel I am influenced by some key concepts of feminist therapy. I think that feminist therapy has something valuable to offer to both women and men, especially when it comes to identifying how gender-role socialization influences our present way of living. Feminist practitioners acknowledge that social and political inequities have negative effects on *all* people. Feminist therapists work to help individuals make changes in their lives, but they also are committed to working toward social change that will liberate all members of society from stereotyping, marginalization, and oppression. Diverse sources of oppression, not simply gender, are identified and interactively explored as a basis for understanding the concerns that clients bring to therapy.

A large part of our work involves talking about what he learned about being a man in his culture. Gabriel becomes aware of many of the messages that he has internalized from society about what it means to be a man, and how accepting gender roles is inhibiting his ability to experience the kind of life he wants for himself. The gang culture impressed upon

him the importance of being able to fight, of never backing down, and never showing any weakness. He spent some time in prison for criminal behavior and here he learned that men need to be made of iron.

His family influenced his vision of what a true man should look like. From his father, he learned the message that showing emotions is a sign of weakness. Gabriel feels that he has never measured up to the expectations of his father and his brother, who both seem to not allow the stress of daily life to derail them. Gabriel describes his brother and his father as being confident and exceptionally strong. When I asked Gabriel if he wants to be like either of these men, he paused and reflected on the values he has received from his family and culture about what men are supposed to be like and how they should act.

We spend considerable time exploring messages he has received about gender roles from the various cultures in which he has lived and about decisions he has made regarding what it means to be a man. People tend to make clear distinctions between women and men, and they divide the world into two categories: male and female. The traditional hypothesis is that women and men are both biologically and psychologically different, so people react differently to them. Merely talking about gender-role messages will not bring about changes in Gabriel's established behavior patterns, but these discussions may result in a shift in Gabriel's thinking about the self-limiting roles that he has accepted without question, which then may translate into him being different in the world. Gabriel begins to increasingly realize how these messages were reinforced by expectations from his parents and young people that he associated with in his gang; he becomes aware of how the damaging self-statements he makes now are related to his early experiences and messages about gender roles that he has not critically examined. These conversations may eventually result in shifts in how Gabriel feels about himself as a man and the ways he allows himself to respond in many different social situations.



Reflection Questions

Reflect on how you might work with Gabriel from a diversity perspective. Consider these questions:

- What are some key differences between you and Gabriel that could either help or hinder the establishment of a good working relationship? Would you bring up what you consider salient differences, or would you invite Gabriel to talk about any of the differences between you that matter to him?
- Assume that Gabriel tells you that he wonders if you can understand him because both of you have such different life experiences. What would you say to him?
- How do you react to what Gabriel has learned about male gender roles?

Concluding Comments

Diversity is a reality that must be factored into an integrative approach to counseling. Regardless of theoretical orientation, both clients' and therapists' underlying values must be taken into account. Some of the values implicit in contemporary counseling theories include an emphasis on individualism, the separate existence of the self, independence, self-determination, becoming your own person, and individuation as the foundation for maturity. It is important to know that these values may not be equally relevant to all people; therapists must recognize that contemporary counseling theories are not value-neutral.

Ethical practice demands that counselors be aware of their own personal values and be able to bracket their beliefs so that their values do not interfere when working with clients whose values differ from their own. Some counselors claim that their personal values do not allow them to provide counseling services to LGBTQI clients and that providing a referral is an acceptable solution to resolving a value conflict with a client. The *ACA Code of Ethics* (2014) clearly states that counselors are to avoid harm and to avoid imposing their values on clients (Sections A.4.a. and A.4.b.). Resorting to a referral because of a conflict of values with a client is considered to be discriminatory practice. The *Code of Ethics* specifies that "counselors do not engage in discrimination based on any form of diversity, be it age, culture, ethnicity, disability, gender, gender identity, sexual orientation, religion/spirituality, or any basis proscribed by law" (Section C.5.). Counselors need to understand that counseling is about the needs of the client, not about the needs of the counselor.

Spiritual or religious values can be considered a dimension of a client's culture. Research has indicated that spirituality is a key component of mental health, so integrating spirituality into the therapy process is appropriate for some clients. Counselors need to be prepared to address spiritual and religious concerns that a client might raise. The key is to include a consideration of a client's spiritual and religious background to determine how these factors may play a role in the individual's treatment. A thorough assessment makes it possible to explore a client's spiritual and religious values without imposing the counselor's values (Young & Cashwell, 2011).

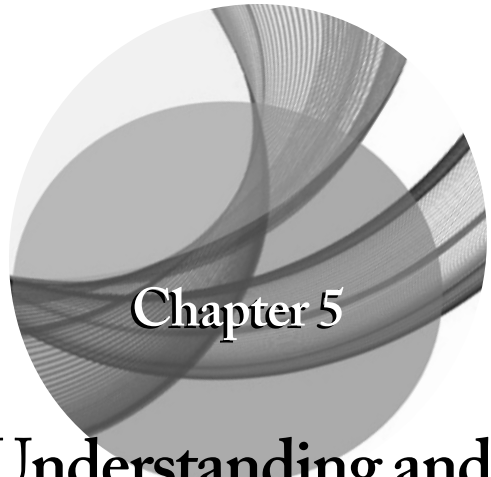
The psychoanalytic, behavioral, cognitive behavior therapies, and existential approaches originated in Euro-American culture and are grounded on a core set of values. The relationship-oriented therapies—such as person-centered theory, existential therapy, and Gestalt therapy—emphasize freedom of choice and self-actualization. If you base your practice on these orientations, you will likely focus on individual responsibility for making internal changes as a way to cope with problems, and you will view individuation as the foundation for healthy functioning.

There is a danger in seeing these values as having universal applicability. Indeed, this is why NTU psychotherapy focuses specifically on

an Afrocentric orientation to communal values and spirituality. Listen to your clients and determine why they are seeking help and how best to deliver the help that is appropriate for them in their unique context.

After reading this chapter on understanding and addressing diversity, take time to reflect on the following questions as a way of clarifying your thoughts on these issues in counseling practice.

- To what extent do you think contemporary counseling theories can incorporate a multicultural perspective?
- What issues and concerns would you need to address in working with individuals who fall upon a wide spectrum of ability, especially those who are in rehabilitation or are part of a cultural community such as the deaf community?
- Do contemporary theories have practical applications when working with LGBTQI clients or with issues brought to counseling by other oppressed groups?
- In what ways should diversity be a central part of the counseling process?
- What do you think it will take for you to become a diversity competent counselor?
- To what extent do you believe that spiritual and religious issues have a place in counseling? Would you be comfortable talking with a client about these topics?
- Would you tend to initiate a discussion of a client's spiritual or religious beliefs, or would you wait for the client to bring up this topic?
- What challenges might you encounter in working with clients who are significantly younger or older than yourself?
- How is gender a key facet of diversity? What importance do you place on exploring the ways gender-role socialization has affected the views your clients have about what it means to be a woman or a man?



Chapter 5

Understanding and Working With Resistance

When working with clients who manifest reluctant and resistive behavior, beginning counselors may interpret these signs in a personal way. They may think, "If I was an effective counselor, I should be able to help anybody who comes to me." If clients reach an impasse, are uncooperative, or don't come back for a second session, counselors may conclude that "this is a sign that I am doing something wrong."

Your self-talk may not be quite so harsh, but you might well blame yourself for what you've done or failed to do when faced with a difficult client. In any event, it is unlikely that you will welcome resistance as the source of productive material for therapeutic work. Take a few moments to think about the meanings you attach to resistance. Is a client's resistance intentional and meant to sabotage your best efforts? Or is resistance part of every counseling venture, regardless of how motivated your client is and how skillful you may be as a counselor?

Most clients with whom you work will test you in some way to determine whether the relationship with you is safe for them. It is essential that you encourage openness on the part of your clients so that they are able to express their hesitations and anxieties. Your clients are likely to have mixed feelings regarding staying in a safe zone versus taking the risk of letting you know them. Both you and your clients need to understand the meaning of resistance and come to view it as something to explore in the counseling process rather than as an enemy to be defeated.

Understanding the Dynamics of Resistance

Some of my colleagues have trouble with the word "resistance," thinking of it as a negative term that implies something is wrong with the

client. They have a good point because resistance is often equated with a stubborn refusal to cooperate with treatment, is viewed as something the client is doing wrong, or is perceived by counselors as a sign of their ineptness. Resistance is typically viewed as a phenomenon that resides within the client, and traditional therapy approaches often assume that clients who are "stuck" are resistant to change. If an impasse in therapy is reached, therapists frequently blame clients and label them as "unmotivated," and clients sometimes judge or blame themselves as well. This way of viewing resistance is limiting and does not facilitate the therapeutic process.

Although resistance is often viewed as an impediment to therapeutic progress, in actuality it can be central to productive work. My view is that resistance is normal and is a fundamental part of the counseling process. Resistance must be recognized, accepted, and explored. Once resistance is identified, it can be addressed cognitively, affectively, and behaviorally in an integrative way. By coming to understand and deal with your own patterns of resistance, you open up possibilities for modifying your behavior and also for developing skills in managing resistance in clients.

In understanding the dynamics of resistance, both clients' and therapists' contributions to the resistance must be considered. It is important that clients do not feel blamed or come to believe that something is inherently wrong with them for experiencing resistance. Nor should therapists feel that resistance always implies a lack of sensitivity or timing on their part. If you are engaged in intense work with a client, he or she might become frightened and stop the process as a form of self-protection against anxiety. Your client may have been psychologically and physically hurt in the past and resorted to defenses as a way to cope with an intolerable situation. Your client's guarded behavior might well be a defense against being wounded again. Past negative experiences are often associated with significant others, and you may need to explore these fears for your client to learn to trust you. It is important that you and your client make a commitment to talk about your present relationship. If you show a willingness to address your client's defensive behavior, the chances are increased that your client will be willing to take a look at various forms of resistive behavior. If you consistently approach what appears to be resistance with respect, interest, compassion, and concern, you increase the chances your client will explore this behavior in counseling sessions.

Respecting and Reframing Resistance

Some Perspectives on Understanding Resistance

Behavioral therapists working with clients value establishing clear and realistic personal goals as a framework for the direction of counseling. What I identify as "resistance" might well be an excuse on my part

for not doing a thorough assessment or for inadequately utilizing techniques. This perspective requires that I look at what I am doing to determine how I might be getting in my client's way. If I impose my vision of what I think the client should be working on, resistance is to be expected.

From a psychoanalytic perspective, resistance is typically defined as the individual's reluctance to bring into conscious awareness threatening material that has been previously repressed or denied. It also can be viewed as anything that prevents individuals from dealing with unconscious material. From a broader perspective, resistance can be viewed as behavior that keeps us from exploring personal conflicts or painful feelings. I agree with the psychoanalytic conceptualization of resistance as a defensive strategy aimed at protecting us and preserving our inner core in the face of anxiety.

From the perspective of time-limited dynamic psychotherapy (TLDP), Levenson (2017) believes resistance is best understood within the dynamic of the client–therapist relationship. Clients are doing the best they can considering how they view the world, and therapists help clients conceptualize their resistance as a form of self-protection. This enables clients to maintain their self-perception, personal integrity, and increase their interpersonal connectedness. When therapists sense they have hit a wall of resistance, Levenson suggests that therapists stand back, appreciate the significance of the wall, and invite clients to look at possible reasons for the wall. This inquiring approach tends to lessen power plays with clients and helps to promote understanding and empathy.

From the perspective of existential-humanistic therapy, Schneider and Krug (2017) address resistance as a safety issue for clients. It is crucial to respect resistance because it is a lifeline for many clients that represents a familiar path. Schneider and Krug urge therapists "to tread mindfully when it comes to resistance, acknowledging to clients both its life-giving and life-taking qualities" (p. 61). Before confronting clients on their defensive and protective patterns of being, a solid therapeutic alliance must be established. If clients do not feel safe, a challenge from the therapist can exacerbate rather than alleviate the situation.

Coming from an interpersonal perspective, Teyber and Teyber (2017) suggest that counselors honor clients' resistance as an outdated coping strategy that at one time served a self-preservative and adaptive function. Resistance can be reconceptualized as an interpersonal process that involves both client and therapist variables. When clients are able to appreciate that a particular coping strategy was their best possible response to a difficult situation at an earlier time in their development, both counselors and clients can recognize that some resistance is normal and makes sense. When clients learn to identify present coping strategies that no longer serve a useful purpose, therapists can assist clients in reframing

their resistance. Too often clients adopt a self-critical stance toward their resistance. They want to avoid talking about the topic because they are afraid of a therapist's critical judgment.

Resistance does not need to be "gotten around quickly" or bypassed. Take time to understand the functions these defenses serve. Clients may demonstrate a variety of resistances, and these defenses need to be understood and therapeutically explored. There are times when people need their defensives in order to survive a crisis situation. At such times, you need to be supportive rather than insist that your clients surrender their protection.

People who seek therapy are often ambivalent about change, and their motivation may ebb and flow during the course of therapy. Although clients may see advantages to making life changes, they also have many concerns and fears about changing. Check in with clients to learn how they are experiencing each session, beginning at the initial meeting. When clients express dissatisfaction or raise questions about their experience in therapy, therapists should welcome these concerns and be willing to discuss them in a straightforward way. Unless counselors ask clients about potential problems they experience with the counseling process, their concerns will likely remain unspoken (Teyber & Teyber, 2017).

Many therapeutic approaches offer ways to work effectively with resistance. Gestalt therapy, solution-focused therapy, narrative therapy, motivational interviewing, reality therapy, and cognitive behavior therapy practitioners all question the validity and usefulness of the way resistance has been viewed. Each of these therapeutic models reconceptualizes the phenomenon of resistance and encourages therapists to pay attention to what is transpiring in the present context of the therapeutic relationship.

Stages of Change

Prochaska and DiClemente (2005) view a client's unwillingness to recognize or own a problem not as an act of resisting the therapist but as resisting change. Similar to the Adlerian notion that resistance occurs when the counselor and the client have different goals, they emphasize the importance of a therapist appreciating how frightening the prospect of change is to many clients. Although therapists are responsible for challenging clients to take action necessary to bring about desired changes, they must avoid instilling blame and guilt in their clients. Prochaska and DiClemente suggest that the therapist become an ally rather than a person attempting to coerce change. Therapists need to develop patience during this sometimes lengthy and frustrating stage of change.

Individuals who seek psychotherapy do not arrive at a therapist's office with identical motivation, preparation, or capacity for behavior change. Krebs, Norcross, Nicholson, and Prochaska (2018) describe a transtheoretical model of change to evaluate a client's readiness for change. In this model, clients progress through a series of five identifiable stages in the counseling process:

Precontemplation stage: client has no intention of changing a behavior pattern in the foreseeable future
Contemplation stage: client is aware of a problem and considering overcoming it but has not yet made a commitment to take action
Preparation stage: client intends to take action in the near future and reports some small behavioral changes
Action stage: client takes steps to modify behavior, experiences, or environment to solve problem
Maintenance stage: client works to consolidate gains and prevent relapse

Clients do not pass neatly through these five stages in linear fashion, and if change is initially unsuccessful, individuals may return to an earlier stage. Effective therapy calls for matching specific interventions with whatever stage of change clients are experiencing. If there is a mismatch between process and stage, movement through the stage will be impeded and is likely to be manifested in reluctant behavior (Krebs et al., 2018).

Working within the framework of the stages of change model has implications for the role a therapist assumes. Krebs, Norcross, Nicholson, and Prochaska (2018) identify the relational stances taken by therapists at different stages. With clients in the precontemplation stage, the role assumed is that of a *nurturing parent*. With clients in the contemplation phase, therapists function as *Socratic teachers* who encourage clients to achieve their own insights. For clients who are in the preparation stage, therapists take the stance of *experienced coach*. With clients who are progressing into action and maintenance, therapists function in the role of *consultant*, being available to provide expert advice and support when action is not progressing smoothly. As termination approaches, therapists are consulted less often as a way to foster client autonomy. Krebs and colleagues claim that stage of change assessment is straightforward and takes but a few minutes during the initial session. They recommend the following research-supported therapist strategies to guide the course of treatment and enhance treatment outcomes:

- Take time to assess the client's stage of change.
- Do not assume all clients are in the action stage and motivated to change.
- Set realistic goals in keeping with the client's readiness to change.
- Deal with clients in the precontemplation stage in a respectful manner.
- Tailor therapeutic interventions to the client's stage: Insight and awareness interventions are useful during the early stages; action-oriented strategies are appropriate at later stages.
- Use the stage-matched relationship style (or role) that will best help the client progress to the next stage of change.

- Expect to recycle through the stages several times before achieving long-term maintenance.
- Integrate readiness to change into treatment resources.

The stages of change model is relevant for many different therapeutic approaches. Clients' readiness for change has implications for all of the theories, and this assessment can help therapists better understanding their clients. Hundreds of studies provide support for the usefulness of this stages of change model with clients of diverse ages, cultures, disability statuses, ethnicities, gender identities, races, religions, and affectional orientations (Krebs et al., 2018).

Understanding How Resistive Behavior Affects You as a Counselor

The key to understanding clients' various forms of defensive behavior is to pay attention to your own reactions that are triggered by this resistance. Many clients will replay their early history with significant others in their present relationship with you. Tune in to what you are feeling as you are working with your clients. If you respond to clients' defensive behavior in a personal way, it will be difficult for you to work with their dynamics. If you respond in an aggressive way, a difficult situation is likely to worsen. Your task is to work cooperatively with clients so that they might learn new and more effective ways of coping.

Clients you label as difficult or resistant may contribute to your own feelings of self-doubt and incompetence and bring out your feelings of inadequacy and impatience. When dealing with difficult behaviors, monitor your responses and potential countertransference to your client. If you quickly become annoyed with clients you view as resistive, you are likely to cut off avenues of reaching them. Do your best to avoid labeling clients and instead describe the behaviors you are observing. Ask clients to consider whether the ways they are behaving are helping or hindering them in getting what they want. When you replace the word "resistant" with a more descriptive and nonjudgmental term—such as being scared, overwhelmed with grief, cautious, or hurt—your own attitude toward these clients may change, making it easier for you to adopt an understanding and respectful stance. This stance will help your clients begin to explore the meaning of their reluctance. By approaching resistance with respect, interest, and understanding, defensive behavior is likely to decrease.

Stricker (2010) cautions therapists to avoid attributing problems in the treatment process to the client's shortcomings because the interactive role between client and therapist often contributes to seeming barriers to treatment. Reflect on your own style of interacting with clients to better understand your part in creating or contributing to resistant or noncompliant client behavior. By viewing resistance as something that emerges from the interactions between clients and counselors, the functions resistance serves are illuminated. "Resistance then can be seen as the friend rather than the enemy of treatment, because it

carries clues as to the client's typical means of functioning" (Stricker, 2010, p. 41).

Becoming the Client: Experiencing Resistance in Yourself

I ask you again to assume the role of client and imagine that you and I are engaged in a therapeutic relationship. I am presenting some key ideas about the experience of resistance in this way because I believe you will come to appreciate the inevitable place of resistance in the therapeutic process if you allow yourself to consider ways you might resist when you are anxious.

I encourage you to involve yourself in personal counseling if you plan to become a professional counselor. You are apt to learn a great deal about helpful and unproductive interventions through your experiences as a client. If you and your therapist work well together, you also will learn quite a bit about how you present yourself to the world. You are bound to respect the courage it takes to forge ahead even though you are frightened, particularly at those times when you wonder if the gain is worth the pain you might be experiencing. In Chapter 10, when we consider the topics of transference and countertransference, I provide greater detail on the importance of opening yourself to some form of personal self-exploration—individual counseling, group counseling, family therapy, or some other pathway toward self-understanding such as a spiritual direction. For now, let me encourage you to be as open as you can be in imagining yourself in the role of a client in counseling with me as you deal with your own ways of resisting.

I will assign a variety of resistive behaviors to you in this section and show how I might intervene. Allow yourself to get into the role of actually experiencing the various forms of defensive behaviors. Some of these scenarios will not fit you, and you may have other creative ways of resisting that I do not describe. See what you can learn about yourself by placing yourself in the center of resistance.

In our client–therapist relationship, my main endeavor is to create and maintain the kind of working relationship that will allow you to take significant risks. Part of this relationship means that I must recognize the signs of resistance, both in you and in myself. If you and I are not dealing well with resistance, it may be a sign that our relationship needs strengthening. (Refer to Chapter 2 on the therapeutic relationship as the foundation for effective counseling for more on this topic.)

The ways you might resist are many, some subtle and others more obvious. Ask yourself if you might engage in any of these resistive or defensive behaviors:

- Forgetting about your counseling appointment
- Frequently showing up late for your sessions
- Not having material to bring into the sessions
- Complaining that you are not being helped by counseling

- Being silent and expecting to be drawn out by the therapist
- Becoming defensive when you get feedback
- Engaging in long-winded stories and leaving out how you are feeling
- Doing a great deal of intellectualizing about why you feel the way you do
- Avoiding emotional expression
- Striving very hard to please the therapist
- Talking in the abstract and remaining global
- Depending on the therapist excessively

Some of these behaviors may be anchored in reality and be a realistic and appropriate response. For instance, your defensiveness regarding feedback from your therapist may be a function of how the therapist presents the feedback to you. Everything that looks like resistance may not actually be resistance. This is why resistance needs to be explored and its meaning sensitively discussed. What are some other ways that you might resist?

Imagine that you are coming for one of the early sessions of counseling with me. Are you experiencing any reluctance? Are you aware of ambivalence regarding changing versus staying as you are? How do you think you would deal with any anxiety you might be experiencing? What will you talk about at the first session? If you are like many in the helping professions, you might have trouble asking for help for yourself. Do you think you should be problem-free if you are going to be an effective counselor? Do you have some pressing problems but feel that you should be able to resolve them on your own without any assistance from anyone?

How open are you? How much do you want from counseling? Students enrolled in counseling programs often go for counseling because it is required as part of the program or because they have been encouraged by their professors (or authors of their textbooks). Some of the reluctance they might be experiencing can be summarized thusly:

> Well, I'm not really sure I need therapy, but I suppose I could learn something about myself from coming in here. To be truthful, though, it is sort of difficult for me to ask for help. In many ways I think I should be able to deal with my problems by myself. After all, everyone has problems, so maybe I could get along fine without counseling. Besides, I've got a lot going for me in my life, and things are great. If I start questioning, who knows what I might find out. Maybe it would be better to let good enough be!

Now I don't expect you or any client to say this much as an opening statement, but it does illustrate some of the ambivalence I see in counseling students when they present themselves for personal counseling. Reflect for a moment about some of the ambivalence you

might be experiencing if you came to counseling because of the urging of your professors or mainly to meet a requirement of your program. If you do have feelings of ambivalence, or negative reactions, to the expectation of receiving personal therapy, how might this relate to your views of counseling in general?

It will be beneficial to spend time exploring some of your beliefs about seeking counseling for yourself. Do you think initiating counseling means that you are not in control of your life? Are you admitting that there is something wrong with you by going for counseling? We can certainly talk about these beliefs and how such beliefs might hinder your openness in this counseling relationship—and how these beliefs might hinder you as a counselor. Talk about what you are experiencing as you come into this session today. Are you feeling excited? Anxious? Hopeful? Frightened? Cautious? Eager? I will continue to center my questions around your feelings, especially if you are experiencing hesitation, to facilitate a deeper understanding of the meaning of your resistance.

We are working well together, and you decide there are some areas of your life in which you feel somewhat stuck and want to understand more fully. This is the fifth session, and you bring up for discussion the gulf that you sense between you and your father:

> My dad has never been emotionally available to me, and I still miss this. You'd think that by this time I should be over needing approval and affection from my father. The truth is there are times when I realize I'm still looking to others for what I've missed with him. I still remember when I was a child and how much I wanted him to notice me and tell me I was special. But I felt he never really knew me and wasn't too interested in spending time with me.

There is a great deal of rich material to pursue here. But let's assume that with every question I pose you draw a blank. You have little to say and seem emotionally reserved. There are long pauses, and many times you grow silent. You give only terse responses to my inquiries. Can you imagine any of this happening?

There are many reasons you might not be ready to work and why you may be hesitant. I want to know about these reasons, but I also want you to understand your own reluctance, and I will encourage you to talk about this. If you can remain nonjudgmental and can accept some ambivalence, there is much you can learn about yourself by exploring these feelings. Resistance typically occurs when a person begins to approach painful or threatening material that, if revealed, could result in feelings of vulnerability. It makes sense that you are not eager to experience feelings of shame, vulnerability, and uncertainty. At the same time, I want to be consistent in inviting you to address the ways you may be holding back. When you get close to painful experiences and become frightened, do you try to avoid these feelings

by some kind of diversion? I will encourage you to challenge your tendency to avoid an uncomfortable state and to go deeper into the feeling or behavior you wish to avoid. In my view, facing and experiencing feelings takes courage. Your willingness to endure the pain or anxiety that is necessary for getting unstuck and making way for new growth is a reflection of that courage.

At this point, I suggest that you participate in a Gestalt experiment. I ask you to bring what you are experiencing with your father into the room at this moment:

> There is an empty chair over here. Would you be that child who so much wanted to be noticed and wanted Dad's approval? Tell him that now. He is sitting in that chair and is ready to listen to you.

Your response is a flat refusal! Although I am not invested in forcing you to participate in any particular technique, I am interested in exploring your reluctance in seeing where this experiment might lead. Talking about what is holding you back seems crucial to me. You respond to my questioning in one of these ways:

- It seems stupid to talk to an empty chair. I would feel foolish doing that.
- I'm afraid of doing what you ask because just thinking about it brings tears to my eyes.
- I'd rather just tell you about my father because it would feel weird for me to talk to him when he is not really here.

Talking about what your cautious behavior means to you is one way to make this setting safer for you. If you'd feel foolish, I ask what it would be like for you to feel foolish in my presence. This could open up useful material. If you indicate you don't want to put your father in the empty chair and talk to him because just imagining this brings you to tears, I follow that lead more fully. What are you crying about? How is it for you to get close to sadness at this moment? If I am able to understand the purpose your resistance serves and respect your hesitation, you will be more likely to come to a new understanding of your defensiveness. I want to show you that I am willing to go as far as you are and that I will not push you to do what you say you do not want to do. I will respect your boundaries, and together we will proceed at the pace that is right for you. Ultimately, you decide which topics you are willing to pursue and how far you will go. By exploring the anxiety underlying an apparent resistance, the soil is being prepared for you to engage in self-exploration in more depth.

In another session, you provide the starting point for our discussion:

> Today I want to talk about the tough time I am having in getting over being rejected in a relationship. Some days I think I'm fine, and other days I really get depressed and start thinking all sorts of horrible things about myself.

There are many ways for us to work with this. Depending on the context of what you are describing, I might pursue a cognitive path with you, or encourage you to get more in contact with the feelings you are experiencing, or talk with you about actual courses of action you might take in doing something different. In fact, in one session it is very possible that we would work with what you are telling yourself about this breakup (cognitions and self-talk), I'd invite you to stay with feelings that are surfacing as you talk, and I'd suggest some behavioral steps you can take in moving in a different direction.

But what if you have mixed reactions about engaging in a dialog on any level? What if your defenses are getting the best of you and you don't seem to be getting far in working cognitively, emotionally, or behaviorally? Put yourself in the following scenario and imagine the ways you might resist.

You feel greatly disappointed over this breakup. This is proof that you're unlovable. I suggest that you examine the validity of your conclusions. You reply that you should have been over this by now. As you reveal some of what you tell yourself about the breakup, I notice you tearing up, yet smiling at the same time. I describe what I'm noticing, and you withdraw. I suggest that you talk more about feeling rejected. Instead you provide an intellectual discussion about the tentative nature of interpersonal relationships and come up with a number of truisms such as "Life is tough" and "You just have to put up with what life gives you." I notice the ways I see you are defending yourself, and I invite you to say more. For instance, what might happen if you were to allow yourself to feel the intensity of what you are feeling about the loss of this relationship—without smiling and without giving intellectual explanations? Following your resistance in a gentle but insistent manner could free you from the impasse you are experiencing. I am not at all sure where your exploration will lead if I ask you to stay with whatever avoidances emerge, yet I have a hunch that doing so will be productive. I encourage you to let down some unnecessary defenses but not to strip away all of your defenses. This process must be done respectfully and carefully, yet persistently.

There are many ways to approach resistance both in session and outside of the sessions. Reflect on some of the main ways you are likely to manifest resistive behavior when your anxiety surfaces and write about this in your journal. Imagine all the ways you might get in your own way when you feel threatened psychologically. What are some defenses you rely on? What are examples of defenses you have used when you felt vulnerable? How did this work for you? To what extent do you think you would challenge your resistance by talking about it? I hope you don't think courage means that you are without any fear. Courage means being afraid yet going ahead nevertheless. At points in your therapy, you may become hesitant because of fear of your emerging feelings. How could you remain in the moment longer when your initial tendency is to withdraw?

By putting yourself in the role of the client, I hope you have come to appreciate how complex the counseling process can be. I suspect that some of your most valuable lessons on how to recognize, understand,

and deal with resistance in your clients will be learned from your own experience with personal counseling.

Understanding Kelsey's Resistance

Kelsey is experiencing problems in her marriage. She is becoming more independent, which is threatening to her husband. He cannot understand why she wants to pursue a graduate program in social work and why she cannot settle for being a stay-at-home mom. He continually tells Kelsey that much needs to be done at home and that is where she is needed. Kelsey is comfortable staying at home and not pursuing her career ambitions, but she also wanted more for herself.

Kelsey makes progress in therapy for the first five sessions, and then she suddenly begins to consider terminating her therapy. She is fearful of the consequences of her choice to pursue educational and career goals, especially because the more independent she is becoming, the more tension is apparent in the family. She is ambivalent about making changes in her life, and she is wondering if doing this is worth the price she might have to pay.

I respect Kelsey's genuine concerns about moving forward versus staying with her present existence. I also wonder if she has fears about failing in her new profession and life, and I inquire about possible ambivalence and fears in changes in these areas. Rather than fighting her initial resistance, hesitation, and ambivalence, I view it as a positive sign of her strength. Originally, Kelsey's defenses served her in adapting to very difficult life circumstances. It is important for her to realize that she is now capable of far more creative and healthy responses to the life tasks she faces.

Overall, Kelsey is willing and motivated. She is insightful, courageous, able to make connections between current behavior and past influences, and is willing to go beyond her comfort level by engaging in new behaviors both in our sessions and in daily life. Even under such favorable and almost ideal circumstances, it is not uncommon for her to experience ambivalence about change and to entertain doubts about the value of counseling. I want to normalize her ambivalence and to proceed cautiously; it shows that she is aware of the risks of changing and the anxiety that is associated with coming to terms with the unpredictable aspects of her life. In one of her sessions, Kelsey discusses whether to continue therapy. She is realizing that many in her family do not like the changes she is making. Her relationships with her husband and children are becoming more strained, which is causing her to doubt the value of what she is doing in counseling.

Working With Kelsey's Resistance Using Other Modalities

Borrowing From a Psychoanalytic View

Kelsey's resistance can be understood in the context of the anxiety over what might arise if she were to make choices for her own life that may be at odds with what others want from her. Together we identify

some of the behaviors that cause her anxiety. I am interested in her interpretation of how she is dealing with threatening situations, and perhaps later I will offer my hunches. I support her in facing her fears. At times, I suggest an interpretation in the form of a hunch to help Kelsey become aware of the reasons for her resistances. For example, "Let me share a hunch about what I see going on and see what you think of it. What we've been talking about is really frightening for you. Getting your family upset over your changes is certainly not comfortable for you. You seem overwhelmed and you seem to want to give up." Kelsey can then reflect on the feedback I offer her and decide how accurate and useful my impressions of her are. I aim to deal with signs of resistance in a collaborative, nonjudgmental, nonthreatening, respectful, straightforward, and therapeutic manner.

Drawing on Other Approaches

I explain to Kelsey that most clients are ambivalent, defensive, and hesitant at some point in the counseling process. If she does not discuss her fears and reservations about being in counseling, this push-pull may result in premature termination of her therapy. She focuses on her husband's displeasure over her changes and the negative reactions from her children, but internal factors also may be affecting Kelsey and contributing to her resistance. Part of her wants to cling to the status quo, but another part of her wants to branch out and become more of the person she would like to be. I ask her to voice her doubts about the value of therapy. It is clear that she is ambivalent and is reluctant to make choices. Talking about her reluctance to participate fully in therapy can provide her with insights about the meaning of her reluctance.

Later in the session I suggest we work with her ambivalence over wanting to change and resisting change, which is a central principle in motivational interviewing. I suggest a role play as a way of exploring her ambivalence. I ask Kelsey to be the side of her that wants to remain the same because that is the feeling that seems strongest in her now. I take on the role of the side of Kelsey that wants to venture forward. In this role play we debate the pros and cons of changing. I do my best to highlight the advantages of taking the risks involved in making significant life changes. She has an opportunity to express out loud what she tends to rehearse silently about the guarantees of sticking with the "old version" of her life. After engaging in this role play for a time, I ask her what she is experiencing. We then reverse roles so that she can deepen her experience of the ambivalent feelings within her. After switching roles for a time, she realizes that it would not be in her best interests to end therapy at this time, even though it is contributing to increased tensions at home.

Toward the end of the session Kelsey admits that she had not realized how scared she is, but she adds that preserving the status quo isn't working for her either. It has become clear to her that she wants more from her life, yet she is frightened when she considers what she might have to do or become to have a more satisfying existence. Because I

accept the existential notion of the client's place in choosing how far to go, it is Kelsey who decides whether she has sufficient motivation to face her reluctance in making some basic changes in her life.

The feminist therapy approach reminds me of how crucial it is for Kelsey to be an active participant in her counseling and to make her own choices. I am committed to ensuring that our therapeutic relationship does not become another arena in which she stays in a passive, dependent role. During our first few sessions, she tended to look to me for answers or advice. I continue to place the responsibility back on her and avoid becoming the expert on her life, and she experiences glimpses of what it is like to trust more in her own power. Some of her self-doubts and anxiety over continuing in therapy relate to a host of feelings that she has contained but that are now emerging within her.

At this point, I attempt to pull together some of the conflicting themes we've talked about in the session and invite Kelsey to reflect on what changes she wants to make. I suggest a homework assignment, as I frequently do, with the expectation that she will gain greater clarity of the options open to her and the choices she is willing to make. (I draw on behavior therapy, reality therapy, and rational emotive behavior therapy.) For one week, I ask Kelsey to write in her journal all the reasons for staying the same versus the reasons for making changes. I ask her to reflect, for one day, on what her life might be like if she quits therapy and to think about the kind of life she might have if she continues on the path she has pursued for much of her life. On another day, I ask Kelsey to write in her journal about how she imagines her life could be if she continues with her therapy and makes some of the changes she desires. She agrees to follow through with this homework, which gives us a good place to continue in a subsequent session.

Reflection Questions

Reflect upon your views of resistance and ambivalence. Consider these questions:

- In what ways have you experienced resistance in yourself? What has been helpful in challenging your resistance?
- If Kelsey were your client, what kind of theories would better enable you to understand and work with her?
- Kelsey is ambivalent about making life changes. Part of her wants to change, and another part of her wants to remain as she is. How would you work with Kelsey's ambivalence?
- What stage of change does Kelsey's behavior suggest? What clues led to your classification?

Understanding Rico's Resistance

Rico is a 45-year-old male who sought counseling because of anxiety and a reluctance to leave his home. He is unmarried, living independently, and a college graduate employed full time. Two years earlier he began experiencing high levels of anxiety leading to a series of panic attacks. His stated goal is to become comfortable again and resume leading an ordinary life.

In early sessions much of the focus was on assessment, treatment planning, and goal setting. Rico now reports being eager to get on with his life and seems committed to the counseling endeavor. He initially followed through with several behavioral recommendations, including a medical evaluation to rule out any physical basis for his symptoms. He developed a consistent daily routine to ensure healthy sleep and nutrition patterns with some level of consistent exercise. He was eager, invested, and quite successful in accomplishing these early goals. By our seventh meeting, however, he began arriving late or rescheduling sessions, and he has taken very little initiative to make gains in our work together. This pattern of lack of follow through on behavioral homework between sessions may be understood as resistance. Rico may be struggling with the hard work and continuous efforts behavioral tasks require outside of the office, and he also may be reacting to the intensity of interpersonal work in our sessions. Instead of focusing specifically on his reluctance, I use solution-focused brief therapy techniques to help him clarify what he wants and assist him in taking the steps necessary to achieve his goals. I suggest, "Let's talk again about what you envision for yourself within the next few months. How would you like your life to look? What would you like to be doing that you aren't currently doing?" Whether Rico's experience is based on fear, avoidance, or being overwhelmed, I convey empathic understanding and acceptance and remain invested in him getting the most from his therapy.

When I shared my observations of what Rico was doing, he appeared guarded and defensive. Rico admits that he measures how much he will disclose in our sessions, and he and I talk about what his hesitation and guarded behavior might mean. He lets me know that he wants to "get it right" when he speaks, and he does not want to look foolish. To protect himself, Rico thinks carefully about what he wants to say and how to say it before he begins to speak to me.

I encourage Rico to say more about what is going on inside of him that he is not expressing with his long pauses, internal rehearsing, and short answers. I find what Rico is not saying to be very interesting, and this seems like a useful avenue for exploration. I might say any or all of these things to Rico: "I hope you will rehearse out loud." "You are quiet, yet it seems like a lot is going on in your head. Are you willing to put words to any of this?" "I'm noticing that you give short answers, that you think a long time before replying, and that you seem to be very cautious. What is it like to be in here now?"

I cannot know what keeps Rico quiet unless he eventually shares his feelings and thoughts with me. From a position of interest and concern, I gently explore underlying feelings Rico may be having about our work together. I share my observation with him that he seems willing to talk about past events that may have created anxiety for him from an intellectual standpoint but tends to avoid connecting emotionally to these events. I want to challenge Rico on the manifestations of his ambivalence, but it is essential that I note how difficult the emotional and behavioral work involved in therapy is, and that I support even small steps he takes toward change.

Rico acknowledges that his behavior is not working for him and that he does not have the kind of life he wants. He recognizes that he does have problems, yet he claims to lack the motivation to take the steps necessary to change his patterns. It is critical for Rico to be willing to identify his ambivalence regarding change and to talk about this in our sessions. Using the spirit of motivational interviewing, I encourage Rico to explore his ambivalence over wanting to change, yet also wanting to stay with familiar patterns. I also reflect on my style of interacting with him to better understand my own part in creating resistant or noncompliant behavior on Rico's part. I want him to know that I am not judging him on his cautiousness and reservations. I adapt my approach and let Rico know that he will likely experience ambivalent feelings about change and that his motivation may ebb and flow during the course of therapy.

I continue to utilize humanistic and experiential concepts to maintain a framework of safety and trust with Rico, which might enable him to experience his feelings on a deeper level. Although he has been relatively unwilling to follow through with behavioral assignments and homework between our meetings, I encourage Rico to role-play in sessions that provide opportunities for behavioral practice and rehearsal.

Finally, I use some concepts from reality therapy to explore ways to modify our goals throughout the process. Together we seek alternatives and different strategies for him to try. I might say,

> So, you didn't achieve your goals this past week because you are feeling overwhelmed. Let's focus for a moment on how you sometimes overwhelm yourself and consider how you can cope with this in the future. How about setting some goals and making specific plans that you are willing to implement this week? What are you willing to do to achieve your goals?

I challenge Rico for not following through on behavioral assignments, yet I let him know I recognize how difficult it is to bring about changes in old patterns. I also want to convey respect for Rico's capability in establishing goals and acting on his own behalf.



Reflection Questions

Reflect on how the therapist worked with Rico's resistance. Consider these questions:

- What signs of resistance can you identify in Rico's case? How would you intervene with some of his resistive behavior?
- What theoretical framework(s) would you find useful in understanding Rico's resistance?
- If Rico were your client, what might you find challenging and why?

Guidelines for Dealing With Resistance in Clients

The various perspectives on resistance presented here provide a foundation for you to use when formulating your own perspective on the role of resistance in the counseling process. Doing so will assist you in respecting the difficulty your clients often experience. A summary of key points follows that may help you decide how to therapeutically work with the reluctance, defensive behavior, and resistance you may encounter as you work with clients.

- Realize that initially many clients are defensive about having to meet with you to deal with their problems. Many people have been given the message that asking for help is a sign of weakness, which may make them hesitant to speak openly.
- Be open and invite your clients to talk about any concerns they may have regarding the progress of therapy and their reactions to you.
- Avoid labeling and judging clients, and instead describe the behavior they are displaying. If you label certain behaviors your clients display as "resistance," they might feel judged and begin to think of resistance as something negative.
- Encourage clients to explore any form of resistance rather than expecting them to give up their resistance.
- State your observations, hunches, and interpretations in a tentative way rather than making dogmatic pronouncements. Approach your clients respectfully and work with their defensiveness and fears therapeutically.
- Distinguish between the phenomenon of resistance, which is occurring in your clients, and your reactions to their resistance. Monitor your reactions to avoid escalating your clients' resistance.
- Do your best to be accepting and nondefensive as you listen to your clients. If you understand and accept your clients and do not react defensively, the intensity of clients' resistance is likely to diminish. If you meet resistance with resistance, you are likely to entrench this pattern. As a practitioner of motivational interviewing would say, "Roll with the resistance."

- Allow clients to express their feelings about prior negative experiences with counseling. Ask them what they would like to do differently with you.
- Remain attentive to the types of resistance displayed by your clients. Talk openly with your supervisor about these patterns so that you can adjust some of your behaviors if they appear to exacerbate clients' resistance.

Concluding Comments

Resistance is not an enemy to be feared or a therapeutic evil to be eliminated; it can be an important facet of the therapeutic process. Clients' resistance and ambivalence may have many sources; do not interpret it as evidence of your professional incompetence. If you are focused on defending yourself against the various forms of resistance and defensiveness you encounter with clients, you deprive your clients of the opportunity to explore the meaning of their reluctant behavior and thus limit what they can learn about themselves. Think of resistance and ambivalence as a normal process involving a lack of readiness on a client's part to get involved in counseling.

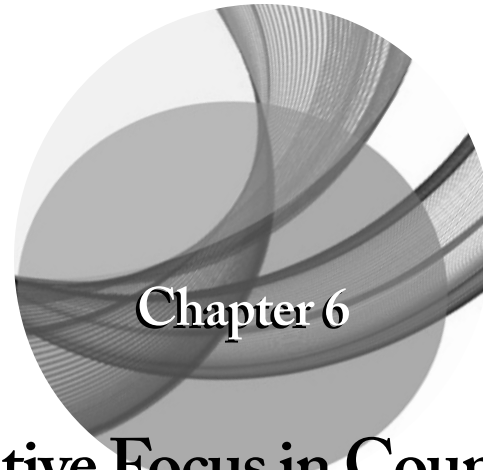
Realize that clients are often ambivalent when it comes to change, and consistently invite your clients to talk openly about their ambivalence. Clients often realize that there are both advantages and disadvantages to giving up some of their comfortable patterns. Clients may want to change certain behavioral patterns, but anxiety is likely to be associated with making changes. They may wonder whether enduring this anxiety is worth the possible gains. Understand what this ambivalence means and explore it with clients rather than demanding that they be different.

To better understand the role resistance plays and how best to deal with it, be open to identifying resistive patterns within yourself. Appreciate your own ambivalence when it comes to change and realize that it is possible to want to change and at the same time to want to remain the same. Remember your difficulty in being open to your own growth, and use this as a model for understanding your clients. If you keep in mind how you deal with your own resistance, you will be less likely to personalize client resistance, which will better enable you to work therapeutically with resistance.

As a way to identify how you deal with resistance in yourself, take some time at this point to reflect on the following questions:

- When you consider yourself in the role of a counseling client, what might lead to resistance for you? What are some of the ways in which you are most likely to resist? What kind of resistive behaviors might you use?
- When you have experienced resistance, can you remember some of your bodily reactions? What kind of self-talk goes on within you when you experience resistance?

- What might help you to reduce your level of resistance or defensiveness? What would you want from your counselor when you are resistant?
- Take a few minutes to consolidate your thinking about resistance. What is your definition of resistance? As a counselor, what will help you to understand and deal with any resistance you might encounter from clients?
- Imagine the characteristics of a client with resistant and difficult behaviors sitting in your office. How does this client affect you? What kind of resistive behavior do you think you would be most challenged by as a counselor? What kind of client is most likely to bring out your defensive reactions? Can you learn anything about yourself by paying attention to your reactions to your most difficult client?



Chapter 6

Cognitive Focus in Counseling

Cognitive behavior therapy (CBT) approaches are quite diverse, but all stress the importance of the links between cognitive processes, emotions, and behavior. The assumption is that how people *feel* and what they actually *do* is largely influenced by their subjective interpretation of situations. The three most popular forms are rational emotive behavior therapy (REBT), founded by Albert Ellis; cognitive therapy (CT), developed by Aaron Beck; and cognitive behavior modification (CBM), developed by Donald Meichenbaum. Meichenbaum's (2007, 2008, 2017) cognitive behavior modification approach combines some of the best elements of behavior therapy and cognitive therapy. This approach shares with REBT and Beck's cognitive therapy the assumption that distressing emotions are typically the result of maladaptive thoughts and that the best way to change behavior is to change thinking processes. Meichenbaum believes the quality of the therapeutic relationship is critical to positive outcomes, and he suggests working in a collaborative fashion with clients to develop the skills necessary to achieve the treatment goals. He believes that expert therapists do not view themselves as experts; instead, they continually seek feedback from their clients, supervisors, and colleagues. Meichenbaum uses a Socratic discovery-oriented approach and the art of questioning to assist clients in reaching their goals. Clients learn that psychological distress is a function of the interdependence of cognitions, emotions, behaviors, and resultant consequences. In therapy, clients learn to change their internal dialog, which serves as a guide to new behavior. Meichenbaum suggests that it may be more effective to *behave* our way into a new way of thinking than to *think* our way into a new way of behaving. A basic premise of CBM is that a prerequisite to behavior

change involves the willingness of clients to notice how they think, feel, and behave and the impact they have on others.

CBT emphasizes a collaborative effort in which therapist and client frame the client's conclusions in the form of a testable hypothesis. Cognitive behavior therapists are continuously active and deliberately interactive with the client; they also strive to engage the client's active participation and collaboration throughout all phases of therapy. Clients adopt a stance of empiricism and apply the scientific method to evaluate their experiences. The client works as a partner with the therapist, assuming the role of a learner in the therapy sessions and engaging in homework that is carried out in daily life (Kazantzis, Dattilio, & Dobson, 2017).

The grandfather of CBT, Albert Ellis, consistently wrote and taught that REBT is highly interactive and that it is impossible to work on just a single dimension of personality. Aaron Beck, the father of cognitive therapy, emphasizes the integration of thinking, feeling, and behaving. Donald Meichenbaum, a leading figure in contemporary CBT, believes that our emotions and thinking are two sides of the same coin; the way we feel affects our way of thinking, just as how we think influences how we feel. With Meichenbaum's approach, clients learn about the role that both cognitions and emotions play in creating and maintaining stress through curious questioning and a process of guided self-discovery.

All of the cognitive behavior therapies place thinking at the core of emotional and behavioral disturbances and treatment, and all of them posit that changes in thinking also affect emotional and behavioral changes. Their differences lie mainly in the style a therapist employs. In these approaches, the client must be active if change is to occur. They have the common goals of providing symptom relief, assisting clients in resolving their pressing problems, changing beliefs and behaviors that maintain problems, and teaching clients skills they can apply in everyday life and in preventing future lapses.

These models all incorporate the Adlerian sequence that behavior is fueled by emotions, and emotions are the result of thinking. Thinking reflects a decision and purpose that the individual has for self and others; it is goal-directed. Thinking can make common sense, or it can reflect the private, mostly irrational, thinking of self-protection.

Benefits and Limitations of a Cognitive Focus

CBT is one of the most widely researched psychotherapies. Over many years, a number of CBT techniques have been developed and standardized; each of these techniques aims at helping people identify, challenge, and modify inaccurate thinking believed to be at the core of various disorders (Rousmaniere, Goodyear, Miller, & Wampold, 2017). Although CBT enjoys wide popularity, Wampold and Imel (2015) report that research has failed to provide evidence that CBT is more effective than most other treatment approaches. When CBT is compared with other therapeutic approaches, no significant differences in effectiveness

are found. CBT is regarded as the therapeutic approach with the most empirical support, however, partly due to the scientific commitment of the approach (Matu, 2018), and the amount of evidence supporting the efficacy and effectiveness of CBT is impressive (DiGiuseppe, Venezia, & Gotterbarn, 2018; Dobson & Dobson, 2017).

Key Concepts and Themes of Cognitive Therapy

Cognitive therapy has many benefits for a diverse client population and a wide variety of problems. Cognitive therapists are interested in what clients think, believe, and the way in which they perceive the world. Basic beliefs may be the product of considerable reflection and questioning, or clients may have acquired a number of beliefs without critically evaluating them. In either case, how clients feel and what they do in certain situations has a lot to do with their basic beliefs and thought patterns. Our thinking is directly connected to our emotional reactions, behaviors, and motivations. When we think about events in erroneous or distorted ways, we experience exaggerated or distorted emotional and behavioral reactions as well. Some beliefs may serve clients well, whereas others may lead to problems for clients.

CT explains psychological problems as an exaggeration of adaptive responses resulting from commonplace cognitive distortions. Albert Ellis (1962) was the first to name irrational ideas that were common in the way people were raised and in how people who were anxious, angry, or depressed thought about life. His original list contained 11 common irrational beliefs. Aaron Beck (1976) looked at the quality of dysfunctional thoughts, and we can now identify 10 cognitive distortions that are common in emotional disorders (Dattilio, 2010). They are:

1. *Arbitrary inference*: conclusions without evidence
2. *Selective abstraction*: forming conclusions based on an isolated event without considering the total context
3. *Overgeneralization*: making a general rule from one or two *occurrences*
4. *Magnification and minimization*: making more or less out an event or situation than is supported by facts
5. *Personalization*: the tendency of individuals to relate external events to themselves, even when there is no basis for making this connection
6. *Dichotomous thinking*: assessments of all or nothing, always or never, good or bad, right or wrong
7. *Labeling and mislabeling*: portraying one's identity on the basis of imperfections and mistakes made in the past and allowing them to define one's true identity
8. *Tunnel vision*: seeing only what one wants to see or what fits the person's current state of mind
9. *Biased explanations*: attributing a negative motive or intent even to positive behaviors
10. *Mind reading*: belief that you know what someone is thinking without evidence or facts

These cognitive distortions become key targets in practicing CT. Many of the interventions in therapy involve helping clients identify these distortions and gathering evidence to aid in reconstructing their way of thinking. Although significant others may have contributed to shaping clients' current lifestyle, clients are responsible for maintaining self-destructive beliefs and attitudes that influence their daily transactions. The goals of CT are to help clients learn; to practice skills they can use to make changes in their thoughts, behaviors, and emotions; and to sustain these changes over time. Cognitive therapists see value in asking clients questions to modify negative thinking such as the following:

- What is the evidence for and against this thought?
- What are some other ways of thinking about this situation?
- What are the implications of thinking this way?

Through a reflective questioning process, the cognitive therapist collaborates with clients in testing the validity of their cognitions, a process called *collaborative empiricism*. In this process, the therapist and the client identify and work together to solve the client's concerns (Sperry & Sperry, 2018). They become co-investigators, examining evidence to support or modify cognitions (Beck & Weishaar, 2019). Once cognitions are identified for intervention, open questions can become a part of the cognitive restructuring process. Cognitive therapists have a wide range of evidence-based methods for shifting automatic thoughts (Dobson & Dobson, 2017). Cognitive therapists often use the *Socratic dialog* method to detect the validity of clients' automatic thoughts. Therapists continually ask clients to examine evidence for and against their belief system. Questions enable the therapist to understand the client's perspective and are raised with sensitivity so that clients may look at their assumptions objectively. The use of Socratic questioning can enable clients to become aware of what their thoughts are, to examine them for cognitive distortions, and to develop constructive thought patterns. Cognitive practitioners emphasize helping clients identify misconceptions for themselves rather than being taught. When used skillfully, Socratic dialog invites a client into a collaborative discussion and provides a foundation for the therapist to use various interventions for modifying thinking (Beck & Weishaar, 2019; Kazantzis et al., 2017).

Adlerian therapy, rational emotive behavior therapy, cognitive therapy, cognitive behavior modification, choice theory/reality therapy, narrative therapy, and solution-focused therapy share the basic assumption that clients' interpretations of situational events are crucial to understanding their clients. Instead of talking about events, therapists with a cognitive focus explore the personal meanings clients attach to these events.

Although clients benefit from a cognitive understanding of their problems, an overemphasis on the cognitive realm can shortchange the emotional dimension. For instance, it may be difficult for clients to

identify and experience what they are feeling. Because of the anxiety of staying with painful emotions, clients might use some form of deflection and engage in intellectualizing. If clients too quickly try to figure out why they are feeling a certain way, they may avoid facing what they are feeling. For example, if a client doesn't get a job she wanted, she might engage in self-deception and rationalizations about why she didn't really want the job in the first place rather than experiencing her feelings and appropriately expressing them in the here and now.

Approaches that highlight cognition typically do not give much attention to exploring a client's past emotional issues. Cognitive therapy is focused on present problems. Working within a cognitive framework, the past may be brought into therapy when the therapist considers it important in understanding how and when certain core dysfunctional beliefs originated, and how these ideas have a current influence on the client's difficulties. Cognitive therapists pay attention to clients' past without getting lost in the past and without assuming a fatalistic stance about earlier traumatic experiences. Past unresolved childhood experiences can be productively explored in therapy if these earlier experiences are connected to a client's present level of functioning. My personal belief is that painful early experiences need to be recognized and worked through in therapy before clients can free themselves of their restrictive influences. Present beliefs about self and clients' current problems are often related to past hurt. Unless clients come to terms with these past traumas, the vestiges of these traumas tend to linger in the background and influence their current ways of being.

From my perspective, the cognitive behavior therapies work best once clients have identified and dealt with their emotional issues. In practice, I don't see how it is effective to work *exclusively* in a cognitive way, or an emotive way, or a behavioral way. We are holistic beings, and all of these domains need to be considered. In fairness, contemporary cognitive behavior therapy is based on the assumption that there is a high degree of interaction among the cognitive, affective, and behavioral domains. Because of these interconnected aspects of human experience, the CBT approaches do not use cognitive interventions alone. All of the cognitive therapies include both cognitive and behavioral interventions; they also include behavioral activities within the session and behavior assignments between sessions (DiGiuseppe, David, & Venezia, 2016; DiGiuseppe et al., 2018).

All integrative approaches to therapy make room for the cognitive dimension—we are all thinking beings. I pay a great deal of attention to thinking as a vital component in counseling because the content of thought processes greatly influences both how we feel and how we act. An integrative approach requires dealing with self-talk, faulty thinking, core beliefs, automatic thoughts, and one's worldview. I find many aspects of CBT very valuable in my work, both during therapy sessions and in a variety of everyday life situations. Psychotherapy is essentially a psychoeducational process, which makes cognitive and behavioral methods most relevant. The psychoeducational focus of

CBT is a clear strength that can be applied to many clinical problems and used effectively in many settings with diverse client populations.

If you are interested in further reading on integrating cognitive therapy into a comprehensive approach, consult J. Beck (2005, 2011), Dobson and Dobson (2017), Ellis and Ellis (2011), Kazantzis, Dattilio, and Dobson (2017), Meichenbaum (2012, 2017), Sperry and Sperry (2018), and Vernon and Doyle (2018). For a general treatment of cognitive perspectives, see Beck and Weishaar (2019, chap. 7) and Corey (2017, chap. 10).

Becoming the Client: Experiencing Cognitive Behavior Techniques

To be most creatively applied, cognitive and behavioral strategies must be tailored to the client's unique needs and situation. You can best appreciate the practical value of cognitive behavior techniques by applying these interventions to yourself personally. Once again, assume that you are my client and consider how you might react to cognitive behavior interventions. To increase cooperation, it is important that I have awareness of and respect your reactions to an intervention. These interventions are tools to be used in the service of you, the client. Here are a few cognitive behavior techniques from my integrative approach that I might employ with you as my client.

Paying Attention to Your Thinking

As a prerequisite to behavior change, I believe you must notice and become aware of how you think, feel, and behave, and the impact you have on others. For change to occur, you need to interrupt the scripted nature of your behavior so that you can evaluate your behavior in various situations. Toward this end, I am likely to ask you questions that get you to critically evaluate your thinking. Do you engage in catastrophic thinking? Do you dwell on the most extreme negative scenarios in many situations? When you get stuck, imagine the worst possible outcome of the situation, and then ask yourself: "What is the worst thing that could occur? If this happens, what would make this such a negative outcome?" You can learn to engage in more realistic thinking, especially if you consistently notice times when you tend to get caught up in catastrophic thinking.

As your therapist, I look for evidence to support or refute some of your core beliefs. Once we identify a number of your self-defeating beliefs, you can begin to monitor the frequency with which these beliefs intrude in situations in everyday life. I function as a guide who helps you understand how your beliefs and attitudes influence the way you feel and act. During counseling sessions, I often ask you, "Where is the evidence for___?" Make it a practice to ask yourself this question, especially as you become more adept at spotting dysfunctional thoughts and paying attention to your cognitive patterns. For

example, the statement "I must be approved of and accepted by all the significant people in my life" can be disputed with statements such as "Where is it written that I must have this approval?" "Why must I have their total approval to feel like a worthwhile individual?" An effective and functional belief might include this statement: "There is no evidence that I must be approved of by everyone, though I would like to be approved of by those whom I respect." You might also tell yourself, "I detest rejection, so if I keep to myself I won't be hurt." How would this belief affect the way you respond to others? I would ask you, "Even though rejection would hurt, could you prevent yourself from being derailed if you experience rejection? How might this fear of being rejected keep you from getting what you want in your relationships?" At this point, make a list of statements that might get in your way at times. What are a few examples of basic conclusions that you could challenge?

Cognitive Restructuring

Cognitive restructuring is the process of identifying and evaluating your cognitions, understanding the negative behavioral impact of certain thoughts, and learning to replace these cognitions with more realistic, appropriate, and adaptive thoughts. As a client in cognitive therapy, I will assist you in identifying self-defeating cognitions and in monitoring your self-talk. Cognitive restructuring is based on the theory that our thoughts are causally linked to our behavior and emotions; the route to changing behavior and emotion is to change negative or self-defeating cognitions. Cognitive restructuring is a major pathway to changing ways of thinking, and it is a key aspect of all the cognitive behavior approaches. Cormier, Nurius, and Osborn (2017) describe the basic components of the cognitive restructuring process:

- Clients are given an orientation to the basic concepts of cognitive restructuring, including a rationale for the procedure.
- Clients learn to identify thoughts pertaining to problem situations.
- Coping thoughts are introduced and practiced.
- There is a shift from self-defeating to coping thoughts.
- Clients learn and practice reinforcing self-statements.
- Homework is carried out, and there is follow up.

Designing Homework or Between-Sessions Assignments

The cognitive behavior approaches place considerable emphasis on putting newly acquired insights into action. Homework is a valuable part of the process leading to change; homework is a necessary component of the cognitive behavior therapies, without which significant change is unlikely to occur (Dobson & Dobson, 2017). In CBT, promoting between-sessions practice helps you generalize to real-world situations what you are learning in your therapy sessions. Although

homework has been consistently linked to treatment outcome, the term "homework" often has negative connotations (Kazantzis et al., 2017; Naar & Safren, 2017). Thus, I will suggest "between-sessions practice" or "between-sessions interventions" or "take home activities." Assignments are designed after each therapy session and should relate to what is addressed in the session. Ideally, you will have a role in designing these between-sessions activities; you also will take the lead by suggesting steps you can take outside the office to move closer to achieving your goals.

Between-sessions activities enable you to practice new behaviors, test your beliefs, and try out different behaviors in daily-life situations. The therapy hour is limited, and activities that you can practice in your daily life can augment therapy sessions. Between-sessions activities involve practicing skills outside of the therapy sessions to promote mastery, the generalization of these skills in the real world, and the extension of treatment benefits after termination (Naar & Safren, 2017). The best activities are those you suggest, especially self-help assignments that grow out of the previous session. It is essential that I tailor between-sessions interventions to your specific problems and that you participate in designing these activities. Again, let me stress that between-sessions activities are geared to what you want for yourself, not what I, as your therapist, think you should want—or what I want for you. Consider for a moment how you would react to carrying out between-sessions activities in your daily life. Would you generally be favorably inclined to practice new skills in everyday life situations? What might get in your way of doing between-sessions activities? What would help you to comply with carrying out your assignments?

Once you have identified some unsupported conclusions and faulty beliefs, between therapy sessions record and think about how your beliefs contribute to your personal problems. In this way, you can work at critically examining your self-defeating cognitions. When you come to the next therapy session, bring up specific situations in which you did well or in which you experienced difficulty. As you consistently question the actual evidence for situations you encounter, you will become more effective in critically evaluating your self-talk. This allows you to determine whether your self-statements are based on accurate or erroneous information.

I will show you ways to carry on your own therapy, largely through between-sessions activities, without my direct intervention. This provides you with tools you can use to continue learning once formal counseling ends. In my view, much of our counseling endeavor will deal with educating you, teaching you coping skills, and enabling you to see the connection between what you are learning in therapy sessions and everyday living. I particularly value the emphasis CBT puts on bibliotherapy and psychoeducational assignments, such as listening to audiobooks, podcasts, using apps, reading self-help books, keeping a record of what you are doing and thinking, and attending workshops.

In this way, you can further the process of change in yourself without becoming excessively dependent on me as your counselor.

Drawing on Adlerian Concepts

Adlerian psychology pays particular attention to the cognitive aspects of personality and in many ways can be considered a cognitive approach to counseling. For Adlerians, feelings are aligned with thinking and are the fuel for behaving. Working within an Adlerian framework, my assumption is that first you think, then you feel, and then you act. Because emotions and cognitions serve a purpose and aim at a central goal in your life, much of our time during counseling is spent discovering and understanding your purpose and reorienting you in a useful way. You can expect to explore what Adlerians call "private logic," which includes concepts about yourself, others, and your life. The core of the therapy experience consists of discovering the purposes of your behavior or symptoms and the basic mistakes associated with your coping.

In therapy let's assume we discover that the structure of your private logic is captured by this syllogism:

- I am basically unlovable.
- The world is filled with people who are quick to reject unlovable persons.
- Therefore, I must keep to myself so I won't be discovered and rejected.

It is easy to see how depression or a sense of hopelessness might follow from this thinking. Learning how to correct such faulty assumptions will be central to your therapy. Through the therapeutic process, you will discover that you have resources and options to draw on in dealing with significant life issues and life tasks.

Working With Marissa From a Cognitive Behavior Perspective

Marissa, a marketing executive in her mid-30s, is taking an extended break from work to begin cognitive behavior therapy. She previously had been in therapy for more than 5 years with several therapists due to her struggles with anxiety and depression. During the last part of her previous therapy she was taking Zoloft under a psychiatrist's supervision. Her previous therapy was mainly with psychoanalytically oriented practitioners. Even though she had weekly therapy sessions, she was not satisfied with her progress. At times, her anxiety would escalate to panic attacks. A friend of hers who had successfully been involved in cognitive behavior therapy recommended that she try this approach.

Recent issues that seemed to catapult Marissa into more intense levels of depression, anxiety, and panic revolved around a breakup

from a man she considered the "love of her life." Dating other men was not successful. She felt shy, not good enough, not pretty enough, and was self-conscious, which resulted in her feeling hopeless and despondent. She is also desperate to have children. If she does not meet a suitable partner soon, she fears she might never be able to have children. Marissa believes she should have found her lifetime partner already, as many of her friends have done, and she is worried that she never will have a long-term intimate relationship. In addition to these concerns, Marissa is struggling with demanding and critical family members telling her what she should and shouldn't be doing with her life. She also reports having perfectionist tendencies at work and at home, feels that she had no purpose, and has lost interest in life.

Cognitive Interventions With Marissa

The therapeutic relationship is key in the practice of cognitive behavior therapy. CBT emphasizes tailoring empirically supported relationship factors that can strengthen collaboration and improve outcomes (Kazantzis et al., 2017). Although my interventions are active and directive, I place great emphasis on establishing and maintaining a therapeutic alliance and strive to include my client as a partner in the therapeutic enterprise. The therapeutic relationship is a team effort, and we share our expertise. Marissa is the expert on her own life, and I am an expert in applying the cognitive model to assist her in achieving her goals. In counseling Marissa, we will explore her belief systems, her thoughts, her attitudes, and her values.

Although I draw mainly from cognitive behavior techniques, I also base my conceptualization of Marissa's situation on an integrative perspective and use concepts from various theoretical models. More specifically, in family systems therapy attention is given to family rules; in behavior therapy attention is given to beliefs and assumptions that have an influence on her behavior. In rational emotive behavior therapy attention is on self-defeating beliefs and self-indoctrination; Adlerian therapy focuses on her basic mistakes. In reality therapy, the emphasis is likely to be on Marissa's values and what she wants in her world; in feminist therapy, we will conduct an assessment of the impact of gender-role messages. In all of these modalities, I zero in on the underlying messages Marissa seems to be hearing now in her life. I assume that her self-talk and her thinking are relevant to her behavior.

I examine the ways in which Marissa's internal dialog and her thinking processes are affecting her day-to-day behavior. I use an active and directive therapeutic style and borrow an array of CBT techniques that have an evidence base to support them (Dobson & Dobson, 2017). These techniques can help Marissa overcome her problems and attain her goals. I favor therapy that is time-limited, present-centered, solution-focused, structured, and psychoeducational. My task is to help Marissa recognize and change her self-defeating thoughts and maladaptive beliefs. Thus we concentrate on the content and process of her thinking by looking for ways to restructure some of her beliefs.

Rather than merely telling Marissa about her faulty beliefs, I encourage her to gather data and weigh the evidence pertaining to certain core beliefs. By using a Socratic discovery-oriented approach and the art of questioning, Marissa and I identify what thoughts, feelings, and behaviors are problematic for her. I assist Marissa in detecting her nonconstructive thinking, in learning ways of correcting her cognitive distortions, and in substituting more effective self-talk and beliefs. Here are a few of the questions I raise for Marissa to ponder and to answer: "What do you suppose it would be like if you were not to live up to the standards others have set for you?" "If you remain the way you are now, what do you imagine your life will be like one year from now?" "How might you feel different if you were able to ease up on yourself?"

Drawing from rational emotive behavior therapy, a main technique I use with Marissa is disputing inaccurate beliefs. Much of her therapy involves her learning ways to challenge unrealistic beliefs and to argue with her internal dialog. Here are some questions aimed at getting Marissa to examine the evidence for the validity of her beliefs: "Does having this belief help or hinder you in your life?" "Where is the evidence for your belief ?" "Who told you that this belief is accurate?"

Not only do I introduce debating methods during the therapy hour, but I also encourage Marissa to pay attention to her internal dialog in daily life and to detect patterns of thinking that become problematic for her. She can then argue with her internal voices when she catches herself reverting to old patterns. I am especially inclined to employ cognitive interventions when it appears that Marissa is giving in to internal voices that reinforce a stance of powerlessness. Such techniques are called for in situations where she might make dire predictions about her future, and when such conclusions are based on negative beliefs.

My interventions are aimed at getting Marissa to reflect on what she is thinking and how this is influencing how she is feeling. My hope is that she will explore the facts involved in these situations, take another look at some of the conclusions she has arrived at, and develop an open stance in assessing new situations. My aim is to get her to critically evaluate her thinking and how it has a pervasive influence on many aspects of her life today. In dealing with her cognitions from an Adlerian perspective, I might focus Marissa's attention on messages she incorporated as a child and on the decisions that she made in her family of origin. Eventually, she is likely to begin thinking about the reasons she made some early decisions. I challenge her to look at these decisions about life, about herself, and about others and to make necessary revisions that can lead her to form new assumptions about life. This is where psychoeducation fits into the picture. I am interested in teaching Marissa the connections between her thinking, emotions, and behaviors and in demonstrating how we create our own emotions (the circumstances and events of our lives are not the cause of them) and that when we think in healthy rational ways we create healthy emotions.

From a cognitive behavior framework, I accept the assumption that it is not an event or a situation in life that actually causes problematic emotions. Rather, it is the evaluation of events and the beliefs people hold about these events that get them into trouble. Drawing from rational emotive behavior therapy, I explain to Marissa the A-B-C model, as developed by Albert Ellis (2001). This concept is based on the premise that A (the activating event) does not cause C (the emotional consequences); rather, it is mainly B (her belief about the activating event) that is the source of her problems. Much of my teaching will be about how she can change B (her belief system) and thereby make some significant emotional and behavioral changes.

A range of cognitive techniques can help Marissa recognize connections between her cognitions and her behaviors. I look for the ways in which she contributes to her painful feelings by what she is telling herself. Through cognitive restructuring work, Marissa can learn new ways of thinking about her actions, new things to tell herself, and new assumptions about life. Our work together emphasizes Marissa's role in self-discovery rather than direct teaching on my part. Lasting changes in her thinking and behavior will come about through her initiative, understanding, awareness, and effort.

Marissa has an underlying dysfunctional belief that she must be perfect in all that she attempts. If she is not perfect, in her mind, there are dire consequences. She is continually rating her performances, and she is bound to think poorly of herself because of her unrealistically high standards. Indeed, there is a judge sitting on her shoulder. I consistently remind her that rating herself as a person will take her on a path toward misery. Although she might continue to rate some of her actions, she can productively reflect on separating what she is doing from her personhood. My assumption is that only through learning to apply rigorous self-challenging methods will she succeed in freeing herself from the self-defeating thinking that contributed to her problems. The goal of cognitive therapy with Marissa is to help her learn practical skills that she can use to make changes in her thoughts, behaviors, and emotions and sustain these changes over time. I hope to teach her practical ways to talk back to her internal critic, to learn a new and functional self-dialog, and to help her reevaluate her experiences as she changes her behavior.

From rational emotive behavior therapy I especially value the emphasis on learning to think rationally. As a highly didactic therapeutic approach, REBT stresses the role of action and practice in combating irrational, self-indoctrinated ideas. It focuses on the role of thinking and belief systems as the roots of personal problems (Ellis & Ellis, 2019). With this focus, I look for ways Marissa contributes to her negative feelings by the process of self-indoctrination with faulty beliefs. I guide her in testing the validity of the negative consequences she predicts. For example, she is extremely self-critical, she demands perfection, she believes that she must live up to what others expect of her at all

times, and she drives herself by performance-oriented standards to the point of exhaustion. Marissa hears this underlying message, which was given to her by her mother: "If you can't do a job well, don't do it at all." Because of internalizing this message, she frequently experiences frustration and guilt. As I am talking with her about her perfectionism, she indicates that she wishes she could be kinder to herself and not feel and believe that she can never make a mistake. Marissa says, "I'd like to ease up on myself." I suggest a role play in which she plays the side that would like to ease up and I play the part of her that is the critical and driving self. Whenever she indicates that she wants to do less and not push herself so hard, I tell her that she cannot afford to get lax or there will be dire consequences. My rationale for this cognitive role play with Marissa is to facilitate an examination on her part of what she believes and where she acquired her beliefs. I expect that Marissa will question some messages that she has uncritically accepted, and she can then determine whether or not her beliefs are serving her at this point in her life. Through a process of debating, disputing, and critically examining some of the messages that Marissa lives by, she will gain increased clarity of the many ways she is trying to measure up to external expectations. By arguing with me as I play a familiar side of her, she will be increasingly able to experience the depth to which her self-talk actually defeats her. We also do reverse role playing, in which Marissa's role is to become as critical as she can be while I engage in a debate with countermessages. This is only one way of helping Marissa replace mistaken beliefs that have no validity with functional and realistic beliefs. Marissa must find for herself a new set of beliefs that enables her to enjoy life more fully. I am operating on the assumption that a reorganization of Marissa's self-statements will result in a corresponding reorganization of her behavior.

I do not provide her with realistic and appropriate beliefs, but I work with her in creating constructive beliefs of her own. Merely examining her faulty logic is not enough for personality change to occur, but critically examining her thinking is an essential component of therapy. One of the aims of therapy is for Marissa to develop greater unconditional self-acceptance. I suggest to Marissa that she remind herself daily that her worth does not depend on being with a man, having children, doing "perfectly" well at work, or being perfect in everything she does.

The cognitive behavior and action-oriented approaches place considerable emphasis on not depending entirely on what takes place in the therapy sessions to bring about lasting change. If Marissa wants to change her thinking, feeling, and behaving, it will be necessary for her to monitor herself outside of the therapy office. I strongly encourage her to keep a journal in which she pays attention to how she is affected by events in everyday life and to find a place where she can write without censoring and rehearsing. I also work collaboratively in designing behavioral homework assignments that put Marissa in situations in

which she is challenged to confront her self-defeating beliefs and her self-limiting behavior. She realizes that her efforts in applying new skills and new ways of thinking in daily life will facilitate the positive changes she desires. It is important for her to focus on goals that she wants to attain, and thus I avoid suggesting what goals she should pursue. In each session we cooperatively work together to generate ideas of what she can do when she leaves the office to maximize her gains in therapy. Of course, each week I begin the session by asking how she did in implementing her homework.

Current Status of Marissa

After 6 months of CBT during which Marissa made an effort to do her homework activities diligently, the changes Marissa is experiencing are largely positive. She no longer feels intensely hopeless, depressed, and anxious. When unhappy events occur, her emotions tend to be the healthy negative emotions such as sadness, disappointment, and concern.

Marissa has dropped her desperation to get the "one and only" man, along with the urgency to have children; she lives more in the here and now and is getting better at accepting herself unconditionally. She still wants a life partner and family, yet she knows she can design a satisfying life for herself even if they don't emerge. She now realizes that it is not her circumstances that create her feelings; rather, she creates her emotions, largely through her thinking. She is less upset when her dates are not satisfying, or if men do not always follow up by asking her for more dates, and these events do not define her worth or attractiveness. Marissa enjoys her journaling and reflection. Her need for perfection remains a strong focus of the therapeutic work we are currently doing, and she reports she is becoming less compulsive but has a way to go. Marissa says that she enjoys life more, even when things don't go the way she thinks they "should." At this point, Marissa recognizes that she will have to keep making an effort to increase and maintain the healthy changes she enjoys, and she is willing to do so.

Reflection Questions

Reflect on a few key concepts of a cognitive behavior approach. Consider these questions as they apply to Marissa:

- From a cognitive perspective, what aspects of Marissa's case do you find most interesting, and why?
- Identify one cognitive technique you might employ in working with Marissa. What is your rationale for selecting this technique, and what do you expect to accomplish?
- What kind of homework assignments do you think would be valuable for Marissa? How could you work collaboratively with Marissa in creating homework?

Working With Erv From a Cognitive Behavior Perspective

Erv is a 71-year-old retiree who is questioning the meaning of his life as he struggles with many losses. Since his wife died 6 years ago, he has lived with his son. Erv suffers from depression and loneliness and often wonders what use he is to anybody. He has little to look forward to and is focused on a past marked by mistakes, lost opportunities, and regrets. Before he retired, Erv taught philosophy at a community college, which he found very meaningful. He has sought out ways to do some teaching by way of being a volunteer. Since the death of his wife and his retirement, Erv ruminates on what he failed to do in his relationship with his wife and how he has been unable to replace his meaningful career in his college teaching. For Erv, retirement is a sign of not being useful to anybody, and he says he feels put out to pasture. He is searching for some meaningful relationship, even though he realizes he will always miss his wife and that a new relationship will not fill this void. He also hopes to find some kind of work where he could put his talents to good use as he did in his teaching position. In spite of his desire to create new relationships and find some form of work, he does not see much hope that he will find a substitute that will bring a sense of vitality and purpose to his daily existence. He has attended social events in the community center and attempted to initiate relationships with women, but he feels awkward talking to people he does not know and is convinced that they would not want a friendship with him. When he has carried on a conversation, he convinces himself others will find him boring and will get away from him as soon as they can.

In working with Erv as a cognitive behavior therapist, I employ a directive, structured, and action-oriented approach. In many ways, the client–therapist relationship is like the student–teacher relationship. I view therapy as a learning process that will afford Erv with an opportunity to explore a host of ways to change his thinking, feeling, and behaving. I assist him in identifying a number of his core beliefs, many of which do not serve him well. Once we have identified some of his major beliefs, I ask him to begin thinking about the decisions he made about himself, others, and the world. I also ask him to reflect on the direction his current behavior is taking him. Because of his insecurities, he tends to isolate and is very hesitant in reaching out to others.

To assist Erv in achieving a constructive set of beliefs and acquiring a self-enhancing belief system, I perform several tasks as his therapist. First, I assist Erv in critically evaluating his self-defeating beliefs regarding his worth. Throughout the therapeutic process, I attempt to actively teach Erv that being overly focused on past mistakes and regrets is the basis for many of his emotional and behavioral problems.

My major focus with Erv is on his thinking. Only through learning to apply self-challenging methods will he succeed in freeing himself

from his defeatist thinking. My intention with Erv is that he not only learns to recognize his self-defeating thought patterns and the resultant feelings but that he takes steps to dispute problematic beliefs and replace these with functional beliefs.

At some point in counseling with Erv, it is likely that we will examine the validity of many of his interpretations about life situations and his conclusions about his basic worth. I am guided by concepts from Aaron Beck's cognitive therapy and use many techniques that will enable Erv to identify and change his negative thoughts and maladaptive beliefs. According to Beck's cognitive model of emotional disorders, to understand the nature of emotional problems it is essential to focus on the cognitive content of a client's reaction to an upsetting event or stream of thoughts. I draw on a range of cognitive, emotive, and behavioral techniques to demonstrate to Erv that he produces his own emotional disturbances by the faulty beliefs he has acquired. In this and other sessions, we explore what cognitive therapists call "cognitive distortions" (Beck & Weishaar, 2019), which include the following:

Arbitrary inferences: Erv makes conclusions without supporting and relevant evidence. He often engages in "catastrophizing," or thinking about the worst possible scenario for a given situation. He believes that even if he were successful in establishing a friendship with a woman, the odds are that the relationship would not endure and that she will lose interest in him when she gets to know him better.

Overgeneralization: Erv holds extreme beliefs based on a single incident and applies them inappropriately to other dissimilar events or settings. For instance, because he has not found a substitute for his teaching position, he is convinced that he is a failure in many aspects of his life.

Labeling and mislabeling: Erv presents himself in light of his imperfections and mistakes. He allows a single problem situation to define his total being. He has been struggling to find some kind of meaningful work since his retirement, and he labels himself as a failure because he did not get a position as a volunteer in a center for older adults. He laments not being able to do any work, and he is sure that people in the center think he has nothing to offer.

Dichotomous thinking: Erv frequently engages in thinking and interpreting in all-or-nothing terms. Either he is a success as a volunteer or he is a total failure in providing any assistance to others.

Personalization: In many instances, Erv draws negative conclusions without support. He applied for a part-time teaching job and was hoping to be able to teach one course. The department chair told Erv that they had no courses they could offer him because enrollment was down and several courses had to be canceled. Erv firmly believed that the chair and the faculty did not want him to be a part of the program and that they were using the excuse of canceled classes as a way to keep him out of the program.

Over a number of sessions we work on specific beliefs. We do this partly by engaging in cognitive role playing. I challenge his conclusions that because he is no longer actively teaching he has nothing of value to offer. He often says that he likes what I say about him being a valuable person in spite of not being engaged in full-time work. I assure him that with consistent effort he will be able to express a new set of beliefs and find a new and more effective voice. The aim is for Erv to critically evaluate the evidence for his conclusions and to replace unsupported conclusions with realistic ones. I let him know that I do not have answers for him. Erv must come to terms with the significant losses in his life and be willing to experiment with ways to create new meaning going forward. The best I can do is to inspire him to take steps in his everyday life to bring about life changes.

I view my role as promoting corrective experiences that will lead to changes in Erv's thinking. I assist him in discovering for himself how to distinguish between functional and dysfunctional beliefs. He can learn this by testing his conclusions. Only through learning to apply rigorous self-challenging methods will he succeed in freeing himself from the self-defeating thinking that contributes to his problems. Although the cognitive dimension is emphasized, this cannot be accomplished without bringing in the behavioral dimension. I particularly like Donald Meichenbaum's (2017) view of what constitutes successful therapy: clients develop their own voices, take pride in what they have accomplished, and take ownership of the changes they are bringing about. Meichenbaum contends that expert therapists teach their clients how to take the therapist's voice with them, and they learn to become their own therapists. Erv is continuing to extend what he learned in therapy, and he is listening to his own voice.



Reflection Questions

Reflect on some of the cognitive behavior techniques that are used in the case of Erv. Consider these questions:

- Identify one faulty belief that is evident in Erv's case. What is at least one cognitive technique you would use in assisting Erv in challenging this self-defeating cognition?
- What value can you see in facilitating cognitive role playing with Erv?
- What concepts of Beck's cognitive therapy do you find useful in encouraging Erv to explore his beliefs and how they affect his behavior?

Concluding Comments

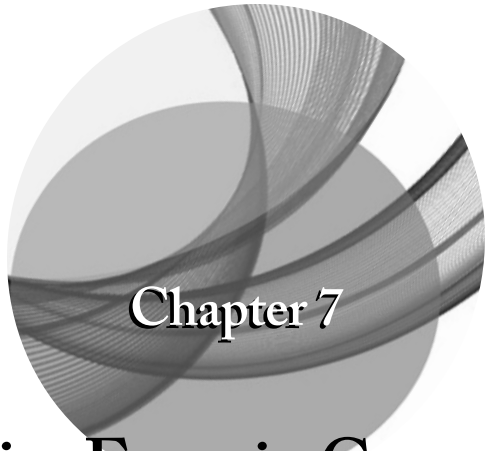
People tend to maintain their core beliefs about themselves, their world, and their future, so the primary focus of cognitive therapy is on cognitive change. Clients are helped to examine their core beliefs

and to begin a process of restructuring beliefs that are no longer functional. The restructuring process can occur by encouraging clients to gather and weigh the evidence in support of their beliefs. In cognitive therapy, clients are active, informed, and responsible for the direction of therapy and are partners in this enterprise. Efficiency is prized, and stress is placed on applying what is learned in therapy to coping with new problems in daily living.

Reflect on some of the key themes I've identified from the cognitive approaches and apply them to yourself. What are some of your basic convictions, core beliefs, and self-talk? Ask yourself where you acquired your basic beliefs and how they appear to be influencing the ways you feel and act today.

Take time to reflect on the following questions as a way of clarifying your thoughts on the cognitive focus.

- What are some of the key concepts and techniques from the cognitive behavior approaches that you most value in understanding yourself?
- How do you imagine it would be for you to be a client in cognitive behavior therapy?
- What key concepts from cognitive therapy fit well with your personal philosophy of counseling?
- What are some cognitive techniques that you would most want to use with your clients?
- How likely are you to employ between-sessions activities as a part of the therapy you provide? How could you collaborate with clients to create meaningful assignments?
- What are some of the main benefits of a cognitive focus?
- What are some potential limitations of a cognitive focus?



Chapter 7

Emotive Focus in Counseling

Paying attention to relational factors and to clients' emotions increases our understanding of our clients. I believe it is extremely important to facilitate an experiential process in clients rather than to immediately focus on modifying their cognitions or behaviors. Bonding in therapy is not facilitated by "thinking together" or by behavioral guidance. Very few people come to therapy saying that they have a thinking disorder or that they need to change a dysfunctional behavior. They come because they feel bad, and they want to feel good. If they can't feel good, they at least want to feel better (Rasmussen, 2010).

Empathy, the foundation of the counselor–client relationship, must be experienced and communicated emotionally to be effective. When the therapist is able to affirm clients' experiences, they feel safe enough to generate new emotional meanings. An empathically attuned and respectful relationship provides the support necessary for clients to allow themselves to experience a range of feelings that they might otherwise block. This then opens the door for clients to modify their patterns of thinking and acting.

Just as cognition is essential to an integrative counseling style, so is the emotional dimension. Too often—especially with brief therapy aimed at discovering solutions to problems as efficiently and quickly as possible—what clients are feeling is relegated to a secondary position. Even in brief therapy, it is possible to be open to exploring emotions. Clients need to have an opportunity to talk before the therapist confronts their mode of thinking or even their way of behaving. Often the best route to getting clients to examine their cognitions is by encouraging them to identify, express, and talk about what they are feeling. However, expressing feelings is not an end in itself. Once clients experience an

emotional catharsis, it is essential to work with the associated insights and cognitions underlying the emotional patterns.

Significant personal changes tend to come about when clients are taught how to transfer what they have learned in therapy sessions to everyday situations. Transferring this learning is not an automatic process. Therapists need to teach clients how to maintain these positive emotional and behavioral changes. If therapy is aimed solely at emotional release, the likely result will simply be an emotional experience. With little attention to cognitive or behavioral follow-up, clients are not likely to acquire a method for transferring their learning to a range of situations they will encounter in their daily life.

Experiential Approaches and the Role of Emotion in Therapy

A number of experiential and relationship-oriented therapeutic approaches emphasize the significance of emotion in the change process. Working with emotion has been recognized as a key aspect of successful therapy across therapeutic orientations (Pascual-Leone, Paivio, & Harrington, 2016). Some of these therapies include emotion-focused therapy, existential therapy, person-centered therapy, and Gestalt therapy.

Emotion-Focused Therapy

Leslie Greenberg (2017) is a prominent figure in the development of emotion-focused therapy (EFT), and the key concepts discussed here are based on his integrative approach. EFT emphasizes the importance of awareness, acceptance, and understanding of emotion and the visceral experience of emotion in creating psychotherapeutic change. In EFT, clients are assisted in identifying, experiencing, accepting, exploring, transforming, and managing their emotions. A premise of EFT is that we can change only when we accept ourselves as we are. Furthermore, emotional change is fundamental to lasting cognitive and behavioral change. The EFT practitioner actively works with emotions after the therapeutic alliance is created using a range of experiential techniques. Through these techniques, new narratives can be created that disrupt maladaptive past emotional schemas, which provides opportunities for positive emotional experiencing. A number of strategies in EFT are aimed at the goals of strengthening the self, regulating affect, and creating new meaning.

EFT is considered as an integrative therapy because it synthesizes aspects of person-centered therapy, Gestalt therapy, and existential therapy. Other therapeutic approaches are increasingly focusing on emotions as well. For example, both psychoanalytic and cognitive behavior approaches are giving more attention to the role of emotions and are rapidly assimilating many aspects of EFT. Greenberg (2017) points out that a strength of EFT is its emphasis on empirically supported treatments, a concept increasingly being stressed in graduate programs.

Existential Therapy

The existential approach provides a foundation for the integration of most other theories. Existential practitioners value the role of emotions in the therapy process. In existential therapy, attention is given to the client's immediate, ongoing experience, especially what is going on in the interaction between the therapist and the client. The client–therapist relationship is of paramount importance because the quality of this relationship offers a context for change. It is not the techniques a therapist uses that make a therapeutic difference; rather, it is the quality of the client–therapist relationship that heals emotional wounds. Instead of prizing therapeutic objectivity and professional distance, existential therapists value being fully present and strive to create caring relationships with clients. The existential therapist strives to "grasp the being" of the client; to be able to do this the therapist must be fully and genuinely present with the client. This therapeutic presence involves aspects of awareness, acceptance, availability, and expressiveness in both counselor and client. Presence is the hub around which evidence-based therapeutic practices revolve (Schneider & Krug, 2017). The existential therapist is a self-revealing *fellow traveler*, meaning that we are all in this together. As a fellow traveler along the same road as the client, the therapist strives to be authentic and genuine. The therapist's most valuable instrument is his or her own self as a vital dimension of the therapy relationship. From the existential therapy perspective, it is the *examined* therapeutic relation that heals (Yalom & Josselson, 2019).

Person-Centered Therapy

Like existential therapy, the person-centered approach stresses the client–therapist relationship and the therapist as a person; techniques are secondary to the therapist's attitudes. This approach minimizes directive techniques, interpretation, questioning, probing, and diagnosis. It maximizes active listening and hearing, empathic understanding, presence, reflection of feelings, and clarification. The person-centered approach focuses on the role of emotions, facilitating the expressing of emotions and following the direction of the client. Person-centered therapy is based on the assumption that the client has the capacity to resolve life's problems effectively without interpretation and direction from an expert therapist. This approach emphasizes fully experiencing the present moment, learning to accept oneself, and deciding on ways to change. More than the therapist or technique, it is the client who makes therapy work. Emphasis is given to understanding and accepting the client, which results in the creation of a safe place to explore feelings, thoughts, behaviors, and experiences.

Gestalt Therapy

Gestalt therapy is another experiential approach that focuses on emotion. In Gestalt therapy, technical expertise is important, but the therapeutic

engagement is key. The focus is not on the techniques employed by the therapist but on who the therapist is as a person and what the therapist is doing. Therapists create experiments within a context of the I/Thou dialog in a here-and-now framework. These experiments are the cornerstone of experiential learning. Gestalt therapists use active methods and personal engagement with clients to increase their awareness, freedom, and self-direction. The counselor assists clients in experiencing their emotions more fully and invites them to make their own interpretations. The therapist does not interpret for clients but focuses on the "what" and "how" of their behavior. Clients identify their own unfinished business from the past that is interfering with their present functioning by reexperiencing past situations as though they were happening at the present moment. Although the therapist suggests experiments, this is a collaborative process with full participation by the client. Gestalt experiments take many forms: setting up a dialog between a client and a significant person in his or her life; reliving a painful event; or carrying on a dialog between two conflicting aspects within an individual. Clients often engage in role playing, which tends to result in gaining increased self-awareness.

Becoming the Client: Experiencing Emotion-Focused Therapy

As I have asked you consistently to do, imagine yourself as the client now to grasp the power of emotion-focused therapy. I prefer to begin therapy by paying attention to what you are experiencing on an emotional level. I attend to how emotions are manifested in your body. I think the truth of many of your struggles can be found in what you are feeling and what your body is telling you. I ask, "What are you aware of at the moment and what do you want to do with that?" Noticing your emotional and bodily states can be significant routes to gaining awareness of what you are experiencing and doing. Without awareness, change is not possible.

Gestalt therapy experiments are designed to expand your awareness and to help you try out new modes of behavior. I create and use interventions, or experiments, to facilitate the exploration of material that emerges from our interactions in the session. Experiments enable you to become aware of aspects of experience that had previously been out of awareness. Within the safety of the therapeutic situation, you are given opportunities to "try on" a new behavior. The interventions I make are geared to your thoughts, feelings, and actions and are routes to supporting your self-exploration. Such interventions have the purpose of facilitating your self-understanding, not promoting my personal agenda as your therapist or to meet my needs. I tend to avoid using planned techniques or exercises as catalysts to open up feelings. Experiments are more powerful when they grow out of the phenomenological context of therapy and when they are chosen for a specific therapy situation.

As a way to demonstrate an experiential, emotionally focused approach, let me present you with some "problems" that I encourage you to "adopt" as my client. You tell me that at this moment you feel a sense of sadness. I ask you to say more and to describe how you experience this sadness. You respond, "I'm feeling sad when I think of how hard I try to get approval from everyone. I'm so caught up in getting you to like me that I forget what it is I want for myself. I feel right now like I felt so much around my parents—always trying to do what they wanted so they would think well of me, yet never feeling that I was able to get what I wanted." You are saying a great deal, and I encourage you to keep talking. As you speak, I direct your attention to what you are experiencing in your body. You state that your heart is heavy and that it feels broken. I suggest that you stay with this bodily feeling as much as possible and put some words to what it is like for you to talk at this time. I might even ask you to give your heart a voice and give expression to your "heavy and broken heart." Where we go next depends on what emerges in your moment-to-moment awareness. Influenced by emotion-focused therapy and Gestalt therapy, I follow the leads you provide and support you in your efforts to stay focused on the images, feelings, thoughts, and sensations that come to the surface. I encourage you to stay with *what is* as fully as possible.

Operating within the framework of the Gestalt approach, I ask you to bring any past or future concerns into the present and directly experience these concerns. Being in the present moment involves a transition between your past and your future. As a way to keep our work emotionally focused, I use experiments that are spontaneously created to fit your present situation. From the perspective of emotion-focused therapy, meaningful dialog often results from staying with the changing flow of your present-centered awareness. By following whatever it is that you are experiencing and showing you how to follow your own energy, you will gradually increase your awareness.

Operating in the spirit of Gestalt therapy to help you make contact with the present moment, I typically ask "what" and "how" questions and rarely ask "why" questions. To promote "now" awareness, I encourage a dialog in the present tense by asking, "What is happening now? What is going on now?" "What are you experiencing as you share your struggle? What is your awareness at this moment?" "How are you experiencing your sadness?" "As you talk of your heavy heart, what is this like for you? If your heart could express itself, what would that be like for you?"

Now imagine yourself in this scenario. You say that you sometimes feel totally inadequate and that you have a hard time liking yourself. I ask you to tell me more about what it feels like to be this way. I may ask you what you are experiencing in your body as you talk about feeling inadequate.

I do not offer you immediate reassurance or tell you that you are an adequate person who is likeable. Although reassuring feedback may make you feel good for a short time, it is doubtful that this feeling

would last very long. Your internal critic will not believe any positive feedback I might offer you—at least not if I attempt to reassure you too soon. I am more interested in assisting you in exploring your feelings. I also want to give you a chance to express what you often do not say. If you are able to talk fully about feeling inadequate and having difficulty liking yourself, you stand a better chance of arriving at your own solutions to the problems you face. I strive to create a therapeutic climate that will enable you to sort out your thoughts and feelings, which will lead to making better decisions and changes. Mere reassurance and advice do not facilitate self-examination, but a willingness to listen to you can encourage you to share what you are feeling in the moment, to engage in significant self-disclosure, and to explore your struggles.

Even though you may have stayed with some intense feelings and allowed yourself to experience the power of your feelings in your body, you might become uncomfortable and want to distance yourself from some feelings that arise as you talk with me. You may express fears about getting involved with me and display reluctance toward any attempt at delving into deeply personal concerns. You tell me, "If I get deep into my feelings, I'm afraid that I'll get stuck and won't be able to get out of it. If I let myself feel that, I'm afraid I'll get out of control. There is a part of me that wants to experience my emotions, yet another part of me wants to get into my head and keep control." Whether or not your fears are expressed, they tend to give rise to some ambivalence: The desire to reveal yourself is balanced by the reluctance to expose yourself. To work with your ambivalence, I suggest a role play in which we each take one of these two sides of yourself.

You will not be able to change an emotion unless you first allow yourself to experience the emotion. If you avoid dealing with your feelings of worthlessness, insecurity, shame, and hopelessness, you cannot reduce these distressing feelings and change them. Greenberg (2017) maintains that an effective way to manage emotions is to help clients become aware of what they are feeling, to express their feelings, and to decide what to do when emotions arise.

Gestalt therapy and psychodrama often employ an *empty chair* or *two-chair technique*. This is another way to act out your present feelings in the therapy session. Having a dialog with various aspects within yourself, or between yourself and another individual, is particularly useful when you are feeling ambivalent about a direction to pursue or when you are in some kind of conflict situation. Rather than talking about a situation, I suggest that you make it present by actually bringing to life a conflict you are experiencing. You tell me that you often feel very young and awkward when you try to relate your accomplishments to your mother. I say to you: "So, sitting in the chair you are in now, become that young and awkward person and talk to this other (empty) chair—to your mother. What would you like your mother to know about your accomplishments? What are you saying to your mother in this scene?" Next I ask you to switch chairs and become your mother

and reply the way you expect her to—or reply in the manner that you hope she would. By bringing emotionally laden material or a conflict into the present through this two-chair exercise, you and I get a better understanding of how you struggle with relationships or with feeling young and foolish. Emotion-focused methods adapted from Gestalt therapy and psychodrama tend to facilitate a deeper understanding and insight, as well as a greater emotional connection to your words.

An alternative way I might work with your feelings about your mother is to suggest that you participate in a *soliloquy*, another psychodrama technique. Imagine yourself in a place where you can think out loud (soliloquize) and say what you are thinking and feeling. This could be a useful follow-up intervention to the two-chair dialog between you and your mother. This intervention facilitates clarification and an open expression of what you may be experiencing internally but not expressing verbally.

Future projection, another psychodrama technique, is designed to help you express and clarify concerns you have about your future. An anticipated event is brought into the present moment and acted out. In this case, you enact a version of the way you hope a situation will ideally unfold between you and your mother. Of course, you could enact a dreaded fear of tomorrow with the most horrible outcome. For instance, your mother might tell you that she does not expect you to ever accomplish anything of any merit. Once you clarify your hopes for a particular outcome, you are in a better position to take specific steps that will enable you to achieve the future you desire.

You may be wondering why it is essential to give primary attention to the role of emotions in our efforts to bring about change. Why is emotional release so important? My answer relates to the connection between physical health and emotional health. Unexpressed feelings often result in physical symptoms such as chronic headaches, stomach pains, muscle tension, and high blood pressure. If we repress our anger, we pay a price for this. If we are chronically under intense stress, our body may react with a host of illnesses. If we hold in our emotional pain, we expend a great deal of energy, and this takes a toll on our body. Clearly, there are links between repressing emotions and symptoms such as headaches, stomach pains, asthma, backaches, arthritis, and muscular tension. It takes a great deal of energy to deny emotional pain, and denied emotions often are expressed in the body. For example, if we are excessively blocked from our grief after a significant loss or death of someone we love, the result can be a chronic sense of pain. This unexpressed pain is likely to prevent us from being emotionally open to experiencing the fullness of other relationships. Experiential techniques borrowed from psychodrama such as empty chair work, soliloquy, future projection, role reversal, and role playing can help to heal an emotional wound. When therapy clients do express their stored-up pain and other unexpressed feelings, they typically report a tremendous physical and emotional release, known as *catharsis*. Through an emotional release of pent-up feelings, the healing process

is facilitated. Although catharsis is a natural part of many of the experiential or emotionally focused therapies (especially emotion-focused therapy, existential therapy, person-centered therapy, Gestalt therapy, and psychodrama), the emotional release is not a goal in itself. Rather, it is an indicator of emotional expansion and integration.

The experiential therapies frequently involve some form of catharsis. Although there is value in catharsis, my experience with therapy groups has taught me time and again how essential it is to provide a context in which clients can come to an understanding of how their bottled-up emotions have affected both them and their relationships. It is important to find links between emotional release and new levels of cognitive and emotional awareness. Insight, or gaining an increased awareness of a problem situation, often follows the process of an emotional release. Insight is the cognitive shift that connects the awareness of the various emotional experiences with some meaningful narrative or growing understanding. Insights that we acquire in our therapy can add a degree of understanding to the catharsis and can allow us to begin the essential process of gaining control over ways of expressing and managing our feelings. Further, our insights may lead to the awareness that we no longer have to continue living as we did before. Integrating insights and developing and practicing more effective behaviors are essential components in the change process. Collaboratively designing homework toward the end of a therapy session is often an effective way to translate insight into action, which can lead to the changes we desire.

For a more detailed discussion of other techniques I might employ with you in an emotionally focused manner, refer to Corey (2017, chap. 8). A useful resource for the topics addressed in this chapter is Greenberg (2017).

Working With Elaine in Identifying and Exploring Feelings

Elaine, a 77-year-old widow who has two children and five grandchildren, has sought counseling for the first time in her life. Although she has always prided herself on being "the rock" in her family, she has been having a hard time coping with her recent diagnosis of breast cancer. Because her family members always seek her guidance in times of crisis, she believes that they will fall apart emotionally when she discloses that she has cancer. She is aware that keeping her secret to herself cannot last because she will need her family's support during her chemotherapy and radiation treatments. She has been having anxiety attacks lately, which led to her decision to heed the advice of her oncologist who recommended counseling.

Counseling is a new experience for Elaine, one with which she is not entirely comfortable. Elaine believes that dwelling on one's feelings is counterproductive and only makes matters worse. She

finds comfort in her role of being the person who others lean on, and she is having difficulty coming to terms with her new reality of needing to depend on others for support. Because she is not used to expressing her needs to others, Elaine has difficulty talking about her emotions, and she tends to feel guilty when she does so, even if only minimally.

The person-centered approach and the other experiential therapies and relationship-oriented therapies stress that one of the first stages in the therapy process involves identifying, clarifying, and learning how to express feelings. I encourage Elaine to talk about any feelings she is aware of, especially those that are a source of difficulty. These feelings may be vague and difficult to identify at first. To facilitate Elaine's experiential emotional process, I must establish a sense of safety in the therapy situation. If I can create a nonjudgmental and accepting environment, I hope that eventually Elaine will be more able to accept herself and all of her feelings. During the early stages of our sessions, I rely on listening with understanding. I encourage her to talk about what most concerns her about receiving the cancer diagnosis and anything else that is on her mind.

If I can really hear Elaine's deeper messages, some of which may not be fully clear to her, I can respond to her in a way that lets her know that I have some appreciation for what it is like in her world. (Grasping the subjective or experiential world of clients is a key concept of many theoretical orientations, including existential therapy, person-centered therapy, Gestalt therapy, and Adlerian therapy.) I need to do more than merely reflect what I hear her saying; I need to share with her my reactions as I listen to her, and perhaps how I am being affected by her in the session. The more I am able to communicate that I understand and accept the feelings she has, the less need she has to deny or deflect her feelings. As a result, her capacity for clearly identifying what she is feeling at any moment gradually increases.

There is a great deal of value in letting Elaine tell her story in the way she chooses. The way she walks into the office, her gestures, her style of speech, the details she chooses to go into, and what she decides to relate and what she withholds provide me with clues to her world. By being attentive and tracking what she is doing in the moment, I am getting a larger picture of her world. Certainly, I am not getting the entire story, but it is possible to grasp significant samples of her story. At this time, she is giving me the "short version" of her life story. I will inquire about her presentation as a way to understand the personal meanings of her thoughts and feelings pertaining to events in her life. My interventions are aimed at checking to make sure that I am understanding her accurately and conveying to her my understanding of her situation. Here I am influenced by concepts from the person-centered approach such as

listening with understanding, presence, and focusing on the subjective aspects of Elaine's experiencing.

If I do too much structuring too soon or if I am too directive, I will interfere with Elaine's typical style of presenting herself. So, at this early stage of counseling, I agree with the person-centered therapists who stress attending and listening on the counselor's part and focusing on the productive use of silence. Although I am not inclined to promote long silences early in counseling, there is value in not jumping in too soon when silences occur. Instead of coming to the rescue, it is better to explore the meanings of the silence.

To help Elaine express and explore her feelings, I draw heavily on Gestalt therapy experiments. I teach her to pay attention to what is emerging in her awareness. I am guided by the shifts in her awareness, and together we create experiments that grow out of her present-centered awareness. The emphasis is on our dialog and the quality of contact we are able to make in the therapy session. Because she has yet to explore her own feelings about having cancer or her fears about how devastated her children and grandchildren will be to get this news, such concerns surface in her therapy.

I invite Elaine to experience her feelings fully and bring whatever she is feeling into the present by reliving an event surrounding these feelings rather than by merely reporting outside events or long-standing themes in her life. For instance, if she says that she is worried when she thinks about how people in her family typically respond to stressful news, I ask that she stay with the worry. If she can, I encourage her to share how she is experiencing her worry in the moment, perhaps by expressing what she is aware of in her body. Before she can change her feelings, she must allow herself to fully experience these feelings. The experiential therapies give me valuable tools for guiding her to the expression of feelings.

In this session, I have made many different interventions, one of which is to ask Elaine to talk to me as if I am one of her children or grandchildren. I ask her to stay with whatever she is experiencing, paying particular attention to her body and to the emotions welling up in her, doing her best to express these emerging feelings. Role-playing Elaine's family members gives me an opportunity to observe how she presents herself to them and to get some sense of how they might receive her verbal and nonverbal messages. Elaine's experiments with role playing are likely to trigger feelings surrounding a specific situation with her children or grandchildren, which brings her work to a deeper level than if she had merely reported a situation.

The Gestalt process for creating experiments is a powerful and effective way to connect Elaine to her emotions. Although Gestalt therapy offers rich pathways to Elaine's emotional experiencing, this approach taps whatever is in her awareness at the moment, and in that sense, it is truly integrative.

Reflection Questions

Reflect on specific aspects of Elaine's case that you find particularly interesting. Consider these questions:

- What theory (or theories) would you find especially useful in assisting Elaine in exploring her feelings?
- How would you go about creating a sense of safety that would enable Elaine to talk about aspects of her life that she typically does not share with others?
- In my work with Elaine, I draw heavily on Gestalt therapy experiments. What are your reactions to this?

Working With Andrew in Identifying and Exploring Feelings

Andrew is a 19-year-old, gay Latino. He is an elite athlete who recently competed in the Olympic trials. He seeks counseling because he failed to make the Olympic team and has been feeling depressed and unmotivated. Andrew reports that he is not out to his family, coaches, or other athletes. He further reports that being a successful athlete has been the only thing that has brought him any sense of accomplishment in his life. His affect in our first session is flat, and he describes his recent challenges with athletic performance and keeping his identity hidden as if he is talking about someone else's life instead of his own.

In our second session, Andrew focuses on his training regime and his relationship with his coach, who has become a father figure for him. When I ask Andrew about his family, his voice becomes distant, and he describes his history of losing his father at a young age to incarceration. His mother would not allow young Andrew to visit his father. When she unexpectedly died, Andrew and his older sister were placed in foster care. He unemotionally disclosed that he and his sister were separated in the foster system. I call attention to the distant manner with which Andrew described what must have been a terrible and frightening ordeal. Andrew responds dryly, "We all have our baggage." Andrew goes on to mention how he met his coach when he was in middle school and that his coach eventually adopted him. Andrew becomes more expressive as he describes the ways in which he is grateful to his coach for "rescuing" him and how all he ever wanted to do was to make his coach proud by making the Olympic team. Andrew fights back tears as he describes the look of disappointment on his coach's face when he saw Andrew lose the Olympic trial. He says, "The only way I have to pay back what he has done for me is to make the team and bring home a medal."

As a first step, I assist Andrew in experiencing some of the feelings he has long denied. We explored extensively the ways in which Andrew, from an early age, started to withhold his true self from all of those around him for fear of rejection and being cast back into the

foster system, which was particularly the case regarding his sexual identity. In the hypermasculine world of elite athletics, the only way Andrew could express gratitude and love for his coach was to win his competitions. The only messages of acceptance and praise he received were times when he excelled at his sport. In many ways, Andrew constricted and closed off his emotional life in order to survive without his family. This made him a strong athlete, but it failed to help him cope when he was defeated in his greatest athletic challenge.

As a way to help Andrew identify and experience his feelings, I use letter writing and role playing with Andrew with positive results. This work with Andrew illustrates a key principle of emotion-focused therapy: to change a feeling, it is necessary to feel the feeling. Simply talking about emotions and problems will not bring about emotional transformation. I draw upon the core therapeutic conditions of the person-centered approach in supporting Andrew in opening himself to what is potentially vulnerable emotional work.

As a homework assignment, Andrew agrees to write a letter to his coach, which he brings with him to the next session. I ask him what it was like for him to write the letter, and he describes the challenges and is pleased that he completed the letter. I then ask Andrew if he is willing to read the letter to me, which he does with a lot of emotion. As a follow-up to him reading his letter and talking about the emotion that he was experiencing in this session while doing so, I invite him to participate in a role-playing situation in which he talks to me as if I am his coach. When Andrew says that he does not know where to begin, I suggest that he tell me (as his coach) how difficult it is for him to know what to say to me. He begins with this, and continues by telling me of many of the ways that he feels he has let down his coach and has not measured up as a prize athlete.

Andrew continued for a time talking in the past tense to me as his coach. When I suggest that he shift to talking to me in the present moment, many intense feelings surface. Andrew gives up some of his control and expresses his disappointment in letting his coach down. After this symbolic role play, we processed what this was like for him and what he still wants from his coach, which accessed even deeper feelings around trying so hard to win his coach's recognition and acceptance. I then suggest that we experiment with a reverse role play in which Andrew "becomes his coach" and enacts both his feared and his hoped-for responses from his coach. He carries out this Gestalt experiment, which deepens his emotional experiencing. He gains increased insight about what he wants to hear from his coach. In describing what it was like for him to participate in this experiment, Andrew describes reactions in his body, such as tearing up, feeling flushed, and a slight shaking of his hands, which we then talk about. He appreciates the opportunity to put words to what he is feeling in his body.

As another homework for Andrew, I suggest that he write a second letter to his coach expressing some of the feelings that just came up for

him in this session. I stress to Andrew that he is not to present this letter to his coach—at least not now. Writing the letter may trigger memories, and he may experience further emotional release. By writing the letter, Andrew is likely to gain clarity and a new perspective on how he wants to approach his coach in real life. This can be a therapeutic exercise because feelings are released that previously were denied.

At our next session, I ask Andrew if he wrote this follow-up letter, and, if he did, what it was like for him to do so. What was he feeling and thinking as he was writing to his coach? How was he affected when he read the letter later? Is there anything that he wants to share with me? The direction of our session will depend on his response, which will provide clues to where we need to go next.

Reflection Questions

Reflect on the emotional aspects of working with Andrew. Consider these questions:

- Would you welcome accepting Andrew as a client? Why or why not?
- In working with Andrew, I suggested several homework assignments involving writing a letter. What are your reactions to this intervention?
- I introduce several role-play experiments with Andrew. How comfortable would you be in asking Andrew to engage in role playing? How would you react if you suggested he engage in a role play and he flatly refused?

Concluding Comments

I have emphasized the role of identifying and expressing feelings in the therapeutic process in this chapter. Let me present a few guidelines for doing experiential, emotionally focused work with your clients. It is important for you to be able to experience your own emotions and to express them in a healthy way. Your ability to be emotionally present for your clients hinges on how grounded you are yourself and whether you have access to your own emotions. If you are frightened by your feelings, you won't be able to facilitate a process with clients that enables them to express and work through their feelings. If you are scared of your own anger or of anger directed toward you, it will be quite difficult for you to assist clients in dealing constructively with their anger. If you have denied your own pain over significant losses, you will not be able to be present for clients as they open painful subjects in their therapy. If you are extremely uncomfortable with conflict, it is unlikely that you will be instrumental in helping clients stay with a conflict long enough to bring resolution to a situation. If you are afraid to cry or lose control,

how can you expect to deal with clients who keep their tears inside lest they get out of control? If your emotions make you uncomfortable, chances are that you will find some way to divert your clients' attention away from intense feelings. If a client is afraid of becoming engulfed in depression and you are frightened of depression, how can you therapeutically engage this individual?

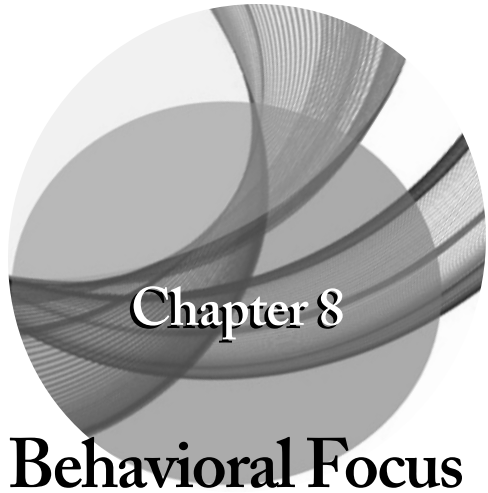
If you are emotionally present for your clients, their stories are bound to affect you personally. Establish clear boundaries so you do not take onto yourself the pain your clients express. When the multiple stories of trauma that clients bring to therapy mirror a therapist's own personal struggles too closely, *empathy fatigue* may result (Stebnicki, 2008). Empathy fatigue shares some similarities with other fatigue syndromes such as compassion fatigue, secondary traumatic stress, vicarious traumatization, and burnout. The symptoms of empathy fatigue are common to professionals who treat survivors of stressful and traumatic events; those who treat people with mood, anxiety, and stress-related disorders; and those who work in vocational settings with people with mental and physical disabilities. Stebnicki believes that being mindful of our empathy fatigue buttons is critical for maintaining our physical, emotional, and spiritual well-being. Counselors who do not take steps to remedy burnout or make changes in how they deal with stress are likely to experience impairment. Self-care is an ethical obligation for those of us in the counseling profession because it protects us from empathy fatigue, burnout, impairment, and other conditions that can detract from our effectiveness as counselors (Corey, Muratori, Austin, & Austin, 2018).

To assist your clients, you need to be able to identify, express, and manage your own emotions. As clients relive painful memories attached to events, you will need to care about them without getting lost in their emotional pain. If you are easily triggered by the emotionally laden stories of your clients, you may not have what it takes to help them work through painful scenarios. As your clients affect you emotionally, be aware of the feelings this evokes in you. Recognize your own emotional reactions and be willing to explore them in your own supervision or therapy sessions. Over time, experienced counselors learn ways to use their emotional reactions to clients as helpful forms of information about the client, the process of therapy, and themselves.

I have placed a great deal of importance on assisting clients in the process of identifying, experiencing, and exploring their emotions. This does not imply that you should insist that your clients always deal with their emotions. Begin where your clients are, and determine what would be most useful for them at that particular time. If you push too hard for emotional expression, clients are likely to become uncomfortable and defensive. Keep in mind that the emotional work needs to be connected with what clients are thinking and doing. It may serve little therapeutic purpose to elicit feelings from clients if this is done mainly for dramatic effect.

After reading this chapter on the emotive focus in counseling, take time to reflect on the following questions as a way of clarifying your thoughts on the emotive focus.

- What are some of the main benefits and limitations of an emotive focus in counseling?
- What are some ways to integrate cognitive and behavioral work in assisting clients in processing an emotional experience?
- If you were a client in counseling, what value would you place on expressing and exploring your emotions?
- What are your reactions to this statement? "Often the best route to getting clients to examine their cognitions is by encouraging them to identify, express, and talk about what they are feeling."



Chapter 8

Behavioral Focus in Counseling

No single theory undergirds the practice of contemporary behavior therapy, nor does this approach take a single form in practice. Behavior therapies include traditional behavior therapy, cognitive behavior therapy (see Chapter 6), and a third wave of behavior therapies focused on mindfulness and acceptance. A panel of psychotherapy experts predict an increase in the next decade in the popularity of integrative therapies such as mindfulness, cognitive behavior, and multicultural approaches (Norcross et al., 2013).

My integrated behavioral focus includes many behavior therapy methods, along with other action-oriented therapies, such as multimodal therapy, rational emotive behavior therapy, cognitive therapy, reality therapy, solution-focused brief therapy, and the mindfulness and acceptance approaches. From an integrative perspective, I address some basic concepts that most of these theoretical orientations share and describe a range of behavioral techniques associated with these other action-oriented models.

The behavioral perspective stands in contrast to the relationship-oriented and experiential approaches described in previous chapters. Experiential approaches place considerable emphasis on clients achieving insight into their problems as a prerequisite for change. Behavior therapists operate on the premise that changes in behavior can occur prior to understanding oneself and that behavioral changes may well lead to an increased level of self-understanding. I draw on a wide variety of behavioral techniques derived from social learning theory, such as reinforcement, modeling, cognitive restructuring, desensitization, relaxation training, self-control procedures, coaching, behavioral rehearsal, relapse prevention, mindfulness exercises, and

acceptance-based procedures. Indeed, behavioral interventions can be incorporated into many of the relationship-oriented therapies, and by doing so clients can consolidate their learning and continue to solve new problems more effectively.

The behavior-oriented therapies assist clients in exploring how past and present thoughts, feelings, and behaviors have worked for them, or have been problematic for them. The action-oriented therapies provide methods that are measurable, plan-specific, and realistic. A key goal of behavior therapy is for clients to learn basic skills that enable them to become their own therapists. Clients are in an active partnership in the therapy process, including establishing therapy goals and setting the agenda for each session (Antony, 2019).

When the emphasis in therapy is on behavioral change, there must be some guidelines for what constitutes a positive change. Some positive changes result in symptom reductions and are obvious: Calmness is better than anxiety; calmness is also better than anger. Happiness is better than depression, even though each person's happiness is likely to be uniquely defined. Some behavioral goals are supported by the sciences associated with nutrition, exercise, health care, and mental wellness. The guidelines for better living discussed in Chapter 3 can also act as a framework for specifying positive changes in behavior. The basic skills in forming friendships, being effective in work, acting responsibly, learning to love and be loved, and raising the next generation, to name a few, can all be specified in behavioral terms in an ethical and operational manner. Learning and implementation of these skills can be assessed and measured. Positive change implies that we are supporting change that is positive not only for ourselves but for all those with whom we come in contact. Positive change is shown when we enact behaviors that increase our sense of belonging and that connect us to others in a manner that others appreciate.

Benefits and Limitations of a Behavioral Focus

The behaviorally oriented models emphasize specifics and the need for a systematic application of therapeutic techniques. Clients often make global statements such as "I feel unloved; life has no meaning." Behavioral approaches aim toward greater specificity so that focused therapy can proceed. Behavior therapists assume an active and directive role in the assessment and treatment process and might reply like this to such a global statement: "Who specifically is not loving you? What is going on in your life to bring about this meaninglessness? What are some specific things you might be doing that contribute to the state you are in? What specific behaviors, thoughts, or feelings would you most like to change?"

One of the major benefits of a behavioral focus is the wide variety of specific techniques available in counseling diverse client populations; these techniques have the advantage of empirical support for their effectiveness. For example, mindfulness, a fundamental aspect

of the third wave behavioral approaches, has been the subject of considerable research and has been found to enhance psychological well-being and positive functioning and to reduce negative functioning (Pantaleno & Sisti, 2018).

The behavioral approach assumes that change can take place without insight into underlying dynamics. The behavioral emphasis is on *doing*, as opposed to merely talking about problems and gathering insights, and clients formulate plans of action for changing behavior. Behavior therapists believe that insight and increased self-understanding will flow from the behavioral changes clients make. Behavioral interventions have been successful in helping people stay with an exercise plan, manage stress, and treat depression, anxiety, and hypertension. In my view, behavior therapy is at its best when it is used in an integrative manner. Clients who look for action plans and behavioral change are likely to cooperate with behavioral approaches because they include concrete methods for dealing with problems of living. A behavioral focus fits well with short-term counseling, which is a clear advantage.

Behavioral practitioners state concepts and procedures explicitly, test them empirically, and revise them continually. Behavioral approaches are grounded on evidence-based practices; they are the most researched and best empirically supported therapies (Antony, 2019). Assessment and treatment occur simultaneously. The specific characteristics of behavior therapy include (a) conducting a behavioral assessment, (b) precisely spelling out collaborative treatment goals, (c) formulating a specific treatment procedure appropriate to a particular problem, and (d) objectively evaluating the outcomes of therapy. These characteristics can be incorporated into other therapeutic approaches, which makes behavior therapy an integrative perspective.

Behavioral practitioners specify problems, focus on key areas, and teach specific cognitive and behavioral skills. When using a behavioral perspective, it is crucial to listen carefully to your clients and allow them to express and explore their feelings and concerns before implementing a treatment plan. Today, most behavioral practitioners stress the value of establishing a collaborative working relationship with clients; they contend that active listening, respect, warmth, accurate empathy, positive regard, authenticity, immediacy, openness, permissiveness, and acceptance are necessary, *but not sufficient*, conditions for producing change in clients' behavior (Beck & Weishaar, 2019; Fishman, 2016; Kazantzis et al., 2017; Matu, 2018). The client–therapist relationship is a foundation on which behavioral strategies are built to help clients make the changes they want. If you are too eager to work toward resolving problems, you may pay little attention to exploring feelings. There are pitfalls in focusing too rigidly on clients' presenting problems and missing the deeper message. Thus, if you are teaching clients assertion skills that they can use in a job interview, remain open to addressing their thoughts (self-talk) associated with going to the interview as well as their emotional reactions (anxiety).

Multimodal Therapy: Assessment and Therapy Goals

Multimodal therapy (MMT) is a comprehensive, systematic, brief, empirically based, holistic approach to behavior therapy originated and developed by the late Arnold Lazarus (1997a, 1997b, 2005, 2006, 2008), a key pioneer in clinical behavior therapy. Multimodal therapy is an outgrowth of behavior therapy that is grounded in social cognitive learning theory. The assessment process is multimodal, yet the cognitive behavior treatment draws upon empirically supported methods. It is an open system that encourages *technical eclecticism* in that it applies diverse behavioral techniques to a wide range of problems. This approach serves as a major link between some behavioral principles and the cognitive behavioral approach that has largely replaced traditional behavioral therapy. The main goal of MMT is to create a specific treatment plan for each client based on a comprehensive assessment of the client's presenting concerns. MMT uses a wide variety of techniques drawn from various theoretical orientations, and these techniques are not anchored to the theories from which they originated (Kelley, 2018). Therapists assess what works best for each client, under what circumstances, and in what context.

Assessment is a crucial step in using a behavioral approach to therapeutic change, and MMT involves a specific and comprehensive assessment across seven modalities of human functioning, all of which are addressed in the treatment process (Kelley, 2018). The BASIC I.D. is the cognitive map that provides systematic attention to each aspect of human functioning. Let's take a closer look at each of the modalities in the BASIC I.D. (Lazarus, 2005).

1. **B***ehavior*. This modality refers primarily to overt behaviors, including acts, habits, and reactions that are observable and measurable. Some questions asked are: "What would you like to change?" "What would you like to start doing?" "What would you like to stop doing?"
2. **A***ffect*. This dimension refers to emotions, moods, and strong feelings. Questions asked include: "What emotions do you experience most often?" "What emotions are problematic for you?"
3. **S***ensation*. This area refers to the five basic senses of touch, taste, smell, sight, and hearing. A typical question asked is: "Do you suffer from unpleasant sensations, such as pains, aches, dizziness, and so forth?"
4. **I***magery*. This area pertains to ways in which we picture ourselves, and it includes memories, dreams, and fantasies. A few questions asked are: "How do you see yourself now?" "How would you like to be able to see yourself in the future?"
5. **C***ognition*. This modality refers to insights, ideas, opinions, self-talk, and judgments that constitute our fundamental values, attitudes, and beliefs. Questions include: "What are the values and beliefs you most cherish?" "What are some negative things you say to yourself?" "What are the main 'shoulds,' 'oughts,' and 'musts' in your life?" "How do they impede effective living?"

6. **I***nterpersonal relationships*. This area refers to interactions with other people. Examples of questions include: "What do you expect from the significant people in your life?" "What do they expect from you?" "Are there any relationships with others that you hope to change?"
7. **D***rugs/biology*. This modality includes more than drugs; it takes into consideration your nutritional habits and exercise patterns. Typical questions are: "Do you have any concerns about your health?" "Do you take any prescribed drugs?" "What are your habits pertaining to diet, exercise, and physical fitness?"

Once this initial assessment is complete, more focused discussions identify problem areas. This leads to identification of therapy goals, the first step in a treatment plan. Not only is multimodal therapy useful from a treatment perspective, but it provides an excellent context for doing a comprehensive assessment that can inform treatment. Multimodal methods enable the counselor to challenge self-defeating beliefs, offer constructive feedback, and provide positive reinforcement. Multimodal therapists function actively and in a directive manner, providing information and instruction. They are constantly adjusting their techniques to achieve the client's goals in therapy. They make a careful attempt to determine precisely what relationship and what treatment strategies will work best with each client and under which particular circumstances. Because individuals are troubled by a variety of specific problems, a multitude of treatment strategies and different therapeutic styles are used to bring about change. Some frequently used techniques in MMT in each of the seven modalities have been identified by Kelley (2018):

1. *Behavior*: behavior rehearsal, modeling, self-monitoring, acting as if, role play
2. *Affect*: anger management training, journaling, bibliotherapy, relaxation training, imagery
3. *Sensation*: meditation, mindfulness training, relaxation training, yoga, massage therapy, anxiety management training
4. *Imagery*: future-rehearsal imagery, positive imagery, guided imagery, time projection
5. *Cognition*: self-instructional training, positive self-talk, reframing, cognitive restructuring, skill training, between-sessions assignments
6. *Interpersonal relationships*: communication training, social skills training, role reversal, bibliotherapy, conflict resolution strategies
7. *Drugs/biology*: healthy eating habits, weight control, sleep habits

MMT has been applied to many different problems, to a range of client populations, and focuses on evidence-based interventions. For further discussion of multimodal therapy, see Corey (2013, chap., 7), Kelley (2018), Lazarus (2005, 2006), and Sharf (2016, chap. 16).

The Third Wave of Behavior Therapy

Mindfulness and Acceptance in Psychotherapy

The third wave of behavior therapy emphasizes considerations that were considered off limits for behavior therapists until recently, including mindfulness and acceptance approaches.

Mindfulness is "the awareness that emerges through having attention on purpose, in the present moment, and nonjudgmentally, to the unfolding of experience moment by moment" (Kabat-Zinn, 2003, p. 145). In mindfulness practice, clients train themselves to intentionally focus on their "present experience with acceptance" (Siegel, 2010, p. 27) and develop an attitude of curiosity and compassion toward present experience. The essence of mindfulness is becoming aware of our mind from one moment to the next, with gentle acceptance. Many therapeutic approaches are incorporating mindfulness and meditation in the counseling process, and this trend seems likely to continue.

Acceptance involves receiving one's present experience without judgment or preference, but with curiosity and kindness, and striving for full awareness of the present moment without efforts to change or avoid these experiences (Germer, 2013). Acceptance is an alternate way of responding to our internal experience. It is not viewed as a strategy to deal with problems; rather, it is seen as a way of being fully in the world (Follette & Hazlett-Stevens, 2016). Mindfulness and acceptance approaches are also good avenues for the integration of spirituality in counseling.

Recent developments in the cognitive behavioral tradition include four major approaches: (a) dialectical behavior therapy (DBT), which has become a recognized treatment for borderline personality disorder (Linehan, 1993a, 1993b, 2015); (b) mindfulness-based stress reduction (MBSR), an 8- to 10-week group program that applies mindfulness techniques to coping with stress and promoting physical and psychological health (Kabat-Zinn, 1990, 2003); (c) mindfulness-based cognitive therapy (MBCT), aimed primarily at treating depression (Segal, Williams, & Teasdale, 2013); and (d) acceptance and commitment therapy (ACT), which encourages clients to accept unpleasant sensations rather than attempting to control or change them (Hayes & Lillis, 2012; Hayes, Strosahl, & Wilson, 2011). All four of these approaches use strategies that have been subjected to empirical scrutiny, a hallmark of the behavioral tradition. The mindfulness-based approaches have a rich empirically based research tradition of positive outcomes (Pantaleno & Sisti, 2018).

Dialectical Behavior Therapy

Dialectical behavior therapy was originally developed to treat chronically suicidal individuals diagnosed with borderline personality disorder, and it is now considered the "gold-standard treatment for adults

with borderline personality disorder, which is primarily characterized by severe emotion dysregulation" (Kelly & Robinson, 2018, p. 237). Developed by Marsha Linehan (1993a, 1993b, 2015), who wanted to alleviate emotional suffering for those who were considering suicide, DBT has been proven effective in treating a wide range of disorders, including anxiety disorders, depression, substance use disorder, post-traumatic stress disorder, eating disorders, and nonsuicidal self-injury (Linehan, 2015).

DBT emphasizes the importance of the psychotherapeutic relationship, validation and acceptance of the client, the etiologic importance of the client having experienced an "invalidating environment" as a child, and the confrontation of resistance. DBT teaches clients who have problems with emotional regulation to recognize and accept the existence of simultaneous, opposing forces. By acknowledging this fundamental dialectic relationship—such as not wanting to engage in a certain behavior, yet knowing they have to engage in the behavior if they want to achieve a desired goal—clients can learn to integrate the opposing notions of acceptance and change, and the therapist can teach clients how to regulate their emotions and behaviors.

DBT is an empirically supported intervention that employs behavioral and cognitive behavioral techniques, including a form of exposure therapy in which the client learns to tolerate painful emotions without enacting self-destructive behaviors. DBT draws upon Zen teachings and practices to integrate mindfulness and acceptance-based techniques in therapy (Kuo & Fitzpatrick, 2015). Some of the Zen Buddhist principles and practices include being aware of the present moment, seeing reality without distortion, accepting reality without judgment, letting go of attachments that result in suffering, developing a greater degree of acceptance of self and others, and entering fully into present activities without separating oneself from ongoing events and interactions (Robins & Rosenthal, 2011).

Dialectical behavior therapists establish a structured, predictable therapeutic environment. Building a strong therapeutic alliance is key, and the relationship between client and therapist is both a vehicle for working with the client and the therapy in and of itself (Linehan, 1993a). A main general goal of DBT is to decrease ineffective coping behaviors and increase effective behaviors through skills training and generalizing these skills to daily life. When clients begin in DBT, they are asked to identify some specific goals in life that would make life worth living (Kelly & Robinson, 2018). Therapists assist clients in using whatever skills they possess to navigate crises more effectively and to address problem behaviors (Robins & Rosenthal, 2011). *Mindfulness* is a fundamental skill in DBT that enables individuals to be aware of and accept the world as it is and to avoid evaluation and judgment. Through mindfulness, clients learn to embrace and tolerate the intense emotions they experience when facing distressing situations. Clients are encouraged to accept and experience emotions as they are rather

than attempting to change them. Acceptance facilitates progress by allowing the client to simply be in the moment as opposed to being stuck in the past or worrying about the future (Kelly & Robinson, 2018). DBT puts a premium on *radical acceptance*, which requires completely accepting oneself as is.

Mindfulness-Based Stress Reduction

Jon Kabat-Zinn, at the University of Massachusetts, developed MBSR in 1979 to see if it was possible to create a training program to relieve medical patients of stress, pain, illness, and other forms of suffering. The 8-week structured group program trains people in mindfulness meditation. The essence of MBSR is realizing that much of our distress and suffering results from continually wanting things to be different than they actually are. MBSR assists people in learning how to live more fully in the present rather than ruminating about the past or being overly concerned about the future. Mindfulness meditation clears the mind and calms the body, which enables individuals to focus on here-and-now awareness in a nonjudgmental way. The focus is on *what is* rather than on *what if*. The approach adopted in the MBSR program is to develop the capacity for sustained directed attention through formal and informal meditation practice. In formal practice, skills taught include sitting meditation and mindful yoga, which are aimed at cultivating mindfulness. The program includes a body scan meditation, which helps clients to observe all the sensations in their body. Clients are encouraged to bring mindfulness into all of their daily activities, and this informal practice includes being mindful when standing, walking, eating, and doing chores. Those who are involved in the program are encouraged to practice formal mindfulness meditation for 45 minutes daily (Kabat-Zinn, 1990, 2003).

The MBSR program is designed to teach participants to relate to external and internal sources of stress in constructive ways, and an ongoing commitment to cultivate and practice its principles in each moment is required. Acquiring a mindful way of being is not a simple behavioral technique; it is a way of being that individuals develop over time as they deepen their focus through disciplined practice. Walsh and Vaughan (2019) describe a practical exercise to develop mindfulness this way: For a day, commit to doing only one thing at a time. For this day, give up all multitasking and direct your full attention to each individual activity. Although this sounds easy to do, you may find it quite challenging. Mindfulness practice can have significant effects on how you approach each day.

For counselors to facilitate mindfulness experiences for their clients, Pantaleno and Sisti (2018) suggest that counselors commit to developing a personal mindfulness practice for themselves. Counselors can gain a great deal by establishing even short periods of time for focusing and on quiet reflection. For mindfulness to generalize beyond the therapy office, mindfulness practices must become integrated in daily living.

Mindfulness-Based Cognitive Therapy

MBCT is a comprehensive integration of the principles and skills of mindfulness applied to the treatment of depression (Segal et al., 2013). MBCT is an 8-week group treatment program of 2-hour weekly sessions adapted from Kabat-Zinn's (1990, 2003) mindfulness-based stress reduction program. MBCT uses acceptance techniques to promote resilience and to prevent recurrences of depressive episodes (Antony, 2019). The program integrates techniques from MBSR with teaching cognitive behavior skills to clients. The primary aim is to change clients' awareness of and relation to their negative thoughts.

Morgan, Morgan, and Germer (2013) cite ample evidence that mindfulness meditation enhances the ability to pay attention in a concentrated and sustained manner. Being able to attend to present experiencing is a route to developing compassion toward oneself and expressing compassion toward others. Mindfulness is something that is *caught* more than something that is *taught*. As we develop the skills of mindfulness, our thinking becomes clearer, and we are better able to increase our awareness of the world around us. Mindfulness goes beyond a formal practice; it is meant to become a way of life. The attitude and behavior of the MBCT practitioner are critical in helping clients acquire an accepting way of being and discarding self-critical and judgmental habits. The brevity of MBCT makes this approach an efficient and cost-effective treatment.

Acceptance and Commitment Therapy

ACT is an empirically based psychological intervention that uses acceptance and mindfulness strategies, together with commitment and behavior change strategies, to foster psychological flexibility. ACT is not a technique; it is an approach to developing and applying interventions within a psychological flexibility model. ACT helps clients accept life's difficulties and move in a committed manner. The main goal of ACT is to help clients remain attuned to the present moment and persist in a value-consistent behavior change that will lead to desired outcomes. By attending to what is present in a focused, voluntary, and flexible fashion, clients learn how to alter the function of a belief without actually modifying its content. ACT teaches clients to notice, accept, and even embrace thought and feelings; ACT also teaches clients how to live a meaningful life in the presence of undesirable thoughts and emotions (Antony, 2019; Hayes & Lillis, 2012; Podina & David, 2018).

In contrast to the cognitive behavioral approaches, in which dysfunctional thoughts are identified and disputed, in ACT there is little emphasis on changing the content of a client's thoughts. Instead, the emphasis is on *acceptance* (nonjudgmental awareness) of cognitions. The goal is for individuals to become aware of and examine their thoughts. Clients accept their thinking as it is and aim to reorient their entire relationship with their thought processes. As clients examine

their thoughts, their reactions to and relationship with their own thinking is viewed as an alternative to restructuring the content of their thinking. Pantaleno and Sisti (2018) report that research is accumulating that validates mindful acceptance, descriptive observation, and nonjudgmental openness to experience as an expedient alterative to cognitive restructuring.

The main role of the ACT therapist is to assist clients in becoming more fully themselves so they can begin to live a more vital life consistent with their values (Hayes & Lillis, 2012). Values imply living in a meaningful way as well as reflecting and choosing life goals (Podina & David, 2018). Toward that goal, ACT practitioners might ask clients, "What do you want your life to stand for?" "What do you truly and deeply care about?" "What kind of life do you want to live?" Therapists and clients collaboratively establish treatment goals as clients become more aware of their values, choose values they want to live by, and take action so that their behavior matches their values (Antony, 2019). Clients are asked to make mindful decisions about what they are willing to do to live a valued and meaningful life. ACT can be summed up with this acronym: A = Accept, C = Choose, and T = Take action. ACT strives to help clients accept life's difficulties and to move in a committed manner in the direction of their chosen values (Podina & David, 2018). Therapists challenge clients to identify what matters to them right now and to pursue this each day (Hayes & Lillis, 2012). It is the role of clients to determine the values that will provide them with a meaningful life; it is not the therapist's role to choose values for clients.

ACT is an effective form of therapy that continues to influence the practice of behavior therapy in many areas of mental health that have been empirically studied, including generalized anxiety disorder, panic disorder, borderline personality disorder, eating disorders, and obsessive-compulsive disorder, as well as behavioral medicine and physical health (Hayes & Lillis, 2012). ACT also has been empirically shown to be effective in the treatment of substance abuse, depression, anxiety, phobias, posttraumatic stress disorder, and chronic pain (Batten & Cairrochi, 2015; Podina & David, 2018).

Using Mindfulness and Acceptance in an Integrative Approach

I appreciate the emphasis of the third wave therapies on psychological health as a counterbalance to traditional medical models of diagnoses. The movement away from a disease model of psychological disorders is a key dimension of the mindfulness and acceptance therapies (Follette & Hazlett-Stevens, 2016). I draw on a number of themes and key concepts of the mindfulness and acceptance approaches for my integrative model. All of these approaches place value on developing and maintaining a therapeutic alliance, and I am in agreement with the notion that a therapeutic relationship is

the foundation for designing any effective therapeutic approach. I appreciate the emphasis not only on establishing and maintaining the therapeutic partnership but also on monitoring the therapeutic alliance for the duration of the therapy.

The third wave therapeutic approaches share common views regarding assisting clients in choosing values they want to live by, establishing specific goals, and taking steps to achieve their goals. Both the behavior therapies and the mindfulness and acceptance therapies are grounded on behavioral concepts and key therapeutic strategies for accomplishing their purposes; these techniques are based on empirical data. Mindfulness-based therapies are effective treatments for stress disorders and may be even more effective when combined with therapeutic lifestyle changes such as exercise, time in nature, diet and nutrition, recreation, providing service to others, relationships, relaxation, and spiritual involvement (Walsh & Vaughan, 2019). The goals of mindfulness-based interventions can be integrated into most approaches to therapy. These goals include improved concentration, a greater sense of executive control, awareness of valued choices, reduction of negative thoughts and emotions, and general emotional resilience (Pantaleno & Sisti, 2018). These newer approaches emphasize experiential learning, in-session practice, learning from feedback, completing between-sessions assignments, and applying what is learned in the program to challenging situations encountered outside of the sessions. These third wave approaches bridge the gap between the humanistic and relationship-oriented therapies and traditional cognitive behavior models.

Becoming the Client: Experiencing Behavior-Oriented Therapy

In this section, I ask you to put yourself in the role of the client to experience the benefits of behavior-oriented therapy. The behavioral methods described in this section can be most useful in translating your insights into concrete action plans.

As my client, I will offer you a range of behavioral strategies, but I still want to attend to what you are thinking and feeling. We explore how your self-talk and thinking influences how you feel, and we explore how your thinking and feeling patterns influence what you are doing. By focusing on what you are doing, you can make an evaluation of the degree to which your present behavior is getting you what you want.

We begin with an assessment to identify patterns using the BASIC I. D. model. Once this initial assessment is completed, we work together to explore problem areas so I can understand you more fully. We are then ready to identify a set of therapy goals that both of us agree are suitable for your therapy. Which of these behavioral goals will you set for yourself?

- Learning to ask clearly and directly for what you want
- Learning to be assertive without becoming aggressive
- Acquiring habits that lead to physical and psychological relaxation
- Developing specific habits for a healthy lifestyle (exercising regularly, controlling eating patterns, reducing stress)
- Monitoring your behavior or cognitions as a means to change
- Recognizing and challenging self-destructive thought patterns or critical self-statements that lead to problematic behaviors
- Learning communication and social skills
- Developing problem-solving strategies to cope with a variety of situations encountered in daily life

Once you have identified your goals, we break down these general goals into specific, concrete, measurable activities that you can pursue in a systematic fashion. For example, if you say, "I'd like to feel more adequate in social situations," I may ask: "What are you doing or not doing that seems to be related to your feeling of inadequacy? What are the conditions under which you feel inadequate? Can you give me some concrete examples of situations in which you feel inadequate? In what specific ways would you like to change your behavior?"

Borrowing from reality therapy, I find Wubbolding's (2000, 2011, 2013, 2016, 2017) WDEP formulation to be especially useful in my work with you.

- **W** stands for exploring wants, needs, and perceptions. I do not tell you what you should change but encourage you to examine what you want.
- **D** stands for exploring the direction of your current behavior and your satisfaction with where you are going.
- **E** stands for evaluation, which consists of you making your own evaluation about what you are actually doing. It is up to you to decide how well your current behavior is working for you.
- **P** stands for designing a plan for change.

Exploring what you want is the first step. Through self-evaluation you determine what you have been doing that is not working. In your self-evaluation, we focus on your explicit and controllable choices and what you are willing to change. This lowers resistance and opens you up to other behaviors or directions. Planning how to bring about this change is the next step. At each of these points in the WDEP model, specific questions get you to look at what you are doing and explore other ways to arrange your life. You may have trouble reaching your goals if your plans are not sufficiently thought out. By developing and assessing behavioral strategies together, we can move in the direction of making your vision a reality.

An aspect I particularly like about behavior therapy and related action therapies is the wide range of behavioral techniques available to assist you in moving in the direction you desire. Here is a sample of the behavioral strategies we might apply to meet your personal goals:

- You indicate that you experience a good deal of anxiety. You find yourself rushing and doing many things at once. I teach you a few basic relaxation procedures that you agree to practice once a day.
- To address your habit of trying to do too many things at once, I teach you the principles of mindfulness, which require that you stay in the here and now, focusing on *what is* rather than *what if*. With this focus, you can observe your moment-to-moment experiencing and concentrate on one thing at a time. Mindfulness is a nonjudgmental way of being and a form of acceptance.
- You say that you want to get better at asking for what you want, without sounding apologetic. I use strategies such as coaching, modeling, and social skills training to teach you how to approach others more effectively.
- You want to improve your time management skills. You find that you procrastinate a great deal and then rush to complete projects. Together we brainstorm possibilities for effectively dealing with procrastination. We come up with a number of specific points for better managing your time as a student: focus on demanding subjects first, set goals in number of pages of a reading assignment, create a reward system, avoid placing guilt on yourself about what you don't do, give yourself credit for what you do, stop the interruptions, and allow for short breaks.
- You would like to reduce your anxiety when you feel you are being tested, which prevents you from going on job interviews. I begin with a specific analysis of the nature of your anxiety by asking how you experience this anxiety in specific situations, including what you actually *do* in these situations: "When did it begin? What are some situations when you most experience this anxiety? What do you do at these times? What are your feelings and thoughts in these situations? How do your present fears interfere with obtaining what you want? What are the consequences of your behaviors in threatening situations?" After this assessment, I introduce strategies to help you reduce your anxiety to a manageable level, and we define some specific behavioral goals. One of your goals is: "I will arrange for one job interview during this week and report back at our next session." I get a commitment from you to work toward this goal, and together we evaluate your progress toward meeting this goal at our next session.
- You are expected to actively work outside of your counseling sessions. At each of your sessions we collaboratively design between-sessions activities that enable you to take into your daily life what you are learning in the office. These assignments are carefully designed to help you carry out positive actions and induce emotional and attitudinal change. This practice extends the value of the brief time we have during a session and fosters an active stance on your part in working on your goals. Toward the end of therapy, I encourage you to review your progress, make plans, and identify strategies for dealing with continuing or potential problems.

Behavior includes emotions, sensations, imagery, cognition, interpersonal relationships, and health. Because you are an integrated being functioning in all of these modalities, a behavioral focus in counseling must attend to more than simply what you are doing. As we work on specific behaviors that we have targeted for exploration, I need to be mindful of the thoughts and feelings you are experiencing pertaining to the behavioral dimension.

Throughout our work together, I encourage you to see the value in actively trying new behavior rather than leaving action to chance. One way of fostering an active stance is to formulate a clear contract that includes writing out a plan of action. In this way, you are continually being confronted with what you want and what you are willing to do.

Developing a Behavioral Contract

Contracts are a useful frame of reference for evaluating the outcomes of counseling, but developing an effective contract is not as easy as it may seem. I borrow some specific aspects of formulating and carrying out a plan from a prominent reality therapist, Robert Wubbolding (2000, 2011, 2017). Here are some specific suggestions for creating an effective plan.

- Plans are based on the client's personal goals. Begin by having the client specify desired changes. Goals should be measurable, attainable, positive, and significant to the client.
- Goals must be translated into target behaviors. Ask what specific behaviors the client wants to increase or decrease. Design plans around the answer to this question.
- Once behavioral changes the client wants to acquire are evaluated, an action program to bring about change is devised.
- Encourage the client to come up with clear plans for what he or she will do today, tomorrow, and the next day to bring about change and to anticipate what might get in the way of these plans.
- It is a good idea to begin the plan as soon as possible. Ask the client, "What are you willing to do today to begin to change your life?" "What are you going to do now to attain your stated goals?"
- It is essential that a client makes a commitment to follow through with the plan. It is useful to discuss what will help in implementing the plan and to identify potential barriers to sticking with the plan.
- Good plans are simple and easy to understand. Plans should be flexible and open to modification as the client gains a deeper understanding of the specific behaviors she or he wants to change.
- The plan should be within the limits of the client's motivation and capacities. Like goals, plans should be realistic, attainable, and reflective of what the client needs and wants.
- Good plans are specific. Develop specificity by addressing questions such as "What?" "Where?" "With whom?" "When?" and "How often?"

- Plans are best stated in positive terms by pointing out what will be done rather than what won't be done.
- It is a good idea to develop plans the client can carry out alone. Plans that are contingent on what others will do or not do can be restrictive and difficult to evaluate.
- In choosing action-oriented steps, it is essential that the client considers his or her internal and external resources and limitations.
- Effective plans are repetitive and, ideally, are performed daily.
- Effective planning involves process-centered activities, such as applying for a job, writing a letter to a friend, taking a yoga class, devoting 2 hours a week to volunteer work, or taking a vacation.
- It may be necessary to revise the plan from time to time. Ask the client, "Is your plan helpful?" If the plan does not work, it can be reevaluated and alternatives considered.
- When a plan is not working, it is often the result of having a mismatch between the client's goals and a realistic plan that is manageable.

Creating and carrying out behavioral plans enables clients to gain effective control over their lives. This is clearly the teaching aspect of counseling, which is best directed toward providing clients with new information and assisting them in the discovery of more effective ways of getting what they want and need.

For further discussion of behavior therapy and other action-oriented approaches, see Corey (2017, chap. 9), Wedding and Corsini (2019, chap. 6, chap. 7), Neukrug (2018, chap. 8), Prochaska and Norcross (2018, chap. 9, chap. 10, chap. 11), Sharf (2016, chap. 8), and Wubbolding (2011, 2017).

Working With Shante Using a Behavioral Focus

Shante, a 32-year-old African American woman, has recently come to counseling after experiencing a setback in her life that has exacerbated her stress and led to considerable weight gain. Several months ago, Shante was involved in a serious automobile accident. She and her partner, Lydia, were driving on the highway when a car suddenly swerved from the next lane, striking them and causing Shante to lose control of her car and crash into another vehicle. Although Shante and her partner were fortunate to survive the accident with relatively minor physical injuries, the driver of the car she hit suffered more serious injuries, leaving Shante feeling guilty and anxious. Although Shante is aware that the driver who hit her car ultimately bears the responsibility for causing the accident, she cannot help feeling upset and concerned about the injured driver. These feelings are contributing to her excessive eating, avoiding exercise, and the resulting weight gain, all of which have had a negative impact on her self-esteem. Moreover, Shante has developed an immobilizing fear of driving for fear of losing control.

My initial focus is on assessing Shante's current behavior. I ask Shante to monitor what she is doing so we can create baseline data to evaluate any changes. We then collaboratively develop concrete goals for her therapy. Her behavioral goals include weight loss and improved dietary habits, increased physical activity, and achieving a return to driving. I use a wide range of cognitive and behavioral techniques to help Shante achieve her goals, including stress-reduction techniques, systematic desensitization, in vivo exposure, flooding, mindfulness and acceptance methods, and relaxation techniques. I emphasize learning new coping behaviors that Shante can use in everyday situations. She practices these activities both during her therapy sessions and outside of the office during the week.

Shante has difficulty focusing her attention on daily tasks because she is consumed with anxiety and guilt about the injured driver. Replaying the car accident in her mind over and over, she often seems lost in thought and disconnected from her body. She frequently eats unhealthy meals and snacks to bring herself comfort and admits that she doesn't pay much attention to the nutritional content of the food she consumes. Her motivation to go to the gym, as she once did on a consistent basis, has decreased dramatically, and she expresses a desire to change this pattern.

To assist her in learning how to direct her attention to whatever she is doing each moment, I introduce mindfulness practice. Although being in the moment sounds simple, most people find it quite challenging to be aware and fully present. I teach Shante specific mindfulness skills in the therapy sessions. I ask her to monitor her experience of mindfulness practice by writing each day in her journal about how she is experiencing what she eats and how her body feels. We then discuss her progress in using mindfulness skills as a way to focus herself and reduce stress in her life. I also suggest that she read Ronald Siegel's (2010) book, *The Mindfulness Solution: Everyday Practices for Everyday Problems.* Shante and I discuss Siegel's notion that by observing how we create our own unnecessary anguish, mindfulness practices teach us how to release painful attitudes and replace them with useful mental habits.

In mindfulness practice, Shante will train herself to focus on her present experience, while at the same time achieving a distance from it. For example, when she becomes aware of guilt feelings related to the car accident, she will train herself to notice these feelings without judging them. My hope is that Shante will acquire an attitude of mindfulness in every aspect of her daily life. I encourage Shante to practice formal mindfulness meditation daily. In our sessions each week, I ask how her practice is going and whether she is progressing on applying these mindfulness principles to her everyday living.

Shante has been relying on her partner for transportation to and from work for the past several months. However, she is aware that this is not a long-term solution. She fears that Lydia will grow tired of doing this and that it will cause a strain on their relationship. Shante feels committed to overcoming her fear of driving, and I work with her using systematic desensitization. This technique requires a good deal of effort, and Shante says she is willing to do the work. Systematic desensitization is an effective and efficient technique for reducing maladaptive anxiety and treating anxiety-related disorders through a gradual exposure process.

A fundamental part of behavior therapy consists of the work clients do outside of the therapy hour. The techniques used are continually verified to determine how well they are working. Actual changes in Shante's behavior provide critical information for making this evaluation. Her own evaluation of how much progress she sees and how satisfied she is by the outcomes is a major factor in assessing therapeutic results.

In a particular counseling session, Shante brings up the subject of her weight. Shante mentions that she does not exercise and that she does not have much energy. She states that she wants to develop a regular exercise program. If her program is to work well for her, she must identify what type of exercise is appropriate for her. Together we determine that walking will be a vital part of her exercise program. It is an activity that she can do that does not require her to drive to the gym, which she currently is resistant to doing. Once her fear of driving has been extinguished, exercising at the gym may become part of her program. We work out the details, including how often she will walk and for how long. I encourage her to use her friend as a source of support in helping her stick with her plans. Furthermore, I strongly recommend that she find ways to monitor her progress and keep herself accountable in following her plans for appropriate eating and exercising habits.

Working from an integrative perspective, it is imperative that I also attend to Shante's thoughts and feelings about her weight and her body image in general. Her self-talk is not helpful to her in this area. She castigates herself about not looking right and about being weak because she is overweight. To worsen matters, she also tells herself that she deserves to feel bad about herself because she caused harm to the driver in the car accident. Her negative self-talk leads to feelings of defeat, depression, and anger with herself. When her clothes don't fit, she tells herself she is totally unattractive and feels upset and discouraged. It will not be enough to simply chart out the steps in a behavioral program for weight control through exercise and dieting. We also need to address her thoughts and emotions. Our focus will shift from one particular dimension to another at various times during her therapy.

Reflection Questions

Reflect on conceptualizing the case of Shante from a behavioral perspective. Consider these questions:

- What are some behavioral interventions that you might employ with Shante's concern about her weight and her body image?
- Think of how you could collaboratively design homework assignments with Shante. What could you do to increase her motivation for implementing these assignments?
- What thoughts do you have about encouraging Shante to engage in mindfulness practice, both in the session and between her therapy sessions?

Working With Marlin Using a Behavioral Focus

Marlin is 20 years old and has been in therapy with me for three sessions. Marlin says he isolates himself at work and would like to feel free enough to be able to carry on conversations with others at his workplace. He had a previous therapy experience and has spent a great deal of time gathering insights into why he is the way he is, yet he has not been successful in making the desired changes in his life. He has explored some of his core beliefs and has begun to acquire a more realistic set of beliefs, but this work has not brought about substantive personality changes. By addressing what Marlin is *doing*, we can apply his feelings and thoughts to real-life situations in various action programs. I greatly value the contributions of Adlerian therapy, behavior therapy, reality therapy, rational emotive behavior therapy, cognitive therapy, and solution-focused brief therapy, all of which give central emphasis to the role of action in the real world as a prerequisite for change.

I have established a good working alliance with Marlin during our initial sessions, and I want to continue to monitor the quality of our therapeutic relationship and the progress of therapy. I rely on systematic feedback from my clients to ensure that what we are doing in our therapy sessions is effective. Scott Miller and his associates at the International Center for Clinical Excellence developed two 4-item instruments to measure client progress and to rate the quality of the therapeutic relationship, and I will use these scales throughout Marlin's therapy. These rating instruments are brief, well-validated, client-rated scales. The *Outcome Rating Scale* assesses the client's therapeutic progress through ratings of a client's personal experience of well-being in his or her individual, interpersonal, and social functioning. The *Session Rating Scale* measures a client's perception of the quality of the therapeutic relationship, which includes the bond with the therapist, the perceived collaboration around specific tasks in therapy, and agreement on goals, methods, and client preferences

(Miller et al., 2015). More information on these scales is available in *Feedback-Informed Treatment in Clinical Practice: Reaching for Excellence* (Prescott et al., 2017). This book contains the latest cutting-edge information about feedback-informed treatment and how to integrate client feedback into therapeutic practice.

Using these rating scales with Marlin provides me with a simple, practical, and meaningful method of documenting the usefulness of his therapy experience. Based on his feedback, I can adjust and accommodate to maximize beneficial outcomes. The action-oriented behavioral approaches rely on evidence-based methods; this kind of systematic feedback from Marlin is essential to enhance our work in the therapeutic enterprise.

Behavior therapy offers many techniques aimed at bringing about behavioral change. In Marlin's case, I am inclined to work with him on developing a self-management program. For example, he complains of often feeling tense and anxious. By learning and practicing daily relaxation procedures, Marlin will gain more control over his physical and psychological tension. Through mindfulness practice, Marlin will experience more success in getting himself centered before he goes to his workplace, meets colleagues, or talks to friends. He is making significant changes by monitoring his behavior in everyday situations. Through this self-monitoring, he is gaining increased awareness of what he tells himself, what he does, and how he feels. He has been writing in a journal, which is therapeutic and is enabling him to behave in novel ways. In his journal, he has been recording events that lead up to his feeling depressed (or anxious or hurt). In addition, he is writing about what he actually did in these situations and what he might have done differently. By paying attention to what he is *doing* in daily life, Marlin is beginning to gain more control of his behavior. I reinforce Marlin's observations of his behavior and thoughts and his journaling by letting him know that there is consistent evidence that self-monitoring leads to improved therapy outcomes (Naar & Safren, 2017).

Marlin would like to engage in social interactions with his colleagues at work, but his negative self-talk gets in the way. He does not like being invisible at work, and he would like to feel free enough to talk with people at work. He is convinced that he is missing opportunities to form friendships, and he wants to change how his fears get in his way. For several therapy sessions Marlin practices initiating conversations with his coworkers through role playing. After several behavioral rehearsals, Marlin feels more confident in engaging in social interactions at work. Together we carefully design between sessions-activities that will enable Marlin to carry his behavioral rehearsals into the work setting. Homework is an excellent way for Marlin to become an active agent in his therapy. He must *do* something in daily situations if change is to occur. The degree to which he will change is directly proportional to his willingness to experiment by trying out new ways of behaving.

Marlin agrees to take the initiative to approach some of the colleagues he would most like to spend time with and get to know. At

the beginning of the following session, I ask Marlin how things went with engaging selected people at work. He is apologetic and informs me that he failed to carry out his assignment. Together we explore what stopped him from doing what he said he wanted to do. If his self-talk and his thoughts are stopping him, it can be helpful for Marlin to learn about the mindfulness principle of recognizing that a thought is just a thought.

Resolutions and plans are empty unless there is a decision to carry them out. It is crucial that Marlin commits to a definite plan that he can realistically accomplish. I draw upon the WDEP model in reality therapy in teaching Marlin what constitutes an effective plan (Wubbolding, 2017). Each week, we discuss Marlin's progress toward meeting his goals and review how well he is completing his assignments. If he does not do an assignment or if he does not like the way he carried out an assignment, we can use this as an opportunity to talk about what this means to him. Rather than judging his behavior in performing assignments, the emphasis is on what can be learned and how he can learn new skills that will lead to success. We spend a good deal of time refining and practicing skills so Marlin can approach situations in a different way. Much of our work together consists of behavioral practice and fine-tuning a plan of action to be implemented between our sessions. The ultimate responsibility for making plans and implementing them rests with Marlin. My task is to consistently encourage Marlin to learn specific information and skills to cope with a range of challenges he may encounter in day-to-day living. Soon Marlin will become his own counselor, applying his new skills for effective living not only to present problems but also to future difficulties.

Reflection Questions

Reflect on conceptualizing the case of Marlin from a behavioral perspective. Consider these questions:

- Accepting the assumption that taking action in the real world is necessary for change to occur, which therapy approach would you find of value in working with Marlin, and why?
- How could you use systematic feedback from Marlin to assess and modify your interventions with him?
- Marlin claims that his negative self-talk prevents him from engaging in social interactions with his colleagues at work. What are some behavioral interventions you would likely use in working with Marlin?

Concluding Comments

An emphasis on evidence-based therapies as a part of the assessment and treatment process is a hallmark of behavior therapy. Behavior thera-

pists are committed to examining the effectiveness of their procedures in terms of their generalizability, meaningfulness, and durability of change. It is up to practitioners to demonstrate that therapy is working. If progress is not being made, therapists look carefully at the original assessment and treatment plan and make modifications as necessary.

A significant feature of behavior therapy is the collaboration between therapist and client. The client is encouraged to become an active participant and to provide regular feedback to the therapist regarding the usefulness of specific interventions. Therapists modify their treatment procedures based on this feedback. Clients have a major stake in formulating therapy goals and are expected to experiment for the purpose of enlarging their repertoire of adaptive behaviors. Clients need to be willing to practice behavioral changes and to continue to implement new behaviors outside of the therapy office. The impact of therapy is not realized until the client transfers the new behaviors from the office to everyday life.

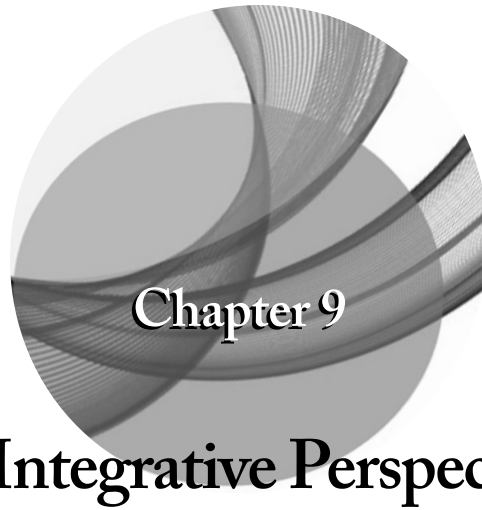
Arnold Lazarus is a key figure in the development of clinical behavior therapy and is the founder of multimodal therapy. Although multimodal therapy has its foundation in social cognitive learning theory, it encourages technical eclecticism, which involves borrowing techniques from a wide range of theoretical approaches. Multimodal therapists take great care to determine precisely what relationship styles and what treatment strategies will work best for each client and under which particular circumstances. Therapeutic flexibility and versatility are highly valued, and multimodal therapists take steps to adjust their procedures to achieve the client's goals. Multimodal therapy begins with a comprehensive assessment of seven modalities of human functioning and the interaction among them. Based on this assessment, therapists teach clients a range of techniques they can use to correct faulty thinking, to learn to manage stressful situations, and to acquire effective interpersonal skills.

Recent developments in the third wave of behavior therapy demonstrate the evolution of the behavioral approaches. Behavior therapy is not a static model; it is an open system that meets a range of needs for diverse client populations. The mindfulness and acceptance theories represent a humanistic perspective that bridges the gap between the relationship-oriented therapies and theories that emphasize evidence-based practices. The new generation of mindfulness and acceptance-based therapies have shifted behavior therapy from treating simple problems to a more complex and complete approach based in behavioral principles.

Review the key themes of the behavioral approaches described in this chapter and apply them to yourself. Apply the WDEP model (of reality therapy) to identify a behavior you are willing to change (Wubbolding, 2017). This might be a behavior you'd like to reduce or a behavior that you'd like to acquire. Once you have selected your target behavior, apply the guidelines for developing an action plan to yourself.

After reading this chapter on the behavioral focus in counseling, take time to reflect on the following questions as a way of clarifying your thoughts on the behavioral focus.

- What is one of your behaviors that is not working for you at this time?
- What is the importance of developing a specific and realistic action plan as a requisite for change?
- What are the characteristics of an effective plan? What are some guidelines you would use to help your clients design a plan for change?
- How does a systematic and comprehensive assessment of the client provide information on which techniques to apply in treating specific problems?
- What aspects of third wave theories would you most want to incorporate in your work with clients?
- What are some ways you could apply mindfulness in your personal life? What value do you place on being more mindful?
- What importance would you place on the therapeutic relationship when working from a behavioral perspective?
- How do you imagine it would be for you to be a client in behavior-oriented therapy?
- What concepts and techniques from the behavioral approaches fit into your personal approach to counseling?



Chapter 9

An Integrative Perspective

In this chapter I present my own perspective on integrative therapy, and I assist you in designing your own theoretical stance. Psychotherapy integration is best characterized as looking beyond and across the confines of single-school approaches to see what can be learned from other perspectives (Norcross et al., 2016). Norcross and Beutler (2019) maintain that effective clinical practice requires a flexible and integrative perspective that is customized to the unique needs and contexts of the individual client. Two common pathways to achieving this integration are technical integration and theoretical integration.

Technical integration is based on a systematic selection of techniques from many approaches, without necessarily subscribing to the theoretical positions that spawned them. Therapists have a variety of tools in their toolkit to use with clients. Clinical effectiveness is determined by the degree of a therapist's flexibility, versatility, and technical eclecticism (Lazarus, 2008).

In contrast, *theoretical integration* is a conceptual or theoretical creation that goes beyond blending techniques. This path creates a conceptual framework that synthesizes the best of two or more theoretical approaches to produce an outcome richer than that of a single theory (Norcross et al., 2016). This approach emphasizes integrating the underlying theories of therapy along with techniques from each. Examples of this form of integration are *dialectical behavior therapy* and *acceptance and commitment therapy*, both of which are described in Chapter 8. *Emotion-focused therapy*, which is informed by the role of emotion in psychotherapeutic change, is also a form of theoretical integration. The most complex and difficult of all types of integration is theoretical integration because it requires bringing together concepts from disparate approaches.

Lazarus (2008) has concerns about theoretical integration because he believes blending aspects of different theories is likely to obfuscate therapeutic practice. He contends that by remaining theoretically consistent, but technically eclectic, practitioners can spell out precisely what interventions they will employ with various clients, as well as the means by which they select these procedures.

A third path to psychotherapy integration is *assimilative integration*. This route is grounded in a single psychotherapeutic orientation but draws techniques from other therapeutic models, which are integrated in as seamless a fashion as possible. Although a single theoretical perspective informs the therapist's understanding of the needs of the client, a diverse range of techniques can be employed in constructing an individualized treatment plan. An example of this form of integration is *mindfulness-based cognitive therapy*, which integrates aspects of cognitive therapy and mindfulness-based stress reduction procedures. As you may recall from Chapter 8, this approach is a comprehensive integration of the principles and skills of mindfulness that has been applied to the treatment of depression (Segal et al., 2013).

I see many advantages to incorporating a diverse range of techniques from many different theories, but I also think it is possible to incorporate key principles and concepts from various theoretical orientations. Some concepts from the experiential approaches blend quite well with the cognitive behavior therapies. For example, the experiential approaches emphasize here-and-now awareness, the therapeutic relationship, and an exploration of feelings—all concepts easily incorporated into action-oriented therapies. Clients can be asked to decide what they want to do with present awareness, including making behavioral plans for change. All the action-oriented therapies depend on a solid therapeutic alliance. Techniques will not take root if there is not a good working relationship, and clients are more likely to cooperate with a therapist's cognitive and behavioral interventions if they believe the therapist is genuinely interested in them.

Searching for Common Factors Across Therapy Schools

Another route to psychotherapy integration is the *common factors approach*, which starts by identifying specific effective ingredients of any group of psychotherapies. This approach to integration searches for common aspects across different theoretical systems. Despite many differences among the theories, a recognizable core of counseling practice is composed of nonspecific variables common to all therapies. Research suggests that *common factors* can become the focal point for integration of seemingly diverse therapy techniques (Lambert, 2011). Some examples of these common factors include empathic listening, a therapeutic alliance, expectations of the therapist and client for a positive therapeutic experience, understanding interpersonal and in-

trapersonal dynamics, learning to be self-reflective, and a systematic way of using techniques to address problems (Norcross et al., 2016). Hubble, Duncan, Miller, and Wampold (2010) summarized research in the field and found four factors that account for change in therapy: client factors: 40%; alliance factors (the therapeutic relationship): 30%; expectancy factors (hope and allegiance): 15%; and theoretical models and techniques: 15%. The common factors are thought to be at least as important in accounting for therapeutic outcomes as the unique factors that differentiate one theory from another (Norcross et al., 2016).

According to Lambert (2011), of all the common factors investigated in psychotherapy, none has received more attention and confirmation than a facilitative therapeutic relationship. The importance of the therapeutic alliance is a well-established critical component of effective therapy. Interpersonal, social, and affective factors common across therapeutic orientations are more critical than techniques employed when it comes to facilitating therapeutic gains. Wampold's (2001, 2010) review of psychotherapy research support's Lambert's findings that no specific form of treatment has been proved to be clearly superior to another. Wampold (2010) concludes that "there is little evidence that the specific ingredients of any treatment are responsible for the benefits of therapy" (p. 71). Specific treatment techniques make relatively little difference in outcome when compared with the value of common factors, especially the human elements (Elkins, 2016). The common factors that are part of all theoretical orientations—such as the therapist's ability to form a working alliance with the client—are critical to therapeutic outcomes. Research indicates that a variety of treatments are equally effective when administered by therapists who believe in them and when they are accepted by the client (Corey, 2015). The various therapy models and techniques work equally well because they share the key factor accounting for change—the client (Bohart & Tallman, 2010).

Creating an integrative approach to practice is not a simple task. You cannot simply pick pieces from theories in an unsystematic manner or based on personal whim. The kind of integration I am suggesting is based on common factors across therapeutic schools. However, blending theoretical constructs is more demanding than utilizing diverse techniques from different schools. If you attempt to blend theoretical constructs from different orientations in your own integrative model, it is critical that these concepts be compatible and that these frameworks lend themselves to a workable merger. Some blending simply does not make much conceptual sense. For instance, psychodynamic theory, which focuses on unconscious factors as the source of present-day problems, does not blend nicely with theories that reject the unconscious, such as rational emotive behavior therapy and reality therapy. Likewise, psychodynamic theories are geared around central concepts such as exploration of past traumatic events, exploration of dreams, working through the transference relationship, and aware-

ness and management of countertransference. Some therapy models do not provide a framework for exploring these theoretical constructs.

A comprehensive work on various perspectives on psychotherapy integration is the *Handbook of Psychotherapy Integration* (Norcross & Goldfried, 2019). I also recommend *Psychotherapy Integration* (Stricker, 2010). These books will give you a sense of the direction being taken by the psychotherapy integration movement. In addition to these sources, the following textbooks dealing with counseling theory have a chapter on integration of therapies: Corey (2017, chap. 15), Norcross, Goldfried, and Arigo (2016), Prochaska and Norcross (2018, chap. 16), Sharf (2016, chap. 16, chap. 17), and Wedding and Corsini (2019, chap. 14).

The Foundation of My Integrative Approach

Existential theory comes closest to my worldview and serves as the foundation for constructing my theoretical orientation, but I also draw heavily from two other related theories: Gestalt therapy and psychodrama. After briefly describing some of the key concepts and themes from the existential, Gestalt, and psychodrama orientations, I discuss how I incorporate basic concepts and techniques from a number of the action-oriented therapies and from systems therapies as well.

Existential Therapy as a Philosophical Base

My own philosophical orientation is strongly influenced by the existential approach, which conceives of counseling as a life-changing process. Existential therapy is the foundation of my integrative approach to counseling, and I operate on its assumption that we each can be the architects of our own lives. If we don't like the design of our present existence, we can take steps to revise the blueprints. Counseling is a journey in which the therapist is a guide who facilitates client exploration.

A number of key themes from the existential approach capture the essence of this therapeutic venture. According to the existentialist view, we are capable of self-awareness, which is the distinctive capacity that allows us to reflect and to decide. With this awareness, we become free beings who are responsible for choosing the way we live, and thus we create our own destiny. I value the emphasis on freedom and responsibility, for this notion challenges us to redesign our lives. I encourage people to look at the choices they *do* have, however limited they may be, and to accept responsibility for choosing for themselves. However, making choices gives rise to existential anxiety, which is another basic human characteristic. This anxiety is heightened when we reflect on the reality of death. Facing the inevitable prospect of eventual death gives the present moment significance as we become aware that we do not have forever to accomplish our goals. The reality of death is a catalyst that can challenge us to create a life that has meaning and purpose.

We strive toward a meaningful life by recognizing our freedom and by making a commitment to choose in the face of uncertainty. Individu-

als often seek therapy because of concerns about their purpose in life (Yalom & Josselson, 2019). An existential view provides me with the framework for understanding universal human concerns, including facing and dealing with the problems of personal freedom, self-alienation and estrangement from others, the fear of death and nonbeing, living with courage, exploring the meaning of life, and making critical life choices. The life themes offer a depth to therapy, and I am convinced that what people bring to therapy are these human concerns.

Both existential therapy and person-centered therapy place central prominence on the person-to-person relationship. Both of these approaches emphasize striving to maintain an equal-power relationship and an attitude of unconditional positive regard. Client growth occurs through a genuine encounter. A key factor influencing the outcome of therapy is the quality of the therapeutic relationship, which is strongly supported by empirical research (Craig, Vos, Cooper, & Correia, 2016). Emphasizing the human quality of the therapeutic relationship lessens the chances of making counseling a mechanical process.

The conceptual propositions of existential therapy can be creatively integrated with many other therapeutic orientations (Schneider, 2008; Schneider & Krug, 2017). In thinking about therapy from an existential perspective, I am not overly concerned with which techniques I might employ or with creating an agenda for my client. I am convinced that it is not the techniques I use that make a therapeutic difference; rather, it is the quality of the relationship with my client that heals. My main interests are in being as fully present as I am able to be for the client, establishing a trusting relationship, creating safety, and moving into the client's subjective world. If my client is able to sense my presence and my desire to make a real connection, then a solid foundation is being created for the hard work that follows.

Because the existential approach is concerned with the basic conditions of human existence and envisions therapy as a shared journey, I do not feel bound by a specific set of techniques. Interventions are used in the service of broadening the ways in which clients live in their world. Techniques are tools to help clients become aware of their choices and their potential for action. Although I incorporate techniques from various therapeutic models, these interventions are made within the context of striving to understand the subjective world of the client.

An excellent resource on the existential approach is *Existential-Humanistic Therapy* by Schneider and Krug (2017). For further discussions of existential therapy, see Corey (2017, chap. 6), Deurzen (2010, 2012), Elkins (2009, 2016), Frew and Spiegler (2013, chap. 5), Neukrug (2018, chap. 5), Prochaska and Norcross (2018, chap. 4), Sharf (2016, chap. 5), and Wedding and Corsini (2019, chap. 8).

Gestalt Therapy: A Holistic Perspective

Gestalt therapy is truly an integrative orientation in that it focuses on whatever is in the client's awareness. From the Gestalt per-

spective, feelings, thoughts, body sensations, and actions are all used as guides to understand what is central for the client in each moment. The centrality of whatever is in the client's awareness is an ideal way to understand the world of the client. I attempt to approach clients with an open mind and focus on what occurs phenomenologically with my client. By paying attention to the verbal and nonverbal cues provided, I have a starting point for exploring the client's world.

Functioning within a Gestalt framework, I view my main goal as striving to increase the client's awareness of "what is." Change occurs through a heightened awareness of what the client is experiencing in the present moment. Gestalt therapy pays attention to *process* as well as content. Awareness, choice, and responsibility are cornerstones of practice. This approach is phenomenological because it focuses on the client's perceptions of reality and is existential because it is grounded in the notion that people are always in the process of becoming, remaking, and rediscovering themselves.

The Gestalt approach is characterized by many key concepts that can be fruitfully blended into other orientations. Gestalt therapy (and psychodrama) techniques encourage clients to bring painful memories and feelings pertaining to both past and present events to center stage. Through the skillful and sensitive use of Gestalt therapy interventions, it is possible to assist clients in heightening their present-centered awareness of what they are thinking and feeling as well as what they are doing; this process can enable them to make decisions about changing the course of living. Jon Frew (2017) contends that it is a common misconception that Gestalt therapy is a tool bag of techniques that a therapist can employ, regardless of his or her theoretical orientation. Gestalt therapists realize that content and process cannot be separated, which is why they pay attention to process and comment on their observations. Frew states that commenting on the process of what is occurring in the present in the therapy relationship invites the client into this relationship. Although the therapist suggests the experiments, this is a collaborative process with full participation by the client. Gestalt experiments take many forms: setting up a dialog between a client and a significant person in his or her life; assuming the identity of a key figure through role playing; or reliving a painful event.

Gestalt therapy utilizes the experiment to move clients from talk to action and experience. This is a perspective on growth and enhancement, not merely a system of techniques to treat disorders. With the emphasis given to the relationship between client and therapist, there is a creative spirit of suggesting, inventing, and carrying out experiments aimed at increasing awareness.

For specific examples of Gestalt experiments when working with the emotive dimension, see Chapter 7 of this book. For further discussion of Gestalt therapy, see Corey (2017, chap. 8), Frew and Spiegler (2013, chap. 7), Neukrug (2018, chap. 6), Polster (2015), Prochaska and Nor-

cross (2018, chap. 6), Sharf (2016, chap. 7), Wedding and Corsini (2019, chap 9), Wheeler and Axelsson, (2015), and Woldt and Toman (2005).

Psychodrama: An Integrative Approach

Psychodrama is primarily an action approach to group counseling in which clients explore their problems through role playing, enacting situations using various dramatic devices to gain insight, discover their own creativity, and develop behavioral skills. The scenes are played as if they were occurring in the here and now, even though they might have their origins in a past event or in an anticipated situation. Although psychodrama is primarily used in group therapy, many psychodrama techniques can be useful in individual counseling. Using psychodrama, the client acts out or dramatizes past, present, or anticipated life situations and roles. This is done in an attempt to gain deeper understanding, explore feelings and achieve emotional release, and develop new ways of coping with problems. Significant events are enacted to help the client get in contact with unrecognized and unexpressed feelings, to provide a channel for the full expression of these feelings and attitudes, and to broaden the role repertoire.

I value psychodrama's active techniques and role playing because these methods lead clients to directly experience their conflicts to a much greater degree than is the case when they *talk about* themselves in a storytelling manner. This direct experiencing tends to bring emotions to the surface. The emotional aspects of an enactment are of therapeutic value, but a degree of cognitive integration is required to maximize the impact of experiencing emotions. Participants in a psychodrama can be encouraged to reflect on how their beliefs and decisions may be contributing to the emotional turmoil they experienced in the psychodrama.

Psychodrama uses a number of specific techniques designed to intensify feelings, clarify implicit beliefs, increase self-awareness, and provide clients with a way to practice new behaviors. One of the most powerful tools of psychodrama is role reversal, which involves the client taking on the part of another person. Through role reversal, people are able to get outside of their own frame of reference and enact a side of themselves they would rarely show to others. In addition, by reversing roles with a significant person, the client is able to formulate significant emotional and cognitive insights into his or her part in a relationship. This technique also creates empathy for the position of the other person.

The concepts and methods of psychodrama offer imagery, action, and direct interpersonal encounters to a range of therapeutic approaches. Psychodramatic techniques can be adapted to fit well within the framework of other theoretical models, including psychoanalytic therapy, cognitive behavior therapies, Gestalt therapy, existential therapy, Adlerian therapy, play therapy, family therapy, and group therapy. The underlying philosophy of psychodrama is consistent with many of the premises of existential therapy, person-centered therapy, and Gestalt

therapy, all of which emphasize understanding and respecting the client's experience and the importance of the therapeutic relationship as a healing factor. Although practitioners who employ psychodramatic methods assume an active and directive role, these techniques are most effective when the counselor adopts a person-centered spirit. Practitioners who are authentic, who are able to make connection and to be psychologically present, who demonstrate empathy, and who exhibit a high level of respect and positive regard for their clients are most effective in implementing a range of psychodrama techniques.

For a more detailed discussion of psychodrama applied to group counseling, see Blatner (1996, 2006) and Corey (2016, chap. 8).

Drawing on the Action-Oriented Therapies

As important as it is to work with the emotional realm, I think it is essential to incorporate concepts and techniques from the action-oriented approaches to bring about both cognitive and behavioral changes. Here are some of the ways I utilize the action-oriented therapies in my integrative model.

Adlerian Therapy

The basic goal of the Adlerian approach is to help clients identify and change their mistaken beliefs about self, others, and life and thus participate more fully in a social world. The therapeutic process helps clients make some basic changes in their style of living, which leads to changes in the way they feel and behave. I especially like the Adlerian perspective on therapy as a cooperative venture. Therapy is geared toward assisting clients in being able to translate their insights into action in the real world.

Adlerians view their work primarily as an educational process—helping people learn better ways to meet the challenges of life tasks, providing direction, helping people change their mistaken notions, and offering encouragement to those who are discouraged. Adlerians focus on strengths, well-being, and encouragement. Adlerian therapy rests on a central belief that our happiness is largely related to social connectedness. Because we are embedded in a society, we cannot be understood in isolation from that social context. We are primarily motivated by a desire to belong. As social beings, we have a need to be of use to others and to establish meaningful relationships in a community. As socially embedded individuals, our lives and contributions flourish when we experience a sense of belongingness to the community.

The Adlerian model lends itself to versatility in meeting the needs of a diverse range of clients (Carlson & Englar-Carlson, 2017; Carlson, Watts, & Maniacci, 2006). Adlerians are not bound to follow a specific set of procedures, which gives them a great deal of freedom in working with clients in ways that are uniquely suited to their own therapeutic style. Adlerian therapists are resourceful in drawing on a

variety of cognitive, behavioral, and experiential techniques that they think will work best for a particular client. Some of these techniques are encouragement, confrontation, summarizing, interpretation of experiences within the family and early recollections, suggestion, and homework assignments.

One of Adler's most important contributions is his influence on other therapy systems. Many of his basic ideas have found their way into other psychological schools, such as family systems approaches, Gestalt therapy, reality therapy, rational emotive behavior therapy, cognitive therapy, person-centered therapy, solution-focused brief therapy, and existentialism. These approaches are all based on a similar concept of the person as purposive and self-determining and as striving for growth and meaning in life.

Adlerian theory addresses the client's past, present, and future. The notion of teleology, or striving for meaning and purpose, is central, and the spiritual concerns of clients can be addressed. Adlerians look for continuity or a pattern in a client's life, but the emphasis is always on the goal-directed nature of behavior. People are constantly striving to reach self-declared goals that they believe will bring them happiness (Carlson & Englar-Carlson, 2017).

The concept of social interest—that we need to contribute to making the world a better place—can be a part of any theoretical system. People who have a developed sense of social interest care as much about the well-being of others as they do about themselves. As an antidote to social isolation and self-absorption, social interest leads to courage, optimism, and a true sense of belongingness. Going beyond the self and getting involved in making a difference in the lives of others is integrative in its very nature. People cannot be understood without considering their social context. As social beings, we have a need to be of use to others and to establish meaningful relationships in a community. As socially embedded individuals, our lives and contributions flourish when we experience a sense of belongingness to the community. All of these concepts can be adapted to any theoretical model.

Contemporary Adlerian theory is an integration of cognitive, psychodynamic, and systems perspectives, and in many aspects resembles the social constructionist theories. Contemporary social constructionist theories share common ground with the Adlerian approach. Some of these common characteristics include an emphasis on establishing a respectful client–therapist relationship, an emphasis on clients' strengths and resources, and an optimistic and future orientation (Carlson & Englar-Carlson, 2017; Carlson et al., 2006).

Miller and Dillman Taylor (2016) examined many of the core tenets of Adlerian theory from a neuroscience perspective. They concluded that neuroscience research supports many of the key concepts of Adlerian theory: attention to the impact of early experiences on development, the role of environmental and systemic factors on well-being, the importance of viewing assessment and treatment from a holistic

perspective, purposefulness of behavior, social embeddedness, the value of psychoeducation, and the belief in goal-directed action. Neuroscience supports the validity of social interest and its relationship to improving mental health and well-being. Miller and Dillman Taylor suggest that there are many possibilities for integrating neuroscience and neurobiological findings with the development of Adlerian psychology. They believe that this integration offers practitioners a deeper understanding of Adlerian principles and techniques as well as providing additional tools in facilitating change within individuals.

For further discussion of Adlerian therapy, see Carlson and Englar-Carlson (2017), Carlson, Watts, and Maniacci (2006), Corey (2017, chap. 5), Neukrug (2018, chap. 4), Prochaska and Norcross (2018, chap. 3), Sharf (2016, chap. 4), and Wedding and Corsini (2019, chap. 3).

Behavior Therapy

A basic assumption of the behavioral perspective is that most problematic cognitions, emotions, and behaviors have been learned and that new learning can modify them. Although this modification process is often called "therapy," it is more properly an educational experience in which individuals are involved in a teaching/learning process. There are many parallels between counseling and education. Counseling is educational in that people develop a new perspective on ways of learning, and they also try out more effective ways of working with their cognitions, emotions, and behaviors. Techniques from the action-oriented approaches such as role playing, behavioral rehearsal, coaching, guided practice, modeling, feedback, mindfulness skills, and homework assignments can be used to attain humanistic goals that characterize the experiential therapies.

Contemporary behavior therapy stresses clients' active role in making decisions about their treatment, and behavior therapists and clients alter goals throughout the therapeutic process as needed. Therapists develop a partnership with clients in formulating specific measurable goals that are clear, concrete, understood, and mutually agreed. In this action-oriented therapy, the focus is on determining specific steps the client can take to achieve her or his goals. Behavioral treatment interventions are individually tailored to specific problems experienced by clients. Behavior therapy differs from other therapies in that it is brief, active, directive, collaborative, and focused on factors maintaining problem behaviors rather than factors that may have triggered a problem initially (Antony, 2019). A defining characteristic of behavior therapy is the adherence to evidence-based practices. Evidence-based strategies that have research support for use with a particular kind of problem are used to promote generalization and maintenance of behavior change. Many of these research-based behavioral interventions can be effectively integrated into other therapeutic approaches.

For further discussion of behavior therapy, see Corey (2017, chap. 9), Craske (2017), Frew and Spiegler (2013, chap. 8), Neukrug (2018, chap. 6), Prochaska and Norcross (2018, chap. 9), Sharf (2016, chap. 8), and Wedding and Corsini (2019, chap. 6).

Cognitive Behavior Therapy

Most of the contemporary therapies can be considered "cognitive" in a general sense because they have the aim of changing clients' subjective views of themselves and the world. However, the cognitive behavior approaches differ from both psychodynamic and experiential therapies in that the major focus of cognitive behavior therapy is on undermining faulty assumptions and beliefs and teaching clients the coping skills needed to deal with their problems.

In many respects, rational emotive behavior therapy can be considered as a comprehensive and integrative therapeutic practice. Numerous cognitive, emotive, and behavioral techniques can be employed in changing one's emotions and behaviors by changing the structure of one's cognitions. Beck's cognitive therapy is also an integrative approach, drawing on many different modalities of psychotherapy. Cognitive behavior therapies provide a structured, focused, present-centered, time-limited, and active approach.

A feature I particularly value in all the cognitive behavior therapies (and in feminist therapy too) is the demystification of the therapy process. Clients are active, informed, and responsible for the direction of therapy. Being based on an educational model, these therapies all emphasize the therapeutic alliance between therapist and client. These approaches encourage self-help, provide for continuous feedback from the client on how well treatment strategies are working, and provide a structure and direction to the therapy process that allows for evaluation of outcomes. Cognitive behavior therapists are concerned with teaching clients how to be their own therapist. Typically, a therapist will educate clients about the nature and course of their problem, about the process of cognitive therapy, and how thoughts influence their emotions and behaviors. The educative process includes providing clients with information both about their presenting problems and about *relapse prevention*, which consists of procedures for dealing with the inevitable setbacks clients are likely to experience as they apply what they are learning to daily life.

For further discussion of cognitive behavior therapy, see J. Beck (2011), Corey (2017, chap. 10), Craske (2017), Dobson and Dobson (2017), Frew and Spiegler (2013, chap. 9), Kazantzis, Dattilio, and Dobson (2017), Ledley, Marx, and Heimberg (2018), Neukrug (2018, chap. 10), Prochaska and Norcross (2018, chap. 10), Sharf (2016, chap. 9, chap. 10), Sperry and Sperry (2018), and Wedding and Corsini (2019, chap. 7).

Reality Therapy/Choice Theory

Choice theory underlies the practice of reality therapy and is grounded on phenomenological and existential premises. From the perspective of choice theory, we choose our goals and are responsible for the kind of world we create for ourselves. We are responsible for what we choose to do, no matter what has happened in the past. Reality therapy is grounded on existential principles; it also shares many concepts with the cognitive behavior therapies.

One concept of reality therapy I find particularly useful is total behavior, which teaches that all behavior is made up of four inseparable but distinct components: *acting*, *thinking*, *feeling*, and the *physiology* that must accompany all of our actions, thoughts, and feelings. The main emphasis is given to acting and thinking, for these aspects of total behavior are easier to change than are the feeling and physiology components. The key to changing a total behavior lies in choosing to change what we are doing and thinking because these are the behaviors that we can control.

I value the basic notion of the need to assume personal responsibility for our feelings that is stressed by reality therapy. This philosophy takes us out of a passive role and challenges us to accept our part in creating our feelings by what we are doing and how we are thinking. For example, depression is not something that simply happens to us but is an active choice we make. Glasser (1998, 2001) speaks of "depressing" or "angering" rather than "being depressed" or "being angry." This process of "depressing" keeps anger in check, and it also allows us to ask for help. By using action words ending in "ing," the therapist emphasizes that feelings are behaviors. Clearly, the emphasis of this theory is on how we act and think, and in this sense, it shares many of the themes of the cognitive behavior therapies. Reality therapy is an integrative approach because many of the skills are compatible with cognitive therapy, motivational interviewing, narrative therapy, solution-focused brief therapy, dialectical behavior therapy, and Adlerian therapy. The techniques of reality therapy are action centered, cognitive centered, and affective centered (Wubbolding, 2017).

The core of reality therapy is learning how to make better and more effective choices and gaining more effective control of our lives. Although the past may have contributed to a current problem, the past is not the focus of therapy. Regardless of what occurred in the past, to function effectively people need to live and plan in the present. As critical as it is for clients to understand where they've come from and to appreciate some of the motivations for their present behavior, this is only part of the story. Understanding the context of early childhood decisions may be useful to clients in getting a better picture of their development, but self-understanding needs to lead to action if clients are to make significant behavioral change. Insight without action will not help clients make life-changing decisions. Reality therapy practitioners focus on what clients are able and willing to do right now to change their behavior. Therapists consistently ask clients to evaluate the effectiveness of what they are choosing to do to determine if better choices are possible. Skillful questioning and various behavioral techniques are employed to help clients make this self-evaluation. The counselor assists clients in making plans to change those behaviors that clients determine are not working for them.

For further discussion of reality therapy and choice theory, see Corey (2017, chap. 11), Frew and Spiegler (2013, chap. 10), Glasser (1998, 2001), Neukrug (2018, chap. 11), Sharf (2016, chap. 11), and Wubbolding (2011, 2013, 2016, 2017).

Solution-Focused Brief Therapy

Solution-focused brief therapy (SFBT) is a brief, future-oriented, goal-directed, and pragmatic approach to counseling and therapy. I particularly value SFBT's optimistic assumption that people are healthy, competent, resourceful, and possess the ability to construct solutions and alternative stories that can enhance their lives. The philosophy of this approach rests on the assumption that people can become mired in unresolved past conflicts and blocked when they focus on past or present problems rather than on present and future solutions. SFBT therapists emphasize strengths and resiliencies by focusing on exceptions to their clients' problems and on creating solutions rather than talking about these problems. Some common techniques that are a part of my integrative approach include the use of miracle questions, exception questions, scaling questions, homework, and summary feedback. Some therapists ask the client to externalize the problem and focus on strengths or unused resources. Others challenge clients to discover solutions that might work.

Solution-focused practitioners adopt a position of curiosity and interest—even fascination—to encourage clients to become the experts on their own lives. In this approach, the therapist-as-expert is replaced by the *client-as-expert*, especially when it comes to what the client wants in life. The role of the counselor is to help clients recognize the resources they already possess, such as resilience, courage, and ingenuity.

Practitioners with a solution-focused orientation tend to engage clients in conversations about what is going well, future possibilities, and what will likely lead to a sense of accomplishment. The focus is on making small steps that can lead to large changes. Therapists often ask clients: "Tell me about times when your life was going the way you wanted it to." These conversations illustrate stories of life worth living. On the basis of these conversations, the power of problems is lessened and new directions and solutions are made possible. A key concept is, "Once you know what works, do more of it." If something is not working, clients are encouraged to do something different. With this approach, clients are involved in the therapeutic process from beginning to end, which increases the chances that therapy will be successful.

For a further discussion of SFBT, see Corey (2017, chap. 13), de Shazer (1985, 1988, 1991), de Shazer and Dolan (2007), Murphy (2015), Neukrug (2018, chap. 13), Prochaska and Norcross (2018, chap. 15), and Sharf (2016, chap. 12).

Motivational Interviewing

Motivational interviewing (MI) is a humanistic, client-centered, directive counseling approach that was developed by William R. Miller and Stephen Rollnick in the early 1980s. Over the past 25 years, clinical and research applications of motivational interviewing have grown at a remarkable pace. The strategies and skills in MI involve using

open-ended questions, employing reflective listening, expressing empathy, affirming and supporting the client, bolstering client self-efficacy, avoiding confrontation, summarizing and linking at the end of sessions, and eliciting and reinforcing *change-talk*. Therapists guide clients to make a commitment to change and help them clarify changes they want to make in their lives (Dean, 2015; Naar & Safren, 2017).

MI is rooted in the philosophy of the person-centered approach, but with a "twist." The twist is that, unlike the unstructured person-centered approach, MI is deliberately directive and has the specific goals of reducing ambivalence about change and increasing intrinsic motivation. In both person-centered therapy and MI, the counselor provides the conditions for growth and change by communicating attitudes of accurate empathy and unconditional positive regard. Naar and Safren (2017) demonstrate how MI strategies can boost client motivation and strengthen the therapeutic relationship, especially when combining motivational interviewing with cognitive behavior therapy. MI has been used to improve outcomes in the practice of cognitive behavior therapy for a range of problems such as anxiety, substance use, and eating disorders (Antony, 2019).

Like solution-focused therapy, MI emphasizes the *relational context* of therapy, which is known as the "MI spirit." This MI spirit involves the counselor establishing a collaborative partnership with the client, drawing on the ideas and resources of the client, and preserving the autonomy of the client (recognizing that all choices ultimately rest with the client rather than with the counselor). MI puts the individual's choices at the forefront of therapy. Practitioners of motivational interviewing believe that change is more likely to occur in a relationship that is collaborative and egalitarian. In MI, the therapeutic relationship is crucial in achieving successful outcomes. It is crucial for therapists to understand and act in accordance with the spirit of MI rather than simply implementing techniques and strategies. MI is much more than a compendium of techniques; it is about a style of interacting with people (Naar & Safren, 2017). MI emphasizes the internal frame of reference of clients, their present concerns, and discrepancies between their values and behavior. MI activates clients' own motivations for change and their desire to adhere to treatment goals. Counselors use ambivalence as a pathway toward assisting clients in making changes that are in accordance with their own values and goals. Clients are encouraged to become their own advocates for change and the primary agents of change in their lives.

MI stresses client self-responsibility and promotes an invitational style for working cooperatively with clients to generate alternative solutions to behavioral problems. MI therapists avoid arguing with clients because they realize that it creates defensiveness in clients, which increases resistance (Dean, 2015). They understand that resistance is the client's best effort to cope with difficulties. Therapists roll with resistance, express empathy, and listen reflectively. By viewing resis-

tance as something that emerges from the interaction between clients and counselors, the functions resistance often serves are illuminated and practitioners can adapt their approach to it. MI therapists do not view clients as opponents to be defeated but as allies who play a major role in their present success.

Miller and Rollnick (2013) describe the five basic principles of motivational interviewing:

1. MI practitioners strive to experience the world from the client's perspective without judgment or criticism. MI practitioners listen carefully to the client's goals for change; reflective listening is a way for practitioners to better understand the subjective world of the client. Goals can be established in the cognitive, affective, behavioral, and interpersonal realms. In setting goals, it is important to establish ways to assess outcomes, so that the client knows when outcomes are successful (Dobson & Dobson, 2017).
2. MI is designed to evoke and explore ambivalence. Counselors pay particular attention to the clients' arguments for changing compared to their arguments for not changing. Change is not easy, and there are good reasons to remain as one is as well as to make life changes. Therapists elicit and reinforce *change-talk* by employing specific strategies to strengthen discussion about change. MI practitioners encourage clients to determine whether change will occur, and if so, what kinds of changes will occur and when.
3. MI therapists strive to enhance client agency about change and are interested in their clients' ideas about change. MI practitioners believe in clients' abilities, strengths, resources, competencies, and inherent ability to make decisions. They are mainly interested in exploring what is working well for clients rather than dwelling on their problems.
4. Although clients may see advantages to making life changes, they also may have many concerns and fears about changing. During the early phase of the MI process it is common for clients to express ambivalence about changing, and individuals may have insufficient motivation to do what is necessary to bring about change. The therapist understands and respects ambivalence, demonstrating compassion for the client. Priority is given to understanding both sides of this ambivalence from a client's perspective.
5. When clients show signs of readiness to change through decreased resistance to change and increased talk about change, a critical phase of MI begins. In this stage, clients may express a desire and the ability to change, show an interest in questions about change, experiment with possible change actions between therapy sessions, and envision a future picture of their life once the desired changes have been made. Therapists shift their focus

toward strengthening clients' commitments to change and helping them implement a change plan. It is critical for therapists to make an accurate assessment of a client's readiness for change. Once clients give evidence of being motivated to change, the role of the therapist is to assist clients in developing their own plan of action. A clinician might ask, "So, what do you see as your next step?" When clients formulate their own plans, with the help of the therapist, they increase their self-efficacy. The plan for change involves strengthening commitment, supporting change, and supporting persistence. A therapist's confidence in a client's ability to change tends to support and build self-confidence (Dobson & Dobson, 2017).

For a comprehensive discussion of MI, see Miller and Rollnick (2013). For an excellent book that shows how integrating motivational interviewing and cognitive behavior therapy can lead to better client outcomes than using either approach on its own, see Naar and Safren (2017).

Drawing on Feminist and Family Systems Approaches

Both feminist and systemic therapies are based on the assumption that individuals are best understood within the context of their relationships. An individual's problems cannot be understood by focusing solely on the individual's internal dynamics. Rather, the individual's dysfunctional behavior grows out of the interactional units of the family, the community, and social systems. Therefore, solutions to an individual's problems need to be designed from a contextual perspective.

Feminist Therapy

Feminist therapy is generally relatively short-term therapy aimed at both individual and social change. At the individual level, therapists work to help women and men recognize, claim, and embrace their personal power. As a consciously political enterprise, feminist therapists work toward social transformation. The major goal is to replace the current patriarchal system with feminist consciousness and thus create a society that values equality in relationships, that stresses interdependence rather than dependence, and that encourages women to define themselves rather than being defined by societal demands.

Feminist therapists are committed to actively breaking down the hierarchy of power in the therapeutic relationship through the use of various interventions. Some of these strategies are unique to feminist therapy, such as gender-role analysis and intervention, power analysis and intervention, assuming a stance of advocate in challenging conventional attitudes toward appropriate roles for women, and encouraging clients to take social action. Therapists with a feminist orientation understand how important it is to become aware of typical gender-role messages clients have been socialized with, and they are skilled in helping clients identify and challenge these messages.

Feminist therapy principles and techniques can be applied to a range of therapeutic modalities such as individual therapy, couples counseling, family therapy, group counseling, and community intervention.

Brown (2018) contends that feminist therapy is an integrative approach because practitioners typically draw upon a diverse range of interventions from other theoretical orientations. Techniques are drawn from psychodynamic, cognitive behavior therapy, mindfulness-based approaches, and humanistic therapies. Interventions from other models include role playing, bibliotherapy, psychoeducation, assertiveness training, behavior rehearsal, cognitive restructuring, psychodrama techniques, identifying and challenging untested beliefs, and journal writing. Interventions are tailored to enhance clients' strengths and to assist them in gaining personal power. Therapists collaborate with clients on the goals of therapy and create a strong working alliance. The primary goals of empowerment, egalitarianism, and an analysis of power, gender, and social location are always kept in mind. According to Brown, the intent is to shift power and privilege to the voices and experiences of those who come to counseling, and away from those who deliver it.

Family Systems Theories

Few individual counseling theories place a primary focus on the role of systemic factors, especially the family, in *influencing* the individual. Operating within the mind-set of family systems therapy, it is important to inquire about the kind of rules clients experienced in their family. Family rules may be spoken or unspoken, but they are powerful influencers on how we think, feel, and act. These rules, which are often couched in terms of "shoulds" or "should nots," become strong messages that govern interactions within a family. Understanding the influence of our family illuminates how we grow up with rules such as these: "Never be angry with your father." "Always keep a smile on your face." "Don't bring attention to yourself." "Never let people see your weaknesses." "Don't talk back to your parents; always try to please them." "Don't talk to outsiders about your family." "Children are to be seen but not heard." "Have fun only when all the work is finished." "Don't be different from other family members." As a child, you may have decided to accept a rule and live by it for reasons of both physical and psychological survival. However, such a pattern may be self-defeating when carried into your adult interactions.

Incorporating concepts from the client's external world is of paramount importance in my integrative approach. Concepts from feminist and family systems approaches add an essential dimension to understanding how individuals can best change by addressing both their internal and external worlds. In my approach, I deal with the systemic (family, community, cultural) variables that contribute to an individual's core problems and draw on these factors as resources to foster the change process.

For further discussion of feminist therapy and systemic therapy, see Bitter (2014), Brown (2018), Corey (2017, chap. 12, chap. 14), Remer (2013), and Sharf (2016, chap. 13, chap. 14).

Becoming the Client: Becoming the Person You Want to Be

Imagine yourself once again as my client as you learn the power of an integrated approach. Awareness is an important first step in the process of changing your ways of thinking, feeling, and behaving. In the early stages of our work together, my focus is on increasing your awareness of your problems and your options for making substantive changes in your life. As therapy progresses, you will have many opportunities to explore the "shoulds" and "shouldn'ts," the "dos" and "don'ts," by which you live. Once you have identified and become aware of these "internalized voices," you will be in a better position to critically examine these messages to determine whether you want to continue living by them.

In the action-oriented phase, I concentrate on helping you translate your insights into action by solving problems and making decisions. Together we will consider possible alternatives and their consequences, evaluate how these alternatives will meet your personal goals, and decide on a specific course of action. The best alternatives and new possibilities for action are ones you are able to generate yourself.

A major part of the counseling process at this time consists of making new decisions based on the expanded information you have acquired in the therapeutic venture. I will ask you to *catch yourself* repeating old patterns that have led to ineffective behavior. I encourage you to set tasks for yourself and do something specific about dealing with your problems. By taking action, your new insights lead to concrete actions. Small steps can lead the way to significant changes.

Operating from my integrative approach, I rely on an Adlerian technique of encouraging you to *act as if* you are the person you want to be, which can serve to challenge your self-limiting assumptions. Spend a few minutes thinking and imagining the person you want to be. Let yourself identify specific characteristics you would have if you actually were the person you are striving to become. Pick a week when you are willing to act *as if* you are this person in a setting, such as home, school, or work. At the end of the week, ask yourself what progress you are making in getting closer to becoming your ideal self. What might be stopping you from becoming the person you want to be?

Working With Petra From an Integrative Perspective

Factors in Case Conceptualization With Petra

Petra is a 39-year-old married woman with two teenagers, coming to therapy for the first time with some anxiety and a host of somatic complaints. She lives with her husband (Rudy, 44) and their children (Robert, 19; and Mark, 17). Petra is the oldest of five children. Her father is a philosophy professor, and her mother is a housewife. She describes her father as distant and authoritarian; as a child, her relationship with him was one of unquestioning adherence to his standards. She

remembers her mother as being critical, and Petra thought she could never do enough to please her mother. In many ways, she took on the role of caring for her younger brother and sisters, largely in the hope of winning the approval of her parents. To a large extent, this pattern of taking care of others has extended throughout her life.

Petra reports general dissatisfaction with her life, seeing it as rather uneventful and predictable. She feels some panic over reaching the age of 39, wondering where the years have gone. For 2 years she has been troubled with a range of psychosomatic complaints, including sleep disturbances, anxiety, dizziness, heart palpitations, and headaches. At times, she must push herself to leave the house. Petra complains that she cries easily over trivial matters, and often feels depressed. Petra reports that she has recently undergone a full medical evaluation, and that medical causes have been ruled out.

Petra has recently become aware that her life is consumed by taking care of others. Living this way leads her to overlook many of her own needs and desires. She has not cultivated healthy interdependence in her relationships and realizes that she plays a role of "superwoman" in all aspects of her life. She feels that she gives abundantly in all of her relationships, to the point that she feels empty. She has a difficult time asking others to attend to her needs. She tries to be the good wife and good mother that her family expects her to be—and that she expects herself to be.

Exploring Family Rules, Messages, and Early Childhood Decisions

Petra and I spend some time identifying and exploring family rules and messages. She comes up with as many family rules as she can recall in growing up as a child. She recollects parental messages such as these: "Don't think for yourself." "Follow the church obediently, and conform to God's will." "Never question what you have been taught." "Live a moral life." "Don't get close to people." "Always be proper and appropriate." We spend time identifying and dealing with gender-role messages Petra still struggles with such as these: "Your main concern should be your family." "Don't put your career needs before what is expected of you as a wife and mother." "Defer to what men want." "Always be ready to nurture those who need care and attention."

In working with Petra's early childhood decisions, I borrow concepts from both the psychoanalytic and family therapy models. The psychoanalytic approach emphasizes reconstructing the past and working through early conflicts that have been repressed to resolve these unconscious conflicts. In some approaches to family therapy, Petra would work to understand family behavior patterns three generations back to unravel patterns and emotional baggage she may have acquired from her family background. An enormously helpful tool to begin this work is to assist Petra in creating a genogram.

I employ a directive and action-oriented approach in working with Petra's early childhood decisions. Functioning as a teacher, attention

is given to what she can learn that will lead to changes in the way she is thinking, feeling, and behaving. Once we have identified some of the major messages she has internalized, I ask her to begin thinking about the decisions she made about herself, others, and the world. I also ask her to reflect on the direction in which the decisions she has made are taking her. Petra sees with increasing clarity that she has lived much of her life in ways that were designed to get her father's approval. She feels that unless she gets her father's acceptance and approval she will not feel good about herself. She reasons that if her own father could not love her then nobody ever could. Drawing from Adlerian and cognitive behavioral concepts, I proceed by getting her to look at themes in her life and by guiding her in critically evaluating several faulty assumptions she continues to make.

At this stage of her therapy, Petra is beginning to challenge her thinking and her value system. She is raising questions about the meaning of her life. She is looking at beliefs and values she has accepted to determine if she still wants to base her life on them. Does she want to spend the rest of her life in a futile attempt to "win over" her father? What will it take for her to finally gain her father's acceptance and love? —if this is possible. What might she think of the person she must become to gain his acceptance? Pursuing these questions gets her to *think* and *decide* for herself on her standards for living.

Petra's Existential Quest

From the existential approach, I draw upon themes in Petra's life for exploration, especially how her values are influencing her way of life. Because Petra has let me know that she wants to reconsider her value system, I have an investment in assisting her in this process. Our counseling requires her to carefully consider what part of her value system she wants to retain and what she wants to modify. The experiential and relationship-oriented approaches emphasize the importance of Petra critically examining her values so that these values can become her own. Authenticity consists of living by values she chooses rather than living by unexamined values given to her by others.

The goal of our therapeutic endeavor is for Petra to become increasingly capable of making self-directed choices and influencing the quality of her future through her choices. She is challenged to re-create herself through her projects. Much of our work together involves my inviting her to raise key questions about her existence, such as "Who am I?" "Who have I been?" and "Who do I want to become?"

A basic existential premise that I accept in my relationship with Petra is that she is not the victim of circumstances. To a large extent, she is what she has chosen and is choosing to be today. For much of her life, Petra has lived a restricted existence. She has tended to see few options for dealing with life situations, and she often reports feeling trapped or helpless. One of my central tasks is to invite her to examine the ways she is describing living a restricted existence.

I encourage Petra to reflect on the direction of her life, to recognize her range of alternatives, and to make decisions without any firm guarantees about the future. Once she becomes aware of factors in her past and of stifling modes of her present existence, she can begin to accept responsibility for changing her future. As she recognizes some of the ways in which she has passively surrendered control, she can start on a path of consciously shaping her own life and designing the future she wants.

Feminist Perspective

The feminist therapy model provides me with a valuable lens to understand some of Petra's core struggles. The intervention of gender-role analysis is especially useful, which can be employed to increase her insight about how societal gender-role expectations have adversely affected her and to help her understand how women and men are socialized differently. Through this process, Petra will learn to identify gender-role messages (verbal, nonverbal, and modeled) she has experienced in her lifetime. Through her therapy, Petra will be in a position to examine the socialization messages she received from society as a whole, from her family of origin, and from her religion. As a result, she will learn that many of her conflicts about her life and identity are due to the fact that she wants to step outside her traditionally defined female gender role. She is now able to identify the positive and negative consequences of following those gender-role messages. Petra is learning how she has internalized certain gender-role messages in conscious and unconscious ways. My hope is that she will be able to acquire a full range of behaviors that are freely chosen rather than those prescribed by gender-role stereotypes.

Encouraging Petra to Act

As much as possible, I structure situations in the therapy session that will facilitate new decisions on Petra's part. It is important that she commit herself to some course of action aimed at changing herself and bringing about environmental change. Here I like the reality therapy emphasis on getting Petra to decide on a plan of action and then making a commitment to carry it out.

Therapy is a place of safety where clients can experiment with new ways of being to see what behavioral changes they really want to make. Petra has learned that insight without action is incomplete. She must apply the lessons learned in therapy to real-life situations. I consistently encourage her to carry out homework assignments that challenge her fears and inhibitions in a variety of practical situations. Because Petra sincerely wants to be different, we use session time for role playing and behavioral rehearsal. Then I ask her to try out her new learning in different life situations, especially with her family. For me, translating what is learned in the sessions into daily life is the essence of what therapy is about.

A Basis for Selecting Techniques With Petra

As a basis for selecting techniques to employ with Petra, I look at her as a thinking, feeling, and behaving person. I work with these three dimensions in an interactive fashion rather than in a linear fashion. Thus, I do not work with Petra's cognitions, then move ahead to her feelings, and finally proceed to behaviors and specific action programs. All of these dimensions are interrelated. When I am working with her on a cognitive level (such as dealing with decisions she has made or one of her values), I am also concerned about the feelings generated in her at the moment and about exploring them with her. Cognitive and emotive dimensions have an interactive influence on actions. Therefore, we explore how her thoughts and feelings are influencing what she is doing and also what she can do differently. Petra can try out new behaviors in the session and practice new skills that she can apply to problems she encounters in daily life. As a basis for this integrative style, I draw on the experiential therapies, which stress expressing and experiencing feelings; on the cognitive therapies, which pay attention to Petra's thinking processes, affecting behavior and beliefs; and on the action-oriented therapies, which stress the importance of creating a plan for behavioral change.

Moving Toward Termination

Petra is starting to think more about getting on with her life without formal therapy. She has learned that she can make new decisions and that she can follow her own lead. Because there are many tasks to accomplish as part of the termination process, more than one additional session will be needed. I ask Petra to think about what she has learned from her counseling and to develop a plan of what she would like to continue doing when she no longer comes in for counseling. In attending to the various aspects of termination, I draw from both the cognitive behavioral and experiential approaches in assisting Petra to move in this new direction in her life. This consolidation process will help Petra put a new perspective on what she is taking away from her therapy experience, and it will continue to guide her work once therapy sessions have ended.

Reflection Questions

Reflect on the case conceptualization with Petra from an integrative perspective. Consider these questions:

- What value do you see in conceptualizing Petra's case from an integrative perspective?
- Petra identifies some messages and early childhood decisions. If you were to select concepts from two different theories to explore these themes, what theories would you draw from and why?
- How could you apply concepts or techniques from a feminist perspective in working with Petra?

Couples Counseling in an Integrative Way: Lani and Rob

The Focus on Working With a Couple

Rob and Lani sought couples counseling after one year of dating and 6 months of marriage. They met in a gym where they frequently worked out. They are both outdoor enthusiasts and share many other interests and goals; they began dating exclusively and married shortly thereafter. Their presenting problems are communication issues, differences in daily lifestyle values, and wanting to explore ways of resolving and better managing their current conflict patterns.

Rob describes himself as a 39-year-old White male, employed steadily as a software engineer with the same company for 17 years. Rob and Lani look forward to having his large group of friends and their families over to watch sports and BBQ in addition to either hiking, running, or kayaking most weekends. Lani is a 30-year-old full-time college student in nursing school who works part-time as a care provider in a nursing facility during the week. Growing up in an Asian American family, the eldest of four siblings, Lani moved several times during her childhood. She states that her parents had a volatile relationship, often requiring her and her brothers to intervene on their mother's behalf.

A recent argument that resulted in a short-term separation of 2 weeks prompted Lani and Rob to seek counseling. There are no significant medical, substance, or legal issues, and they both report being committed to their marriage. Rob believes Lani needs to begin to settle down now that she is married and 30 years old, spend more time at home during the week rather than "being all over the place," such as school, the library, at work, or with friends. Rob reports feeling unappreciated, but when he approaches her about this she becomes defensive, angry, and often explosive, leaving him no choice but to retreat and sometimes leave. He is confused about Lani "being so intelligent, yet unable to understand" his position, and he feels that her "unstructured and disorganized" lifestyle is a reflection of her lack of respect for him and their marriage.

Lani states that Rob is set in his ways and inflexible, expecting her to become like him. She was a student when they met, and he used to enjoy the fast-paced excitement of her life. She sees him becoming critical, not able to see her side of things, and less willing to help her with family issues when they arise. When they argue, Lani admits she fights just like her parents did. She doesn't know how to get Rob to listen to her instead of him "lecturing" her. Lani states feeling "devastated" when Rob resorts to leaving, fearing he doesn't really love her and that he will eventually leave for good.

Although the arguments are increasing in frequency and periods of separation are becoming longer each time, they report still loving each other, wanting to work out the difficulties in their relationship, enjoying the positive aspects of their relationship, and believing they have a strong commitment to each other.

Applying an Integrative Approach in Couples Therapy

Because Rob and Lani appear to have the capacity for insight and are committed to change, I devote the initial sessions to establishing a safe and trusting environment where they can openly explore and share thoughts and feelings about their relationship. As a foundation for working with this couple, I rely on the person-centered approach to establish a trusting relationship in our therapy. I listen to Rob's and Lani's concerns and attempt to understand their perception of their relationship. They seem open to gaining increased insight and awareness. Rather than quickly trying to solve their problems, I strive to demonstrate that I can hear their individual concerns as well as their struggles as a couple.

I use a number of cognitive behavior techniques to assist Rob and Lani in identifying inaccurate beliefs and encourage them to practice replacing these nonconstructive beliefs with new and more functional beliefs. Rob was able to see how Lani's busy and sometimes erratic schedule during the week does not mean he is not important or appreciated. Lani challenged her belief that when Rob withdraws during times of conflict, this does not imply that he no longer loves or appreciates her.

I employ behavioral approaches in teaching this couple how to practice active listening skills and acquire positive communication patterns. I teach assertiveness training techniques and initiate behavior rehearsals in our sessions. I collaboratively create homework assignments for Lani and Rob to carry out between our therapy sessions.

Rob and Lani are good candidates for solution-focused brief therapy. I encourage them to evaluate the main changes each of them want in their relationship. My work with them rests on the premise that each of them has a good sense of what has or has not worked in the past, as well as what might work in the future. I want Lani and Rob to become better observers of their interactions and to report what might have gone differently. I teach this couple that they can become mired in unresolved past conflicts and become blocked when they focus on past or present problems rather than on present and future solutions. I emphasize their strengths and resiliencies by focusing on exceptions to their problems. Rather than putting the focus on their problems, I encouraged each of them to pay attention to small steps that each of them might have made during the week to move their relationship in a positive direction. I frequently ask at the beginning of a session: "What did you do differently since we last met? What has changed?" I realize that their relationship is not static, so I encourage them to reflect on what each of them did to make their relationship better. "Tell me about times this week when you enjoyed each other's company."

Motivational interviewing is helpful in counseling this couple. I work with Rob and Lani in the presence of the other in examining their ambivalence regarding change. They each want to make changes in the way they relate to each other, but they often rely on old patterns when they become anxious. Motivational interviewing is instrumental in helping this couple understand that, like most people, they vacillate between

wanting to change problematic behaviors and at the same time having a desire to continue problematic behaviors. There is a discrepancy between what they claim they want in their marriage and what they are actually doing. For example, there is a discrepancy between their ineffective ways of dealing with conflict and being able to cope with conflict situations in a constructive manner. I find it useful to ask questions about specific ways they would hope their situation might change. When they state how they would like to relate to each other, I ask them what they can do to bring this about. I offer support for new decisions and challenge them to establish specific goals and new directions they are able to agree upon.

Creating an Action Plan as a Couple

Both Lani and Rob are committed to making changes in the shared value and investment in their marriage, but they need to develop specific action plans to bring about the changes they want. Both Rob and Lani are learning important lessons through the process of couples counseling. Rob is learning to accept uncritically his frustration and discomfort during conflict while remaining committed to communicating openly as opposed to emotionally withdrawing or actually leaving. Lani is increasingly able to identify and accept her anxiety about Rob not loving her when he becomes upset. In counseling this couple, I use cognitive behavior strategies to assist them in becoming their own therapist. A large part of my work consists of educating the couple about the nature and course of their problem and about the process of cognitive therapy. The educative process entails providing Lani and Rob with information about *relapse prevention*, which consists of procedures for dealing with the inevitable setbacks they may experience once they *terminate* formal couples therapy. Toward the final phase of therapy with Lani and Rob, I use reality therapy procedures in developing an action plan. I assist them in designing an action plan they can use when they will no longer be coming for couples sessions, but the plan is based on what each of them wants, both individually and as a couple.

Reflection Questions

Reflect on working with a couple from an integrative perspective. Consider these questions:

- Several different theoretical approaches are described in applying some techniques from an integrative model. Which of these theories would you be most interested in building upon if you were to work with this couple?
- In what ways would your counseling approach differ in working with this couple from working with an individual client?
- What ideas do you have for assisting Rob and Lani in creating an action plan designed to help them make the changes they want to make in their relationship?

Study a Primary Theory—But Be Open to Integration

As you think about your clients and the therapy sessions to come, ask yourself this question: "*What* treatment, by *whom*, is the most effective *for this* individual with *that* specific problem, and under *what* set of circumstances?" (Paul, 1967, p. 111). Regardless of what model you may be working with, you must decide *what* techniques, procedures, or intervention methods to utilize, *when* to use them, and with *which* clients. You will most likely encounter a diverse range of client populations. For counseling to be effective, it is necessary to utilize techniques and procedures in a manner that is consistent with clients' values, worldview, life experiences, and cultural background. Although it is unwise and possibly unethical to stereotype clients because of their cultural heritage, it is useful to assess how the cultural context has a bearing on their problems. Some techniques may be contraindicated because of clients' socialization. Clients' responsiveness (or lack of it) to certain techniques is a critical barometer in judging the effectiveness of these methods.

A pivotal assessment question is, "What does this particular person most need in order to more effectively cope with his or her problems?" The techniques that you will use can best be determined by your assessment of the client, which is why it is important to integrate assessment with treatment. Once you know what your client's target problems and goals are, the next step is to design specific techniques that can be effective with that client, and preferably your techniques will be evidence-based. Wampold (2010) offers sound advice in stating that if clients do not respond to a particular approach, then the therapist must adjust the therapy accordingly. He adds that therapists need to be competent in providing multiple treatment approaches to accommodate variability in clients' characteristics, attitudes, values, and preferences. "The process of becoming an effective therapist is a lifelong pursuit involving deliberate practice. The goal is not to select the 'right' theory but to use a particular theory—or more likely theories—effectively" (Wampold, 2010, p. 113). Because diverse theoretical approaches all have something of value to offer, students and practitioners do well to remain open to thinking from an integrative perspective.

Benefits and Limitations of Integration

I believe it is needlessly restrictive to apply only a few techniques from a single theory to most clients. I like to incorporate a wide range of procedures in my therapeutic style. However, unless you have an accurate, in-depth knowledge of theories, you cannot formulate a true synthesis. You cannot integrate what you do not know. The integrative approaches have provided an effective response to the growing demand for short-term, evidence-based treatments (Norcross & Beutler, 2019; Norcross et al., 2016). Constructing your integrative orientation

to counseling practice is a long-term process. Becoming an integrative counselor is a lifelong endeavor that is refined with clinical experience, reading, reflection, and engagement with colleagues. The challenge for you is to think and practice in an integrative manner—but to do so critically.

It is important to avoid the trap of emerging with a hodgepodge of unamalgamated theories thrown hastily together. An integrative approach can be an excuse for sloppy practice—a practice that lacks a systematic rationale for what you actually do in your work. If you merely pick and choose without a rationale, it is likely that your selections will reflect your biases and preconceived ideas. At its best, however, integration can be a creative synthesis of the unique contributions of diverse approaches, dynamically integrating concepts and techniques that fit the uniqueness of your personality and style.

Concluding Comments

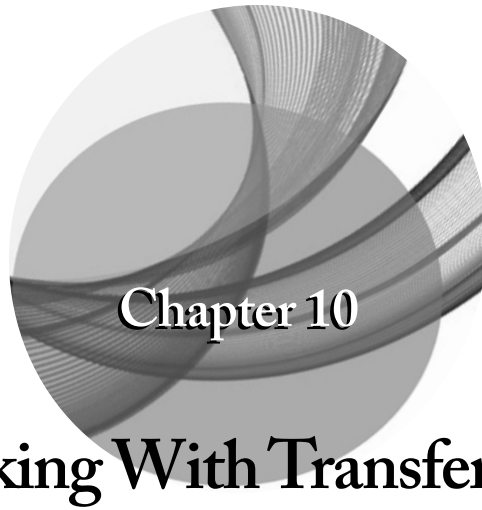
Throughout this book, I have presented the advantages of constructing a systematic, consistent, personal, and disciplined approach to integrating various elements in your counseling practice. Whatever the basis of your integrative approach to counseling, you need to have a basic knowledge of various theoretical systems and counseling techniques to work effectively with a wide range of clients in various clinical settings. Sticking strictly to one theory may not provide you with the therapeutic flexibility that is required to deal creatively with the complexities associated with clinical practice.

One reason for the trend toward integrative counseling is the recognition that no single theory is comprehensive enough to account for the complexities of human behavior, especially when the range of client types and their specific problems are taken into consideration. Consider the contributions of the various counseling models and work toward creating your own integrative perspective. If you are open to an integrative perspective, you may find that several theories play a crucial role in your personal approach. By accepting that each theory has strengths and weaknesses and is, by definition, "different" from the others, you have some basis to begin developing a counseling perspective that fits you.

Take time to reflect on the following questions as a way of clarifying your thoughts on using an integrative approach in counseling practice.

- What do you understand by the concept of an integrative approach to counseling?
- What are the differences between technical eclecticism and theoretical integration?
- What are some potential advantages of incorporating a diverse range of techniques from various theories?
- In what way is the client–therapist relationship a common denominator among the various theoretical models?

- What are the main benefits and limitations of an integrative approach?
- Is an integrative therapy approach a good fit for you?
- What have you learned thus far that will help you in designing your own integrative approach to counseling practice? What do you need to learn about approaches and methods in developing an integrative approach?



Chapter 10

Working With Transference and Countertransference

Transference occurs in many different kinds of relationships, but especially in intensive forms of therapy. Clients bring unresolved feelings with significant others into the therapeutic relationship and project them onto their counselors. Regardless of whether your theory explicitly makes room for concepts of transference and countertransference, it is my belief that these factors operate in most counseling relationships.

Contrasting Views of Transference

Transference includes any feelings clients project onto their counselor, whether the source of these feelings is in past or present relationships. Transference may be the result of the client's interpretation of the therapist's behavior and is not necessarily a distortion carried over from the client's past. This is common in the therapeutic process, and it is essential that you understand what transference means and that you know how to deal with it ethically and effectively. In developing your own integrative approach, you will need to conceptualize a way to understand both transference on a client's part and your reactions to the client's transference with you—or your countertransference.

Countertransference involves the therapist's reactions to a client's transference, and it may be influenced by the therapist's unresolved conflicts from other past or present relationships rather than being tied to any feature of the relationship with the client. The therapist's unconscious emotional responses to the client may result in a distorted perception of the client's behavior. In the broad sense, countertransference includes any projections that can potentially get in the way of the therapist helping the client.

In psychodynamic approaches, transference and countertransference are viewed as central to the therapy process. An unconscious process, transference typically has its origins in the client's early childhood, and it constitutes a repetition of past conflicts. Because of this unfinished business, the client perceives the counselor in a distorted way. When these reactions and feelings of the client are transferred to the counselor, the intensity of the feelings have more to do with unresolved issues in the client's life than to the present counseling situation. For example, the client may transfer unresolved feelings toward a stern and unloving father to the therapist, who, in the client's eyes, becomes stern and unloving. Or the client may develop a positive transference and seek the love, acceptance, and approval of an all-powerful therapist. A central part of psychoanalytic therapy involves the therapist becoming a psychological substitute for significant others.

Transference allows clients to understand and resolve unfinished business from these past relationships through the present relationship with the therapist. In essence, working through a similar emotional conflict in the therapeutic relationship counteracts the effects of a client's early negative experiences. Reflect on these questions: Do you remember times in your personal counseling when you experienced transference toward your counselor? What was this like for you? What did your counselor do that was helpful to you? If you have not been in personal counseling, think about transference reactions you have experienced toward authority figures such as teachers, employers, and supervisors. What were some of the reactions you had toward them? What might have been useful to you in understanding and dealing with these reactions so that you could perceive these individuals more realistically?

The psychoanalytic model offers the oldest perspective for grasping the implications of both transference and countertransference, and these concepts are a basic part of my integrative approach. Altman (2013) contrasts the classical psychoanalytic model with the contemporary relational model of psychoanalytic therapy. In classical psychoanalysis, the analyst remains outside the relationship and strives for a detached and objective stance from which to make observations and interpretations of the client's psychodynamics. In contrast, the immediacy of the therapeutic relationship is much more central in the contemporary relational approach to psychoanalytic therapy. The modern analytic model recognizes that the participation of the therapist in the therapeutic process cannot be eliminated.

All interventions that therapists make involve elements of their subjective experiences as well as their objective observations. Furthermore, clients influence the experience of therapists as much or more than therapists influence clients. In the contemporary psychoanalytic relational model, basic concepts such as transference and countertransference pertain to a more general mode of here-and-now interactions within the therapeutic situation.

Modern psychoanalytic therapists, therefore, divide many feeling responses into one of four categories: (a) positive feelings that result from an actual interaction within the counseling relationship; (b) positive transference, the result of projecting positive feelings from the unconscious onto the therapist; (c) negative feelings that result from an actual interaction within the counseling relationship; and (d) negative transference, the result of, again, projecting negative feelings from the unconscious onto the therapist. The key word is *projection*, a feeling coming from somewhere else and attached to the therapist or from the therapist to the client. If a feeling is an expected response to an experience in the counseling process, then it is not transference or countertransference; it is merely a feeling or a reaction. For example, if a client catches a counselor daydreaming for a few moments in a session, the client may say, "Are you listening to me? I feel really upset when you seem to zone out." This is not transference on the client's part. Anyone might feel the same way if the counselor was indeed zoning out. However, the reason the therapist may have been zoning out might significantly involve countertransference, depending on what stimulated the daydreaming in the first place.

As a general rule, psychoanalytic therapists talk about, but do not place much meaning on, positive or negative feelings. But transference and countertransference is another matter. Typically, therapists initially tend to let positive transference go because it helps to build a working alliance, but they would not let a session end without addressing negative transference.

Psychoanalytic practitioners consider the transference situation to be valuable because its manifestations provide clients with the opportunity to reexperience a variety of feelings that would otherwise be inaccessible. Transference also provides many clues to here-and-now interactions between therapist and client. Transference can be understood and worked with in other therapy systems as well. I mention this so that you do not get the picture that the only way of understanding and exploring transference in the therapeutic relationship is via the psychoanalytic route.

Reality therapy, cognitive therapy, rational emotive behavior therapy, and behavior therapy do not have a framework for addressing transference and countertransference. Reality therapy, as taught by William Glasser (1998, 2001), rejects the concept of transference. Since the inception of reality therapy, Glasser has consistently maintained that reality therapists should make no effort to be anyone but themselves. He contends that transference is a way for people to avoid taking personal responsibility in the present.

Rational emotive behavior therapists tend to take a dim view of working with transference phenomena. In REBT transference is not encouraged, and when it does occur, the therapist is likely to confront it. The therapist wants to show that a transference relationship is based on the irrational belief that the client must be liked and loved by the

therapist (or parent figure). Ellis believes that devoting any length of time to reliving earlier traumatic situations or exploring transference feelings is "indulgence therapy," which might result in clients feeling better but will rarely aid them in getting better (Ellis & Ellis, 2011, 2019).

Although cognitive behavior therapists may not use the terms "transference" and "countertransference," I think many of them recognize that these phenomena do occur in the therapeutic relationship, and they address these issues. According to Kazantzis, Dattilio, and Dobson (2017), cognitive behavior practitioners place considerable emphasis on the working alliance and a collaborative partnership. Ruptures in this working alliance are distressing for both the client and therapist because both are aware of this situation. When ruptures are due to what is going on in the here-and-now therapeutic interaction, CBT practitioners are generally inclined to discuss any factors that might be impeding the therapeutic relationship. If this discussion is avoided, meaningful therapeutic work will likely be limited.

The Connection Between Transference and Countertransference

Your clients will bring past reactions to significant others with them and place them on you in the present. A client may view you with a mixture of positive and negative feelings, and at different times the same client may express love, affection, resentment, rage, dependence, or ambivalence; indeed, this is the therapeutic definition of some personality disorders, especially when the difference in feeling expression happens within a matter of minutes. Transference can be a path that enables clients to gain insight into how they operate in a wide range of relationships. Clients may experience transference with many people, not just with you as their counselor.

The psychoanalytic perspective holds that it is essential for you as a therapist not only to know your clients, but also to know yourself. One way of attaining this self-knowledge is to monitor and reflect upon your behavior in the context of the here-and-now interactions with your clients. Be attentive to your own reactions toward clients and how you work with them therapeutically. The transference reactions your clients have toward you will very likely evoke reactions in you as a counselor. These reactions can become problematic if they result in countertransference. It is important to be alert to the possibility of countertransference so that "your problem" does not become your client's problem. However, if you can recognize and monitor your countertransference, this phenomenon does not have to be a destructive aspect in the therapeutic relationship. In fact, as Levenson (2017) points out, from the viewpoint of time-limited psychodynamic therapy, *interactive countertransference* can be viewed as a more universal reaction to the client's style dynamics. Levenson maintains that the therapist's internal and external reactions often provide important sources of information for understanding the client's lifelong dysfunctional pat-

terns and his or her dynamics. From this vantage point, the therapist's countertransference can be used to further therapeutic gains rather than being an impediment to the progress of therapy.

Working With Transference Therapeutically

Put yourself in the role of a counselor as you read this section. What kind of client behaviors will be particularly challenging for you? Reflect on a client's transference toward you and how you are likely to deal with these reactions. When you encounter clients whom you perceive as being problematic, do not put the exclusive focus on them. Rather than zeroing in on their problematic behaviors, reflect on your reactions to your clients. See what these reactions tell you, both about yourself and about your client.

Examples of Transference

This section describes some transference situations you are likely to encounter with clients whom you counsel. Ask yourself what your response might be to a client's feelings toward you, and what feelings are likely to be evoked in you.

Clients Who Make You Into Something You Are Not

Some clients will want you to be a parent substitute for them. They may see you as a savior who will take care of them and solve all their problems. They see you as an all-knowing person who will provide them with answers. Other clients may immediately distrust you because you remind them of a former spouse, a critical parent, or some other important figure in their life. Some clients will not let themselves get emotionally close to you because they feel that as children they were abandoned by people they cared about. Clients who feel their parents did not care for them and who felt abandoned may be very careful about letting you into their lives.

Clients Who See You as Perfect

A client may use superlative adjectives to describe you. Consider a client who sees you as always understanding and supportive. He is sure that you have the ideal family and cannot imagine that you might have any personal difficulties. He credits you with the changes he is making in his counseling with you. How would you deal with this client and all the adulation he is directing toward you?

Clients Who Become Excessively Dependent on You

Some clients may not make decisions without first finding out what you think. They want to call you at any time of the day or night. They may want to run over the allotted time for their session. Dependent clients may view you as all knowing and all wise. They are convinced that you have the right answers for them and at the same time never think of finding their own answers. They are likely to want you to affirm them and convince them that they are special in your eyes.

You can inadvertently foster client dependence in subtle ways. Even though you may intellectually understand the value of promoting autonomy, there may be payoffs for encouraging dependence. For example, if your clients depend on you and tell you how important your input is to them, this can reinforce your need for feeling important and for being needed. Do you have any needs that could be met by allowing certain clients to develop dependence on you? How can you monitor this situation?

Clients Who Are Not Able to Accept Boundaries

Some clients have problems understanding or accepting appropriate boundaries. Much of their behavior within the counseling relationship may be aimed at testing you so that they know how far they can go with you. Some of your clients may transgress therapeutic boundaries and want to enter into a friendship or develop some kind of social relationship with you. Wanting more than a client–therapist relationship can be a manifestation of transference. As children, these clients knew no boundaries, and now they are likely to be lost and to feel anxious because they are not certain where they stand with you. In working with such clients, it is essential not to fall into the trap of allowing them to treat you as they did a parent. One way you can do this is by being clear about your own boundaries. Know your role and function in the therapeutic relationship, and avoid relaxing appropriate boundaries to be liked by your clients.

Clients Who Displace Anger Onto You

Some clients will strike out at you with displaced anger. These clients are likely to tell you that because you are supposed to be helping them you have no right to express your own feelings. Recognize that you are probably getting more of this client's anger than you deserve. Avoid getting into a debate. If you take what you are getting too personally, you are bound to begin to react defensively, and possibly with countertransference.

Clients Who Fall in Love With You

Some clients will make you the object of their affection. They may see you as the ideal person and want very much to become a person just like you. They are convinced that they could find a resolution to their problems if only they found a person like you who would love and accept them. How might you respond to being the object of adulation? This may be an appropriate time for self-disclosure on your part. For example, if you feel uncomfortable with accolades being showered on you by clients, you can share with them that it is difficult to hear some of what they are telling you. For example, you might say: "I do appreciate your liking and valuing me, yet sometimes I become uncomfortable with it. At times, I sense that you perceive me as being without faults. By elevating me as high up as you do, it is likely that at some point I will fall far short of your expectations. That can be a very difficult

moment for both of us." It is extremely important for counselors to establish clear boundaries in dealing with a client's expression of affection or adulation. Counselors need to be aware of their own needs and not allow themselves to foster a client's feelings.

These few illustrations of transference behaviors demonstrate how essential it is for you to gain awareness of your own needs and motivations. Dealing with which of these transference behaviors would require the most skill? If you became entangled in a client's transference reactions, how open would you be to seeking supervision or personal therapy? If you are unaware of your own dynamics, you might tie into your clients' projections and get lost in their distortions. You are likely to avoid focusing on key therapeutic issues and instead focus on defending yourself. If you understand your own reactions to clients, you'll have a better frame of reference for understanding their reactions to you.

As you reflect on ways that you may be emotionally triggered in working with certain clients, think about how you are affected by those clients you perceive as being especially difficult. How do you respond to the different forms of transference? What kind of transference tends to elicit intense emotional reactions on your part? Do you take intense reactions of a client in a personal way? Do you blame yourself for not being skillful enough? Do you become combative with clients who project a range of feelings toward you?

When clients appear to work very hard at getting you to push them away, it can be therapeutically useful to explore what they are getting from this self-defeating behavior or how what they are doing serves them. You might say to such a client: "You know, I would very much like to work with you. Sometimes I get the sense that you are trying hard to get me not to like you. Are you aware of trying to get me to push you away?" Handled properly in the therapeutic relationship, clients can experience and express feelings toward you that more properly belong to others who have been significant in their lives. When these feelings are productively explored, clients become aware of how they are keeping old patterns functional in their present relationships.

It is a mistake to think that all feelings your clients have toward you are signs of transference. Clients may become realistically angry with you because of something you have done or not done. Their anger does not have to be an exaggerated response triggered by past situations. If you are consistently late for sessions, for example, clients may become upset with you over not giving them the time and respect they deserve. Their reactions may well be justified and should not be "explained away" as a mere expression of transference.

Likewise, clients' affection toward you does not always indicate transference. Perhaps some clients genuinely like your traits and enjoy being with you. You can err both by being too willing to accept unconditionally whatever clients tell you and by interpreting every-

thing they tell you as a sign of transference. In short, it is difficult to identify certain reactions as being rooted either in transference or in reality. Practice and experience will help you develop your intuitive abilities, making this discernment easier over time.

Addressing Countertransference Issues

The other side of transference is countertransference, and it occurs when therapists are triggered into emotional reactivity, respond defensively, or lose their ability to be present in a relationship because their own issues become involved. It is important for you to consider countertransference as a potential source of difficulty that may develop between you and a person with whom you are working. It is crucial for you to be aware of how your own problems may affect the quality of your working relationships with clients.

Simply having feelings toward a client does not automatically mean that you are having countertransference reactions. You may feel deep empathy and compassion for some of your clients as a function of their life situations. Countertransference occurs when your needs become too much a part of the relationship or when your clients trigger old wounds of yours. Just as clients may have some unrealistic reactions to you and may project onto you some of their unfinished business, you may have some unrealistic reactions to them. Your own vulnerabilities will be opened up as you are drawn into some of the transference reactions of those you help. Norcross and VandenBos (2018) suggest that practitioners would do well to "leave their clients at the office." Learning to manage our reactions to clients who exhibit problematic behavior is an essential aspect of self-care.

Countertransference can have both positive and negative effects on the counseling process. If you use your own feelings as a way of understanding yourself, your client, and the relationship between the two of you, these feelings can be a positive and healing force. Countertransference becomes problematic when it is not acknowledged, understood, monitored, and managed. Recognizing countertransference is the most important first step in learning how to manage it. Hayes, Gelso, and Hummel (2011) consider countertransference to be inevitable because all therapists have unresolved conflicts, personal vulnerabilities, and unconscious "soft spots" that are activated through their professional work. Monitor your feelings during therapy sessions, and use your responses as a source for increased self-awareness. Doing so will enable you to better understand your clients so you can help them come to a deeper understanding of themselves.

Countertransference is more likely to enter the counseling relationship when the counselor is having significant problems in other parts of her or his life; when stress is high enough to become distress; or when a particular client stimulates a "triggered" reaction that has been common in other parts of the counselor's life. This is why it is extremely

important for working counselors to have their own therapists and to have a place to process feelings and transference issues that arise. The most obvious countertransference, having sexualized feelings for a client, can be problematic, and acting on it is always unethical. The more subtle countertransference experiences, however, can be just as problematic, and processing them with one's own counselor can be meaningful, both personally and professionally.

Your countertransference reactions can teach you a great deal about yourself. Be alert to the subtle signs of countertransference, and do not be too quick to attach the blame for your reactions on your clients. For example, you may find that certain clients evoke a parental response in you. Their behavior can bring out your own critical responses to them. Knowing this about yourself will enable you to work through some of your own projections or places where you get stuck. If you become aware of certain symptoms—such as a strong aversion to certain types of clients or a strong attraction to other types of clients—seek consultation, participate in supervision, or enter your own therapy for a time to work out these personal issues that stand in the way of your clinical effectiveness.

Your own supervision is a central factor in learning how to deal effectively with both transference and countertransference reactions. Your blind spots can easily hamper your ability to deal with "difficult clients" or with your own old wounds. Focus on yourself in your supervision sessions rather than talking exclusively about a client's problem. Spend some time exploring your thoughts, feelings, and reactions toward certain clients. A good way to expand your awareness of potential countertransference is by talking with colleagues and supervisors about your feelings toward clients. This is essential if you feel stuck and don't quite know what to do in some of your sessions.

Self-knowledge is your most basic tool in dealing effectively with transference and countertransference. It is good to remember that being instrumental in the changes your clients are making will influence you. If you are reluctant to acknowledge your own personal problems, it will be difficult for you to challenge clients to work through their transference issues.

Personal Therapy as a Way to Understand Our Countertransference

Working with clients who manifest problematic behaviors can affect us in personal ways. Whether we are students in training, or new professionals, or seasoned therapists, it is of the utmost importance to recognize and deal effectively with our countertransference, which can usefully be explored in our personal psychotherapy. We all have personal vulnerabilities that are frequently triggered when we begin working with clients. Making use of personal therapy and clinical supervision can be most helpful to you in better understanding how your internal reactions influence the therapy process and how you

can use your countertransference reactions to further the progress of therapy (Hayes et al., 2011).

Another form of countertransference relates to becoming overly involved with a client. One colleague presents her trainees with this guideline to help them detect countertransfence: "When you find yourself working harder than your clients, this is a sign of your countertransference." When we begin practicing, it may be extremely difficult to psychologically leave our clients at the office, yet we need to learn how to do this if we expect to work effectively with our clients.

Becoming the Client: The Value of Personal Therapy

Instead of becoming my client as you did in earlier chapters, in this section I encourage you to consider the value of experiencing your own personal therapy. By becoming a client, you will learn about yourself personally and also acquire a great deal of knowledge and skill that you can apply as a professional. I believe it is important that you have the experience of being a client at some time, if for no other reason than to learn to appreciate the courage and persistence it takes to be on the receiving end of the therapy process. Yalom and Josselson (2019) address the value of personal therapy not only for students but for therapists at many stages of their career. Personal therapy enables student trainees to become aware of their own blind spots and permits them to experience the therapeutic process from the client's vantage point. In your personal therapy, you may experience transference and thus know firsthand how it is to view your therapist as a parent figure. Making use of personal therapy could be an extremely valuable resource, both as a way to resolve some of your personal issues and as way to increase your self-awareness and ability to work with others. Your own therapy provides you with an avenue to explore and process your countertransference reactions to clients.

Personal counseling is useful in helping you recognize how you may overidentify with certain clients, how you might meet some of your needs at the expense of your clients, and how control issues may be played out between you and a client. I am convinced that to the extent that you have not dealt with your past problems or unresolved interpersonal conflicts, these issues will influence your reactions to your clients. If you have not explored your own pain over certain events in your life, you may be carried away on a client's emotional tidal wave. Personal therapy provides an opportunity for you to achieve a balance between your personal and professional life (Corey et al., 2018). A commitment to working on your own current and past problems is a crucial part of self-care. It is essential to remember that your degree of self-care is directly related to your effectiveness as a therapist. Your personal therapy is not simply a way for you to resolve your problems; your personal therapy also instills practical wisdom and promotes human flourishing (Woolfolk, 2015).

There will always be residual feelings attached to certain events you've experienced in your life, and you will have your own sources of vulnerability. You should be aware of what your conflicts are and how they are likely to affect you as a counselor. It is critical for you to recognize when you are triggered. For example, if you have great difficulty in dealing with anger, chances are that you will do something to dilute these emotions when they occur in your clients. If you are extremely uncomfortable with conflict and find yourself withdrawing in the face of conflict, it is a good bet that you are likely to find ways to skirt around conflict, even when your clients bring up conflicts they are having outside of the session. If you have not allowed yourself to grieve over some significant loss, you may find it challenging to remain present with clients when they are dealing with feelings surrounding loss and grief. How can you be present for your clients and encourage them to express feelings that you are so intent on denying in yourself?

Participating in a process of self-exploration can reduce your chances of assuming an attitude of arrogance. Even if your program does not require any form of personal counseling, I hope you will find ways to avail yourself of therapeutic experiences. Through various therapeutic modes, such as individual and group counseling, you can come to a better understanding of how you operate in your interpersonal world. Indeed, experiencing counseling as a client is very different from merely reading about counseling theories and techniques. By reflecting on your own experience as a client, you can identify aspects of the counseling process that are foundational. You will learn what attitudes and behaviors of therapists facilitate working through resistance with clients. This personal and experiential dimension will enhance your knowledge and skill base in the process of becoming a counselor.

For more information on the topic of personal therapy for mental health practitioners, I recommend *The Psychotherapist's Own Psychotherapy: Patient and Clinician Perspectives* (Geller, Norcross, & Orlinsky, 2005) and *The Value of Psychotherapy: The Talking Cure in an Age of Clinical Science* (Woolfolk, 2015). For an in-depth discussion of counselor self-care, I recommend two books: *Leaving It at the Office: A Guide to Psychotherapist Self-Care* (Norcross & VandenBos, 2018) and *Counselor Self-Care* (Corey et al., 2018).

Working With Transference and Countertransference With Matt

Working With a Challenging Client

Matt is a 17-year-old, European American, heterosexual male. He comes from a family that does not encourage emotional expression, except those that are congruent with masculine gender norms. Matt has been in trouble with the law for the past 2 years and is currently court-ordered to participate in psychotherapy and outpatient substance abuse treatment for adolescents as part of his probation. Past experiences in therapy contribute to Matt's lack of trust in mental health

providers. A former therapist did not inform Matt about the limits of confidentiality, including what would be reportable to his parents. When he disclosed that he had been smoking marijuana, the therapist informed his parents without Matt's knowledge or permission. Currently, Matt is associating with a group of individuals who are engaging in various types of criminal behavior, along with regularly abusing substances and alcohol.

In the past 2 years, Matt has progressed from smoking marijuana to using methamphetamine, cocaine, and alcohol. His isolation and repressed emotions have led him to act out violently at school and in the community. He was recently informed that if he does not turn his life around, his current behavior would land him in a juvenile detention center or in prison (once he turns 18).

The Interplay of Transference and Countertransference

When Matt arrives in my office for his first session, he appears to be quite agitated. I can see that he notices the molding clay I put on the table in front of him. I invite him to use the clay while we talk to give his hands something to do so our time together can feel less like the interrogation he is likely expecting. He mumbles something and looks at me with disdain. I begin by giving Matt an overview of the components of this initial session, and I explain the ways this session will be different from our later sessions because we need to discuss business, legal, and ethical components. I devote the first part of the session to explaining my informed consent policy, which includes confidentiality and the limits of confidentiality. Knowing he is coming in with a history of substances, I explain that I may occasionally have to break confidentiality and involve his parents if I believe his safety is at risk. I invite him to ask me hypothetical questions of behavior that I would have to report, and I provide examples of potential situations based on his presenting details and my experience. An early area of my concern is his combined use of alcohol and cocaine, which can produce a synergistic effect with a real potential of death from a heart attack or stroke. I explain that I would strive not to break confidentiality without first informing him and that I would encourage and support him in making these disclosures to his parents himself in my presence. I tell him that his progress letter to the court will simply state the degree of his participation in therapy. Although I work for the "system," I tell Matt that my priority is his welfare and that I hope to help him learn to make more effective choices.

During this first session with Matt I am taken aback by his rudeness and vulgar language. He makes it clear that he does not need to be in therapy and that he sees me as part of the "system" intent on putting him in jail. He is certainly judging me, and possibly experiencing transference, without giving me a chance to be of any use to him. It is important that I monitor my potential countertransference of not being appreciated and being quickly dismissed. My immediate reaction is

that I am receiving the blame for all of his previous negative experiences, especially with authority figures. I need to remind myself that I am not responsible for his negativity and not to take his criticisms in a personal way. I use a person-centered approach with Matt as a way to attempt to understand his world and to create a working alliance. I am reminded that his resistance to treatment and being vulnerable has probably been a useful defense mechanism in his life. Therefore, I let Matt know that I hear and understand his concerns, feelings, and thoughts, even though I do not condone his self-destructive actions. While explaining how much the "system" has harmed him, Matt mentions his former therapist and the event of telling his parents about his marijuana use. I can understand the grounds for his anger and frustration and tell him, "That was frustrating and upsetting that your former therapist told your parents about smoking marijuana without even consulting you. I can see how you feel let down and betrayed." We talk about how he would like to have had a different experience with his former therapist.

Two very important things have come out of this encounter. First, I realize that I have the ability to empathize with Matt. I do not have to get lost in the quagmire of his lashing out at all therapists for being untrustworthy. I have an obligation to not let my personal reactions interfere with Matt's progress in therapy. What I see when I look at Matt is a person who put his trust in a professional and was disappointed. Second, Matt was taken aback by my comments that I could understand how he felt let down. He was expecting me to side with the former therapist and explain his obligation to tell his parents that he was involved in dangerous situations. Matt felt understood by my expression of concern for him, which helped in establishing our relationship.

By the end of the session, Matt is less angry and more likely to trust me. I can empathize with his reluctance to trust and be vulnerable. At this point, Matt is fully aware of those situations that will require me to breach confidentiality. Given these guidelines, he is in charge of what he is willing to disclose.

Therapeutic Interventions

In the beginning of therapy, I focus on building the relationship, gaining trust, and exploring Matt's goals. Over the next few months, I integrate multiple methods to help Matt achieve his treatment goals. Rather than relying solely on traditional talk therapy, I utilize an array of age-appropriate games and art activities during our sessions. I draw from the client-centered approach, and Matt chooses the activity we engage in while we talk. As is commonly seen with adolescents, Matt has an affinity for technology. I show Matt apps he can download on his phone to help with coping strategies such as an app for mindfulness relaxation exercises.

I also implement motivational interviewing techniques in my sessions with Matt. Despite building rapport and trust with him, he is

still reluctant to work on the behaviors that brought him to therapy. The first, and most important, technique I implement is "rolling with resistance." An important component of motivational interviewing is identifying goals by understanding the problem. Therefore, I ask questions such as, "Where do you see yourself in five years?" "Tell me what you think about your current legal troubles?" and "How do you think these sessions can be most helpful?" I also draw from reality therapy in focusing on what Matt wants and the direction that his current behavior is taking him. Is what he is doing working for him? What does he want out of life? In the spirit of reality therapy, I do not evaluate his behavior but encourage him to evaluate the degree to which his present actions are getting him what he wants in his life.

After the first several sessions, Matt is beginning to become more vulnerable and honest with me about his feelings. I affirm his progress and any feelings that he brings to the session. During these interactions I try to encourage "change-talk" by exploring the problem with Matt and then moving on to what life looks like without the problem. Slowly, Matt starts to express his desire to get out of the legal justice system. I encourage him to elaborate and then summarize what he tells me so we are both on the same page.

Matt and I collaboratively design homework assignments. He expresses a dislike for writing, so we modify journaling to best meet his needs. Instead of writing his thoughts and feelings, Matt decides to journal through the use of art and symbols. This method produces some rich material and insights for a while. However, 2 weeks have now gone by since Matt has touched his art journal. I gently inquire about what is possibly getting in his way of doing this work, and he suddenly blows up at me. His body becomes tense, and he yells: "You just keep pushing and *pushing*! . . . this is *stupid*! . . . *you* are stupid! . . . *Nothing* I ever do is good enough for you!!!!" His rage transitions to tears, and we just sit together for a while. We both become acutely aware that Matt's outburst has little, if anything, to do with me, as I have always approached him with empathy, compassion, and unconditional positive regard. We are able to process this transference reaction, discovering that it is more about some unresolved feelings toward his father that he has directed toward me. As painful as this experience is, we use this as a gift and an opportunity to explore and eventually heal old wounds.

After several months, Matt opens up and tells me about his experiences of being sexually abused by a neighbor when he was 8 years old. No one was aware of the abuse, and Matt has been troubled by keeping this situation a secret. Although Matt wishes to break his silence and talk about what happened, he fears that nobody would understand and is worried about what people would think of him if he reveals this secret. I am emotionally present and attend to his sharing this difficult time in his life. I let him know that hearing this saddens me and that it is also sad to hear about how he is tormented by what happened to him. By my responding in a nonjudgmental and accepting way, Matt feels a shift in his attitudes.

I provide him with some psychoeducation based on cognitive behavior therapy (CBT) to explain how perpetrators of sexual abuse can make victims feel that they did something wrong by participating in the act. We also talk about the possibility that some people might not believe his story. We spend a lot of time on this topic, and by the end of the session, with Matt's knowledge, I inform him that I will call child protective services to file a suspected child abuse report regarding the sexual abuse from the neighbor. In an effort at transparency and to empower Matt, I ask him if he would like to participate in, or to be present, for the call. He declines to be present for the child protective services call. Matt mentions that at some point he would like to tell his parents about the event, and we discuss working toward a family session in the future in which I will be present to support him in his disclosures. We target his negative self-belief system over the next few months and work through topics of shame, anger, and trust.

If I had let my own anger and defensiveness get the best of me during our initial sessions, I simply would have been one more therapist who disappointed Matt. Instead, I utilized the therapeutic relationship, motivational interviewing techniques, reality therapy, and CBT to help Matt learn how he built walls of resistance, anger, and substance use to protect himself from being hurt. Matt is learning about the value of expressing and sharing painful events, which is giving him a new perspective. He is also discovering resources within himself that he did not know existed. We talk about his resilience and how he has been able to survive, even in the face of traumatic events and a difficult past.



Reflection Questions

Reflect on ways that client transference could trigger your countertransference reactions. Consider these questions:

- What aspects of Matt's case do you think you would find most challenging, and why?
- Which techniques would you be inclined to employ in counseling Matt, and what would you hope to accomplish?
- Several therapeutic interventions were described in counseling Matt. Which intervention did you find most interesting, and why?

Working With Transference and Countertransference With Bonnie

Bonnie has been in counseling for 2 years to address issues with the men in her life. She was raised in a strict family and always felt that she was in trouble for some reason or another. As she often said, "I could never do anything right, especially in my father's eyes." She feels that this relationship set the tone for subsequent relationships with men. To complement the therapeutic work we are doing, I encourage Bonnie to

express her thoughts in a journal (she enjoys writing) and ask her to bring it to every session. Although Bonnie says she is excited to keep a journal and thinks it will help her get the most out of her therapy, I notice that she often forgets to bring it to sessions, and when she does, she has written very little. In my efforts to explore and process this with her, I wonder out loud, "What makes it difficult for you to write in your journal?" Although my intention is to help her understand her reluctance to do something that she believes will be useful, she interprets my question as a personal attack.

Bonnie says this to me in the session: "No matter how hard I try, I'm not able to please you. I fear you are judging me, and I'm not enough, just like with my Dad. I feel that I can't please you or meet your expectations." Bonnie's reactions toward me appear to indicate transference of her feelings about her father onto me. I think there is therapeutic merit in encouraging Bonnie to express herself in some detail, and her explanations will help me gain a further understanding of her dynamics. I ask her to explore the connections she is making between her father and me.

Exploring her feelings toward me, and what she is attributing to me, can lead to insights regarding unresolved conflicts with her father. I frequently suggest to Bonnie that she talk about her perceptions of me as they are happening in the present moment and that she share her experience of being in therapy. The psychoanalytic approach offers a rich perspective in helping Bonnie understand how her father is still operating in her life. Her transference provides valuable clues in the here-and-now interactions between us. The transference situation gives Bonnie an opportunity to re-experience a variety of feelings that would otherwise be inaccessible, and it also allows her to understand and deal with unfinished business from her past relationships. By productively exploring her transference reactions, Bonnie can gain insight into how she is keeping old patterns functioning in her present relationships.

If I deal with Bonnie's transference reactions toward me appropriately, this can be an avenue of increased self-understanding for her. The psychodynamic relational approach emphasizes the importance of a therapist being aware of, monitoring, and managing countertransference reactions. I am aware that I tend to get impatient and defensive when clients tell me how I am and then treat me on the basis of their perceptions—without including me in this process. If Bonnie continues to make assumptions about who I am and what I think of her, I may react defensively because I feel misunderstood by her. If I am expending a great deal of energy defending myself, I will not be able to deal with her reactions in a therapeutic manner.

In this instance, I recognize my own vulnerability, and I let Bonnie know how I am affected by hearing what she is saying and how it affects me when I feel judged by her. Bonnie says to me, "I can't imagine that you'd want to invest the energy and time in really listening to my pain or understanding me, especially when I don't follow through with the homework you assign." I respond, "I'd like to hear more about how you came to this conclusion about me." If I can suspend my tendency to convince Bonnie that I am different from the person she perceives me to be, we can work

through this transference therapeutically. I invite more exploration of the transference rather than negating it.

I also engage in self-disclosure and let Bonnie know that some of her feelings of "not being enough" touched me personally. Operating on the assumption that I can use my personal reactions in a therapeutic manner by appropriately disclosing to Bonnie, I tell her: "There are times when I also feel that I am not enough. When I am told how I am, and when I feel judged, this taps into old issues for me. When I am extending myself and feel that I am not appreciated, it is easy for me to tell myself that I am being judged as inadequate." Disclosing this opened up a dialog that she could use to reflect on her experience in therapy.

Later in the session, I tell Bonnie that I do not always say the right thing, nor am I always as sensitive as I might be. I admit that perhaps some of my shortcomings make it difficult for me to acknowledge the work that she has done in these sessions. I also admit that the tone of my questions may not always reflect my intention in asking them, which could understandably leave the wrong impression. My self-disclosure had an impact on Bonnie, for she becomes silent and then gets teary. I ask her to talk about the tears. She indicates that she is surprised that I am not blaming her and that I am willing to consider my part in how she is perceiving and reacting to me. One of Bonnie's fears is that I would tell her that what she is telling me is just about her father. Furthermore, it was affirming for her to hear that we both had in common the struggle with feeling good enough personally.

I am increasingly able to recognize countertransference patterns in myself, and I take steps to monitor my reactions. I certainly need to be cognizant of the ways my reactions could blur my vision and hamper my effectiveness with Bonnie. Through supervision, and perhaps personal therapy, I can learn to identify and accept some immediate reactions that are stirred up inside me when I feel judged or negatively evaluated. It may not be important or appropriate for me to share this with my client, but knowing this myself is critical to maintaining a sense of objectivity.



Reflection Questions

Reflect on specific aspects of transference in Bonnie's case. Consider these questions:

- What aspects of the interaction between the client's transference and the counselor's countertransference most stood out for you, and why? What did you learn from this case presentation?
- What theoretical framework(s) would you be inclined to adopt in understanding Bonnie's transference?
- Identify one source of countertransference that you may have to manage in working with clients. What are your ideas about how to best understand and manage your countertransference?

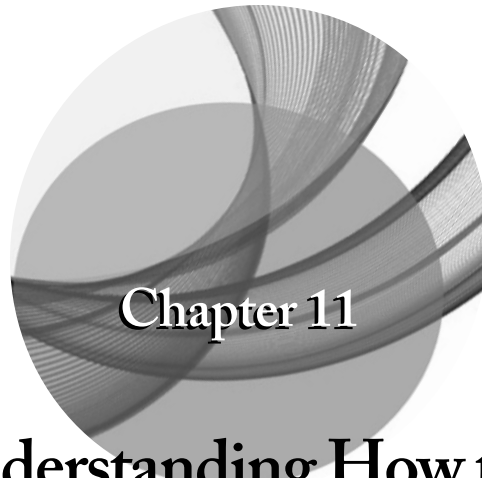
Concluding Comments

I encourage you to reflect on how you might be affected by the intensity of your professional work. If you can remain open to identifying and exploring areas of your countertransference, this will surely influence your ability to function effectively with others. Being able to deal therapeutically with clients' reactions to you is a major challenge in the client–therapist relationship. It is imperative for you to recognize and deal with internal conflicts and past pain that are triggered by your work as a counselor.

The emotionally intense relationships we develop with clients can be expected to tap into our own vulnerability. Our clients' stories and pain are bound to affect us, and we need to find some way to deal with what is opened up for us. We don't have to numb ourselves to the pain experienced by our clients, and, indeed, we can be touched by their stories and express compassion and empathy. However, we have to realize that it is their pain and not carry it *for them* lest we become overwhelmed by their life stories and render ourselves ineffective in working with them.

After reading this chapter on working with transference and countertransference, take time to reflect on the following questions as a way of clarifying your thoughts on how these concepts may be used therapeutically in counseling practice.

- What is your understanding of the concept of transference?
- What value do you place on working with transference in the therapeutic relationship?
- What is your understanding of countertransference?
- If you became aware of countertransference reactions to a client, what course of action would you be inclined to take?
- What value do you see in therapist self-disclosure?
- What are your thoughts about requiring (or recommending) psychotherapy for those studying to become counselors?
- What value do you place on receiving your own personal therapy?
- How is therapist self-care related to a therapist's ability to provide effective services to clients?



Chapter 11

Understanding How the Past Influences the Present and the Future

Some approaches, such as reality therapy, behavior therapy, rational emotive behavior therapy, cognitive therapy, and solution-focused brief therapy, do not examine the historical determinants of behavior. Their premise is that present conditions, rather than traumas or faulty learning during early childhood years, influence clients' problems, and that cognitive and behavioral techniques can change the relevant current factors that influence clients' behaviors. However, I believe that the past offers a context for understanding and dealing with a client's present cognitive, emotional, and behavioral difficulties; and I value working with the past, as well as with the present and the future, in my integrative model.

Understanding How the Past, Present, and Future Are Intertwined

Bringing the Past Into the Present

The psychoanalytic model holds that the shadow of the past can haunt the present, and I continue to see the vital connection between the past and the present in the work I do. Typical problems that people bring to counseling include an inability to freely give and accept love; difficulty recognizing and dealing with anger, resentment, rage, hatred, and aggression; an inability to direct one's own life; difficulty separating from one's parents and becoming a unique person; a need

for and a fear of intimacy; and difficulty accepting one's own sexual identity. From the psychoanalytic perspective, these problems of adult living have their origin in early development. Early learning is not irreversible, but to change its effects, we must become aware of how certain early experiences have contributed to our present personality functioning. I have incorporated these basic psychoanalytic concepts in my personal integrative approach to counseling.

A common misconception about psychoanalytic therapy is that it resembles an archaeologist digging out relics from the past, dwelling on the past to the exclusion of present concerns. Otto Kernberg (1997), a key figure in psychoanalytic theory, indicates that there is an increasing interest by contemporary psychoanalytic therapists in focusing on the unconscious meanings in the here and now before attempting to reconstruct the past. Among the contemporary psychoanalytic therapists are those who subscribe to object-relations theory and view the internal and external world of relationships as central to therapy. The object-relations model of therapy is largely based on the early relations of a child and mother and how this early relationship shapes the child's inner world and later adult relationships. As the therapist moves back and forth in time, the aim is to understand how early patterns are repeated in the present. Modern psychodynamic practitioners are interested in their clients' past, but they intertwine that understanding with the present. From a relational analytic perspective, there is a shift from focusing on the there and then to the here and now, which enables therapeutic work to proceed in a more enlivened and accelerated way (Levenson, 2017).

Insight can be a vehicle that enables clients to relinquish old behaviors from the past that intrude into the present. It is useful for clients to understand and use historical data in their therapy, but they also need to be aware of the pitfalls of getting lost in their past by recounting endless and irrelevant details of their early experiences. A preoccupation with the past can be uselessly time consuming and can inhibit progress. Discussion centering on childhood events is less useful than dealing with the past in relation to here-and-now interactions between client and therapist.

Envisioning the Future

Our vision of our future can have an impact on our present functioning, just as our past can. A number of therapeutic approaches emphasize future strivings. Along with valuing the Gestalt and psychodrama approaches for dealing with the future, I find a number of useful concepts in narrative therapy, solution-focused therapy, Adlerian therapy, reality therapy, and existential therapy—all of which have concepts dealing with future aspirations.

Solution-focused therapy eschews the past and emphasizes the present and, especially, the future. It is so focused on what is possible that it has little interest in or understanding of the client's problem. de Shazer

(1991) has suggested that therapists do not need to know a problem to solve it and that there is no necessary relationship between problems and their solutions. By focusing on the future and using techniques such as the miracle question (de Shazer & Dolan, 2007), solution-focused therapists encourage clients to move toward their vision of a better life.

Narrative therapy is both present-centered and future-oriented. The therapist believes that clients have abilities, talents, positive intentions, and life experiences that can be the catalysts for new possibilities for future action. The counselor needs to demonstrate faith that these strengths and competencies can be identified, even when the client is having difficulty seeing them. A narrative therapist is likely to ask a client to speculate about what kind of future could be expected from the strong, competent person that is emerging. As the client becomes free of problem-saturated stories of the past, he or she can envision and plan for a less problematic future.

Both solution-focused and narrative therapists rely on questioning as the centerpiece of therapy. Open-ended questions about the client's attitudes, thoughts, feeling, behaviors, and perceptions are one of the main interventions in these therapies. Especially useful are future-oriented questions that challenge clients to think about how they are likely to solve potential problems they encounter in the future. In narrative therapy, *unique possibility questions* enable clients to consider future possibilities and to reflect upon what they have achieved in the present. Based on their successes, they can consider what their next steps might be. Here are some examples:

- Given your present discoveries and understandings, what do you think your next steps might be?
- Having observed people responding in a supportive manner to your self-caring, what plans do you have for yourself to not let go of your ability to nurture yourself?
- Now that you are prepared to question and reexamine the direction your life is taking, what does this behavior suggest to you about the personal qualities and abilities you are exhibiting right now?

I very much agree with the Adlerian premise that clients can be understood best by looking at where they are going and what they are striving to accomplish. Adlerians are interested in the future, but they do not minimize the importance of past influences. They assume that our decisions are based on our experiences, on the present situation, and on the direction in which we are moving. I appreciate the perspective of the Adlerians, who look for continuity, or a pattern, in life. Adlerians look for this continuity in the client's lifestyle, which reveals ways the past is still operating in the present. Even some Gestalt therapists have come to appreciate the importance of continuity, realizing every here and now was preceded by a there and then and anticipates the next (Polster, 1987).

Becoming the Client: Examining Your Past, Present, and Future

We have looked at how the past, present, and future are inseparable. Now would be a good time for you to consider how your own past is affecting your present and ways that your past experiences could influence your future. In this section I ask you to "become a client," and I encourage you to reflect on the personal applications of dealing with your past, present, and future. Your past experiences have implications for who you are now. What you are striving to become also has implications for the person you are today. Your past is still alive in the person you are today, and it can influence your future direction. As your therapist, I ask you to reflect on the following questions:

- What themes stemming from your childhood are still a part of your personality today?
- What unresolved conflicts from your childhood get in the way of your enjoyment? To what extent do your present behavior or current problems indicate areas of unfinished business?
- What events occurred during your childhood or adolescence that you might bring into your personal counseling sessions?
- Is there a significant person in your life that you would like to bring into this session? Are you willing to engage in a role-play session with me acting as that person?
- What value do you see in reexperiencing some difficult situation you have been through earlier and talking about how you are feeling in the moment in therapy?

Think of one particular relationship in your life right now that you'd like to improve. Imagine that you are having a dialog with that person one year from now. What would you most want to say to him or her? What would you most hope to hear from this person?

Exploring your vision of your future would certainly be a factor in my work with you. How valuable do you think that imagining yourself in the future might be for you personally? The miracle question, a technique often used in solution-focused brief therapy, is one way I help you clarify your future goals and strivings. In our therapy session I ask, "If a miracle happened and the problem you have was solved overnight, how would you know it was solved, and what would be different?" I then encourage you to enact "what would be different" in spite of the perceived problems. By formulating this future perspective, you are focusing on what you want in your life and how you can go about getting it.

Working With Tricia's Past, Present, and Future

Dealing With the Past in an Integrative Style

Tricia, age 43, received many negative messages about herself as a child. Today Tricia is in the process of divorcing her second husband. She readily admits that she has problems with intimacy in her romantic

relationships. She believes that her pattern of attracting critical and emotionally unavailable men is rooted in her relationship with her father.

Tricia's father, a strict parent and a religious man, viewed her emotional outbursts and behavioral problems after her mother died as sinful and embarrassing. He often scolded her in public and seemed hostile toward her, which intensified her anger and increased her sense of isolation. Tricia recalls spending many hours every week at the church because her father was the choir director, and she still resents the cold treatment she received from other members of the congregation. She believes that her father is largely responsible for the way others perceived her because he had a lot of power and influence in the church. As an adolescent, she became increasingly rebellious as she realized that she could not gain the approval of her father, and he ultimately sent her to live with her aunt.

I begin by using a psychodynamic approach to explore Tricia's early history as it is manifested in her current problems. I am interested in assisting her in gaining insight into how her past connects to some of the problems she is experiencing today. When the past seems to have a significant bearing on Tricia's present attitudes or behavior, we bring it into the present as much as possible through the use of Gestalt experiments. I heavily construct my integrative practice around Gestalt therapy concepts as a way to illuminate the connections between the past and the present.

When Tricia speaks about a past event, I ask her to bring whatever feelings surface into the present by reliving them through a variety of role-playing techniques. I suggest that she become the hurt child and talk directly to her father in fantasy. I say: "Bring your father into this room now, and let yourself go back to the time when you were a child. Tell him now, as though he were here and you were that child, what you most want to say." Although she was rebellious as a teenager and acted out, she never had the opportunity to express what she wanted to say to her father. She might tell her father what she most wanted from him then and what she still wants with him now. The theoretical rationale for most of these techniques is rooted in the assumption that the emotions that were overwhelming to her as a child were dealt with by some form of distortion or denial. By reliving an experience as though it were happening now, emotions that were repressed can come to the surface.

In the supportive, accepting, and safe therapeutic environment, Tricia can allow herself to experience feelings that she has cut off from awareness, and she can now express these feelings that are keeping her stuck. By challenging her assumptions of how her father is, she is able to establish a new basis for relating to him. For instance, while expressing the anger and pain she felt as a child, she may now recognize that her father was also grieving the loss of his wife and didn't have the best coping strategies. Through this process, she relives the hurt, yet she also can experience understanding and resolution of a painful situation. By symbolically dealing in the here and now with people in her life with whom she feels unfinished, she can bring closure to some painful events.

Let's look at a few critical situations between Tricia and her father, along with how I intervened. One critical incident took place when

she was 9 years old. She reported, "One day during a choir rehearsal, I wasn't feeling well, so I kept interrupting my father to find out when we would be going home. In front of everyone, he lost his temper and told me that I was interfering with his work. He told me that I was always needy and to just stop it! I felt rejected and ashamed."

To initiate our work, I introduce a mindfulness relaxation exercise or help Tricia focus on the moment, and then I provide a guided imagery exercise to deepen the experience. I then transition to Gestalt methods, asking Tricia to be herself (at the age of 9) and talk to me now (as her father). I say to her: "Imagine I'm your father, and I'm not going to say anything. I'll just listen to you. Tell me what is going on with you right now." As she talks, I simply listen because it is important for her to express feelings that she has kept to herself for so long. Tricia tells me how embarrassed and angry she feels for being publicly humiliated; she feels devastated that she can't ever be good enough in the eyes of her father. It is clear that her rigid upbringing continues to shape her self-image. She is convinced that she is perceived as very bad in the eyes of her father and her church.

Because Tricia brings up the matter of her church, we explore how her religious upbringing influences her today. I suggest a second role-play in which she becomes the voice of her church, the past, and her father. I ask her to talk to me from that stance. I assume the more accepting part and the voice that she would like to hear. She is able to get into her part with great conviction because she is saying aloud what she heard for so many years growing up and what she has told herself silently. She has incorporated the critical and judgmental voice of her father and the church without really examining the validity of the messages she is receiving from either of these sources. In role-playing the "accepting side," I want to get across that "even though I have my faults and am imperfect, I am certainly not a bad person." As Tricia's assertive side, a central message I'm striving to get across is that Dad's approval costs her much unhappiness: "I have been striving all my life, Dad, to get your approval. But to be what you want I have to be scared and always feel guilty for not measuring up. I don't want to do that anymore. To get your approval costs too much."

This gives Tricia an idea of what she would like to be able to say and really believe. She says: "I sure wish I could talk to my father that way!" We devote several sessions to discussing how her father played a central role in shaping her expectations about what to expect in a mate. Eventually, Tricia gets the insight that she does not want to live by her father's demands for her, nor does she want to accept for herself the messages he continues to give her about how she would be more successful in life if she controlled her emotions and was more agreeable and compromising in relationships. She wants to be accepted by others for who she truly is and not get caught up in pursuing relationships with men who want her to be an ideal that she cannot possibly measure up to. By bringing the past into the here and now, Tricia was able to express beliefs and feelings that have roots in

her early childhood. My intervention enabled her to clarify how she wants to think, feel, and believe today.

Along with Gestalt methods, I draw on concepts and techniques from psychodrama to help Tricia understand her past. In psychodrama, she enacts conflicts as if they were occurring in the present moment rather than narrating past events. For instance, I ask her: "Show me what happened when you were a young child and your father scolded you in front of the church parishioners and choir members." Psychodrama offers encouragement to her to speak in the present tense and to use action words. Placing the client in the present, regardless of when the scenes actually happened, reduces verbal reporting and assists clients in directly experiencing an event. When she engages in showing me what she is thinking or feeling, she moves toward concrete experiencing and cuts through her defenses. Through this process, she can assign new meaning to the past event and develop a new and different ending to an earlier situation.

Dealing With the Present in an Integrative Style

I am counseling Tricia from a present-centered perspective. Most of our discussions deal with her thoughts and beliefs, her emotional states, and what she is doing currently. Even though I am open to exploring some of her past experiences, my focus is on examining ways that her past still has a present influence. Tricia and I can direct attention to her immediate feelings as well as to her thoughts and actions. It seems essential to me that we work with all three dimensions—what she is thinking, how her thoughts and behaviors affect her feeling states, and what she is actually doing. By directing her attention to what is going on with her during our sessions, Tricia becomes aware of how she interacts in her world apart from therapy.

Dealing With the Future in an Integrative Style

The technique of making a past situation a present one can be applied to future events as well. The rationale for doing this is the same as working with the past. If the future comes alive through enactment, there is less chance of dealing with abstractions. I frequently utilize both Gestalt and psychodrama techniques to assist clients in experiencing their hopes, fears, and expectations for the future. If Tricia is afraid of a future confrontation with her father, I ask her to live her expectations in the here and now by speaking directly to her father in a role-play situation. By expressing her fears and hopes in the safety of the therapy context, she is likely to gain clarity about what she would like to say to her father in real life. Thus, she may say in a role play to her symbolic father: "I want to tell you how much I'd like to be close to you, but I'm afraid that if I do so, it won't matter to you. I'm afraid of saying the wrong things and pushing you even further away. I often think I have disappointed you because I haven't become the person you expected."

Psychodrama is also highly useful for exploring some anticipated event in the future. In psychodrama, the past, present, and future are

all significant tenses, yet the action is played out in the present moment. Psychodrama can enable Tricia to bring the future into the now: "Show me how you'd like to be able to talk with your father. Let me be your father and say some of what you'd like to be able to say to him."

A technique in psychodrama known as future projection can be appropriate in counseling Tricia, especially when she brings up wanting improvements in her significant relationships. When I ask her to participate in living out a scenario in the future, my goal is to help her express and clarify concerns she has about the future. Her future concerns are not merely discussed; an anticipated event is brought into the present moment and acted out. These concerns may include her wishes and hopes, fears of tomorrow, and goals that provide some direction to her life. In implementing this technique, I have her imagine a future time and place with selected people, bring this event into the present, and get a new perspective on a problem. Tricia may act out either a version of the way she hopes a given situation will ideally unfold or her version of the worst possible outcome.

Once Tricia clarifies her hopes for a particular outcome, she is in a better position to take specific steps that will enable her to achieve the future she desires. For example, she can be asked to carry on the kind of dialog with her father that she would ideally like to have with him at some time in the future. A role-reversal technique can be powerful if used in a timely manner. I might suggest to her: "Let yourself become your father. You know the words you'd most like to hear from him. As your father, speak to me (as Tricia) and tell me what you wish he would say and how you would hope he would act." Through reversing roles with her father, Tricia is able to formulate significant emotional and cognitive insights into the situation involving her father. She can also project herself forward and tell him how she has acted differently toward him during the previous year. If she gets a clearer sense of the kind of relationship that she would like with him and if she accepts her own responsibility for the quality of this relationship, she can begin to modify some of the ways in which she approaches her father.

Reflection Questions

Reflect on how the past, present, and future can be explored in an integrative way with Tricia. Consider these questions:

- What aspects of Tricia's past do you think need to be explored to help her better understand her present functioning?
- Techniques from Gestalt therapy and psychodrama were used in counseling Tricia. What techniques might you apply from different theories in your work with Tricia?
- How important is considering future strivings in working with Tricia? What is one aspect of her future that would interest you?

Working With Jasper's Past, Present, and Future

Forming and Maintaining Intimate Relationships

Jasper is a 51-year-old male who was adopted at age 2. This is Jasper's fifth session, and his presenting problem was his difficulty in forming and maintaining intimate relationships, with both men and women. Jasper grew up in a family with two older biological brothers, and he felt they were accepted and treated by the parents in a much more favorable way than he was treated. There was a good deal of tension between his adoptive mother and adoptive father, which sometimes led to domestic violence. Jasper believed his adoptive father singled him out as the target to vent his frustrations and anger, and he was physically and emotionally abusive toward Jasper. What Jasper is hoping to get from his counseling is the ability to have a close family at some point in his future. In this session, I work with a series of Jasper's early recollections. This Adlerian technique shows how a client's early memories are often connected to present-day themes.

Dealing With Jasper's Past

I am very interested in exploring Jasper's early history and assisting him in understanding how his past is connected to his current problems of not being able to establish personal relationships. I believe we can illuminate themes that run through our life if we are able to come to terms with significant experiences from our childhood. Through identifying key aspects in his past and the meaning Jasper attached to past events, he is becoming more aware of how his work in therapy will consist of reframing his perceptions and conclusions about himself, others, and the world. My emphasis is on having Jasper begin to question these conclusions. Here are a few of Jasper's mistaken conclusions and self-defeating perceptions that grew out of his earlier experiences:

- If I get close to people, I will experience rejection and hurt.
- Because my biological parents didn't want me and my adoptive parents were abusive to me, I'll never be loved by anybody.
- If only I were a better person and more lovable, maybe people would accept me.
- Because my biological parents abandoned me and my adoptive parents were rejecting of me, this proves that I am an unworthy person.

Erv Polster (1987), a key figure in Gestalt therapy, stresses the importance of having clients flesh out their stories, which may include working with the past, the present, and the future. In keeping with this framework, my work with Jasper involves an integration of his past, present, and future. This involves listening to stories from his childhood, current concerns, and constructing a future story of his life.

One way of getting a better sense of significant events from Jasper's past is to ask him to report early recollections, which I use as part of the assessment process. Early recollections are *specific*, one-time incidents that Jasper can recall, along with the feelings and thoughts that accompanied these childhood incidents. These recollections are useful in better understanding Jasper's past and how it is influencing him today. Jasper reports an early memory when he was 9: "My dad and mom were arguing and yelling, and my dad is pushing my mom against the wall. I am really scared, yet I yell at my dad to stop pushing my mom. He then turns his anger toward me and takes his belt off and is hitting my legs."

Jasper reports feeling a sense of helplessness in conflict situations, but he begins to see that not having power as a child does not have to define him today. He is realizing that he was not responsible for the decisions of his biological parents or for the emotional upheaval in his adoptive family. He is beginning to recognize that his early conclusions about himself are not accurate.

Dealing With Jasper's Present

As I continue working with Jasper, I keep the focus on his present life. I am interested in his past history mainly as a way to get a richer appreciation for his present struggles. Whether he is talking about something from his past or an anticipated future situation, I consistently bring Jasper into the here and now. My assumption is that power is in the present. One of the most important contributions of Gestalt therapy is the emphasis on learning to appreciate and fully experience the present. The past is gone, and the future has not yet arrived, whereas the present moment is alive with possibilities.

Dealing With Jasper's Future

Being influenced by the Adlerian philosophy, I operate on the assumption that Jasper lives by goals and purposes, is moved by anticipation of the future, and strives to create meaning. Jasper's past, present, and future are dynamically interrelated: his decisions are based on what he has experienced in the past, on his present situation, and on the goals that he is moving toward. As Jasper and I look for continuity in his life, the emphasis is on the goal-directed nature of all his behavior. I am especially interested in Jasper's new decisions about his future. What does Jasper want for himself? He says he wants safety and a no-conflict or low-conflict existence, but it too often leaves him feeling sidelined and ineffective. If he decides that his present behavior is not getting him what he wants, he is in a good position to think about the changes he would like to make and what he can do now to actualize his aspirations. The present-oriented behavioral emphasis of the action therapies is a good reference point for getting Jasper to speculate on what he would like to say about his life in a few years. Once Jasper has a clearer picture of the kind of future he wants, he is in a good

position to formulate a systematic plan that will help him achieve his goals. Here choice theory and reality therapy are most useful in the development of a realistic action plan to move toward accomplishing his desired goals. Although Jasper cannot change his past experiences, he can be empowered by taking small steps that will lead to the goals he wants for his future.

Reflection Questions

Reflect on how the past, present, and future can be explored in an integrative way with Jasper. Consider these questions:

- How might you intervene in working with Jasper's past beliefs and experiences as it pertains to gaining a better understanding of his present struggles?
- How can thinking about Jasper's past, present, and future be relevant in deciding which interventions to apply?
- If you had to select one main theory to better understand how the past, present, and future are interrelated, which theory would this be, and why?

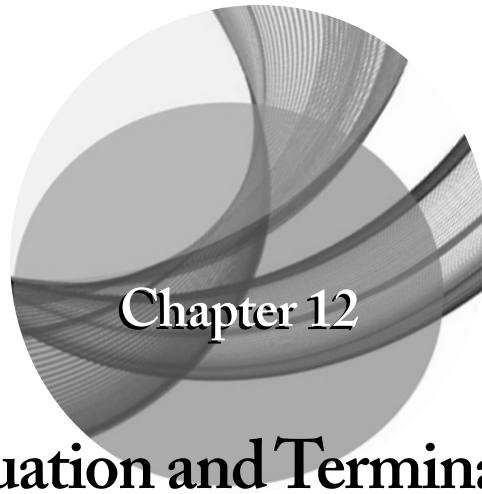
Concluding Comments

It is one thing to read about the concepts and techniques I've described in my integrative approach, but it is quite another matter to apply them to yourself. The more you are able to identify personal themes of significance associated with your past, present, and future, the greater will be your ability to appreciate some of these concepts. If you see value in counseling clients from the perspective I've described, you are likely to find ways of using techniques aimed at bringing both past and future concerns into the present with your clients. To engage clients in experiential role-playing scenarios in the here and now, you need to have the skills to make these interventions. Although the emphasis of this chapter has been on identifying and expressing feelings, there is plenty of room for exploring cognitive and behavioral patterns associated with the past, present, and future.

After reading this chapter on understanding how the past influences the present and the future, take time to reflect on the following questions about how your own past has influenced your present and future goals.

- What are some of your past experiences that continue to influence who you are today?
- If you could revise one of the chapters of your life, how would you rewrite it?
- To what extent do you see your past as giving you an appreciation of the struggles of the clients with whom you will work?

- Consider three aspects of your current existence that you would most like to change. If you were successful in changing them, how do you imagine that your life would be different?
- What kind of life would you like to have 5 years from now? Are there any steps you can take now to move you closer to your goals?
- How much time do you devote to thinking about future goals and aspirations? To what degree do you think that your vision of your future has an influence on how you think, feel, and act today?
- How do you think your own experience with your past could influence the way you work with a client?
- What emphasis would you place on your client's past, present, and future? Which would gain your primary focus?



Chapter 12

Evaluation and Termination

Just as the initial session sets the tone for the therapeutic relationship, the ending phase enables clients to maximize the benefits from the relationship and decide how they can continue the change process. The process of termination begins with planning for the eventual end of the relationship during the early phases of therapy. This preparation for termination is ongoing as you and your client collaboratively monitor progress over time (Davis & Younggren, 2009). As a counselor, your goal is to work with clients in such a way that they can terminate the professional relationship with you as soon as possible and continue to make changes on their own. From your first contact with clients, it is important to convey the idea that your intention is to assist them to function effectively without you. The goal of therapy is the resolution of the problems or concerns that brought the client into therapy. Ideally, therapy ends when the goals that were initially established have been met (Dobson & Dobson, 2017). It is not helpful for therapy to continue indefinitely, although clients may return intermittently after the first therapeutic experience has been completed.

Toward the end of the therapy experience, the therapist functions as a consultant and assists clients in consolidating gains and identifying potential future problem areas. *Termination* is the process of taking clinically and ethically appropriate steps to bring closure to the therapeutic relationship (Davis & Younggren, 2009). Terminating therapy is a positive step, for it means that clients have acquired the necessary skills to deal with the challenges they may face. Some of the key tasks of termination are helping clients assess what they have accomplished in therapy, summarizing the counseling experience, generalizing the lessons they learned in therapy to the real world, dealing with feelings

regarding ending the relationship, and exploring their strategies for dealing with future problems or setbacks.

Guidelines for Effective Termination Strategies

Norcross, Zimmerman, Greenberg, and Swift (2017) studied the core termination behaviors of experienced therapists across diverse theoretical orientations and identified several tasks that were supported by 90% of practitioners. Among these tasks were identifying the gains and areas of growth for the client during therapy, talking about what went well in psychotherapy, encouraging the client to take credit for the gains made, establishing future goals and creating plans to accomplish these goals, and following the ethical guidelines.

The action-oriented approaches (reality therapy, behavior therapy, cognitive behavior therapy, solution-focused brief therapy, and motivational interviewing) have very good guidelines for consolidating learning and transferring what was learned in therapy to daily living. Consider these tasks when planning therapeutic endings:

- Actively plan for the completion of therapy at various phases of the therapy. Discuss the termination process early, and remind clients of the approaching end of the therapy relationship. Ask clients what they'd most like to talk about in the final few meetings with you. At a session prior to the last one, ask clients for their feedback: "If this were our last meeting, how would that be for you?"
- If you are not limited to a specified number of sessions, and both you and your client determine that termination is appropriate, suggest spacing out the final few sessions, perhaps meeting every 3 weeks, so clients have an opportunity to practice new behaviors and to prepare for termination.
- Encourage clients to take the lead in reviewing the course of their treatment. What lessons did they learn, how did they learn them, and what do they intend to do with what they have learned? What did they find most helpful in the sessions with you? How do they evaluate their own participation in this process? It is good for clients to take the lead in addressing these questions. It is critical for clients to take ownership of the gains they have made and to discuss how positive changes are the result of the therapeutic partnership. The final sessions should include a discussion of ways to maintain treatment gains and to continue their work after termination.
- Encourage clients to talk about their feelings of separation and loss. Just as clients may have had fears about seeking help, they may have anxieties or concerns about terminating the counseling relationship.
- Be clear about your own feelings about endings. You may be ambivalent about letting go of certain clients. Reflect on the degree

to which you may need your clients more than they need you. Monitor any signs of countertransference so that your needs do not make closure difficult for your clients. Talk to a supervisor or a trusted colleague about these feelings.

- A guiding principle of counseling is helping clients to achieve self-directed behavior. Pay attention to clues your clients give about ending the client–counselor relationship and be willing to discuss this at appropriate times. Respect clients' decisions regarding termination, but be willing to talk about any reservations you may have about the timing of termination.
- If you are an effective counselor, you'll eventually put yourself out of business—at least with your current clients. Your task is to help clients get moving on their own, not to keep them coming to you for advice. Give clients the tools that empower them to become their own counselors. You may feel some sadness when ending therapy with some clients, but take time to acknowledge your role in helping clients become self-reliant.
- Discuss potential future difficulties, and help clients plan for ways to effectively cope with challenges once therapy ends.
- The aim of counseling is not to work until every problem in life has been addressed and resolved. Neither perfection nor a life without problems is possible. Counseling is an ongoing and evolutionary process. At a later period of development, clients may be ready to address new problems or concerns that they are not ready to take up now.
- Encourage clients to return at a later time should they feel a need for further learning. Clients may need only a session or a few sessions to get refocused. Be sure clients know that asking for help in the future is not a sign of failure but an indication of new levels of strength.
- Assist clients in translating their learning into action programs for continuing the process of personal growth. If clients have been successful, the ending stage is a commencement; they now have some new directions to follow in dealing with problems as they arise. Discussing available programs and making referrals are especially timely tasks toward the end of your work with clients. In this way, termination of the professional relationship can lead to new beginnings in a client's personal relationships.

These guidelines are a good start in bringing about a successful ending to counseling. What other topics would you want to raise with your clients at the final phase of therapy? What goals are essential for an effective and positive ending of a counseling relationship? What are some techniques for closure you would use? How would you prepare your clients for relapse prevention? What challenges do you expect you may encounter in the termination phase with clients?

Becoming the Client: Taking Credit for Your Changes

You have become my client through various stages of counseling, and I ask you to once more become my client and think about the termination of this relationship. What is it like for you to face the end of this therapeutic relationship? The final phase of your counseling may be a difficult time, but you are the decision maker of when to terminate your therapy. I want to give you an opportunity to express your feelings associated with ending this therapeutic experience.

Positive endings include discussing your thoughts and feelings about your experience in the therapeutic journey. Are you concerned that you won't be able to carry into your everyday life some of the central things you learned in therapy? We discuss what you can do to maintain the gains you have made in therapy and how you can continue to generalize these behaviors in your everyday life. We also spend some time in preparing for lapses and address how you can cope with any slips or setbacks. If you experience any setbacks in implementing your skills in your daily life, I don't want you to think that you have failed. Viewing slips or lapses as learning opportunities improves the chances of long-term maintenance of the changes you have made (Naar & Safren, 2017). During the termination phase I rely on techniques from behavior therapy and reality therapy to assist you in consolidating your learning. It is of the utmost importance that you understand what *you* did to make your therapy experience meaningful. Take credit for decisions you made and the steps you took in accomplishing your goals in therapy.

During our final session, I ask you to imagine your life in some ideal future circumstance, a technique used by both Gestalt and psychodrama approaches. I might suggest to you any of the following:

- Imagine that you are coming in for a follow-up session 5 years from now and that we are meeting to discuss how your life has changed. What do you most want to be able to say to me?
- Let yourself think about all the ways in which you want to be different in your everyday life once you leave formal therapy. Close your eyes and carry on a silent dialog between yourself and the people who are most special to you. What are you telling them? What are they replying?
- Imagine that a year has passed since you ended your counseling and nothing has changed in your life—you have continued the way you have always been. Try to picture how you would feel.

During this final phase of therapy, I suggest that you continue keeping a journal—writing down the problems you are encountering, describing how you feel about yourself in specific situations, and listing your successes and difficulties in following through with your

contracts. Reading the right book at the right time can be a powerful catalyst in making changes, so I encourage you to read as a way of continuing to work on yourself. I ask you to formulate a contract that spells out what you will do to maintain your gains and accomplish new goals you are setting for yourself. To implement your contract, I ask you to continue giving yourself homework assignments.

Narrative therapists sometimes write letters to clients between sessions to provide feedback and to reinforce changes clients are making (White & Epston, 1990). These letters can promote and empower progress by keeping the conversation alive between sessions. I might say any of the following to you in a brief letter: "I am impressed by your willingness to take risks." "I am wondering how you are doing with the homework that we agreed upon at our last session." "I hope you will continue writing in your journal." "I appreciate your determination in creating an action plan and making a commitment to follow through with your plan." Before we have our termination session, I might write a more detailed letter summarizing some of my observations about the changes I have seen in you since you began therapy.

During our final session, I am likely to give you this existential message to take with you. "I hope you have become aware of your role in bringing about change in your life. You can assume power by focusing on changing yourself rather than trying to get others to be different. You have become aware of the choices that are open to you; thus, you can now reflect on the decisions you will make. Even if you decide to remain largely as you are, you are now aware that you *can* choose differently. Although choosing for yourself can provoke anxiety, it does give you a sense that your life is yours and that you have the power to shape your own future."

Finally, I want you to know that you are welcome to call for follow-up sessions if the need arises. Although bringing closure to our work together is essential, I want you to feel welcome to call for future contacts if this is in your best interest.

Evaluation and Termination With Chelsea

Chelsea has been in counseling for a period of one year. She initiated the process after finding herself frequently bursting into tears both at home and at work. Although her mother suffered from clinical depression and anxiety, Chelsea believed that she was not like her mother and could handle her life without succumbing to feelings of despair when stressful and upsetting situations occurred. She often prided herself on her ability to hold in her feelings and to be strong for everyone in her life: for her husband and children, her friends and extended family members, and even for her coworkers. All of these individuals came to expect Chelsea to support them in times of crisis, but on those rare occasions when she needed their support, no one was there to provide support for her. She is convinced that they believed

she was so strong and self-sufficient that she would be offended by their offering of support.

Throughout the counseling process, Chelsea and I discuss her progress in therapy. We especially look at the degree to which she is getting what she wants from counseling (and from me). If for some reason she is not successfully meeting her therapeutic objectives, we explore possible factors that might be getting in the way of her progress. At our initial session, I made clear to Chelsea that a main aim of our relationship is to help her to function without me as her counselor. In our journey together, we have been working toward the ultimate goal of Chelsea becoming her own counselor. Given Chelsea's tendency to view herself as a strong individual who does not need to rely heavily on others for support, she appreciates this focus. Although through counseling she has recognized the importance of letting people in her life know what she needs from them, she does not want to become overly dependent on our therapeutic relationship.

When is it time for Chelsea to terminate therapy? Chelsea has attained a degree of increased self-awareness, has succeeded in making some cognitive shifts, and has acquired some specific behavioral skills in meeting present and future problems. During the previous session, she brought up a desire to discontinue counseling. We talk about her readiness to end therapy and her reasons for considering termination. To encourage Chelsea to remain in therapy after she has made significant changes on many levels could result in needlessly fostering her dependence on me, which certainly would not lead to her empowerment. Termination of therapy is as important as the initial phase, and the main task is to consolidate her gains and put into practice what she has learned in the sessions by applying new skills and attitudes to daily social situations.

In our final sessions, Chelsea spends considerable time talking about what she learned in her counseling sessions, how she learned these lessons, and what she wants to do with what she has learned now that she will be terminating counseling. What Chelsea will do once her therapy formally ends is as important as the sessions in the office. In Adlerian terms, we are in the reorientation phase. At this time, the central task is to encourage Chelsea to translate her insights into acting in new and more effective ways. We discuss some concrete plans that she can put into action. We again explore some contracts that she has established as a way to provide herself with a useful direction.

The final sessions address specific changes Chelsea has made over the course of her therapy and what she considers to be the most helpful aspects of her therapy. Chelsea indicates that the focus of our work shifted at different times. Sometimes the emphasis was on her thoughts, other times on her feelings, other times on insights she was gaining, and other times on new behaviors she was practicing. At times, she needed to simply experience and express her feelings, other

times she needed to take action, and sometimes she needed to reflect on her beliefs, thoughts, and decisions. From Chelsea's vantage point, she has gained a new appreciation for herself as a thinking, feeling, and doing individual.

Together we utilize methods from reality therapy to develop an action plan and talk about how she can best maintain her new learning. I also share with her my perceptions of the directions I have seen her take. We talk about steps Chelsea can take to involve herself in other self-development programs. She could find support in a variety of social networks. Drawing from the Adlerian concept of social interest, we create a plan for Chelsea to get involved in some form of social action program. Finding ways she can involve herself to benefit others could be an important way to maximize the benefits of her therapy. In essence, she can continue to challenge herself by doing things that are difficult for her, yet at the same time broaden her range of choices.

One of Chelsea's core struggles was with members of her family, whom she felt too often took advantage of her. A referral to a family therapist may be in order. She might decide to invite her husband and children to be part of a few family therapy sessions. Certainly, her changes have not been easy for her husband, her children, and herself. From a family systems perspective, her changes have implications for others in her family. Even a session or two with the entire family could be instrumental in renegotiating some shifts in roles and expectations. If Chelsea decides not to broach the subject of family therapy with her family, she can still continue to behave in different ways with each member of her family. Instead of placing the emphasis on changing others in her family, she can focus on herself and relate differently to her husband and children. Indeed, if she presents herself in new ways and keeps the focus on herself, others may be affected by her new way of being.

Before she entered counseling, Chelsea was unaware of some patterns of her life that were not serving her well. As a result of her therapy, she has a greater appreciation that she possesses the resources within her to manage her life and to draw upon external resources, such as the support of loved ones, when the need arises. Once Chelsea leaves formal therapy, I expect that she will continue to increase her awareness of times when she is thinking, feeling, and acting in ways that are not serving her well. When she catches herself in these patterns, Chelsea knows how to make a shift and do something different. When she engages in self-talk about the futility of reaching out for support, she can become aware of this negative voice and shift to a new dialog. When she experiences depression or anxiety, she will notice this and reflect on tools she has learned to cope with her symptoms and to practice self-care. The process of catching herself at those times when she becomes ensnared in self-defeating patterns enables her to become her own counselor.

To empower Chelsea on her continuing journey, together we explore possible stumbling blocks and ways to cope with them. *Relapse prevention* is an integral part of the final phase of therapy. Chelsea is likely to experience possible setbacks once formal therapy ends, and the goal of relapse prevention is to help her maintain the gains she has made by using what she has learned in therapy when she needs it in the future (Dobson & Dobson, 2017; Marlatt & Donovan, 2007). At times, it is inevitable that Chelsea will revert to old patterns and experience self-defeating thoughts and behaviors. The point is not that she never experiences setbacks, but that she learns to catch herself when she slips into old familiar patterns that no longer work for her. The expectation is that Chelsea will avoid slipping into discouragement and will focus on how she can get back on track.



Reflection Questions

Reflect on what you consider to be crucial tasks to address during the ending of a therapy relationship. Consider these questions:

- Which theories would you find most useful during the termination phase of counseling with Chelsea, and why?
- What are some specific topics you would discuss with Chelsea during the termination process?
- If Chelsea were reluctant to terminate therapy with you, even though she had accomplished her goals, how would you deal with her concerns about ending the relationship?

Evaluation and Termination With Charles

Charles is a 35-year-old, divorced, heterosexual, White male. He was raised in an economically depressed part of Chicago and was an only child to parents who did not complete high school. Charles's father was a career criminal who took young Charles with him to commit robberies, collect outstanding debts, and make drug deals. Charles's mother was a victim of domestic violence and resented young Charles for the "burden" he placed on her. Charles learned to get by in life by stealing, intimidating, and otherwise manipulating others in a community that was rife with violence and crime. When Charles had a problem with another kid on the block, Charles's father would insist that Charles settle the difficulty via violence. As an adult, Charles had become a successful drug dealer who was admired and feared by those around him. He had plenty of money and frequently got his way through intimidation or violence.

Eventually, Charles was arrested and convicted of drug crimes. He served 2 years in prison where the need to engage in violence

and intimidation in order to survive reinforced his view of people as untrustworthy, dangerous, and "threats to be managed." Charles's father died while Charles was in prison. He felt guilty about not being available to his father when he died. This experience had a major influence on Charles's decision to go to college and "make something good" of his life. He decided that he wanted a different kind of life than his father had. Charles came to the conclusion that he did not have to be bound by his father's rules and approach to living. Charles also realized that he did not want to return to prison, and he increasingly realized that this would be his destiny if he did not change some of his behaviors.

Soon after starting college, Charles began to struggle with thoughts that everyone he met could tell that he had been to prison and had been a "bad person." He also struggled with much of what he was learning about human nature and solving disputes with reason and verbal skills instead through intimidation and violence. Powerful emotions began to surface as Charles began to learn about the ways in which he was born into his earlier life of crime. He began to question his reasons for looking up to his father. He came to therapy questioning whether he could ever redeem himself for the crimes he had committed. He struggled with how much to share in therapy and worried out loud whether I would betray his trust and whether I believed he was a "bad person."

Charles and I worked together in therapy for quite some time before we mutually agreed to terminate services. Charles used our time together to explore thoughts, feelings, and behaviors that had served as obstacles to transcending his past and generating new possibilities for his life. He demonstrated remarkable courage in authentically facing the shame he experienced during his incarceration, and he worked through his fear that he would never be "good enough" to successfully complete a college degree. As he approached graduation from a 4-year university, we started preparations to end our work together.

As the date of our final session approached, old symptoms of anger and impulsivity reemerged. We revisited the cognitive and affective strategies he had implemented early in his therapy. Charles was able to recognize his increased awareness and willingness to approach these old reactions in new ways. We reflected on gains made both in the therapy sessions and in his everyday life. Though he initially wanted to give all the credit to me as his therapist, he was able to own the significant role he played in the changes he accomplished. It is of paramount importance that Charles be able to recognize what he actually did, both in the therapy sessions and in his life outside, to bring about the changes he is experiencing in his life now. His growth is not due to magic on my part, but to his own hard work in therapy. By recognizing his part in the success of his therapy, Charles will be able to continue making positive strides.

Evaluating the process and outcomes of therapy are key parts of termination. We mapped his path from the first session to the last session, paying attention to important turning points in the therapy and what each meant to both of us. I ask Charles to reflect on what he has learned ("What stands out the most for you? What did you learn that you consider especially useful to you?") and to consider how he can consolidate his learning ("What can you do now to keep practicing new ways of thinking and new behaviors?"). We reviewed the gains he made and explored his future goals, identifying specific steps he can continue to take to achieve these new goals. Charles and I talk about how he can be different with others in his life, even if they are not different with him. We also explored ways he can deal with problems differently, and I ask Charles about changes he would like to see in his life. Reviewing processes are always summative, and the very deliberate use of summaries helps to bring therapy to a useful end.

We also explored some potential difficulties he expects to face when he no longer comes to weekly counseling sessions ("What will you do if you experience any setbacks?"). We anticipated that old patterns of self-blame would emerge when Charles was faced with challenging situations, and he wrote down the important ways he had dealt with these situations in therapy. I introduce some *relapse prevention strategies* to help Charles cope constructively with future problems. These cognitive behavior techniques improve the retention, accessibility, and implementation of adaptive coping responses once treatment has ended and are designed to help clients maintain the longevity of self-managed treatment gains (Newring, Loverich, Harris, & Wheeler, 2008). By addressing potential problems and stumbling blocks that he might have to deal with, Charles is less likely to get discouraged if he experiences any setbacks.

We also talked about how Charles would know if he needed to return to therapy. He identified what he learned in therapy and how he planned to keep these lessons alive outside of the consulting room. Once Charles decides what direction he wants to move toward, he will need to develop a realistic plan of action and make a commitment to carry out his plan. Referrals and suggestions for continued growth are particularly useful at this time. I explore with Charles ways he can continue building on the changes he has made in his therapy, even though our professional relationship is ending. I let Charles know that termination of formal therapy does not mean that he cannot return for future counseling sessions if he thinks that returning would be useful for him. A follow-up session to assess his progress after termination may be scheduled at this time as well. As our final session comes to a close, we acknowledge what our work meant to us and reflected on the ways each of us has been affected by Charles's work.

Reflection Questions

Reflect on how you can assist clients in the termination and evaluation of a therapeutic relationship. Consider these questions:

- What did you find was the most interesting aspect of the case of Charles, and why?
- What theoretical framework(s) would you draw from in working with Charles in evaluating his experience in therapy?
- What value do you see in incorporating relapse prevention strategies with Charles during the final phase of therapy? Identify a few relapse prevention strategies you might introduce to Charles. How could you work with him collaboratively in ways to predict and deal with potential setbacks?

Your Thoughts on Ending the Therapeutic Relationship

Take time to reflect on the following questions as a way of clarifying your thoughts on evaluating client outcomes and termination of the therapeutic relationship.

- In what ways can termination of therapy be a positive step?
- What guidelines do you think are most important for the effective termination of a therapy relationship?
- What are some ways you might prepare your clients for ending the therapy relationship?
- When assisting clients in exploring termination issues, what difficulties might you face because of your own issues over loss or experiences with endings?
- What are your thoughts about how to evaluate the meaning of a therapy experience?

Some Final Thoughts on Psychotherapy Integration

The Future of Psychotherapy Integration

What is the future of psychotherapy integration? It may be reassuring to you to know that there is no one "right" approach to the integration of counseling practice. Integrative counseling is best viewed as an evolving framework rather than as a fixed approach. Stricker (2010) reminds us that providing the best service possible to clients should be the ultimate goal of the psychotherapy integration movement, stating that "psychotherapy integration is a creative

endeavor and promises to continue to contribute in a realistic and flexible manner to the development of the psychotherapeutic enterprise" (p. 109). There is a growing recognition that counseling can be most effective when contributions from various approaches are integrated. Although to date the bulk of psychotherapy integration has been based on theoretical and clinical foundations, evidence-based practice will become the organizing force for integration in the future (Goldfried et al., 2011). Norcross, Goldfried, and Arigo (2016) contend that integrative therapies offer the evidence and responsiveness to meet the needs of individual clients in their unique contexts. "Integrative psychotherapies are intellectually vibrant, clinically popular, and demonstrably effective" (p. 124).

The Role of Research in Developing an Integrative Perspective

Both theory and research need to be considered in developing your way of working with clients. Become familiar with the research literature in the psychotherapy field and consider how you can apply research findings in your practice. Today, there is an increasing demand in most agencies for practitioners to use techniques that are supported by research. An advantage of practicing within an integrative framework is the vast amount of research that attests to the efficacy of psychotherapy and points to its differential effectiveness with certain types of clients. Programmatic research supports the effectiveness of a disciplined eclecticism that prescriptively matches different treatments for different individuals (Prochaska & Norcross, 2018).

Accountability is being increasingly emphasized, but counseling is a more complex process than merely picking out techniques that are empirically validated. The therapy relationship, the therapist's personality and therapeutic style, the client, and environmental factors are all vital contributors to the success of treatment. *Evidence-based practice* (EBP) tends to emphasize only one of these aspects—selecting interventions based on the best available research. Norcross, Hogan, Koocher, and Maggio (2017) advocate for inclusive evidence-based practices that incorporate the three pillars of EBP: looking for the best available research; relying on the clinician's expertise; and taking into consideration the client's characteristics, culture, and preferences. It is important to keep in mind that the involvement of an active, informed client is crucial to the success of therapy services. Based on your clinical expertise, you will be expected to make judgments regarding particular interventions, and these decisions need to be made in the context of considering your client's values and preferences.

For a more detailed treatment of EBP, see *Evidence-Based Practice of Cognitive-Behavioral Therapy* by Dobson and Dobson (2017) and *Clinician's Guide to Evidence-Based Practices: Behavioral Health and Addictions* by Norcross, Hogan, Koocher, and Maggio (2017).

Designing Your Personal Integrative Approach

For those of you currently in a counseling program or who are beginning your counseling career, my general suggestion is to select the primary theory closest to your basic beliefs, learn that theory as thoroughly as you can, and at the same time be open to examining other theories in depth. If you begin by working within the parameters of a favored theory, you will have an anchor point from which to construct your own counseling perspective. Remember that designing your counseling approach does not have to be completed by the time you finish this book or a specific course or even a graduate degree. Do not think that simply because you adhere to one theory you can use the same techniques with all of your clients. Even if you adhere to a single theory, you will need to be flexible in the manner in which you apply the techniques that flow from this theory as you work with different clients. Integrative therapies attempt to tailor both the techniques used and the relationship style to the needs of individual clients (Norcross et al., 2016). In short, relationship styles of choice are at least as important as techniques of choice.

Concluding Comments

Throughout this book you have "become a client" as I applied various interventions with you. By thinking of yourself as a client, you can continue studying these theories from a personal perspective. Take the key concepts of several theories that have personal relevance for you and apply these ideas to your own life. What aspects of the different theories would most help you as a client in understanding yourself? In addition, ask yourself what aspects of each theory would be most useful to you as a therapist in working with diverse client groups. What basic concepts are essential aspects of your theoretical orientation? What concepts would most help you make sense of the work you are doing with your clients? Try to answer these questions by experimenting within the parameters of one or more theories. I have described the concepts and techniques I employ in my integrative approach, both as I worked with you as a client and with a variety of cases. Reflect on these case examples as a way to create your own counseling style.

Commit yourself to a reading program and consider attending a variety of professional workshops. Reading is a realistic and useful way to expand your knowledge base and to learn new ideas on how to create, implement, and evaluate techniques. Take time to review the resources given throughout this book and also those listed in the "References" at the end of the book. Attend workshops dealing with different aspects of the counseling process and as a way to learn about the implementation of techniques. Just about every theoretical system I've mentioned is associated with one or more professional organizations where you can get further training in

the particular orientation. (Consult the "Where to Go From Here" sections in *Theory and Practice of Counseling and Psychotherapy* [Corey, 2017] for places to contact for training and supervision on specific therapy approaches.)

As you attend workshops, be open to ideas that seem to have particular meaning for you and that fit the context of your work. Don't adopt ideas without first putting them through your personal filter. As you experiment with many different counseling techniques, avoid using techniques in a rigid or "cookbook" method. Techniques are merely tools to assist you in effectively reaching your clients. Personalize your techniques so they fit your style and the needs of your clients, and be open to feedback from your clients about how well your techniques are working for them.

Whether you are a student in a counseling program or a new professional, be open to supervision throughout your career. Talk with supervisors and colleagues about what you are doing. Discuss some of your interventions with other professionals and think of alternative approaches you could take with clients. Remain a lifelong learner and continue thinking about alternative theoretical frameworks. Continue reflecting on what fits for you and what set of blueprints will be most useful in creating an emerging model for practice. Although you will have a solid foundation consisting of theoretical constructs, realize that the art of integrative counseling consists of personalizing your knowledge so that how you function as a counselor is an expression of your personality and life experiences. No prefabricated model will fit you perfectly. Customize your approach to fit your therapeutic style and the needs of your clients. Even with the same client, you will likely need to adapt your methods at different phases of therapy. Psychotherapy integration rests on the assumption that different clients require different methods, yet this blending of concepts and methods needs to be done systematically.

Perhaps the most important thing you can do is to engage in *deliberate practice* (Rousmaniere, 2016): record and study your own work and reflect on the effects that it has on clients. Deliberate practice is focused, systematic, and carried out over an extended period of time. Those who practice deliberately often review the recordings of their work; they engage in reflecting on feedback they receive; they seek feedback from experts and watch experts at work every chance they get; they practice very specific, often behaviorally defined, skills; and they evaluate their own effectiveness. Deliberate practice involves a commitment to repetitively practicing specific skills until they become routine (Rousmaniere et al., 2017). There is a key difference between having years of experience and having a year of experience many times over.

After reading this book, you may be able to begin putting the theories together in some meaningful way for yourself. I hope you have seen something of value that you can use from the various pathways

to integration. You will not be in a position to conceptualize a comprehensive integrative approach to your counseling practice, but you may have the knowledge to *begin* the process of integration. With additional study and practical experience, you will continue to expand and refine your emerging personal perspective on counseling.



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